

Moody's: Aaa
Standard & Poor's: AA+
See "Ratings" herein

NEW ISSUE – Book-Entry Only

Interest on the Bonds is not excludable from the gross income of holders under Section 103 of the Internal Revenue Code of 1986, as amended. See "TAX MATTERS" herein for a more detailed summary of certain U.S. federal income tax consequences of owning the Bonds.



\$100,535,000

MAIMONIDES MEDICAL CENTER

GNMA COLLATERALIZED TAXABLE REVENUE BONDS, SERIES 2013

Dated: Date of Delivery

Due: As shown below

Maimonides Medical Center (the "Issuer") is issuing \$100,535,000 aggregate principal amount of its GNMA Collateralized Taxable Revenue Bonds, Series 2013 (the "Series 2013 Bonds") pursuant to a Trust Indenture, dated as of March 1, 2013 (the "Indenture"), by and between the Issuer and U.S. Bank National Association, located in New York, New York, as trustee (the "Trustee"). The proceeds of the Series 2013 Bonds will be used to (i) acquire the GNMA Securities described below, and (ii) pay certain expenses incurred in connection with the issuance of the Series 2013 Bonds.

The Series 2013 Bonds will be issued as fully registered bonds in denominations of \$5,000 or any integral multiple thereof. Interest on the Series 2013 Bonds will be payable on each March 20 and September 20, commencing September 20, 2013 until their maturity or earlier redemption thereof by check mailed to the registered owners of the Series 2013 Bonds as of the Record Date, as described herein. Principal payments will be payable on March 20 and September 20 as described herein.

The Series 2013 Bonds will be issued initially under a Book-Entry Only System, registered in the name of Cede & Co., as nominee for The Depository Trust Company ("DTC"). Individual purchases of beneficial interests in the Series 2013 Bonds will be made in book-entry form (without certificates). So long as DTC or its nominee is the registered owner of the Series 2013 Bonds, (i) principal payments and Redemption Price of, and interest on the Series 2013 Bonds will be made directly to DTC or its nominee, and (ii) all references in this Official Statement to Bondowners or registered owners shall mean Cede Co., not the beneficial owners of the Series 2013 Bonds. Disbursement of payments to DTC participants is the responsibility of DTC and disbursement of such payments to the beneficial owners is the responsibility of DTC participants. See "THE SERIES 2013 BONDS - Book-Entry Only System" herein.

The Series 2013 Bonds have not been registered under the Securities Act of 1933, as amended (the "Securities Act"), or any state securities laws. The Series 2013 Bonds constitute exempt securities under Section 3(a)(4) of the Securities Act and, as a result, the Series 2013 Bonds and their offering to the public are exempt from registration under the Securities Act of 1933.

The Issuer will enter into a Financing Agreement (the "Financing Agreement") with the Trustee and Prudential Huntoon Paige Associates, LLC, a Delaware limited liability company (the "Lender"), pursuant to which the Lender will (i) modify two existing FHA-insured mortgage loans with the Issuer (the "Mortgage Loans") and (ii) issue and deliver to the Trustee two fully modified pass-through mortgage-backed securities in respect of the Mortgage Loans (the "GNMA Securities") which are guaranteed as to timely payment of principal and interest by the Government National Mortgage Association ("GNMA"). See "SECURITY AND SOURCES OF PAYMENT FOR THE SERIES 2013 BONDS" herein.

Timely payment of principal of, and interest on, the GNMA Securities is guaranteed by GNMA pursuant to Section 306(g) of Title III of the National Housing Act. Section 306(g) provides that "The full faith and credit of the United States is pledged to the payment of all amounts which may be required to be paid under any guaranty under this subsection" and an opinion, dated December 12, 1969, of an Assistant Attorney General of the United States, recites that such guaranties "constitute general obligations of the United States backed by its full faith and credit".

The Series 2013 Bonds are payable solely from the amounts available under the Indenture, primarily constituting moneys derived from the GNMA Securities and certain funds held by the Trustee, including the investment income thereon. The Series 2013 Bonds do not constitute a debt of, or a pledge of the faith and credit of, the United States of America, the Department of Housing and Urban Development ("HUD"), GNMA, the Federal Housing Administration ("FHA") or any other federal governmental agency.

The Series 2013 Bonds are subject to redemption prior to maturity as more fully described in this Official Statement and in such event, investors who purchase Series 2013 Bonds at a price in excess of par risk the loss of any premium paid. See "THE SERIES 2013 BONDS – Redemption of the Series 2013 Bonds – Early Redemption and Loss of Premium" herein.

\$100,535,000	3.30% Term Bond Due March 20, 2032	Price 103.196%	CUSIP 56033AAA8¹
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The Series 2013 Bonds are offered when, as and if issued and received by the Underwriter. The offer of the Series 2013 Bonds may be withdrawn or modified at any time without notice and is subject to the approval of legality by Eichner Norris & Neumann PLLC, Washington, D.C., Transaction Counsel, and to certain other conditions. Certain legal matters will be passed upon for the Underwriter by its counsel, Harris Beach PLLC, New York, New York; for the Issuer by its counsel, Arent Fox LLP, New York, New York, and by its Senior Vice President for Legal Affairs and General Counsel, Joyce A. Leahy, Esq.; and for the Lender by its special counsel, Krooth & Altman LLP, Washington, D.C. The Series 2013 Bonds are expected to be delivered in book-entry only form on or about March 21, 2013.

BofA Merrill Lynch

February 27, 2013

1. Copyright American Bankers Association. CUSIP numbers have been assigned at the time of issuance of the Series 2013 Bonds by Standard & Poor's, CUSIP Service Bureau, a division of The McGraw-Hill Companies, Inc., an independent company not affiliated with the Issuer. CUSIP numbers are provided solely for the convenience of the holders of the Series 2013 Bonds and no representations are made with respect to the selection or uses of, nor does any party undertake any responsibility for the accuracy of, the CUSIP numbers now or at any time in the future. The CUSIP number is subject to being changed after the issuance of the Series 2013 Bonds as a result of various subsequent actions including, but not limited to, a refunding in part or as a result of the procurement of secondary market portfolio insurance or other similar enhancement by investors for such maturity.

No dealer, broker, salesperson or other person has been authorized by the Issuer or the Underwriters to give any information or to make any representations with respect to the Series 2013 Bonds, other than the information and representations contained in this Official Statement. If given or made, such information or representations must not be relied upon as having been authorized by either of the foregoing.

This Official Statement does not constitute an offer to sell or the solicitation of an offer to buy nor shall there be a sale of the Series 2013 Bonds by any person in any jurisdiction in which it is unlawful for such person to make such offer, solicitation, or sale. The Series 2013 Bonds have not been registered under the Securities Act of 1933, as amended, and the Indenture has not been qualified under the Trust Indenture Act of 1939, as amended, in reliance upon exemptions contained in such acts. The registration or qualification of the Series 2013 Bonds in accordance with applicable provisions of securities laws of the states in which the Series 2013 Bonds have been registered or qualified, if any, and the exemption from registration or qualification in other states cannot be regarded as a recommendation thereof. Neither these states nor any of their agencies have passed upon the merits of the Series 2013 Bonds or the accuracy or completeness of this Official Statement. This Official Statement has not been reviewed, approved or disapproved, nor has the accuracy or adequacy of the information set forth herein been passed upon, by the Securities and Exchange Commission (the "SEC") or any state securities administrator. Any representation to the contrary may be a criminal offense.

This offering is subject to withdrawal, cancellation or modification by the Issuer without notice. The Issuer reserves the right, in its sole discretion, to cancel this offering in whole or in part for any reason.

No person, other than as provided for herein, has been authorized to give any information or to make any representations other than those contained in this Official Statement in connection with the offer being made hereby, and if given or made, such information or representations must not be relied upon as having been authorized by the Issuer. Although the Issuer has an internet website, the information contained thereon is not a part of this Official Statement and should not be relied upon in making an investment decision.

Prospective investors are not to construe the contents of this Official Statement as legal, investment or tax advice. Prospective investors should consult their advisors as to legal, investment, tax and related matters concerning an investment by such prospective investors in the Series 2013 Bonds.

To the extent that this Official Statement contains summaries of certain provisions of documents or of various provisions of relevant statutes and regulations, such summaries do not purport to be complete and are qualified in their entirety by reference to the texts of the original documents, statutes and regulations that are available upon request. If descriptions or terms in this Official Statement are inconsistent or contrary to descriptions or terms in such agreements or statutes, then the full agreement or statute, and not the summary contained in this Official Statement, shall control.

Certain information contained herein has been obtained from published sources and/or prepared by other parties. While such sources are believed to be reliable, neither the Issuer nor any other person assumes any responsibility for the accuracy of such information.

Certain of the information contained herein represents or is based upon forward-looking statements or information. The Issuer believes that such statements and information are based upon reasonable estimates and/or assumptions. However, forward-looking statements and information are inherently uncertain, and factors such as those described in "RISK FACTORS AND REGULATORY PROVISIONS WHICH MAY AFFECT THE MEDICAL CENTER OR THE SERIES 2013 BONDS" and other factors may cause actual events or results to differ from those projected. Therefore, undue reliance should not be placed on such forward-looking statements and information.

The Issuer has reviewed the information in this Official Statement, including the Appendices thereto except for (i) the sections of this Official Statement described in the next two paragraphs, (ii) the information in the sections of this Official Statement entitled "THE SERIES 2013 BONDS — Book-Entry Only System, "TAX MATTERS" and "RATINGS"), and (iii) the information in Appendices D through F. The Issuer will certify, as of the date of sale and the date of delivery of the Series 2013 Bonds, that such information (other than the excluded information described in clause (i) through (iii) or this paragraph) does not contain any untrue statements of a material fact and does not omit any material fact necessary to make the statements made therein, in light of the circumstances under which the statements are made, not misleading. The Issuer makes no representation as to the accuracy or completeness of any other information included in this Official Statement.

The Lender has reviewed the information in the sections of this Official Statement entitled "INTRODUCTORY STATEMENT - Description of Plan of Refinancing" (to the extent it relates to its role as Lender, the issuance of the GNMA Securities, the GNMA Guaranty Agreement and the FHA Documents), "PLAN OF REFINANCING" (to the extent it relates to its role as Lender, the issuance of the GNMA Securities, the GNMA Guaranty Agreement and the FHA Documents), "GNMA SECURITIES", "CERTAIN PROVISIONS OF THE FHA DOCUMENTS", "THE LENDER" and "RISK FACTORS AND REGULATORY PROVISIONS WHICH MAY AFFECT THE MEDICAL CENTER OR THE SERIES 2013 BONDS" (to the extent it relates to its role as Lender, the issuance of the GNMA Securities, the GNMA Guaranty Agreement and the FHA Documents), and

will certify, as of the date of sale and the date of delivery of the Series 2013 Bonds, that such information does not contain any untrue statements of a material fact and does not omit any material fact necessary to make the statements made therein, in light of the circumstances under which the statements are made, not misleading. The Lender makes no representation as to the accuracy or completeness of any other information included in this Official Statement.

The Underwriter has reviewed the information in the section of this Official Statement entitled “UNDERWRITING”, and will certify, as of the date of sale and the date of delivery of the Series 2013 Bonds, that such information does not contain any untrue statements of a material fact and does not omit any material fact necessary to make the statements made therein, in light of the circumstances under which the statements are made, not misleading. The Underwriter makes no representation as to the accuracy or completeness of any other information included in this Official Statement.

References in this Official Statement to the Indenture, the GNMA Guaranty Agreement, the FHA Documents, the Financing Agreement and the GNMA Securities do not purport to be complete. Reference is made to the Indenture, the GNMA Guaranty Agreement, the FHA Documents, the Financing Agreement and the GNMA Securities for full and complete details of their provisions. Copies of the Indenture, the primary FHA Documents and the Financing Agreement are on file with DAC and the Trustee. Information regarding the GNMA Securities is on file with the Lender.

The order and placement of material in this Official Statement, including its appendices, are not to be deemed a determination of relevance, materiality or importance, and all material in this Official Statement, including the appendices, must be considered in its entirety.

Under no circumstances shall the delivery of this Official Statement, or any sale made after its delivery, create any implication that the affairs of the GNMA, FHA, HUD, the Lender or the Issuer have remained unchanged after the date of this Official Statement.

IN CONNECTION WITH THE OFFERING OF THE SERIES 2013 BONDS, THE UNDERWRITER MAY OVERALLOT OR EFFECT TRANSACTIONS WHICH STABILIZE OR MAINTAIN THE MARKET PRICE OF SUCH BONDS AT LEVELS ABOVE THOSE WHICH MIGHT OTHERWISE PREVAIL IN THE OPEN MARKET. SUCH STABILIZING, IF COMMENCED, MAY BE DISCONTINUED AT ANY TIME.

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OFFICIAL STATEMENT

relating to

\$100,535,000

MAIMONIDES MEDICAL CENTER GNMA COLLATERALIZED TAXABLE REVENUE BONDS, SERIES 2013

INTRODUCTORY STATEMENT

Purpose of the Official Statement

The purpose of this Official Statement, including the cover page and appendices hereto, is to set forth certain information concerning Maimonides Medical Center (the “Issuer” or the “Medical Center”) and its \$100,535,000 GNMA Collateralized Taxable Revenue Bonds, Series 2013 (the “Series 2013 Bonds”).

The following is a brief description of certain information concerning the Series 2013 Bonds, the GNMA Securities, the Issuer and Prudential Huntoon Paige Associates, LLC, a Delaware limited liability company (the “Lender”). A more complete description of such information and additional information that may affect decisions to invest in the Series 2013 Bonds are contained throughout this Official Statement, which should be read in its entirety.

Authorization of Issuance

The Series 2013 Bonds will be issued pursuant to the Trust Indenture dated as of March 1, 2013 (the “Indenture”), by and between the Issuer and U.S. Bank National Association, located in New York, New York, as trustee (the “Trustee”). All references to funds and accounts in this Official Statement are to those funds and accounts authorized to be created pursuant to the Indenture. See “THE SERIES 2013 BONDS”.

Description of Plan of Refinancing

The Issuer will enter into a Financing Agreement (the “Financing Agreement”) with the Trustee and the Lender, pursuant to which the Lender will modify two existing mortgage loans with the Issuer that are insured by the Federal Housing Administration (“FHA”), an organizational unit within the United States Department of Housing and Urban Development (collectively the “Mortgage Loans”) pursuant to Section 242 and Section 241, respectively, of the National Housing Act of 1934, as amended (the “National Housing Act”). The Mortgage Loans are evidenced by the Issuer’s mortgage notes (collectively, as amended, the “Mortgage Notes”), and secured by FHA-insured mortgages on certain real property owned by the Issuer. The FHA-insured mortgages made with respect to the Mortgage Loans (collectively, as amended, the “Mortgages”) currently secure the Mortgage Notes which evidence two loans made with the proceeds of the Dormitory Authority of the State of New York (“DASNY”) Maimonides Medical Center FHA-Insured Mortgage Hospital Revenue Bonds, Series 1996 (the “Series 1996 Bonds”) and the Maimonides Medical Center FHA-Insured Mortgage Hospital Revenue Bonds, Series 2004 (the “Series 2004 Bonds”). The Mortgages and the Mortgage Notes will be assigned by DASNY to the Lender upon the defeasance of the Series 1996 Bonds and the Series 2004 Bonds on the date of delivery of the Series 2013 Bonds. Following the assignment of the Mortgages and the Mortgage Notes to the Lender, the Lender will issue, sell and deliver to the Trustee two fully modified pass-through

mortgage-backed securities in the form of Project Loan Certificates (the “PLCs” or the “GNMA Securities”) in respect of the Mortgage Loans. The GNMA Securities are guaranteed as to timely payment of principal and interest by the Government National Mortgage Association (“GNMA”) and such guaranty is further backed by the full faith and credit of the United States of America. Substantially all of the proceeds of the Series 2013 Bonds (including those derived from original issue premium), will be used to purchase the GNMA Securities from the Lender. See “ESTIMATED SOURCES AND USES OF FUNDS” herein. As a result, in the event the GNMA Securities are not delivered by April 4, 2013 (unless such date shall be extended pursuant to the terms of the Indenture), the Series 2013 Bonds shall be subject to a Special Mandatory Redemption (described herein), and investors will lose any original issue premium paid in connection with their purchase of the Series 2013 Bonds.

The GNMA Securities acquired by the Trustee, on behalf of the Issuer, will be deposited by the Trustee into the Bond Fund and pledged as security for the payment of principal of and interest on the Series 2013 Bonds. The GNMA Securities will be dated and bear interest from the first day of the month in which they are issued. Interest on the GNMA Securities for any given month is payable on the fifteenth day of the following month commencing and is payable at the Pass-Through Rate (which is equal to the interest rate on the Mortgage Notes less the GNMA guaranty fee and the servicing fee paid to the Lender).

The principal of, and redemption premium, if any, and interest on the Series 2013 Bonds are primarily payable from the payments on the PLCs. In addition to the pledge of the PLCs, the Issuer will pledge and grant a security interest to the Trustee in all moneys, securities and instruments held from time to time under the Indenture. The Series 2013 Bonds and the security therefor are more fully described below under “THE SERIES 2013 BONDS”.

The Series 2013 Bonds are payable solely from the amounts available under the Indenture, primarily constituting moneys derived from the GNMA Securities and certain funds held by the Trustee, including the investment income thereon. The Series 2013 Bonds do not constitute an obligation or indebtedness of, and the payment of the Series 2013 Bonds is not insured or guaranteed by, the United States of America or any agency or instrumentality thereof, including GNMA, HUD or FHA.

The Series 2013 Bonds

The Series 2013 Bonds will be dated their date of delivery and will bear interest from such date, payable on September 20, 2013 and on each March 20 and September 20 thereafter until their maturity or earlier redemption thereof, at the rates set forth on the cover page of this Official Statement. See “THE SERIES 2013 BONDS”.

The Issuer

The Issuer is a not-for-profit corporation which is exempt from federal income tax as an organization described in Section 501(c)(3) of the Internal Revenue Code of 1986, as amended (the “Code”). See “APPENDIX A – MAIMONIDES MEDICAL CENTER”.

PLAN OF REFINANCING

Application of Series 2013 Bond Proceeds and Other Moneys

Substantially all of the proceeds of the Series 2013 Bonds (other than those derived from original issue premium) will be deposited in the Acquisition Fund. In the event that the PLCs are not delivered by April 4, 2013 (the “GNMA Securities Delivery Date”) (unless such date shall be extended pursuant to the

terms of the Indenture), amounts deposited into the Acquisition Fund shall be transferred to the Bond Fund for the redemption of the Series 2013 Bonds on a date not later than April 19, 2013. See “THE SERIES 2013 BONDS - Redemption of the Series 2013 Bonds – Special Mandatory Redemption”.

The funds deposited in the Acquisition Fund will be invested in Qualified Investments. It is currently expected that funds deposited in the Acquisition Fund will be applied to the purchase of the PLCs from the Lender on or before the GNMA Securities Delivery Date. A portion of the amounts on deposit in the Bond Fund will be used to pay accrued interest on the PLCs upon purchase thereof. In the event the GNMA Securities become subject to a Special Mandatory Redemption (described herein) are not delivered by April 4, 2013 (unless such date shall be extended pursuant to the terms of the Indenture), and the Series 2013 Bonds shall be redeemed on a date not later than April 19, 2013, investors will lose any original issue premium paid in connection with their purchase of the Series 2013 Bonds

The New York State Department of Health approved the plan of refinancing on October 25, 2012.

Payment of the Series 2013 Bonds

The GNMA Securities will bear interest at the Pass-Through Rate, and the Lender will pass through principal payments on the Mortgage Notes as principal payments on the GNMA Securities. See “THE SERIES 2013 BONDS - Redemption of the Series 2013 Bonds”.

The Indenture provides that the Trustee shall apply amounts on deposit in the Bond Fund on each Interest Payment Date (or any other date on which Series 2013 Bonds are to be redeemed) to pay the principal of (whether at maturity or by mandatory redemption), premium, if any, and interest on the Series 2013 Bonds that are payable in accordance therewith.

THE SERIES 2013 BONDS

Description of the Series 2013 Bonds

The Series 2013 Bonds will be issued as fully registered bonds in the aggregate principal amount set forth on the cover page hereof. The Series 2013 Bonds will be dated their date of delivery and will bear interest from such date payable on September 20, 2013 and on each March 20 and September 20 thereafter until their maturity or earlier redemption thereof, and will bear interest at the rates and mature on the dates set forth on the cover page hereof. Interest on the Series 2013 Bonds shall accrue based upon a 360-day year of twelve 30-day months.

The Series 2013 Bonds will be issued in denominations of \$5,000 or any integral multiple thereof. The Series 2013 Bonds will be registered in the name of Cede & Co., as nominee of DTC, pursuant to DTC’s Book-Entry Only System. Purchase of beneficial interests in the Series 2013 Bonds will be made in book-entry form, without certificates. See “THE SERIES 2013 BONDS - Book-Entry Only System”.

Principal of and premium, if any, on the Bonds shall be paid only upon presentation and surrender thereof for cancellation at the Trust Office of the Trustee. Payment of the interest on each Bond shall be made by check mailed by first class mail to the person entitled thereto at its address as it appears on the Bond Register. For so long as the Bonds shall not be held in book-entry only form, upon the request of a registered owner of at least \$1,000,000 principal amount of Outstanding Bonds, all payments of interest on the Bonds owned by such registered owner shall be paid by wire transfer of immediately available funds to an account designated in writing by such registered owner. CUSIP number identification with appropriate dollar amounts for each CUSIP number must accompany all payments of principal, interest or premium, whether by check or by wire transfer.

Security for the Series 2013 Bonds

Prior to delivery of the GNMA Securities, moneys on deposit in the funds and accounts under the Indenture will be invested in Qualified Investments.

Within approximately 10 day following the assignment of the Mortgages and the Mortgage Notes to the Lender, the Lender will issue, sell and deliver to the Trustee, and the Trustee will purchase with substantially all of the net proceeds of the Series 2013 Bonds, two GNMA Securities that will secure the Series 2013 Bonds.

Upon delivery of the GNMA Securities, the Series 2013 Bonds will be secured by a pledge of (i) the GNMA Securities and (ii) such other moneys and funds as may be made available to the Trustee pursuant to the Financing Agreement and the Indenture. If the GNMA Securities are not delivered by the GNMA Securities Delivery Date (as such date may be extended pursuant to the Indenture), the Series 2013 Bonds shall be called for Special Mandatory Redemption on April 19, 2013 (or such other date permitted by the Indenture). See “THE SERIES 2013 BONDS - Redemption of the Series 2013 Bonds – Special Mandatory Redemption”. The Issuer’s monthly payments on the Mortgage Notes (less the servicing fee and the GNMA guaranty fee), which are secured by the Mortgages will be passed through monthly by the Lender to the Trustee. The Trustee will hold such monthly payments and apply them to interest and principal payments due on the Series 2013 Bonds.

The Series 2013 Bonds are payable from the above-described sources. The guaranty of GNMA of the payment of principal of and interest on the GNMA Securities relate only to the timely payment of principal of and interest on the GNMA Securities and does not provide a guaranty of the payment of principal of and interest on the Series 2013 Bonds; provided, however, that a failure to make a principal amortization payment on the Series 2013 Bonds when due is an event of default under the Indenture. Based on the Cash Flow Statement (as defined in the Indenture), it is anticipated that the interest on and principal of the GNMA Securities and the Qualified Investments as described above will be sufficient to pay interest on and principal of the Series 2013 Bonds.

In the event the Issuer defaults under the provisions of the Mortgages and Mortgage Notes, the Lender may make a claim for FHA mortgage insurance benefits. Mortgage insurance benefits for an insured loan are payable by FHA in cash or debentures in an aggregate amount approximately equal to the outstanding principal amount of a Mortgage Note. If FHA elects to make partial payments in cash, the Lender may elect to pass through the initial payment to the Trustee in prepayment of the GNMA Securities and to pay the remaining balance of the GNMA Securities by making the regularly scheduled payments under the original amortization schedule without recasting the mortgage until the final payment is received from FHA. When the final cash payment is received from FHA for the mortgage insurance benefits, the Lender will use such cash, together with other funds of the Lender, to redeem the GNMA Securities at par plus accrued interest. In the alternative, the Lender may elect to use such initial partial cash payment, together with other funds of the Lender, to redeem the GNMA Securities in full the month following receipt of such amount. Such moneys, together with any moneys on deposit in any Fund established under the Indenture, shall be used to redeem the Series 2013 Bonds in accordance with the terms of the Indenture. If FHA pays a claim in debentures, the Lender must transfer the debentures to GNMA in return for GNMA reimbursing the Lender for interest on and principal of the Mortgage Note.

The Series 2013 Bonds are payable solely from the amounts available under the Indenture, primarily constituting moneys derived from the GNMA Securities and certain funds held by the Trustee, including the investment income thereon. The Series 2013 Bonds do not constitute an obligation or indebtedness of, and the payment of the Series 2013 Bonds is not insured or guaranteed by, the United States of America or any agency or instrumentality thereof, including GNMA, HUD or FHA.

Redemption of the Series 2013 Bonds

The Series 2013 Bonds are subject to redemption prior to maturity as follows:

Optional Redemption. The Series 2013 Bonds are subject to optional redemption prior to maturity by the Issuer on any date on or after March 20, 2023, in whole or in part, from payments on the GNMA Securities representing voluntary prepayments on the Mortgage Loans pursuant to the Financing Agreement, whether from the proceeds of refunding bonds, other funds of the Issuer or otherwise, at a redemption price of 100% of the principal amount thereof, plus accrued interest to the redemption date.

Scheduled Mandatory Redemption. The Series 2013 Bonds are subject to scheduled mandatory redemption on the respective Interest Payment Dates set forth in the table below, at a redemption price equal to the principal amount thereof plus accrued interest to the redemption date, in the following principal amounts, subject to pro rata reduction of such scheduled mandatory redemption payments pursuant to the Indenture as described below under the heading “Selection of Bonds for Redemption” to the extent that such Bonds are redeemed prior to maturity otherwise than pursuant to such scheduled mandatory redemption:

<u>Payment Date</u>	<u>Scheduled Mandatory Redemption of Series 2013 Principal</u>	<u>Payment Date</u>	<u>Scheduled Mandatory Redemption of Series 2013 Principal</u>
Sept. 20, 2013	\$1,855,000	March 20, 2023	\$2,470,000
March 20, 2014	2,170,000	Sept. 20, 2023	2,415,000
Sept. 20, 2014	2,205,000	March 20, 2024	2,460,000
March 20, 2015	2,250,000	Sept. 20, 2024	2,500,000
Sept. 20, 2015	2,285,000	March 20, 2025	2,545,000
March 20, 2016	2,330,000	Sept. 20, 2025	2,590,000
Sept. 20, 2016	2,365,000	March 20, 2026	2,635,000
March 20, 2017	2,410,000	Sept. 20, 2026	2,685,000
Sept. 20, 2017	2,450,000	March 20, 2027	2,730,000
March 20, 2018	2,500,000	Sept. 20, 2027	2,780,000
Sept. 20, 2018	2,540,000	March 20, 2028	2,830,000
March 20, 2019	2,585,000	Sept. 20, 2028	2,875,000
Sept. 20, 2019	2,630,000	March 20, 2029	2,935,000
March 20, 2020	2,680,000	Sept. 20, 2029	2,980,000
Sept. 20, 2020	2,725,000	March 20, 2030	3,035,000
March 20, 2021	2,780,000	Sept. 20, 2030	3,090,000
Sept. 20, 2021	2,820,000	March 20, 2031	3,145,000
March 20, 2022	2,875,000	Sept. 20, 2031	3,200,000
Sept. 20, 2022	2,925,000	March 20, 2032 [†]	3,255,000

[†] Maturity.

In the event of a partial redemption of Series 2013 Bonds, the Issuer agrees to deliver a revised mandatory redemption schedule.

Special Mandatory Redemption. The Series 2013 Bonds are subject to special mandatory redemption on the earliest practicable date for which notice of redemption can be given by the Trustee pursuant to the Indenture, at a redemption price equal to the principal amount thereof redeemed plus accrued interest to the redemption date, but without premium, as follows:

(a) In whole, on the fifteenth (15th) day after the GNMA Securities Delivery Date (as such date may be extended pursuant to the provisions the Indenture), if one or both of the GNMA Securities are not delivered to the Trustee on or before the GNMA Securities Delivery Date (as such date may be extended pursuant to the provisions the Indenture).

(b) As a whole or in part (and if in part, in incremental amounts of one dollar notwithstanding the minimum authorized denomination of \$5,000) on the earliest practicable date (but in no event longer than 15 days from when the Trustee receives the funds) to the extent that the Trustee receives payments on the GNMA Securities in excess of regularly scheduled payments (except payments representing optional prepayments on the Mortgage Loans) including, but not limited to, payments representing:

- (i) casualty insurance proceeds, condemnation awards or other amounts applied to the prepayment of the Mortgage Loans following a partial or total destruction or condemnation of either of the projects secured by the Mortgages in the event and to the extent that such amounts are not applied to the repair or restoration of the relevant project in accordance with the FHA Documents;
- (ii) mortgage insurance proceeds or other amounts received with respect to the Mortgage Loans following the occurrence of an event of default under the FHA Documents;
- (iii) a mandatory prepayment of the Mortgage Loans required by the applicable rules, regulations, policies and procedures of FHA or GNMA (including the possible exercise by HUD of its right to override the prepayment and premium provisions of the Mortgage Notes under certain circumstances); or
- (iv) a prepayment of the Mortgage Loans if HUD determines that prepayment will avoid a mortgage insurance claim and is therefore in the best interest of the federal government.

Selection of the Bonds for Redemption. In the event the Series 2013 Bonds are redeemed in part and not in whole (other than from principal amortization payments resulting in scheduled mandatory redemption of Series 2013 Bonds), the Series 2013 Bonds to be redeemed shall be selected by lot and the scheduled mandatory redemption requirements for each maturity shall be adjusted so that the resulting debt service on the Series 2013 Bonds (including scheduled mandatory redemption payments) during each six month period commencing on each Interest Payment Date is proportional, as nearly as practicable, to the payments on the GNMA Securities during each such six month period, provided however, that any such adjustment to the debt service on the Series 2013 Bonds shall be conditioned upon written confirmation from the Rating Agencies of the then current respective ratings on the Series 2013 Bonds.

Except as otherwise described above, any Series 2013 Bonds to be called for redemption shall be selected by the Trustee by lot, such selection to be made prior to the date on which notice of such redemption must be given and Series 2013 Bonds shall be redeemed as soon as practicable after an event causing a redemption shall have occurred.

Notice of Redemption. Except as provided below, notice of redemption shall be mailed, first-class postage prepaid, by the Trustee, not less than 10 days nor more than 30 days prior to the redemption date (except as otherwise described below), to (a) the respective holders of any Series 2013 Bonds designated for redemption at their addresses appearing on the bond registration books of the Trustee as of the Special Record Date, and (b) Digital Assurance Certification LLC (“DAC”) as provided at www.dacbond.com. Each notice of redemption shall state the date of such notice, the Regular Record Date or Special Record Date, as applicable, the redemption date, the redemption price, the place or places of redemption (including the name and appropriate address or addresses of the Trustee), the CUSIP number (if any), the distinctive numbers of the Series 2013 Bonds to be redeemed and, in the case of Series 2013 Bonds to be

redeemed in part only, the respective portions of the principal amount thereof to be redeemed. Each such notice shall also state that on said date there will become due and payable on each of said Series 2013 Bonds the redemption price thereof or of said specified portion of the principal amount thereof in the case of a Series 2013 Bond to be redeemed in part only, together with interest accrued thereon to the redemption date, and that from and after such redemption date interest thereon shall cease to accrue, and shall require that such Series 2013 Bonds be then surrendered at the address of the Trustee specified in the redemption notice.

Notice of redemption of Series 2013 Bonds shall be given by the Trustee for and on behalf of, and at the expense of, the Issuer. The Trustee shall mail a second notice to Bondholders with respect to any Series 2013 Bond called for redemption but not tendered for redemption within 60 days of the redemption date.

Failure by the Trustee to give notice pursuant to the Indenture to DAC shall not affect the sufficiency of the proceedings for redemption. Failure by the Trustee to mail notice of redemption pursuant to the Indenture to any one or more of the respective holders of any Series 2013 Bonds designated for redemption shall not affect the sufficiency of the proceedings for redemption with respect to the Bondholder or Bondholders to whom such notice was mailed.

Notwithstanding the foregoing or any other provision of the Indenture, in the event of a redemption pursuant to the Indenture, the Trustee shall give notice of redemption of Series 2013 Bonds not less than 10 days nor more than 15 days prior to such redemption immediately upon receipt of notice of prepayment of the Mortgage Loans, in each case from the Lender in accordance with the Financing Agreement. However, such notice of redemption shall not be required if the circumstances do not permit the Trustee to give such notice in accordance with the preceding sentence, provided that the Trustee shall give such notice as soon as practicable.

Early Redemption and Loss of Premium

Purchasers of Series 2013 Bonds who purchase a Series 2013 Bond at a price in excess of its principal amount or who hold such a Series 2013 Bond trading at a price in excess of par, should note that the Series 2013 Bonds are subject to certain early redemptions at a redemption price equal only to their principal amount plus accrued interest to the redemption date without any redemption premium or the then market value of the Series 2013 Bonds. This could occur, for example, in the event (i) the GNMA Securities are not delivered as currently contemplated, (ii) the Mortgage Notes are prepaid as a result of a default, (iii) there is a default under the Mortgages, or (iv) condemnation or insurance payments with respect to the Projects are received.

No Additional Bonds

The Indenture does not authorize the issuance of other Series of Bonds.

Book-Entry Only System

Unless otherwise noted, the description which follows of the procedures and record-keeping with respect to beneficial ownership interests in the Series 2013 Bonds, payment of interest and other payments on the Series 2013 Bonds to DTC Participants or Beneficial Owners of the Series 2013 Bonds, confirmation and transfer of beneficial ownership interests in the Series 2013 Bonds and other bond-related transactions by and between DTC, the DTC Participants and Beneficial Owners of the Series 2013 Bonds is based solely on information provided on DTC's website and presumed to be reliable. Accordingly, the Issuer, the Trustee, and the Underwriter do not and cannot make any representations concerning these matters.

The Depository Trust Company (“DTC”), New York, New York, will act as securities depository for the Series 2013 Bonds. The Series 2013 Bonds will be issued as fully-registered securities in the name of Cede & Co. (DTC’s partnership nominee) or such other name as may be requested by an authorized representative of DTC. One fully-registered Series 2013 Bond certificate will be issued for the Series 2013 Bonds, in the aggregate principal amount of the Series 2013 Bonds, and will be deposited with DTC.

DTC is a limited-purpose trust company organized under the New York Banking Law, a “banking organization” within the meaning of the New York Banking Law, a member of the Federal Reserve System, a “clearing corporation” within the meaning of the New York Uniform Commercial Code, and a “clearing agency” registered pursuant to the provisions of Section 17A of the Securities Exchange Act of 1934. DTC holds and provides asset servicing for over 3.5 million issues of U.S. and non-U.S. equity issues, corporate and municipal debt issues, and money market instruments (from over 100 countries) that DTC’s participants (“Direct Participants”) deposit with DTC. DTC also facilitates the post-trade settlement among Direct Participants of sales and other securities transactions in deposited securities, through electronic computerized book-entry transfers and pledges between Direct Participants’ accounts. This eliminates the need for physical movement of securities certificates. Direct Participants include both U.S. and non-U.S. securities brokers and dealers, banks, trust companies, clearing corporations, and certain other organizations. DTC is a wholly-owned subsidiary of The Depository Trust & Clearing Corporation (“DTCC”). DTCC is the holding company for DTC, National Securities Clearing Corporation and Fixed Income Clearing Corporation, all of which are registered clearing agencies. DTCC is owned by the users of its regulated subsidiaries. Access to the DTC system is also available to others such as both U.S. and non-U.S. securities brokers and dealers, banks, trust companies, and clearing corporations that clear through or maintain a custodial relationship with a Direct Participant, either directly or indirectly (“Indirect Participants”). The DTC Rules applicable to its Participants are on file with the Securities and Exchange Commission. More information about DTC can be found at www.dtcc.com and www.dtc.org.

Purchases of Series 2013 Bonds under the DTC system must be made by or through Direct Participants, which will receive a credit for the Series 2013 Bonds on DTC’s records. The ownership interest of each actual purchaser of each Series 2013 Bond (the “Beneficial Owner”) is in turn to be recorded on the Direct and Indirect Participants’ records. Beneficial Owners will not receive written confirmation from DTC of their purchase. Beneficial Owners are, however, expected to receive written confirmations providing details of the transaction, as well as periodic statements of their holdings, from the Direct or Indirect Participant through which the Beneficial Owner entered into the transaction. Transfers of ownership interests in the Series 2013 Bonds are to be accomplished by entries made on the books of the Direct and Indirect Participants acting on behalf of Beneficial Owners. Beneficial Owners will not receive certificates representing their ownership interests in the Series 2013 Bonds, except in the event that use of the book-entry system for the Series 2013 Bonds is discontinued.

To facilitate subsequent transfers, all bonds deposited by Direct Participants with DTC are registered in the name of DTC’s partnership nominee, Cede & Co., or such other name as may be registered by an authorized representative of DTC. The deposit of bonds with DTC and their registration in the name of Cede & Co. or such other DTC nominee effect no change in beneficial ownership. DTC has no knowledge of the actual Beneficial Owners of bonds; DTC’s records reflect only the identity of the Direct Participants to whose accounts bonds are credited, which may or may not be the Beneficial Owners. The Direct Participants and Indirect Participants will remain responsible for keeping account of their holdings on behalf of their customers.

Conveyance of notices and other communications by DTC to Direct Participants, by Direct Participants to Indirect Participants, and by Direct Participants and Indirect Participants to Beneficial Owners will be governed by arrangements among them, subject to any statutory or regulatory requirements as may be in effect from time to time. Beneficial Owners of the Series 2013 Bonds may

wish to take certain steps to augment the transmission to them of notices of significant events with respect to the Series 2013 Bonds, such as redemptions, tenders, defaults, and proposed amendments to the Series 2013 Bond documents. For example, Beneficial Owners of Series 2013 Bonds may wish to ascertain that the nominee holding the Series 2013 Bonds for their benefit has agreed to obtain and transmit notices to Beneficial Owners. In the alternative, Beneficial Owners may wish to provide their names and addresses to the registrar and request that copies of notices be provided directly to them.

Redemption notices shall be sent to DTC. If less than all of the Series 2013 Bonds within a maturity are being redeemed, DTC's practice is to determine by lot the amount of the interest of each Direct Participant in such maturity to be redeemed.

Neither DTC nor Cede & Co. (nor any other DTC nominees) will consent or vote with respect to the Series 2013 Bonds unless authorized by a Direct Participant in accordance with DTC's MMI procedures. Under its usual procedures, DTC mails an omnibus proxy (the "Omnibus Proxy") to the Trustee as soon as possible after the record date. The Omnibus Proxy assigns Cede & Co.'s consenting or voting rights to those Direct Participants to whose accounts the Series 2013 Bonds are credited on the record date (identified in a listing attached to the Omnibus Proxy).

Principal, redemption premium, if any, and interest payments on the Series 2013 Bonds will be made to Cede & Co., or such other nominee as may be requested by an authorized representative of DTC. DTC's practice is to credit Direct Participants' accounts upon DTC's receipt of funds and corresponding detail information from the Trustee, on payable dates in accordance with their respective holdings shown on DTC's records. Payments by Direct and Indirect Participants to Beneficial Owners will be governed by standing instructions and customary practices, as is the case with securities held for the accounts of customers in bearer form or registered in "street name", and will be the responsibility of such Participant and not of DTC, the Trustee, or the Issuer, subject to any statutory or regulatory requirements as may be in effect from time to time. Payment of principal, redemption premium, if any, and interest to Cede & Co., or such other nominee as may be requested by an authorized representative of DTC, is the responsibility of the Issuer or the Trustee, disbursement of such payments to Direct Participants shall be the responsibility of DTC, and disbursement of such payments to the Beneficial Owners shall be the responsibility of Direct and Indirect Participants.

DTC may discontinue providing its services as securities depository with respect to the Series 2013 Bonds at any time by giving reasonable notice to the Issuer or the Trustee. Under such circumstances, in the event that a successor securities depository is not obtained, Series 2013 Bond certificates will be printed and delivered.

The Issuer may decide to discontinue use of the system of book-entry-only transfers through DTC (or a successor securities depository). In such event, Series 2013 Bond certificates will be printed and delivered.

The information in this section concerning DTC and DTC's book-entry system has been obtained from DTC's website and is presumed to be reliable, but neither the Issuer, the Trustee nor the Underwriter takes any responsibility for the accuracy thereof or as to the absence of material adverse changes in such information subsequent to the date hereof.

NEITHER THE ISSUER, THE TRUSTEE NOR THE UNDERWRITER WILL HAVE ANY RESPONSIBILITY OR OBLIGATION TO ANY DTC PARTICIPANT, INDIRECT PARTICIPANT OR ANY BENEFICIAL OWNER OR ANY OTHER PERSON WITH RESPECT TO: (1) THE SERIES 2013 BONDS; (2) THE ACCURACY OF ANY RECORDS MAINTAINED BY DTC OR ANY DTC PARTICIPANT OR INDIRECT PARTICIPANT; (3) THE PAYMENT BY DTC OR ANY DTC PARTICIPANT OR INDIRECT PARTICIPANT OF ANY AMOUNT DUE TO ANY BENEFICIAL OWNER IN RESPECT OF THE PRINCIPAL, OR REDEMPTION PRICE OF OR INTEREST ON THE

SERIES 2013 BONDS; (4) THE DELIVERY BY DTC OR ANY DTC PARTICIPANT OR INDIRECT PARTICIPANT OF ANY NOTICE TO ANY BENEFICIAL OWNER WHICH IS REQUIRED OR PERMITTED UNDER THE TERMS OF THE INDENTURE TO BE GIVEN TO BONDHOLDERS; (6) ACTION TAKEN BY DTC AS BONDHOLDER; (5) THE SELECTION OF THE BENEFICIAL OWNERS TO RECEIVE PAYMENT IN THE EVENT OF ANY PARTIAL REDEMPTION OF THE SERIES 2013 BONDS; OR (6) ANY CONSENT GIVEN BY DTC OR OTHER ACTION TAKEN BY DTC AS BONDHOLDER.

THE ISSUER, THE TRUSTEE AND THE UNDERWRITER CANNOT AND DO NOT GIVE ANY ASSURANCES THAT DTC, THE DTC PARTICIPANTS OR THE INDIRECT PARTICIPANTS WILL DISTRIBUTE TO THE BENEFICIAL OWNERS OF THE SERIES 2013 BONDS (i) PAYMENTS OF PRINCIPAL OR REDEMPTION PRICE OF OR INTEREST ON THE SERIES 2013 BONDS, (ii) CERTIFICATES REPRESENTING AN OWNERSHIP INTEREST OR OTHER CONFIRMATION OF BENEFICIAL OWNERSHIP INTERESTS IN SERIES 2013 BONDS, OR (iii) REDEMPTION OR OTHER NOTICES SENT TO DTC OR CEDE & CO., ITS NOMINEE, AS THE REGISTERED OWNER OF THE SERIES 2013 BONDS, OR THAT THEY WILL DO SO ON A TIMELY BASIS OR THAT DTC PARTICIPANTS OR INDIRECT PARTICIPANTS WILL SERVE AND ACT IN THE MANNER DESCRIBED IN THIS OFFICIAL STATEMENT. The current “rules” applicable to DTC are on file with the Securities and Exchange Commission, and the current “procedures” of DTC to be followed in dealing with DTC Participants are on file with DTC.

So long as Cede & Co. is the registered owner of the Series 2013 Bonds, as nominee for DTC, references herein to the Bondholders or registered owners of the Series 2013 Bonds shall mean Cede & Co., as aforesaid, and shall not mean the Beneficial Owners of the Series 2013 Bonds.

GNMA SECURITIES

General

The summary and explanation of the GNMA Mortgage-Backed Securities Program (the “GNMA Program”) and the other documents referred to herein do not purport to be complete, and reference is made to the GNMA I Mortgage-Backed Securities Guide (GNMA Handbook 5500.3, as amended, hereafter, the “GNMA Guide”) and to said documents, copies of which are on file with the Lender, for full and complete statements of their provisions.

GNMA is a non-stock corporate instrumentality of the United States of America within the Department of Housing and Urban Development (“HUD”) with its principal office in Washington, D.C.

The GNMA Securities will be “fully modified pass-through” mortgage-backed securities in the form of Project Loan Certificates (the “PLCs” or the “GNMA Securities”) in respect of the Mortgage Loans that will be issued and serviced by the Lender. By the 15th day of each month commencing with the 15th day of the month next following the date of issue of the GNMA Securities, the Lender will be required to pass through to the Trustee, as the holder of the GNMA Securities, the monthly scheduled installments of principal and interest on the Mortgage Notes (less the GNMA guaranty fee and the Lender’s servicing fee), whether or not the Lender receives payment on the Mortgage Notes from the Issuer, plus any unscheduled prepayments of principal of the Mortgage Notes received by the Lender. GNMA guarantees the timely payment of the principal of and interest on the GNMA Securities.

GNMA Guaranty

GNMA is authorized by Section 306(g) of Title III of the National Housing Act, to guarantee the timely payment of the principal of, and interest on, securities which are based on and backed by mortgage

pools consisting of a single mortgage insured by FHA pursuant to Sections 242 and 241 of the National Housing Act. Section 306(g) of the National Housing Act further provides that “[T]he full faith and credit of the United States is pledged to the payment of all amounts which may be required to be paid under any guaranty under this subsection”. An opinion, dated December 9, 1969, of the then Assistant Attorney General of the United States of America, states that such guaranties under Section 306(g) of mortgage-backed securities of the type being delivered to the Trustee on behalf of the Issuer are authorized to be made by GNMA and “would constitute general obligations of the United States backed by its full faith and credit”.

Pursuant to such authority, GNMA, upon delivery of the PLCs to the Trustee in accordance with the related GNMA Guaranty Agreement, will have guaranteed the timely payment of principal of and interest on such PLCs.

GNMA Borrowing Authority

In order to meet its obligations under such guaranty, GNMA, in its corporate capacity under Section 306(d) of Title III of the National Housing Act, may issue its general obligations to the United States Treasury Department (the “Treasury”) in an amount outstanding at any one time sufficient to enable GNMA, with no limitations as to amount, to perform its obligations under its guaranty of the timely payment of the principal of and interest on the GNMA Securities. The Treasury is authorized to purchase any obligations so issued by GNMA and has indicated in a letter dated February 13, 1970, from the then Secretary of the Treasury to the then Secretary of HUD that the Treasury will make loans to GNMA, if needed, to implement the aforementioned guaranty.

GNMA warrants to the holder of the PLCs, as described in the GNMA Prospectus, that in the event it is called upon at any time to make good its guaranty of the payment of principal of and interest on the PLCs, it will, if necessary, in accordance with Section 306(d), apply to the Treasury for a loan or loans in amounts sufficient to make payments of principal and interest on the PLCs.

Servicing of Mortgage Loans

The Lender is responsible for servicing and otherwise administering the Mortgage Loans in accordance with generally accepted practices of the mortgage banking industry and the GNMA Guide.

The monthly remuneration of the Lender, for its servicing and administrative functions, and the guaranty fee charged by GNMA, total 0.25% (25 basis points) per annum (payable monthly) of the unpaid principal amount of the Mortgage Notes outstanding on the last day of the month preceding such date of calculation. The Mortgage Notes carry an interest rate that is 0.25% per annum greater than the interest rate on the GNMA Securities because the 0.25% total of the Lender’s servicing fee and the GNMA guaranty fee are deducted from payments on the Mortgage Notes prior to making payments on the GNMA Securities. It is not necessary for GNMA to receive its guaranty fee for the guaranty to be effective for holders of GNMA Securities.

It is expected that interest and principal payments on the Mortgage Notes will be the source of moneys for payments on the GNMA Securities. If such payments are less than what is due, the Lender may advance its own funds to ensure timely payment of scheduled installments of principal and interest due on the GNMA Securities. GNMA guarantees such timely payment in the event of the failure of the Lender to pass through such scheduled principal and interest payments when due.

The Lender is required to advise GNMA in advance of any impending default on scheduled payments on the GNMA Securities so that GNMA as guarantor will be able to continue such payments as

scheduled on the 15th day of each month. If, however, such payments are not received as scheduled, the Trustee, on behalf of the Issuer, has recourse directly to GNMA.

The Guaranty Agreement to be entered into by GNMA and the Lender in connection with the PLCs (the “GNMA Guaranty Agreement”) will provide that, in the event of a default by the Lender, including (a) a request to GNMA to make a payment of principal or interest on the PLCs when the Issuer is not in default under the Mortgage Notes, (b) insolvency of the Lender, or (c) default by the Lender under any other guaranty agreement with GNMA, GNMA will have the right, by letter to the Lender, to effect and complete the extinguishment of the Lender’s interest in the Mortgage Notes, and the Mortgage Notes will thereupon become the absolute property of GNMA, subject only to the unsatisfied rights of the holder of the PLCs. In such event, the GNMA Guaranty Agreement will provide that on and after the time GNMA directs such a letter of extinguishment to the Lender, GNMA will be the successor in all respects to the Lender in its capacity under the GNMA Guaranty Agreement and the transaction and arrangements set forth or arranged for therein, and will be subject to all responsibilities, duties and liabilities (except the Lender’s indemnification of GNMA) theretofore placed on the Lender by the terms and provisions of the GNMA Guaranty Agreement, provided that, at any time, GNMA may enter into an agreement with any other eligible issuer of the PLCs under which the latter undertakes and agrees to assume any part or all of such responsibilities, duties or liabilities theretofore placed on the Lender, and provided that no such agreement will detract from or diminish the responsibilities, duties or liabilities of GNMA in its capacity as guarantor of the PLCs, or otherwise adversely affect the rights of the holders thereof.

The PLCs do not constitute a liability of, nor evidence any recourse against, the Lender as the issuer of the PLCs, but recourse thereon is solely against GNMA as provided in the GNMA Guaranty Agreement and the GNMA Securities. The GNMA Guaranty Agreement provides that the full faith and credit of the United States of America is pledged to the payment of all amounts required to be paid pursuant to the GNMA guaranty of such securities.

Payment of Principal and Interest on the GNMA Securities

Each PLC is a “fully-modified pass through” security which requires monthly payments by the Lender, as the issuer of the GNMA Securities, to the Trustee, on behalf of the Issuer, of principal and interest at the rate stated thereon on the aggregate unpaid principal balance, whether or not the Lender receives payments on the Mortgage Notes underlying such GNMA Securities, plus any prepayments of principal of the Mortgage Notes received by the Lender.

Following the issuance of the PLCs and commencement of the payment of principal thereon, the PLCs will be payable in monthly installments of not less than the interest due on the PLCs the rate specified thereon, together with any scheduled installments of principal, whether or not collected from the Issuer and any prepayments or early recovery of principal and premium, if any. Each installment on the GNMA Securities shall be applied first to interest and then in reduction of the principal balance of the GNMA Securities then outstanding. Interest shall be paid at the specified rate on the unpaid portion of the principal. The amount of principal due on the GNMA Securities shall be in an amount equal to the scheduled principal amortization currently due on the Mortgage Note. Final payment shall be made to the Trustee upon surrender of the outstanding GNMA Securities.

Liability of the Lender

The GNMA Securities will not constitute a liability of nor evidence any recourse against the Lender. Recourse may be had by the Trustee only to GNMA in the event of any failure of timely payment as provided for in the GNMA Guaranty Agreement appended to the GNMA Securities.

CERTAIN PROVISIONS OF THE FHA DOCUMENTS

The Mortgage Notes

Upon compliance with the terms and conditions of the Lender Commitment, and pursuant to the terms and conditions specified in the Financing Agreement, the Lender agrees to make a loan to the Issuer in the aggregate amount of \$100,533,682.88 at an interest rate of 3.595% per annum, to be evidenced by the Mortgage Notes, which are currently and will continue to be insured by FHA. The Mortgage Loans (i) are insured by FHA pursuant to and in accordance with the provisions of Section 242 and Section 241, respectively, of the National Housing Act and applicable regulations thereunder, as evidenced by the endorsement by FHA of the Mortgage Notes; (ii) will have an aggregate outstanding principal balance as of March 1, 2013 of \$100,533,682.88; (iii) bear interest at an interest rate of 3.595% per annum; (iv) as to the Section 242 Mortgage Loan (defined herein), has a term of 115 months and matures on October 1, 2022 following the modification thereto pursuant to the Second Allonge to Mortgage Note, dated March 21, 2013, and, as to the Section 241 Mortgage Loan (defined herein), has a term of 228 months and matures on March 1, 2032 following the modification thereto pursuant to the Allonge to Mortgage Note, dated March 21, 2013; (v) is payable in equal monthly installments of principal and interest following Commencement of Amortization (as defined in the Indenture); (vi) is secured on a nonrecourse basis pursuant to the FHA Documents; and (vii) is not subject to prepayment prior to February 28, 2023; provided, however, that (A) the Mortgage Loans are subject to mandatory prepayment in whole or in part at any time from the proceeds of any casualty insurance, condemnation award or other amounts received following a partial or total destruction or condemnation of the Project, in the event and to the extent that such casualty proceeds, condemnation awards or other amounts are not applied to the repair or restoration of the Projects in accordance with the FHA Documents; (B) the Mortgage Loans are subject to prepayment from the proceeds of mortgage insurance or other amounts received with respect to the Mortgage Loans following the acceleration thereof upon the occurrence of an event of default under the Mortgages; (C) the Mortgage Loans are subject to prepayment to the extent, if any, required by applicable rules, regulations, policies and procedures of FHA and GNMA (including the possible exercise by HUD of its right to override the prepayment lockout and premium provision of the Mortgage Loans under certain circumstances following an event of default thereunder); and (D) the Mortgage Loans are subject to optional prepayment on the last day of the month on or after February 28, 2023 at a prepayment price of 100%, together with accrued interest to the end of the month of prepayment.

If the Mortgage Loans are prepaid subsequent to the date referenced in clause (vii) above or are prepaid in accordance with clauses (A) through (D) above, the Lender will give written notice of such prepayment to the Trustee no later than the fifth day of the month in which the resulting prepayment of the GNMA Security is to occur.

In the event of an optional or mandatory prepayment of the Mortgage Loans, in part, pursuant to the Mortgage Notes, the Issuer unconditionally covenants in the Financing Agreement to exercise its option to proportionally reamortize the outstanding principal balances of the Mortgage Notes in accordance with the terms of the Mortgage Notes.

The Mortgages

Upon the defeasance of the Series 1996 Bonds and the Series 2004 Bonds, the Mortgages will be modified and assigned to the Lender, as FHA mortgagee, and modified to reflect the terms of the loan to be made by the Lender. Each Mortgage grants a lien on the properties identified therein together with all buildings, improvements and fixtures thereon, rents, issues and profits thereof, and all building materials, equipment, furnishings and other property incident to use and occupancy thereof. Until the final payment of each Mortgage Note, the Issuer agrees not to sell, encumber or alienate the property subject to each such Mortgage in any way without the consent of the Lender and FHA, or as otherwise permitted in each

Mortgage or in the related Regulatory Agreement (as defined herein). The Issuer also covenants that it will not voluntarily create or permit to be created any other lien or liens against the Property subject to the Mortgages, except for permitted encumbrances.

The Issuer agrees that, in addition to payments for debt service due on the Mortgage Notes, it will pay monthly amounts to provide for the payment when due of premiums on the FHA mortgage insurance, casualty insurance, water rates, and taxes and assessments. In the event the Issuer fails to pay any of the sums required to be paid under the Mortgages, the Lender, at its option, may pay such amounts. The Mortgages provide that all sums paid by the Lender may be added to the principal amount of the Mortgage Notes, bear interest at the rate set forth in the Mortgage Notes and will be due and payable on written demand. The Issuer covenants that it shall keep the projects secured by the Mortgages in good repair, and may not make structural alterations without the consents of the Lender and FHA, which consent shall not be unreasonably withheld, delayed or conditioned.

Each Mortgage incorporates by reference a regulatory agreement between the Issuer and FHA (the “Regulatory Agreement”; and collectively with the Mortgage Note, Mortgage and all other documents required in connection with the modification of a Mortgage Loan by FHA for FHA mortgage insurance, the “FHA Documents”). The Regulatory Agreement sets forth certain of the Issuer’s obligations in connection with the management and operation of the project financed with the related Mortgage Loan (the “Project”). Among its provisions it restricts the use of the Project and the revenues which it generates and prohibits or restricts transferring, encumbering or altering the Project.

Under the Mortgages and the applicable FHA regulations, the following are events of default on the FHA Documents: (i) any default in any payment on the Mortgage Notes or under the Mortgages or (ii) any failure in the observance or performance of any other covenant in the Mortgages, if the Lender, because of such failure, has accelerated the debt. If such defaults as defined in (i) and (ii) above continue uncured after a grace period of 30 days, the Lender shall be entitled to receive mortgage insurance benefits, provided that within 30 days after the end of the grace period it notifies FHA of the default, and within 45 days, or any permitted additional time, after the end of the grace period it files with FHA a notice of its intention to file an insurance claim and its election either to assign the Mortgage Notes and the Mortgages to FHA or to acquire and convey title to the Projects to FHA. A default solely under the Regulatory Agreement will not constitute a default under the Mortgages unless declared by the Lender at the request of FHA. *An event of default under the Mortgages is not an Event of Default under the Indenture and will not automatically result in an acceleration of the Series 2013 Bonds.* Under GNMA regulations, the Lender has broad latitude to enter into workout arrangements and to negotiate modifications of the Mortgages subject to the approval of FHA and GNMA. So long as the Lender continues to make regularly scheduled payments of principal and interest on the GNMA Securities no default will exist as a result of any such mortgage modifications.

FHA and GNMA Documents and Regulations Control

To the extent that any provision in the Financing Agreement or the Indenture shall be inconsistent or in conflict with the provisions of the National Housing Act, FHA regulations, related FHA administrative requirements, the GNMA regulations, the GNMA I Mortgage-Backed Securities Guide, the FHA Documents, the GNMA Securities or the GNMA Guaranty Agreement, as the case may be, the provisions of the National Housing Act, FHA regulations, related FHA administrative requirements, the GNMA regulations, the GNMA I Mortgage-Backed Securities Guide, the FHA Documents, the GNMA Securities or the GNMA Guaranty Agreement, as the case may be, shall govern. No provision of the Financing Agreement or the Indenture or of any instruments referred to in the Financing Agreement or the Indenture shall alter the non-recourse nature of the Lender’s obligations under the GNMA Securities.

FHA-INSURED INDEBTEDNESS OF THE ISSUER

In 1996, DASNY issued bonds to finance a mortgage loan to the Issuer for FHA Project No. 012-13058 in the amount of \$32,676,000, which mortgage loan is secured by a FHA-insured Section 242 mortgage (the “Section 242 Mortgage Loan”) and evidenced by a mortgage note (the “Section 242 Mortgage Note”). Slightly more than half of the proceeds of the mortgage loan was used to finance expansion and modernization of medical care facilities at the Issuer’s main hospital campus (the “Mortgaged Property”) while slightly less than half of the proceeds of the mortgage loan was used to refinance outstanding FHA-Insured bonds issued by DASNY in 1977 and 1983. The Section 242 Mortgage constitutes a first mortgage lien on the Mortgaged Property. As of March 1, 2013, the Section 242 Mortgage Loan will be outstanding in the amount of \$9,787,311.10.

In 2004, DASNY issued bonds to finance a mortgage loan to the Issuer for FHA Project No. 012-10030 in the amount of \$104,704,000, which mortgage loan is secured by a FHA-insured Section 241 mortgage (the “Section 241 Mortgage Loan”) and evidenced by a mortgage note (the “Section 241 Mortgage Note”). The proceeds of the loan financed the modernization of the main hospital campus through new construction and renovations, including relocating selected inpatient services from the Gellman, Kronish and Aron Buildings providing much needed expansion space for existing acute care services, simplifying service delivery to patients through consolidation of services and upgrading clinical departments. Specifically, this modernization resulted in an enlargement of both ambulatory and inpatient surgery facilities, new and improved state-of-the-art medical technologies, and the reorganization and enlargement of the Issuer’s Cardiac Institute, the Infants and Children’s Hospital, and the Emergency Department. The lien of the Section 241 Mortgage is subordinate to the lien of the Section 242 Mortgage. As of March 1, 2013, the Section 241 Mortgage Loan will be outstanding in the amount of \$90,746,371.78.

The Section 242 Mortgage Loan and the Section 241 Mortgage Loan are collectively referred to in this Official Statement as the “Mortgage Loans”. The Section 242 Mortgage Note and the Section 241 Mortgage Note are collectively referred to in this Official Statement as the “Mortgage Notes”.

THE ORDER AND PRIORITY OF LIENS OF THE MORTGAGE SECURING THE MORTGAGE LOANS WILL REMAIN UNCHANGED AFTER THE CLOSING OF THE SERIES 2013 BONDS.

ESTIMATED SOURCES AND USES OF FUNDS

Set forth below is a description of the sources and uses of funds in connection with the issuance of the Series 2013 Bonds.

Sources of Funds	
Principal Amount of Series 2013 Bonds.....	\$100,535,000.00
Original Issue Premium	<u>3,213,098.60</u>
Total Sources	<u>\$103,748,098.60</u>
Uses of Funds	
Deposit to Acquisition Fund	\$100,535,000.00
Deposit to Bond Fund	750,000.00
Deposit with DASNY for defeasance of the Series 1996 Bonds and Series 2004 Bonds	536,059.56
Deposit to Cost of Issuance Fund.....	<u>1,927,039.04</u>
Total Uses	<u>\$103,748,098.60</u>

THE LENDER

Prudential Huntoon Paige Associates, LLC, a Delaware limited liability company (the “Lender”), originates, underwrites and funds mortgage loans and Prudential Asset Resources, Inc. (“PAR”), a sister company, services mortgage loans, including those backing mortgaged-backed securities that are insured, guaranteed or purchased by governmental agencies and sponsored enterprises, including HUD, FHA, GNMA, Fannie Mae and Freddie Mac. The mailing address of PAR is 2100 Ross Avenue, Suite 2500, Dallas, Texas 75201. Further information about the Lender may be obtained by written request to 3560 Lenox Road, Suite 1400, Atlanta, GA 30346.

To be approved by GNMA to issue modified pass-through securities with respect to long-term mortgages on a hospital project, a GNMA issuer is required to have a net worth (based on audited financial statements) equal to at least \$1,000,000 plus 1.0% of any securities outstanding, plus the amount of available commitment authority and unexpected construction loan draws in excess of \$25,000,000, but less than \$175,000,000, plus an additional 0.2% of any securities outstanding, plus the amount of available commitment authority and unexpected construction loan draws in excess of \$175,000,000.

RISK FACTORS AND REGULATORY PROVISIONS WHICH MAY AFFECT THE MEDICAL CENTER OR THE SERIES 2013 BONDS

The following discussion of risks to holders of the Series 2013 Bonds, in addition to other risks described throughout this Official Statement, is not intended to be exhaustive, but rather to summarize certain matters which could affect payment of the Series 2013 Bonds or the financial condition of the Medical Center.

The revenue and expenses of the Medical Center are affected by the changing health care environment. These changes are a result of efforts by the federal and state governments, managed care organizations, private insurance companies and business coalitions to reduce and/or contain health care costs including, but not limited to, the costs of inpatient and outpatient care, physician fees, capital expenditures and the costs of graduate medical education. In addition to matters discussed elsewhere herein, the following factors may have a material effect on the operations of the Medical Center to an extent that cannot be determined at this time.

General

The Series 2013 Bonds are payable solely from amounts available under the Indenture, including moneys derived from the GNMA Securities, and the funds and accounts held by the Trustee pursuant to the Indenture and certain investment income thereon. No representation or assurance can be made that revenues will be realized from the Medical Center in amounts sufficient to provide funds for payment of principal of and interest on the Mortgage Notes when due and to make other payments necessary to meet the obligations of the Medical Center. Further, there is no assurance that the revenues of the Medical Center can be increased sufficiently to match increased costs that may be incurred.

There are certain risks which might prevent the Medical Center from obtaining sufficient revenues to meet all of its obligations, including its obligations under the Mortgage Notes and the Financing Agreement. Future revenue and expenses of the Medical Center are subject to conditions which may change to an extent that cannot be determined at this time. Some of the possible changes in future conditions and other risks are discussed below.

The receipt of future revenues by the Medical Center is subject to, among other factors, federal and state regulations and policies affecting the health care industry; and the policies and practices of managed care providers, private insurers and other third-party payors, and private purchasers of health care services, all of which may affect rates, costs, third-party payments, and governmental regulations and policies concerning payment. The effect on the Medical Center of future changes in federal, state and private policies cannot be determined at this time. Loss of established managed care contracts by the Medical Center could also adversely affect the future revenues of the Medical Center.

Future revenues and expenses of the Medical Center and the resulting ability of the Medical Center to make payments under the Mortgage Notes may be affected by events and economic conditions, which may include an inability to control expenses in periods of inflation, as well as other conditions such as demand for health care services; the capability of the management of the Medical Center; the receipt of grants and contributions; referring physicians' and self-referred patients' confidence in the Medical Center; and increased use of discounted payment schedules through contracts with health maintenance organizations ("HMOs"), preferred provider organizations ("PPOs") and other payors. Other factors which may affect revenues and expenses include the ability of the Medical Center to provide services required by patients; the relationship of the Medical Center with physicians; the success of the Medical Center's strategic plans; the degree of cooperation among and competition with other hospitals in the Medical Center's area; changes in levels of private philanthropy; malpractice claims and other litigation; economic and demographic developments in the United States and in the service areas in which facilities of the Medical Center are located; changes in interest rates that affect the investment results; and changes in rates, costs, third-party payments (including, without limitation, Medicare and Medicaid program reimbursement) and governmental regulations concerning payment. All of the above-referenced factors could affect the Medical Center's ability to make payments requirements pursuant to the Indenture. See "APPENDIX A – MAIMONIDES MEDICAL CENTER" herein.

Adequacy of Revenues; Early Redemption and Loss of Premium

While the Medical Center is obligated to the Lender under the Financing Agreement to satisfy any deficiency with respect to the payment of the principal of and interest on the Mortgage Notes, the primary security for the Series 2013 Bonds are the GNMA Securities and moneys on deposit in various funds and accounts established pursuant to the Indenture. A default by the Medical Center on the Mortgage Loans that remains uncured will result in a prepayment of the GNMA Securities and, consequently, a redemption of all or a portion of the Series 2013 Bonds.

In addition, purchasers of Series 2013 Bonds who purchase a Series 2013 Bond at a price in excess of its principal amount or who hold such a Series 2013 Bond trading at a price in excess of par,

should note that the Series 2013 Bonds are subject to certain early redemptions at a redemption price equal only to their principal amount plus accrued interest to the redemption date without any redemption premium or the then market value of the Series 2013 Bonds. This could occur, for example, in the event (i) the GNMA Securities are not delivered as currently contemplated, (ii) the Mortgage Notes are prepaid as a result of a default, (iii) there is a default under the Mortgages, or (iv) condemnation or insurance payments with respect to the Projects are received. See “THE SERIES 2013 BONDS - Redemption of the Series 2013 Bonds”.

Health Care Reform

In March 2010, President Obama signed into law comprehensive health reform through the Patient Protection and Affordable Care Act (Pub. L. 111-148) and the Health Care and Education Reconciliation Act of 2010 (Pub. L. 111-152), together known as the Affordable Care Act (“ACA”). ACA was intended by its supporters to be transformative and includes numerous provisions affecting the delivery of health care services, the financing of health care costs, reimbursement of health care providers, and the legal obligations of health insurers, providers, employers and consumers.

The U.S. Supreme Court decided the constitutionality of certain ACA provisions in *National Federation of Independent Business v. Sebelius*: (i) the “individual mandate” that requires individuals to purchase health insurance starting in 2014 or be penalized, and (ii) the expansion of the Medicaid program. On June 28, 2012, the Supreme Court upheld the individual mandate but held that states have the option to reject the Medicaid expansion provisions without the risk of losing their federal Medicaid funding.

Controversy over ACA continues even after the Supreme Court’s decision. While the reelection of President Obama and the continued Democratic control in the U.S. Senate will be a major force in defending ACA, the Supreme Court’s consideration in its 2012-13 term of a challenge to ACA’s employer insurance requirements may negatively affect ACA’s future. Additionally, the U.S. House of Representatives has voted several times to repeal ACA, in whole or in part, and to cut funding for various ACA provisions; however, none of these efforts have passed, and after the recent elections are more unlikely to pass, in the U.S. Senate. Regardless, even if the ACA’s future is more secure, it is impossible to predict its potential impact on the health care industry.

As ACA and its implementing regulations will be phased in over several years on both a federal and a state level, the full consequences of ACA on the health care industry have not been immediately recognized. Therefore, it is difficult to predict whether the net impact of specific provisions of ACA will be positive or negative on the Medical Center, its revenues and its financial condition. Moreover, uncertainties regarding the implementation of ACA on a national level and in New York State create unpredictability for the strategic and business planning efforts of the Medical Center. Some of the specific provisions of ACA that may affect the Medical Center’s operations, financial performance or financial condition are described below. *This listing is not intended to be, nor should be considered by Series 2013 Bondholders as comprehensive. ACA is complex, and includes a myriad of new programs, initiatives, and changes to existing programs, policies, practices and laws. For a comprehensive understanding, Series 2013 Bondholders should review ACA in its entirety.*

Insurance Changes

Under ACA, new state-based health insurance exchanges, increased employer insurance coverage requirements, and Medicaid expansions will likely decrease the number of uninsured patients. This increase in insured patients could result in lower levels of bad debt and increased utilization or profitable shifts in utilization patterns for hospitals generally. Nonetheless, it remains unclear how these ACA

insurance changes will impact the Medical Center. For example, the ultimate provider rates under, and the comprehensiveness of, the New York exchange plan are still uncertain.

While statements by Governor Cuomo indicate that New York is leaning towards participating in the Medicaid expansion available under ACA, it is still uncertain whether New York will participate, and exactly how this Medicaid expansion will impact New York hospitals. In August 2012, the New York State Department of Health (“DOH”) submitted its Medicaid Redesign Team (“MRT”) Waiver Amendment to CMS seeking \$10 billion in federal funds over a five year period for investment in New York State’s health care system to be funded from Medicaid redesign savings. If obtained, these funds will be used to implement MRT and ACA provisions, including the Medicaid expansion. See “State Budget” herein.

Payment Rules

ACA will increasingly link payments to hospitals under public programs to quality and coordination of care. Accordingly, if the Medical Center does not meet the quality status requirements, the Medical Center and its revenues could be adversely impacted. Regardless of the hospital’s performance, beginning in federal fiscal year 2013, Medicare inpatient payments to hospitals, including Medicare HMO payments, will be reduced by 1% progressing to 2% by federal fiscal year 2017. Beginning in fiscal year 2013, hospitals will be subject to Medicare payment withholds or bonuses contingent on satisfaction of specific performance measures related to common and high-cost medical conditions under a new value-based purchasing program and face payment reductions based on the dollar value of preventable readmissions for certain medical conditions. Furthermore, ACA allows the Centers for Medicare & Medicaid Services (“CMS”) to expand the conditions measured for readmission rate penalties beginning in federal fiscal year 2015.

In addition, beginning in federal fiscal year 2015, Medicare inpatient payments to hospitals that are in the top quartile nationally for frequency of certain “hospital-acquired conditions” will be reduced by 1% for all discharges for the applicable federal fiscal year. As of July 1, 2011, CMS no longer provides federal funding to states for any amounts expended by providers in treating so-called provider-preventable conditions. CMS also directed states to submit amendments to their Medicaid state plans to require payment denials for the cost of treating such conditions, consistent with the prohibition on federal reimbursement. DOH issued an emergency regulation, effective December 6, 2011, that denied payment for several “potentially preventable negative outcomes.”

ACA also contains reductions in Medicare market basket updates and cuts in Medicare and Medicaid disproportionate share hospital (“DSH”) payments for providing care to low income and uninsured patients. Generally, Medicare payment rates to hospitals are adjusted annually based on a “market basket” of estimated cost increases, which have averaged approximately 2-4% annually in recent years. ACA provides for a general reduction of a specified percentage (up to 0.75% per year) in each federal fiscal year through 2019. There is also a productivity adjustment based on national economic productivity statistics which resulted in a 1% reduction in the market basket update for federal fiscal year 2012 and is expected to have a similar result in future years. Separately, beginning in federal fiscal year 2014, Medicaid DSH allotments to each state will also be reduced. See “Medicare and Medicaid Reimbursement–DSH Payments” herein for background on DSH funding.

While the House of Representatives has voted to repeal the Medicare Independent Payment Advisory Board (“IPAB”), ACA directs the IPAB to provide recommendations to Congress on ways to curtail growth of Medicare expenditures beginning in 2014. If Congress does not adopt alternative legislation to meet the savings targets, ACA provides for the IPAB’s recommendations to be automatically implemented. Starting in 2020, hospitals will be subject to the IPAB’s cost reduction proposals.

ACA contained significant changes to federal payments to Medicare health management organizations – Medicare Advantage plans (“MAPs”) – that could have an effect on MAPs’ payments to hospitals. Federal payments to MAPs are tied to the level of fee-for-service spending in the applicable county, resulting in a reduction for certain MAPs. These reduced payments could in turn affect the scope of coverage of these plans or cause plan sponsors to negotiate lower payments to providers. See “APPENDIX A – MAIMONIDES MEDICAL CENTER–Sources of Patient Service Revenue” herein.

Fraud and Abuse

ACA included significant changes to health care fraud and abuse laws that will intensify the risks and consequences of enforcement actions. These changes include: expansion of the Federal False Claims Act; lessening of the intent requirements under the federal Anti-Kickback Statute; and new funding and expanded powers to investigate fraud, including through expansion of the Medicare Recovery Audit Contractor (“RAC”) program and the requirement that states create a Medicaid RAC program. New York’s State Plan Amendment for the Medicaid RAC program was approved in March 2011 and designated HMS as its RAC contractor in April 2011. ACA also creates enhanced penalties for noncompliance, including increased criminal penalties and expansion of administrative penalties under Medicare and Medicaid. In addition to these potential noncompliance penalty costs, all hospitals will eventually be required to establish and maintain compliance and ethics programs that satisfy certain federal requirements as a condition of enrollment in Medicare, Medicaid and the Children’s Health Insurance Program. The New York’s Office of the Medicaid Inspector General 2012-13 Work Plan emphasizes enforcement of ACA requirements, specifically discussing the identification and refunding of overpayments. See “Federal Fraud and Abuse Laws and Regulations” and “Federal and State False Claims Acts” in this section and “APPENDIX A – MAIMONIDES MEDICAL CENTER – Material Pending Audits” herein.

Budget Control Act

The Budget Control Act of 2011 was enacted August 2, 2011, thereby bringing an end to the debt ceiling crisis that had threatened to lead the United States into sovereign default on or about August 3, 2011. The law specifies \$917 billion of cuts over 10 years in exchange for the initial debt limit increase of \$900 billion, with approximately \$21 billion of cuts to be applied in the fiscal year 2012 federal budget. In order to achieve those budget cuts, and to try to reduce the long-term federal debt, the law introduces several complex mechanisms. Since Congress failed to produce a deficit reduction bill with at least \$1.2 trillion in cuts over the coming 10 years by December 23, 2011, then any further increase in the debt ceiling will trigger across-the-board spending cuts equally split between security and non-security programs. The across-the-board cuts would apply to mandatory and discretionary spending in the years 2013 to 2021 in an amount of \$1.2 trillion, and are scheduled to be automatically implemented on January 2, 2013. In September 2012, the White House Office of Management and Budget issued a report detailing the possible cuts, with Medicare beneficiaries having no changes in their benefits but providers, including hospitals, having an across-the-board 2% cut to Medicare reimbursements. While the Medical Center cannot predict whether across-the-board cuts will be triggered, substantial reductions in federal health care spending through Medicare, Medicaid and other programs seem probable, and any such reductions are likely to negatively impact the Medical Center and its revenues.

Economic Turmoil

The current economic turmoil, including the downgrade of the long-term credit rating of the United States by Standard & Poor’s Ratings Service, has had and will continue to have negative repercussions upon the United States and global economies. In the last few years, this turmoil has particularly affected the financial sector, prompting a number of banks and other financial institutions to seek additional capital, to merge, and, in some cases, to cease operating. These events collectively have led to a scarcity of liquidity

and credit, lack of confidence in the financial sector, volatility in the financial markets, fluctuations in interest rates, reduced economic activity, increased business failures, and increased consumer and business bankruptcies.

Hospitals are required to provide emergency care without regard to a patient's ability to pay. Poor economic conditions and increased unemployment can enlarge the population that does not have health care coverage and thus cannot pay for care out-of-pocket, which in turn can increase the uncompensated care provided by the Medical Center. Tax-exempt hospitals, in particular, often treat large numbers of indigent patients who are unable to pay in full, or perhaps at all, for their medical care. In addition, poor economic conditions and increased unemployment can lead patients to postpone or forego elective procedures, thereby reducing volume and revenue. See "Not-for-Profit Status – IRS Examination of Compensation Practices and Community Benefit" and "Internal Revenue Code Limitations – Charity Care" herein.

If the current economic turmoil continues and the economy further weakens, health care providers could be materially and adversely impacted in a number of ways, including reduced investment income, reduced philanthropic donations, reduced access to the credit markets, difficulties in obtaining new liquidity facilities or extensions of existing liquidity or other short-term facilities, significant draws on internal liquidity due to difficulties with remarketing existing variable rate bonds and commercial paper, increased risk of acceleration on variable rate bonds or rollover risk of short-term indebtedness, increase in bad debt expense and charity care write-offs, and increased borrowing costs, any of which may negatively affect the operations or financial condition of the Medical Center.

Legislative, Regulatory and Contractual Matters Affecting Revenue

The health care industry is heavily regulated by the federal and state governments. A substantial portion of revenue comes from governmental sources. Governmental revenue sources are subject to legislative and policy changes by the governmental and private agencies that administer Medicare, Medicaid, other third-party payors and governmental payors, and actions by, among others, The Joint Commission, CMS and other federal, state and local government agencies. These agencies have broad discretion to alter or eliminate programs that contribute significantly to revenues of the Medical Center. In the past, there have been frequent and significant changes in the methods and standards used by government agencies to reimburse and regulate the operation of hospitals. See "Health Care Reform" herein for more information on current and proposed future changes in hospital reimbursement. No assurances can be given that further substantial changes will not occur in the future or that payments made under such programs will remain at levels comparable to the present levels or be sufficient to cover all existing costs. While changes are anticipated, the impact of such changes on the Medical Center cannot be predicted. The Medical Center is exploring the possibility of forming accountable care organizations (and has formed a health home) and entering into risk based (capitation) agreements, which would change the mentality of care delivery to one of promoting the wellness of a population of patients, rather than treating diseases and other conditions that result from poor health maintenance. The expectation of this evolving philosophy is to ultimately reduce the cost of health care and, therefore, benefit patients and providers alike.

The Medical Center has established estimates, based on information presently available, of amounts due to or from Medicare and non-Medicare payors for adjustments to current and prior years' payment rates, based on industry-wide and Medical Center-specific data. The current Medicaid, Medicare and other third-party payor programs are based upon extremely complex laws and regulations that are subject to interpretation. Medicare cost reports, which serve as the basis for final settlement with government payors, have been settled through 2008. The Medical Center is not aware of any allegations of noncompliance that could have a material adverse effect on its financial statements and believes that it is in compliance, in all material respects, with all applicable laws and regulations.

Legislation is periodically introduced in Congress and in the New York State Legislature that could result in limitations on the Medical Center's revenue, third-party payments and costs or charges, or that could result in increased competition or an increase in the level of indigent care required to be provided by the Medical Center. From time to time, legislative proposals are made at the federal and state level to engage in broader reform of the health care industry, such as ACA. The effects of future reform efforts on the Medical Center cannot be predicted.

State Budget

In January 2011, Governor Andrew M. Cuomo issued Executive Order No. 5 creating the Medicaid Redesign Team and setting in motion a process of substantial reform of New York's Medicaid program. The Medicaid Redesign Team, comprised of health care professionals, stakeholders in the industry and legislators, was charged with reducing Medicaid costs and improving patient care. In February 2011, the Medicaid Redesign Team issued a report containing findings and recommendations for cost reductions of over \$2.3 billion to the Governor for consideration in the budget negotiation process. The majority of these recommendations (so-called "Phase I" proposals) were included in the Final Budget for 2011-12 and passed by the New York legislature on March 31, 2011. The Final 2012-13 Budget, passed by the New York legislature on March 30, 2012, included a number of Phase II proposals designed to continue the reformation of Medicaid within New York.

The 2011-12 Final Budget included a series of changes and cost-containment measures such as programmatic reforms to Medicaid payments and program structures; the elimination of annual statutory inflation factors for hospitals, nursing homes and home and personal care providers (\$185 million); a 2% across-the-board rate reduction and other industry-specific measures (\$345 million); the acceleration of certain payments to take advantage of additional enhanced Federal Medical Assistance Percentage payments (\$66 million); mandatory managed care enrollment of previously exempt populations; changes in the benefit package and reimbursement for certain overused benefits; and creation of new integrated care models anticipated to save Medicaid dollars in the long run by improving patient care. The 2012-13 Final Budget continued the work of the Medicaid Redesign Team and included provisions calling for further redesign of the basic benefit package; additional initiatives to provide integrated care; and a state takeover of Medicaid administration from local governments.

The 2012-13 Final Budget assumes a targeted growth rate for Medicaid equal to the ten-year average change in the medical component of the Consumer Price Index (currently four percent) and grants the Executive authority to hold Medicaid spending to this rate. If spending is projected to exceed the budget cap, DOH and the State Department of Budget have the authority to develop and implement a plan of action to bring spending in line with the cap, which could include modifying or reducing reimbursement methods or program benefits. This authority expires after the 2013-14 budget year; however, the cap remains in place and the statutory authority likely will be extended in subsequent years. For 2012-13, the budget cap requires DOH to achieve savings of \$3.3 billion. The 2011-12 New York State budget year ended with Medicaid spending \$14 million below the global spending cap. According to DOH, Medicaid spending was 1% below projections and remained under the global spending cap through July 2012. Although successful in meeting the budget cap in the first 18 months, higher-than-average Medicaid enrollment threatens the ability of DOH to continue meeting the ambitious savings goal in the future.

Although the 2012-13 Final Budget contains the statutory tools necessary to implement the recommendations of the Medicaid Redesign Team, there can be no assurance that these proposals will achieve the level of gap-closing savings anticipated or limit the rate of annual growth in DOH State Funds Medicaid spending. In addition, many of the cost-saving initiatives are dependent upon timely federal approvals, appropriate amendments to the existing systems and processes and a collaborative working relationship with health care industry stakeholders. Recently, DOH submitted its Medicaid Redesign

Team Waiver Amendment which is in the process of being reviewed by CMS (see “Health Care Reform – Insurance Changes” herein).

The effect of the Medicaid redesign process on the Medical Center will depend significantly on participation in new models of integrated care delivery, the ability to collaborate with different types of providers and relationships with Medicaid managed care plans, as those plans will play an increasingly larger role over the next several years.

Medicare and Medicaid Reimbursement

Medicare is a federal health benefits program administered by CMS, fiscal intermediaries and carriers. Available to individuals age 65 or over, and certain other classes of individuals, the Medicare program provides, among other things, health care benefits that cover, within prescribed limits, the major costs of most medically necessary care for such individuals, subject to certain deductibles and co-payments.

Medicare Part A covers inpatient services and certain other services, Medicare Part B covers certain outpatient services, medical supplies, durable medical equipment and physician services, and Medicare Part C (also known as the Medicare Advantage Program) covers services for persons enrolled in Medicare managed care organizations. Medicare pays most acute care hospitals for most services provided to inpatients under a payment system known as the “Prospective Payment System” or “PPS.” Under PPS, the amounts payable for services are established in advance of the services being rendered. Separate PPS payments are made for inpatient operating costs and inpatient capital-related costs. Some costs are also paid on the basis of “reasonable cost.”

Medicaid is a federal health benefits program, also funded by states to various degrees, that is state administered. Medicaid is available only to certain low-income individuals and families who fit into an eligibility group that is recognized by federal and state law. Each state establishes the type, amount, duration and scope of services under Medicaid and the rate of payment for such services. DOH administers the New York Medicaid Program for New York State. Services are provided through use of a Medicaid card or through a Medicaid managed care plan.

Health care providers have been and will likely continue to be affected significantly by changes in federal and state health care laws and regulations, particularly those pertaining to Medicare and Medicaid. The Medicare Prescription Drug, Improvement and Modernization Act of 2003 contained many significant changes to the Medicare program, including the availability of prescription drug coverage. The Deficit Reduction Act of 2005 also contained significant changes including, among other things, various provisions to decrease spending growth in the Medicare program while increasing health care providers’ focus on quality and efficient delivery of health care services. ACA has continued this trend toward greater cost containment and performance-based payments. See “Health Care Reform” herein. Diverse and complex statutory and regulatory mechanisms, the effect of which is to limit the amount of money paid to health care providers under both the Medicare and Medicaid programs, have been enacted and approved in recent years. It is impossible to predict what effect, if any, current and future legislative initiatives related to Medicare and Medicaid may have on the operations of the Medical Center. See “APPENDIX A – MAIMONIDES MEDICAL CENTER–Sources of Patient Service Revenue” herein.

Inpatient Operating Costs

Under PPS, acute care hospitals are paid a specified amount towards their operating costs based on the Medicare Severity Diagnosis Related Group (“MS-DRG”) to which each Medicare inpatient service is assigned, which is determined by the diagnoses, procedures and other factors for each particular inpatient stay. The amount paid for each MS-DRG is established prospectively by CMS as a part of the

Medical Center's PPS, and is not related to a hospital's actual costs. For each MS-DRG, CMS assigns a weighting factor that reflects the relative hospital resources used with respect to discharges classified within that group compared to discharges classified within other groups. Each MS-DRG weight represents the average resources required to care for cases in that particular MS-DRG, relative to the national average resources consumed per case by the "average" hospital. CMS is required to adjust, or recalibrate, on a budget-neutral basis, the MS-DRG weights annually to reflect changes in treatment patterns, new technologies and other factors affecting the use of hospital resources.

To calculate the payment for a particular discharge, the MS-DRG weight is multiplied by a "standardized amount" that reflects the operating and labor costs particular to the geographic region where the hospital is located. The standardized amounts are adjusted annually based upon an annual update factor. The annual update factor is based on a hospital "market basket" index, or the percentage by which the cost of the mix of goods and services for the cost reporting period at issue will exceed the cost of such mix of goods and services for the preceding 12-month cost reporting period. Congress can apply a statutory adjustment to the market basket index for any given year. For every year since 1983, Congress has modified the increases and given substantially less than the increase in the market basket index. ACA provides for additional reductions to the market basket update, as well as other payment adjustments, in future years. See "Health Care Reform—Payment Rules" herein. There is, therefore, no assurance that future updates in MS-DRG payments will keep pace with the increases in providing inpatient hospital services. Additional payments are available, where applicable, for the direct and indirect costs of medical education, for hospitals serving a disproportionate share of patients subsidized by federal funds and for certain atypical or "outlier" cases. With the exception of outlier cases, PPS payments are not adjusted for actual costs or variations in service or length of stay. The PPS amount and adjustments described above are calculated using formulae established by CMS that are revised periodically pursuant to federal budgetary policy. There is no assurance that the Medical Center will be paid amounts that adequately reflect the actual cost of providing health care or the cost of the health care technologies available to patients.

Outpatient Services

Under Section 1833(t) of the Social Security Act, hospital outpatient services, including hospital operating and capital costs, are paid on a prospective basis under a methodology known as the outpatient prospective payment system ("OPPS"). Certain hospital supplier services, however, including certain physician and non-physician practitioner services, ambulance, physical and occupational therapy, and speech pathology services are reimbursed pursuant to fee schedules rather than pursuant to the hospital OPPS. Under hospital OPPS, predetermined amounts are paid for designated services furnished to Medicare beneficiaries. CMS classifies outpatient services and procedures, which are comparable clinically and in terms of resource use, into ambulatory payment classification ("APC") groups. Using hospital outpatient claims data from the most recent available hospital cost reports, CMS determines the median costs for the services and procedures in each APC group. Payment is made on the basis of the APC group rather than on the cost of the individual service.

Payments made under OPPS are adjusted annually based on the hospital inpatient market basket percentage increase. In addition, hospitals must submit specified quality data in order to receive the full market basket update in any given year. Failure to report such data results in a 2% reduction for all OPPS services for the subsequent year.

There can be no assurance that the hospital OPPS rate will be sufficient to cover the actual costs of the Medical Center allocable to Medicare patient care. In addition to the APC rate, there is a predetermined beneficiary coinsurance amount for each APC group. There can be no assurance that the beneficiary will pay this amount.

Medical Education Costs

Medicare pays for certain costs associated with both direct and indirect medical education (including portions of the salaries of residents and faculty and other overhead costs directly attributable to medical education programs for training residents, nurses and allied health professionals) under Section 1886(h) of the Social Security Act. Payment for direct graduate medical education (“DGME”) reimburses hospitals for the direct costs of their medical education programs, including faculty and resident salaries and other costs incurred directly and in support of the teaching programs. The payment amount for DGME costs for a cost reporting period is based on the hospital’s number of residents in that period and the hospital’s costs per resident in a base year, multiplied by the hospital’s Medicare “patient load.” Payment for the operating costs of indirect medical education is made as an adjustment to a hospital’s MS-DRG payment and based on a statutory formula determined in part by the ratio of a hospital’s number of full-time equivalent residents to its average number of staffed beds. There can be no assurance that payments to the Medical Center for providing medical education will be adequate to cover the costs attributable to medical education programs for training residents, nurses and allied health professionals.

Physician Payments

Certain physician services are paid on a national fee schedule called the “resource-based-relative-value scale” (“RB-RVS”). The RB-RVS fee schedule establishes payment amounts for all physician services, including services of provider-based physicians, and is subject to annual updates. The Sustainable Growth Rate (“SGR”), which is a limit on the growth of Medicare payments for physician services, is linked to changes in the U.S. Gross Domestic Product over a ten-year period. SGR targets are compared to actual expenditures in order to determine subsequent physician fee schedule updates. Since 2003, Congress has passed legislation to delay application of the SGR. The latest legislative act postponing the implementation of SGR cuts, which averted an approximate 25% reduction to all physician payments reimbursed under fee schedules, is effective only until December 31, 2012. There can be no assurance that Congress again will intervene to prevent the SGR payment reduction or, if delayed again, when the SGR formula will take effect, if ever.

Capital Costs

Hospitals are paid on a fully prospective basis for capital costs (including depreciation and interest) related to the provision of inpatient services to Medicare beneficiaries. Thus, capital costs are paid exclusively on the basis of a standard federal rate (based on average national costs), subject to certain adjustments (such as for disproportionate share, indirect medical education and outlier cases) specific to the Medical Center.

There can be no assurance that the prospective payments for capital costs will be sufficient to cover the actual capital-related costs of the Medical Center allocable to Medicare patient stays or to provide adequate flexibility in meeting the Medical Center’s future capital needs.

DSH Payments

In addition to making payments for services provided directly to beneficiaries, Medicare makes additional payments to hospitals that treat a disproportionately large number of low income patients. These DSH payments are determined annually based on certain statistical information submitted to the U.S. Department of Health and Human Services (“DHHS”) and are applied as a percentage addition to MS-DRG payments. Hospitals receiving Medicare DSH payments may also receive Medicaid DSH payments. The federal government distributes federal Medicaid DSH funds to each state based on a statutory formula. The states then distribute DSH payments among qualifying hospitals.

Annual Cost Reports

All hospitals participating in the Medicare and Medicaid programs must meet specific financial reporting requirements, which involve submission of annual cost reports to identify expenses associated with the services provided to Medicare and Medicaid beneficiaries. These cost reports are subject to routine audits, which may result in adjustments to the amounts ultimately determined to be due in reimbursement. The audit process may be prolonged, and it may take several years to reach the final determination of allowable amounts.

Compliance and Reimbursement

Hospitals must comply with standards called “Conditions of Participation” to be eligible for Medicare and Medicaid reimbursement. CMS is responsible for ensuring that hospitals meet these regulatory Conditions of Participation. Under applicable Medicare rules, hospitals accredited by The Joint Commission are deemed to meet the Conditions of Participation. Failure to maintain The Joint Commission accreditation or to otherwise comply with the Conditions of Participation or other applicable state licensing requirements could have a material adverse effect on the revenues of the Medical Center. However, there can be no assurance that the Medical Center will continue to receive The Joint Commission accreditation in the future.

CMS also has announced an initiative to require all Medicare-certified providers, including hospitals, to revalidate their Medicare enrollment records by March 2015 in order for CMS to implement new screening criteria mandated by ACA. Under this initiative, Medicare contractors will send mandatory revalidation requests to providers, who will have a limited time to respond to the requests. Failure to timely revalidate Medicare enrollment records for any hospital facility could result in deactivation or termination of a hospital’s provider agreement, which could adversely affect the hospital’s patient services revenues and financial performance. The Medical Center recently revalidated for its psychiatric exempt unit and is in the process of revalidating for the remainder of the hospital.

Managed Care and Other Private Initiatives

Currently, the term “managed care” refers to all commercial relationships between payors and providers. The term covers the negotiated arrangement for prices and payment terms that a health care provider will accept from a payor on behalf of a covered individual. All prices and terms are carefully articulated in contracts between providers and payors. Prices and terms differ for each hospital and for each payor and, usually, for each product sold by each payor. For example, a payor may sell HMO, PPO, Medicare and Medicaid products to various populations. That payor will then have a unique price established with each individual hospital for every covered service offered for each product sold.

Typical payment methodologies that have been established include severity-adjusted case neutral rates; per diem rates for stays in a Medical/Surgical Unit, Intensive Care Unit, and Cardiac Care Unit; case rates for obstetric deliveries, open heart surgeries and other tertiary level services; discounts from full charges; and set fees for outpatient services. Capitation and risk arrangements, which carry significant risk for providers, are not currently a material segment of the Medical Center’s payment arrangements. Management believes that the Medical Center, on a yearly contracting basis, has developed equitable pricing arrangements with most of the significant payors with which it contracts. As part of these negotiated contracts, the Medical Center has developed payment terms limiting the extent to which a payor may retroactively deny payments for services, which has been a common practice among managed care companies. The contracts also define requirements for insurers/managed care payors to conduct concurrent and prospective reviews. Some contracts contain provisions for advances and Periodic Interim Payments (PIP) as well as other terms that are financially acceptable to the Medical Center. However, these contracts have finite terms and are subject to renegotiation, and managed care payors are expected to continue to seek

ways to reduce the utilization of health care services. Traditional insurance companies and managed care organizations in the State are increasingly offering managed care programs, including various payment methodologies and utilization controls through the use of primary care physicians. Payment methodologies include per diem rates, per discharge rates, discounts from established charges, fee schedules and capitation payments. Enrollment in managed care programs has increased, and managed care programs are expected to have a greater influence on the manner in which health care services are delivered and paid for in the future. In addition, some managed care organizations have been delaying reimbursements to hospitals, thereby affecting cash flows. The Medical Center's financial condition may be adversely affected by these trends. See "APPENDIX A – MAIMONIDES MEDICAL CENTER – Patient Insurers/Managed Care" herein.

Medicare and Medicaid Managed Care

The Medicare program has encouraged the development of managed care products for Medicare beneficiaries. Enrollment in a Medicare managed care product is voluntary and enrollees may disenroll and re-enroll in the traditional fee-for-service Medicare system. Managed Medicare plans may be structured as HMOs, PPOs, private fee-for-service-plans or Provider Sponsored Organizations.

The federal Medicare program pays each managed Medicare plan a pre-established monthly premium for each Medicare beneficiary who voluntarily enrolls in the plan. The premium levels are set at a regional average price adjusted by each enrollee's age, gender and other considerations. In return for the premium, the plan pays for all the covered and medically necessary services delivered to the enrollee in the month. The plan is at full financial risk for costs incurred for caring for its enrollees in the given month, as described above.

The establishment of the Medicare Advantage program in 2003 increased reimbursement to managed Medicare plans, many of which in turn offered increased benefits coverage to beneficiaries. ACA, however, provides for reductions to managed Medicare plan payments, with the intention of aligning managed Medicare per capita premium payments with expenditures in the traditional Medicare fee-for-service program.

The Medical Center also participates in the federal and New York State Medicaid program. In order to control Medicaid expenditures, the State has sought to enroll large numbers of Medicaid patients in managed care programs because experience in other states has shown that inpatient utilization decreases for Medicaid recipients who are enrolled in such programs. The rules for the enrollment of Medicaid patients in managed care programs, premium payments to managed care organizations, and the resulting and potential financial risks to the Medical Center are similar to those already discussed above for Medicare managed care programs.

New York State's program for mandatory Medicaid managed care enrollment, The Partnership Plan (also known as the 1115 Waiver), was approved by CMS in July 1997, allowing the State to begin enrolling most Medicaid recipients in managed care plans. Mandatory Medicaid managed care enrollment programs were instituted throughout New York City, and a significant portion of the Medicaid eligible population has been enrolled in managed care plans. Since 1997, the Partnership Plan 1115 Waiver has since been extended several times, most recently as of August 2011, effective through December 31, 2014, although the waiver authority for New York's Medicaid managed care program extends only through March 31, 2014. However, New York State is requesting a 5-year extension in October 2012. The latest amendments to the Partnership Plan 1115 Waiver have further extended the groups eligible and required to enroll in Medicaid managed care, which will likely result in an increase in Medicaid managed care admissions.

The Medical Center has been attempting to retain volume and reasonable reimbursement through its ownership interest in Healthfirst and contracts with a few other large Medicaid managed care insurers. In particular, the Medical Center is working with health plans, social service agencies and others to ensure that

Medicaid patients currently cared for by the Medical Center will continue to have access to these facilities throughout the managed Medicaid enrollment process. Despite these efforts, Medicaid patient volume at the Medical Center may be reduced, partially attributable to competition from other health networks and the uncertainty relating to this historic change in the process for treating Medicaid. The teaching component of Medicaid and managed Medicaid reimbursement is expected to continue to be paid by the State directly to the hospitals.

Future actions by the federal and state governments are expected to continue the trend toward more restrictive limits on payment for hospital services. Management of the Medical Center cannot assess or predict the ultimate effect of any such legislation or regulation, if enacted or adopted, on its operations. See “APPENDIX A – MAIMONIDES MEDICAL CENTER – Patient Insurers/Managed Care” herein.

Certain Accreditations

The Medical Center has received accreditation from The Joint Commission and is subject to periodic review by The Joint Commission. No assurance can be given as to the effect on future operations of existing, or subsequently amended, laws, regulations and standards for certification or accreditation. See “APPENDIX A – MAIMONIDES MEDICAL CENTER – Licensure and Accreditation” herein.

In addition, the Medical Center sponsors programs of graduate medical education (“GME Programs”), training residents and fellows, which programs are accredited by the Accreditation Council for Graduate Medical Education (“ACGME”) (for medical programs) and by the American Dental Association (“ADA”) (for dental programs). All GME Programs are subject to periodic review by the applicable specialty Residency Review Committee of the ACGME, or by the ADA, as appropriate. No assurance can be given as to (i) the outcome of future reviews of these GME Programs (ii) such programs’ continued accreditation or (iii) the continuing eligibility of the costs associated therewith for graduate medical education reimbursement.

Competition

Payments to the hospital industry have undergone rapid and fundamental changes triggered by the deregulation of the acute care hospital reimbursement system and the requirement to negotiate, all nongovernment contracts and prices. This may further increase competitive pressures on acute care hospitals, including the Medical Center. The Medical Center faces and will continue to face competition from other hospitals, integrated delivery systems, ambulatory care providers, rehabilitation facilities, freestanding independent diagnostic treatment facilities and increasingly sophisticated physician group practices, among others that offer similar health care services as well as expanded preventive medicine treatment.

The competition for physicians has intensified in recent years, with frequent recruitment efforts by hospitals both locally and nationally to attract physicians away from competing hospitals in order to bolster admissions and profitability attributable to the patients such physicians frequently bring with them or are able to attract.

There are many limitations on the ability of a hospital to increase volume and control costs, and there can be no assurance that volume increases or expense reductions needed to maintain the financial stability of the Medical Center will occur.

Management believes that insurers will encourage competition among hospitals and providers on the basis of price, payment terms and quality. Payors have used the threat of patient steerage, restrictive physician contracting, carve outs and network exclusion to drive provider prices lower. This may lead to

increased competition among hospitals based on price where insurance companies attempt to steer patients to the hospitals that have the most favorable contracts.

Workforce Shortages

Historically, workforce shortages have affected health care organizations at the local, regional and national level though shortages have abated due to current economic conditions. There can be no assurance that such workforce shortages will not reoccur when economic conditions stabilize or as the population continues to age; such workforce shortages could adversely affect the Medical Center's ability to control costs and its financial performance in the future.

In order to recruit and retain professional and nursing staff to strengthen clinical services, the Medical Center has offered, and in the future may have to offer, competitive salaries to both newly recruited individuals and existing staff. In some years such salaries have increased, and in the future may be required to increase, more than the rate of inflation. Such increases in the future may exceed increases in the Medical Center's rates of payment.

Labor Relations and Collective Bargaining

Hospitals and other health care providers often are large employers with a wide diversity of employees. Increasingly, employees of hospitals and other providers are becoming unionized, and many hospitals and other providers have collective bargaining agreements with one or more labor organizations. Employees subject to collective bargaining agreements may include essential nursing and technical personnel, as well as food service, maintenance and other trade personnel. Renegotiation of such agreements upon expiration may result in significant cost increases to the affected members. In addition, employee strikes or other adverse labor actions may have an adverse impact on the Medical Center. See "APPENDIX A – MAIMONIDES MEDICAL CENTER – Collective Bargaining Agreements/Employee Relations" herein.

Multiemployer Pension Plans

Certain Medical Center employees are covered by defined benefit multiemployer pension plans (each, a "Plan") to which the Medical Center makes contributions pursuant to collective bargaining agreements. Each Plan covers employees of multiple unrelated employers, and employers do not typically have access to complete and current information concerning the funding status of a Plan. Plans carry with them the risk that benefit liabilities associated with one participating employer may, over time, be shouldered by other participating employers through increased contributions payable by them, for example where a participating employer is unable to make its required contributions (e.g., due to bankruptcy). Further, under pension regulations, all members of a "controlled group," including such a participating employer, determined under Internal Revenue Service rules, generally are jointly and severally liable together with such participating employer to make contributions to the Plan.

Entities that participate in a Plan, including the Medical Center, are subject to various risks, including but not limited to lack of transparency concerning the full extent of the funding status of the Plan; lack of transparency concerning creditworthiness of other employers participating in the Plan (and attendant risk liability for shortfalls in funding by such other employers); unpredictable spikes in pension cost upon renewal of collective bargaining agreements due to underfunding of the Plan resulting from failure by other employers to contribute to the Plan as required or other causes such as adverse investment results with respect to Plan assets or increases in Plan liabilities due to benefit increases or changes in actuarial assumptions; withdrawal liabilities as described above; and other factors which may be outside the knowledge or control of the Medical Center.

Under current generally accepted accounting principles, the extent of any funding shortfall in a Plan is not recorded as a liability of a participating employer on its financial statements, although the amount of such funding shortfall which may be allocated to such participating employer may be material.

Changes in generally accepted accounting principles which took effect for the Medical Center's fiscal year ended December 31, 2011 required that an employer's financial statements reflect certain additional information concerning the extent of its participation in a Plan, the most recent certified funding "zone" status of the Plan, and certain other information, not including the dollar amount of any current underfunding of such Plan (which, as noted above, is not generally known by the employers on a current basis). See "APPENDIX A – MAIMONIDES MEDICAL CENTER – Collective Bargaining Agreements/Employee Benefits" and "–Pension Plans and Post-Retirement Health Care Benefits" herein.

Federal Fraud and Abuse Laws and Regulations

The federal Anti-Kickback Law is a criminal statute that prohibits anyone from knowingly or willfully offering, paying, soliciting or receiving any remuneration, directly or indirectly, in return for or to induce business that may be paid for, in whole or in part, under a federal health care program including, but not limited to, the Medicare or Medicaid programs. ACA amended the Anti-Kickback Law to provide that a claim that includes items or services resulting from a violation of the Anti-Kickback Law now constitutes a false or fraudulent claim for purposes of the False Claims Act. Violation of the Anti-Kickback Law is a felony, subject to a maximum fine of \$25,000 for each criminal act, imprisonment for up to five years and exclusion from the Medicare and Medicaid programs. The Office of Inspector General of the United States Department of Health and Human Services (the "OIG"), the enforcement arm of DHHS, can also initiate an administrative exclusion of a provider from the Medicare and Medicaid programs. In addition, civil monetary penalties of \$50,000 for each act in violation of the Anti-Kickback Law or damages equal to three times the amount of prohibited remuneration may be imposed and violation of this law also renders the violator civilly liable under the False Claims Act. The scope of prohibited payments in the Anti-Kickback Law is broad and includes many economic arrangements involving hospitals, physicians and other health care providers, including, but not limited to, joint ventures, space and equipment rentals, recruitment of physicians, purchases of physician practices, and management and personal services contracts. However, OIG has established regulatory safe harbors for various payment and business practices that are safe from prosecution though they may be prohibited from the Anti-Kickback Law.

The outcome of any government efforts to enforce the Anti-Kickback Law against health care providers is difficult to predict due, in part, to government discretion in pursuing enforcement and the lack of significant case law.

Federal and State False Claims Acts

The federal criminal False Claims Act ("criminal FCA") makes it illegal to submit or present a false, fictitious or fraudulent claim to the federal government. Violation of the criminal FCA can result in imprisonment and/or a fine. The federal civil False Claims Act ("civil FCA"), is one of the government's primary weapons against health care fraud. Under the civil FCA, those who knowingly submit, or cause another person or entity to submit, false claims for payment of government funds are liable for three times the government's damages plus civil penalties ranging from \$5,500 to \$11,000 per false claim.

On May 20, 2009, the Fraud Enforcement and Recovery Act of 2009 ("FERA") was signed into law. It included significant amendments to the civil FCA. Among other items, FERA expanded the scope of potential civil FCA liability, increased the Attorney General's power to delegate authority to investigate a civil FCA case prior to intervening in a civil FCA action and increased protections for whistleblower plaintiffs beyond employees. ACA also amended the civil FCA by expanding the numbers of activities that are subject to enforcement as violations of the civil FCA including, among other actions,

failure to report and return to a federal health care program a known overpayment within 60 days of having identified the overpayment or, for cost-reporting entities, the date (if later) on which a hospital cost report is due.

The State of New York also has a False Claims Act which closely tracks the federal civil FCA. It imposes penalties and fines on individuals and entities that file false or fraudulent claims for payment from any state or local government, including health care programs such as Medicaid. The civil FCA and the New York State False Claims Act also permit individuals to initiate actions on behalf of the government in lawsuits called *qui tam* actions. These *qui tam* plaintiffs, or “whistleblowers,” can share in the damages recovered by the government.

Under the civil FCA and the New York State False Claims Act, health care providers may be liable if they take steps to obtain improper payments from the government by submitting false claims or failing to refund known overpayments. Civil FCA and the New York State False Claims Act violations have been alleged solely on the existence of alleged kickback or self-referral arrangements. Even in the absence of evidence that literally false claims have been submitted, these cases argue that the improper business relationship tainted the subsequently submitted claims, thereby rendering the claims false under the civil FCA and the New York State False Claims Act. Other FCA and New York State False Claims Act cases have proceeded on a theory that providers are liable for the submission of false claims when they are not in full compliance with applicable legal and regulatory standards. It is impossible to predict with certainty whether courts will uniformly hold that regulatory non-compliance and anti-kickback or self-referral violations are subject to prosecutions as false claims. If a provider is faced with a civil FCA and New York State False Claims Act prosecution based on one of these theories, however, allocation of the funds required to contest or settle the matter could have a material adverse impact on the Medical Center and, potentially, its affiliates.

Limitations on Certain Arrangements Imposed by Federal Ethics in Patient Referrals Act

The federal Ethics in Patient Referrals Act (known as the “Stark Law”) prohibits the referral of Medicare and Medicaid patients for certain “designated health services” to entities with which the referring physician (or an immediate family member of such physician) has a financial relationship. The statute also prohibits the entity furnishing the “designated health services” from billing the Medicare or Medicaid program for designated health services furnished pursuant to a prohibited referral. The designated health services subject to these prohibitions are clinical laboratory services, physical and occupational therapy services, radiology services (including magnetic resonance imaging, computerized tomography and ultrasound), radiation therapy services and supplies (not including nuclear medicine), durable medical equipment and supplies, parenteral and enteral nutrients (including equipment and supplies), orthotic and prosthetic devices and supplies, speech language pathology, home health services, outpatient prescription drugs, and inpatient and outpatient hospital services (not including lithotripsy).

The New York Health Care Practitioner Referral Law (the “State Provisions”) is similar to the Stark Law; however, it covers all patients (irrespective of payor) and prohibits practitioners from referring a patient to a health care provider for clinical laboratory services, x-ray imaging services, radiation therapy services, physical therapy or pharmacy services if the referring practitioner (or an immediate family member) has a financial interest in the health care provider.

A financial relationship, for purposes of the Stark Law and State Provisions (the Stark Law and State Provisions are hereinafter collectively referred to as “Stark”), is defined as either an ownership or investment interest in the entity or a compensation arrangement between the practitioner (or immediate family member) and the entity. An ownership or investment interest may be through equity, debt, or other means and includes an interest in an entity that holds an ownership or investment interest in an entity providing the designated health services. Many ordinary business practices and economically

desirable arrangements with physicians would constitute “financial relationships” within the meaning of Stark.

Stark provides certain exceptions to these restrictions, but these exceptions are narrow and an arrangement must fully comply with an exception. If the relationship (which would include compensation arrangements such as employment and other professional services relationships, and ownership or investment interests) between a physician/practitioner and the hospital cannot be made to fit within the exceptions, the hospital will not be permitted to accept referrals for designated services from the physician/practitioner who has such financial relationship.

Violations of Stark can result in denial or refund of payment, substantial civil money penalties, and exclusion from the Medicare and Medicaid programs. In certain circumstances, knowing violations may also create liability under the civil FCA or criminal FCA. Enforcement actions for any such violations could have a material adverse impact on the financial condition of a health care provider, including the Medical Center.

Regulation of Patient Transfers

Federal and New York laws require hospitals to provide emergency treatment to all persons presenting themselves with emergency medical conditions. Congress enacted the Emergency Medical Treatment and Active Labor Act (“EMTALA”) in response to concerns regarding inappropriate hospital transfers of emergency patients based on the patient’s inability to pay for the services provided. EMTALA requires hospitals with emergency rooms, including the Medical Center, to treat or conduct an appropriate and uniform medical screening for emergency conditions (including active labor) on all patients and to stabilize a patient’s emergency medical condition before releasing, discharging or transferring the patient to another hospital.

Failure to comply with EMTALA can result in exclusion from the Medicare and/or Medicaid programs as well as civil penalties of up to \$50,000 per violation. In addition, a hospital will be liable for any claim by an individual who has suffered harm as a result of such violation.

Civil Monetary Penalty Act

The federal Civil Monetary Penalty Act (“CMPA”) provides for administrative sanctions against health care providers for a broad range of billing and other abuses. A health care provider is liable under CMPA if it knowingly presents, or causes to be presented, improper claims for reimbursement under Medicare, Medicaid and other federal health care programs. A hospital that participates in arrangements known as “gain sharing” by paying a physician to limit or reduce services to Medicare fee-for-service beneficiaries that is done on an improper basis also would be subject to CMPA penalties. A health care provider that provides benefits to Medicare or Medicaid beneficiaries that the provider knows or should know are likely to induce the beneficiaries to choose the provider for their care also would be subject to CMPA penalties. CMPA authorizes imposition of a civil money penalty and treble damages. ACA amended CMPA to establish various new grounds for exclusion and civil monetary penalties, as well as increased penalty thresholds for existing civil monetary penalties.

Health care providers may be found liable under CMPA even when they did not have actual knowledge of the impropriety of their action. Knowingly undertaking the action is sufficient and ignorance of the Medicare regulations is no defense. The imposition of civil money penalties on a health care provider could have a material adverse impact on the provider’s financial condition.

Exclusions from Medicare or Medicaid Participation

The Secretary of DHHS is required to exclude from governmental program participation (including Medicare and Medicaid) for not less than five years any individual or entity who has been convicted of a criminal offense relating to the delivery of any item or service reimbursed under Medicare or a state health care program, any criminal offense relating to patient neglect or abuse in connection with the delivery of health care, felony fraud against any federal, state or locally financed health care program or an offense relating to the illegal manufacture, distribution, prescription or dispensing of a controlled substance. DHHS also may exclude individuals or entities under certain other circumstances, such as an unrelated conviction of fraud, theft, embezzlement, breach of fiduciary duty or other financial misconduct relating either to the delivery of health care in general or to participation in a federal, state or local government program. The New York State Office of the Medicaid Inspector General (“OMIG”) also has the authority to exclude individuals and entities from participation in Medicaid. Providers are excluded for reasons that may include program-related convictions, patient abuse or neglect convictions, and licensing board disciplinary actions. ACA authorizes the Secretary of DHHS to exclude a provider from participation in Medicare and Medicaid, as well as to suspend payments to a provider pending an investigation or prosecution of a credible allegation of fraud against the provider.

Health Insurance Portability and Accountability Act

The Health Insurance Portability and Accountability Act of 1996 (“HIPAA”) established civil and criminal sanctions for health care fraud which expanded upon prior health care fraud laws and applies to health care benefit programs.

HIPAA also provides for punishment of a health care provider for knowingly and willfully embezzling, stealing, converting or intentionally misapplying any money, funds, securities, premiums, credits, property or other assets of a health care benefit program. A health care provider convicted of health care fraud could be subject to mandatory exclusion from the Medicare program.

HIPAA also required DHHS to adopt national standards for electronic health care transactions, including:

- (i) standardized electronic transaction formats and code sets to allow standardized electronic transmission of health care claims and information;
- (ii) unique identifiers to support these standard transmissions;
- (iii) comprehensive privacy standards establishing a minimum threshold for determining when to allow access to or disclosure of personal health information (the “Privacy Rule”); and
- (iv) security mechanisms to guard against unauthorized access to health information (the “Security Rule”).

HIPAA imposes civil monetary penalties for violations and criminal penalties for knowingly obtaining or using individually identifiable health information. The penalties range from \$50,000 to \$250,000 or imprisonment for up to 10 years if the information was for a violation of willful neglect or for a violation related to the intent to sell, transfer or use the individually identifiable health information for commercial advantage, personal gain or malicious harm.

Compliance with HIPAA has required changes in information technology platforms, major operational and procedural changes in the handling of data, and vigilance in monitoring of ongoing

compliance with the various regulations. The Medical Center has implemented HIPAA training and ongoing monitoring, which have been in place since April 2003. The financial cost of compliance with the “administrative simplification” regulations is substantial.

On February 17, 2009, President Obama signed into law the Health Information Technology for Economic and Clinical Health Act (the “HITECH Act”). The HITECH Act expands the scope and application of the administrative simplification provisions of HIPAA, and its implementing regulations, (i) extending the reach of the Privacy Rule and Security Rule to business associates, (ii) imposing a written notice obligation upon covered entities for security breaches involving “unsecured” protected health information, (iii) limiting certain uses and disclosures of protected health information, (iv) increasing individuals’ rights with respect to protected health information, (v) increasing penalties for violations, and (vi) providing for enforcement of violations by State attorney generals. Many of the HITECH Act’s provisions became effective on February 17, 2010, but other provisions require implementing regulations and may become effective at some point in 2013 or thereafter. While the effects of the HITECH Act cannot be predicted at this time, the obligations imposed thereunder could have a material adverse effect on the financial condition of the Medical Center.

Under the HITECH Act, the Electronic Health Record (“EHR”) Incentive Program provides incentive payments to eligible professionals, eligible hospitals and critical access hospitals participating in Medicare and Medicaid to adopt and successfully demonstrate meaningful use of certified EHR technology. The Medical Center has received incentive payments for 2012 and expects to receive incentive payments for 2013 and 2014. CMS has begun audits to determine if recipients were entitled to the incentive payments and, starting in 2015, non-compliant recipients will have payment adjustments made to their Medicare reimbursements.

The Office for Civil Rights of DHHS (OCR) is putting increased emphasis on enforcement. In 2012, OCR entered into a number of highly publicized, high value settlements with HIPAA-covered entities stemming from alleged violations of HIPAA. The settlements are also noteworthy because they indicate that OCR is interested in enforcing violations of the HIPAA Security Rule, not just the HIPAA Privacy Rule. In addition, at the end of 2011, OCR launched a HIPAA Privacy & Security Audit Program pursuant to which audits conducted by an outside consultant, KPMG, are now underway across the country. There have also been additional cases where state attorneys general, exercising the powers given them under the HITECH Act, have brought actions against covered entities for alleged HIPAA violations seeking significant penalties.

Security Breaches and Unauthorized Releases of Personal Information

State and local authorities are increasingly focused on the importance of protecting the confidentiality of individuals’ personal information, including patient health information. Many states have enacted laws requiring businesses to notify individuals of security breaches that result in the unauthorized release of personal information. In some states, notification requirements may be triggered even where information has not been used or disclosed, but rather has been inappropriately accessed. State consumer protection laws may also provide the basis for legal action for privacy and security breaches, and frequently, unlike HIPAA, authorize a private right of action. In particular, the public nature of security breaches exposes health organizations to increased risk of individual or class action lawsuits from patients or other affected persons, in addition to government enforcement. Failure to comply with restrictions on patient privacy or to maintain robust information security safeguards, including taking steps to ensure that contractors who have access to sensitive patient information maintain the confidentiality of such information, could consequently damage a health care provider’s reputation and materially adversely affect business operations.

New York State has numerous laws impacting dissemination of specific health-related information. For example, the New York State Information Security Breach and Notification Act, effective December 8, 2005, requires any entity conducting business in New York State to disclose any “breach of the security of a system” to any New York resident whose unencrypted personal information was acquired by an unauthorized person. Personal information means computerized unencrypted identifying information including any of the following three data elements: social security number, driver’s license number, and account number or credit or debit card number if the personal identification number was also disclosed. The Attorney General of the State of New York can sue to recover actual damages including financial losses incurred by New York residents, who are entitled to receive notification and a civil penalty for a knowing or reckless violation of up to the greater of \$5,000 or \$10.00 per instance of failed notification up to \$150,000. Statutory remedies are in addition to other lawful remedies such as private actions. New York State also has a number of laws protecting the use and disclosure of social security numbers pursuant to which civil penalties in the amount of \$1,000 for a single violation and not more than \$100,000 for multiple violations may be imposed. For a second or subsequent violations penalties run from not more than \$5,000 for a single violation to not more than \$250,000 for multiple violations resulting from a single incident.

Enforcement Activity

Enforcement activity against health care providers and funding for such enforcement activity has increased, and enforcement authorities are adopting more aggressive approaches. ACA provided \$10 million per year for fiscal years 2011-2020 and an additional \$250 million over fiscal years 2011-2016 for various health care fraud initiatives. In the current regulatory climate, it is anticipated that many hospitals will be subject to billing or false claim investigations, audits or inquiries. Due to the complexity of the Federal and State fraud and abuse laws, the instances in which an alleged violation may trigger such investigations, audits or inquiries are increasing and could result in enforcement action against the Medical Center.

Enforcement authorities are sometimes in a position to compel settlements from providers charged with, or being investigated for, false claims violations by withholding or threatening to withhold Medicare, Medicaid or similar payments or by threatening the possibility of a criminal action. In addition, the cost of defending such an action, the time and management attention consumed thereby and the facts of a particular case may dictate settlement. Therefore, regardless of the merits of a particular case or cases, the Medical Center could experience materially adverse settlement costs, as well as materially adverse costs associated with the implementation of any settlement agreement. Prolonged and publicized investigations could be damaging to the reputation, business and credit of the Medical Center, regardless of the outcome, and could have material adverse consequences on the financial condition of the Medical Center.

Increased Enforcement Affecting Academic Research

In addition to increasing enforcement of laws governing payment and reimbursement, the federal government has also increased enforcement of laws and regulations governing the conduct of clinical trials at hospitals. DHHS elevated and strengthened its Office of Human Research Protection, one of the agencies with responsibility for monitoring federally funded research. In addition, the National Institute of Health (“NIH”) significantly increased the number of facility inspections that these agencies perform. The United States Food and Drug Administration (“FDA”) also has authority over the conduct of clinical trials performed in hospitals when these trials are conducted on behalf of sponsors seeking FDA approval to market the drug or device that is the subject of the research. Moreover, OIG, in past “Work Plans” has included several enforcement initiatives related to reimbursement for experimental drugs and devices (including kickback concerns) and has issued compliance program guidance directed at recipients of extramural research awards from the NIH and other agencies of the U.S. Public Health Service. The

Medical Center receives payments for health care items and services under many of these grants and is subject to complex and, at times, ambiguous coverage principles and rules governing billing for items or services it provides to patients participating in clinical trials funded by governmental agencies and private sponsors. These agencies' enforcement powers range from substantial fines and penalties to exclusion of researchers and suspension or termination of entire research programs, and errors in the billing of Medicare for care provided to patients enrolled in clinical trials that are not eligible for Medicare reimbursement can subject the Medical Center to sanctions as well as repayment obligations.

Increased Costs and State-Regulated Reimbursement

In recent years, substantial cutbacks in personnel and other cost-cutting measures have been instituted at hospitals throughout the State. Generally, these cutbacks have been instituted to address the disparity between rising medical costs and State-regulated reimbursement formulas, including those for Medicaid, Blue Cross and Blue Shield, and other third-party payors. Rising health care costs resulted from, among other factors, health care costs exceeding inflation, staff shortages, increased pharmaceutical and medical device costs, and the highly technical nature of the industry. The Medical Center has been affected by the impact of such rising costs, and there can be no assurance that the Medical Center would not be similarly affected by the impact of additional unreimbursed costs in the future.

Department of Health Regulations

The Medical Center is subject to regulations of DOH. Compliance with such regulations may require substantial expenditures for administrative or other costs. The Medical Center's ability to add services or beds and to materially modify existing services is also subject to DOH review and approval. Approvals can be highly discretionary, may involve substantial delay and may require substantial changes in the proposed request. Accordingly, the Medical Center's ability to make changes to its service offerings and respond to changes in the environment may be limited.

New York State Executive Order

On January 18, 2012, Governor Andrew Cuomo signed an Executive Order limiting spending for administrative costs and executive compensation at state-funded service providers. The Medical Center may be subject to the limitations contained in the Executive Order. The Executive Order limits reimbursement with State funds for executive compensation to \$199,000 annually per executive and requires that 75% (increasing to 85% by April 1, 2015) of State-authorized payments be directed to direct care or services, rather than administrative costs. On May 30, 2012, DOH published proposed regulations to implement the Executive Order, effective January 1, 2013. However, the way in which the final regulations may impact the Medical Center remains unclear. Accordingly, it is impossible at this time to predict what changes in accounting or practices might be required of the Medical Center as a result of these regulations.

Other Governmental Regulation and Accreditation Activity

The Medical Center is subject to regulatory actions and policy changes by those governmental and private agencies that administer the Medicare and Medicaid programs, and actions by, among others, the U.S. Department of Labor, the National Labor Relations Board, professional and industrial associations of staff and employees, applicable professional review organizations, The Joint Commission, the Environmental Protection Agency, the Internal Revenue Service ("IRS"), other federal, state and local governmental agencies, and by the various federal, state and local agencies created by the National Health Planning and Resources Development Act and the Occupational Safety Health Act. Hospitals, including the Medical Center, are also periodically subject to audits and other investigations relating to various segments of their operations which could result in expensive and prolonged enforcement actions.

Renewal and continuation of certain of these licenses, certifications and accreditations are based on inspections, surveys, audits, investigations or other reviews, some of which may require or include affirmative activity or response by the Medical Center. These activities generally are conducted in the normal course of business of health facilities. Nevertheless, an adverse result could cause a loss or reduction in the Medical Center's scope of licensure, certification or accreditation, could reduce the payment received or could require repayment of amounts previously remitted to the provider.

OIG and OMIG Compliance Guidelines

On February 23, 1998, OIG published Compliance Program Guidance ("CPG") for the hospital industry. In recognition of the significant changes in the delivery and reimbursement for hospital services that have occurred since the CPG's publication, OIG published Supplemental Compliance Program Guidance on January 31, 2005. These issuances (collectively, the "Guidances") provide recommendations to hospitals for adopting and implementing effective programs to promote compliance with applicable federal and state law and the program requirements of federal, state, and private health plans, and they include a discussion of significant risk areas for hospitals. Compliance with the Guidances is currently voluntary, but as discussed in "Health Care Reform" above, every hospital will soon be required to establish and maintain a compliance program that meets the Guidances. Regardless, compliance with the Guidances is an important factor in controlling risk because OIG will consider the existence of an effective compliance program that pre-dated any governmental investigation when addressing the appropriateness of administrative penalties. The Medical Center maintains a corporate compliance program that is designed to assist the Board and staff to meet or exceed applicable standards established by federal and state laws and regulations. However, the existence of a compliance program or compliance therewith does not guarantee that health care providers, such as the Medical Center, will not be investigated by one or more federal or state agencies that enforce health care fraud and abuse laws or that they will not be required to make repayments to various health care insurers (including the Medicare and/or Medicaid programs).

Since October 2009, hospitals in New York have been required by statute and regulation to have an effective compliance program that meets applicable statutory regulatory Medicaid program requirements. The compliance program must include, among other things, a chief compliance officer, written policies and the conduct of audits after the identification of risk areas. OMIG may conduct audits of compliance programs and assess their effectiveness. Under New York law, each year the Medical Center must certify that it has a compliance program in place and that it has been effective, and management of the Medical Center has advised that it will so certify this year.

Not-for-Profit Status

As a non-profit tax-exempt organization, the Medical Center is subject to federal, state and local laws, regulations, rulings and court decisions relating to its organization and operation, including its operation for charitable purposes. At the same time, the Medical Center conducts large-scale complex business transactions and is a significant employer in its geographic area. The rules designed to regulate a wide range of charitable organizations are required to be integrated into the day-to-day operations of a complex health care organization.

Recently, an increasing number of the operations or practices of health care providers have been challenged or questioned to determine if they are consistent with the regulatory requirements for non-profit tax-exempt organizations. These challenges, in some cases, are broader than concerns about compliance with federal and state statutes and regulations, such as Medicare and Medicaid compliance, and instead, in many cases, are examinations of core business practices of the health care organizations. Areas that have come under examination have included pricing practices, billing and collection practices, charitable care, executive compensation, exemption of property from real property taxation and others.

For example, in August of 2011, the real estate tax exemption of three Illinois-based hospitals was revoked for failing to provide sufficient charity care. These challenges and questions have come from a variety of sources, including state attorney generals, the IRS, labor unions, Congress, state legislatures, consumer groups and patients, and in a variety of forums, including hearings, audits and litigation.

Revision of IRS Form 990 for Not-for-Profit Corporations

The IRS Form 990 is used by 501(c)(3) not-for-profit organizations (including the Medical Center) to submit information required by the federal government for reporting purposes. The revised Form 990 requires detailed public disclosure of compensation practices, corporate governance, loans to management and others, joint ventures and other types of transactions, political campaign activities, and other areas the IRS deems to be compliance risk areas. The revised form also requires the disclosure of a significantly greater amount of both hard data and anecdotal information on community benefit information on Schedule H. It establishes uniform standards for reporting of information relating to tax-exempt bonds, including compliance with the arbitrage rules and rules limiting private use of bond-financed facilities, including compliance with the safe harbor guidance in connection with management contracts and research contracts. The redesigned Form 990 is intended to result in enhanced transparency as to the operations of exempt organizations. It is also likely to result in enhanced enforcement, as the redesigned Form 990 will make a wealth of detailed information on compliance risk areas available to the IRS and other enforcement agencies. Completion of the revised Form 990 has placed an additional burden on the Medical Center.

IRS Examination of Compensation Practices and Community Benefit

In 2004, the IRS began a new compliance program to measure compliance by tax-exempt organizations with requirements that they not pay excessive compensation and benefits to their officers and other insiders. In February 2009, the IRS issued its Hospital Compliance Project Final Report (the “IRS Final Report”) that examined tax-exempt hospitals’ practices and procedures with regard to compensation and benefits paid to their officers and other defined “insiders.” The IRS Final Report indicates that the IRS (1) will continue to heavily scrutinize executive compensation arrangements, practices and procedures of tax-exempt hospitals and other tax-exempt organizations; and (2) in certain circumstances, may conduct further investigations or impose fines on such organizations.

The IRS has also undertaken a community benefit initiative directed at hospitals. The most recent IRS report on this initiative determined that a lack of uniformity in definitions of community benefit used by reporting hospitals, including those regarding uncompensated care and various types of community benefit, made it difficult for the IRS to assess whether any particular hospital is in compliance with current law. The revised Form 990 includes a new schedule, Schedule H, which hospitals must use to report their community benefit activities, including the cost of providing charity care and other tax-exemption related information. Proposals have also been made within Congressional committee to codify the requirements for hospitals’ tax-exempt status, including requirements to conduct a regular community needs analysis and to provide minimum levels of charity care.

ACA also contains new requirements for tax-exempt hospitals. Under ACA, each tax-exempt hospital facility is required to (i) conduct a community health needs assessment at least every three years and adopt an implementation strategy to meet the identified community needs, (ii) adopt, implement and widely publicize a written financial assistance policy and a policy to provide emergency medical treatment without discrimination, (iii) limit charges to individuals who qualify for financial assistance under such tax-exempt hospital’s financial assistance policy to no more than the amounts generally billed to individuals who have insurance covering such care and refrain from using “gross charges” when billing such individuals, and (iv) refrain from taking extraordinary collection actions without first making reasonable efforts to determine whether the individual is eligible for assistance under such tax-exempt

hospital's financial assistance policy. In addition, the U.S. Treasury Department is required to review information about each tax-exempt hospital's community benefit activities at least once every three years, as well as to submit an annual report to Congress with information regarding the levels of charity care, bad debt expenses, unreimbursed costs of government programs, and costs incurred by tax-exempt hospitals for community benefit activities. The periodic reviews and reports to Congress regarding the community benefits provided by 501(c)(3) hospitals may increase the likelihood that Congress will require such hospitals to provide a minimum level of charity care in order to retain tax-exempt status and may increase IRS scrutiny of particular 501(c)(3) hospital organizations. In addition, on June 26, 2012, the IRS proposed regulations providing guidance on the requirements for Section 501(c)(3) hospitals, under Section 501(r) of the Code, relating to financial assistance and emergency care policies, charges for certain care provided to individuals eligible for financial assistance, and billing and collections, by, among other things, limiting extraordinary collection actions by Section 501(c)(3) hospitals.

Internal Revenue Code Limitations

Private Inurement and Excess Benefit Transactions

The Internal Revenue Code of 1986, as amended (the "Code") contains restrictions on the issuance of tax-exempt bonds for the purpose of financing and refinancing different types of health care facilities for not-for-profit organizations, including facilities generating taxable income. Consequently, the Code could adversely affect the Medical Center's ability to finance its future capital needs and could have other adverse effects on the Medical Center that cannot be predicted at this time. The Code continues to subject unrelated business income of non-profit organizations to taxation.

As a tax-exempt organization, the Medical Center is limited with respect to the use of practice income guarantees, reduced rent on medical office space, below market rate interest loans, joint venture programs, and other means of recruiting and retaining physicians. The IRS has recently intensified its scrutiny of a broad variety of contractual relationships commonly entered into by hospitals and affiliated entities, including the Medical Center, and has issued detailed hospital audit guidelines suggesting that field agents scrutinize numerous activities of hospitals in an effort to determine whether any action should be taken with respect to limitations on, or revocation of, their tax-exempt status or assessment of additional tax. The IRS has also commenced intensive audits of select health care providers to determine whether the activities of these providers are consistent with their continued tax-exempt status. The IRS has indicated that, in certain circumstances, violation of the fraud and abuse statutes could constitute grounds for revocation of a hospital's tax-exempt status.

Any suspension, limitation, or revocation of the tax-exempt status of the Medical Center or assessment of significant tax liability could have a material adverse effect on the Medical Center.

Section 501(c)(3) of the Code specifically conditions the continued exemption of all Section 501(c)(3) organizations upon the requirement, among others, that no part of the net earnings of the organization inure to the benefit of any private individual. Any violation of the prohibition against private inurement may cause the organization to lose its tax-exempt status under Section 501(c)(3) of the Code. The IRS has issued guidance in informal private letter rulings and general counsel memoranda on some situations that give rise to private inurement, but there is no definitive body of law and no regulations or public advisory rulings that address many common arrangements between exempt health care providers and nonexempt individuals or entities. There can be no assurance concerning the outcome of an audit or other investigation given the lack of clear authority interpreting the range of activities undertaken by the Medical Center.

Intermediate sanctions legislation enacted in 1996 imposes penalty excise taxes in cases where an exempt organization is found to have engaged in an "excess benefit transaction" with a "disqualified

person.” Such penalty excise taxes may be imposed in lieu of revocation of exemption or in addition to such revocation in cases where the magnitude or nature of the excess benefit calls into question whether the organization functions as a public charity. The tax is imposed both on the disqualified person receiving such excess benefit, and on any officer, director, trustee or other person having similar powers or responsibilities who participated in the transaction willfully or without reasonable cause, knowing it will involve “excess benefit.” “Excess benefit transactions” include transactions in which a disqualified person receives unreasonable compensation for services or receives other economic benefit from the organization that either exceeds fair market value or, to the extent provided in regulations yet to be promulgated, is determined in whole or in part by the revenues of one or more activities of such organization. “Disqualified persons” include “insiders” such as board members and officers, senior management and members of the medical staff, who in each case are in a position to substantially influence the affairs of the organization, their family members and entities which are more than 35% controlled by a disqualified person.

Although the Medical Center believes that the sanction of revocation of tax-exempt status is likely to be imposed only in cases of pervasive excess benefit, the imposition of penalty excise tax in lieu of revocation, based upon a finding that the Medical Center engaged in an excess benefit transaction, is likely to result in negative publicity and other consequences that could have a materially adverse effect on the operations, property or assets of the Medical Center.

Charity Care

Hospitals are permitted to have tax-exempt status under the Code because the provision of health care historically has been treated as a “charitable” enterprise. This treatment arose before most Americans had health insurance, when charitable donations were required to fund the health care provided to the sick and disabled. Some have posited that, with the onset of employer health insurance and government reimbursement programs, there is no longer any justification for special tax treatment for the not-for-profit health care sector, and the availability of tax-exempt status should be eliminated. Management of the Medical Center cannot predict the likelihood for such a dramatic change in the law. Federal and state tax authorities are beginning to demand that tax-exempt hospitals justify their tax-exempt status by documenting their charitable care and other community benefits. See “Not-for-Profit Status – IRS Examination of Compensation Practices and Community Benefit” herein.

Tax Audits

Taxing authorities historically have conducted tax audits of non-profit organizations to confirm that such organizations are in compliance with applicable tax rules and in some instances have collected significant payments as part of the settlement process. The Medical Center is not currently under any audit.

Antitrust

Enforcement of the antitrust laws against health care providers is becoming more common. Antitrust liability may arise in a wide variety of circumstances including medical staff privilege disputes, payor contracting, physician relations, joint ventures, merger, affiliation and acquisition activities, and certain pricing and salary setting activities. Actions can be brought by federal and state enforcement agencies seeking criminal and civil penalties and, in some instances, by private litigants seeking damages for harm arising out of allegedly anti-competitive behavior. Common areas of potential liability include joint action among providers with respect to payor contracting, medical staff credentialing and issues relating to market share. Liability in any of these or other trade regulation areas may be substantial, depending on the facts and circumstances of each case.

Prompted by ACA, which promotes the formation of accountable care organizations among otherwise independent providers and provider groups, the U.S. Federal Trade Commission and U.S. Department of Justice issued a policy statement on October 28, 2011. The policy statement outlines, among other things, a rule of reason approach to whether a collaboration through these organizations is likely to have anticompetitive effects, and, if so, whether the collaboration's potential procompetitive efficiencies are likely to outweigh these effects.

Some judicial decisions have permitted physicians who are subject to disciplinary or other adverse actions by a hospital at which they practice, including denial or revocation of medical staff privileges, to seek treble damages from the hospital under the federal antitrust laws. The Federal Health Care Quality Improvement Act of 1986 provides immunity from liability for discipline of physicians by hospitals under certain circumstances, but courts have differed over the nature and scope of this immunity. In addition, hospitals occasionally indemnify medical staff members who incur costs as defendants in lawsuits involving medical staff privilege decisions. Some court decisions have also permitted recovery by competitors claiming harm from a hospital's use of its market power to obtain unfair competitive advantage in expanding into ancillary health care businesses. Antitrust liability in any of these contexts can be substantial, depending upon the facts and circumstances involved. There can be no assurance that a third party reviewing the activities of the Medical Center would find such activities to be in full compliance with the antitrust laws.

Environmental Matters

Health care providers are subject to a wide variety of federal, state and local environmental and occupational health and safety laws and regulations. These requirements govern medical and toxic or hazardous waste management, air and water quality control, notices to employees and the public, and training requirements for employees. As an owner and operator of properties and facilities, the Medical Center may be subject to potentially material liability for costs of investigating and remediating the release of any such substances either on, or that have migrated off, the property, or that have been improperly disposed of off-site. Typical health care provider operations include, but are not limited to, in various combinations, the handling, use, storage, transportation, disposal and/or discharge of hazardous, infectious, toxic, radioactive, flammable and other hazardous materials, medical and other wastes, pollutants or contaminants. As such, health care provider operations are particularly susceptible to the practical, financial and legal risks associated with the obligations imposed by applicable environmental laws and regulations. Such risks may result in damage to individuals, property or the environment; may interrupt operations and/or increase their cost; may result in legal liability, damages, injunctions or fines; may result in investigations, administrative proceedings, civil litigation, criminal prosecution, penalties or other governmental agency actions; and may not be covered by insurance. There can be no assurance that the Medical Center will not encounter such risks in the future, and such risks may result in material adverse consequences to the operations or financial condition of the Medical Center.

Affiliation, Merger, Acquisition and Divestiture

As part of its ongoing planning and property management functions, the Medical Center reviews the use, compatibility and financial viability of many of its operations, and from time to time, may pursue changes in the use, or disposition, of its facilities. Likewise, the Medical Center may receive offers from, or conduct discussions with, third parties about the potential acquisition of operations or properties that may become part of the Medical Center in the future, or about the potential sale of some of the operations and properties of the Medical Center. Discussions with respect to affiliation, merger, acquisition, disposition, or change of use, including those that may affect the Medical Center, are held on an intermittent, and usually confidential basis. As a result, it is possible that the assets currently owned by the Medical Center may change from time to time, subject to the provisions in the financing documents that apply to merger, sale, disposition or purchase of assets.

Insurance

The dollar amounts of patient damage recoveries remain potentially significant. A number of insurance carriers have withdrawn from this segment of the insurance market citing underwriting losses, and premiums have increased in the last several years. The effect of these developments has been to significantly increase the operating costs of hospitals, including the Medical Center.

The Medical Center currently carries malpractice, directors' and officers' liability and general liability insurance, which management of the Medical Center considers adequate, but no assurance can be given that the Medical Center will maintain coverage amounts currently in place in the future, that the coverage will be sufficient to cover all malpractice judgments rendered against the Medical Center or settlements of any such claims or that such coverage will be available at a reasonable cost in the future. For a discussion of the insurance coverage of the Medical Center, see "APPENDIX A – MAIMONIDES MEDICAL CENTER – Insurance" herein.

Litigation and Claims

The Medical Center is involved in litigation and claims which are not considered unusual to their business. While the ultimate outcome of these lawsuits cannot be determined at this time, it is the opinion of management that the ultimate resolution of these claims will not have a material adverse effect on the Medical Center. The Medical Center, like most other employers in the current economic climate, has experienced a recent increase in the number of employment-related actions. See "APPENDIX A – MAIMONIDES MEDICAL CENTER – Litigation" and "– Material Pending Audits" herein.

Risks Related to Interest Rate Swap Agreements

The Medical Center has entered into interest rate swap agreements related to indebtedness of the Medical Center (the "Swaps"). The Swaps are and will be subject to periodic "mark-to-market" valuations and at any time may have a negative value to the Medical Center. The Swap counterparties may terminate the Swaps upon the occurrence of certain "termination events" or "events of default." The Medical Center may terminate the Swaps at any time. If either the counterparty to one of the Swaps or the Medical Center terminates any of the Swaps during a negative value situation, the Medical Center may be required to make a termination payment to such Swaps counterparty, and such payment could be material. In addition, failure of the Swap counterparties to perform could lead to the Medical Center bearing the risk of variable rate interest rate fluctuations. See "APPENDIX A – MAIMONIDES MEDICAL CENTER – Discussion of Outstanding Indebtedness – Interest Rate Swaps" herein.

Bankruptcy

The Series 2013 Bonds are payable from the sources and are secured as described in this Official Statement. The practical realization of value from the collateral for the Series 2013 Bonds described herein upon any default will depend upon the exercise of various remedies specified by the Indenture, the Financing Agreement and the Mortgages. These and other remedies may, in many respects, require judicial actions which are often subject to discretion and delay.

Under existing law, the remedies specified by the Indenture, the Financing Agreement and the Mortgages may not be readily available or may be limited. A court may decide not to order the performance of the covenants contained in those documents. The legal opinions to be delivered concurrently with the delivery of the Series 2013 Bonds will be qualified as to the enforceability of the various agreements and other instruments by limitations imposed by State and federal laws, rulings and decisions affecting remedies and by bankruptcy, reorganization or other laws affecting the enforcement of creditors' rights generally.

The rights and remedies of the holders of the Series 2013 Bonds are subject to various provisions of Title 11 of the United States Code (the “Bankruptcy Code”). If the Medical Center were to file a petition for relief under the Bankruptcy Code, the filing would automatically stay the commencement or continuation of any judicial or other proceedings against the Medical Center and its property, including the commencement of foreclosure proceedings under the Mortgages. The Medical Center would not be permitted or required to make payments of principal or interest under the Mortgage Notes, unless an order of the United States Bankruptcy Court were issued for such purpose. In addition, without an order of the United States Bankruptcy Court the automatic stay may serve to prevent the Trustee from applying amounts on deposit in certain funds and accounts held under the Indenture from being applied in accordance with the provisions of the Indenture, and the application of such amounts to the payment of principal and Sinking Fund Installments of, and interest on, the Series 2013 Bonds. Moreover, any motion for an order canceling the automatic stay and permitting such funds and accounts to be applied in accordance with the provisions of the Indenture would be subject to the discretion of the United States Bankruptcy Court, and may be subject to objection and/or comment by other creditors of the Medical Center, which could affect the likelihood or timing of obtaining such relief. The automatic stay may also extinguish the Trustee’s continuing security interest in the Medical Center’s revenues arising subsequent to the filing of the bankruptcy petition, adversely affect the ability of the Trustee to exercise remedies upon default, including the acceleration of all amounts payable by the Medical Center under the Indenture, the Financing Agreement and the Mortgages, and may adversely affect the Trustee’s ability to take all steps necessary to file a claim under the applicable documents on a timely basis.

The Medical Center could file a plan for the adjustment of its debts in a proceeding under the Bankruptcy Code, which plan could include provisions modifying or altering the rights of creditors generally, or any class of them, whether secured or unsecured. The plan, when confirmed by the United States Bankruptcy Court, would bind all creditors who have notice or knowledge of the plan and would discharge all claims against the Medical Center provided for in the plan. No plan may be confirmed unless certain conditions are met, among which are that the plan is in the best interests of creditors, is feasible and has been accepted by each class of claims impaired thereunder. Each class of claims has accepted the plan if at least two-thirds in dollar amount and more than one-half in number of the allowed claims of the class that are voted with respect to the plan are cast in its favor. Even if the plan is not so accepted, it may be confirmed if the court finds that the plan is fair and equitable with respect to each class of non-accepting creditors impaired thereunder and does not discriminate unfairly.

In the event of bankruptcy of the Medical Center, transfers of property by the bankrupt entity, including the payment of debt or the transfer of any collateral, including receivables and revenues on or after the date which is 90 days (or, in some circumstances, one year) prior to the commencement of the case in bankruptcy court, may be subject to avoidance or recoupment as preferential transfers. Under certain circumstances a court may have the power to direct the use of revenues to meet expenses of the Medical Center before paying principal or interest on the Mortgage Notes.

Secondary Market

There can be no assurance that there will be a secondary market for the purchase or sale of the Series 2013 Bonds. From time to time there may be no market for them depending upon prevailing market conditions, including the financial condition or market position of firms who may make the secondary market, the evaluation of the Medical Center’s capabilities and the financial conditions and results of operations of the Medical Center.

Other Risk Factors

In the future, the following factors, among others, may adversely affect the operations of health care providers, including the Medical Center, or the market value of the Series 2013 Bonds, to an extent that cannot be determined at this time:

- (i) Adoption of legislation that would establish a national or statewide single-payor health program or that would establish national, statewide or otherwise regulated rates;
- (ii) Increased unemployment or other economic conditions in the service area of the Medical Center, which could increase the proportion of patients without health insurance benefits or who are unable to pay fully for the cost of their care;
- (iii) Continued efforts by insurers and governmental agencies to limit the cost of hospital and physician services, to reduce the number of beds and to reduce the utilization of hospital facilities by such means as preventive medicine, improved occupational health and safety and outpatient care, or comparable regulations or attempts by third-party payors to control or restrict the operations of certain health care facilities;
- (iv) Efforts by employers to reduce the costs of health insurance by having employees bear a greater portion of their health care costs, causing employees to be more selective and cost conscious in choosing health care services;
- (v) Reduced demand for the services of the Medical Center that might result from decreases in population or innovations in technology;
- (vi) Bankruptcy of an indemnity/commercial insurer, managed care plan or other payor; and
- (vii) The occurrence of a natural or man-made disaster, including but not limited to acts of terrorism, that could damage the facilities of the Medical Center, interrupt utility service to the facilities, result in an abnormally high demand for health care services or otherwise impair the operations and the generation of revenues from the Medical Center's facilities.

TAX MATTERS

Notice Pursuant to Treasury Department Circular 230

TO ENSURE COMPLIANCE WITH TREASURY DEPARTMENT CIRCULAR 230, PURCHASERS OF THE SERIES 2013 BONDS ARE HEREBY NOTIFIED THAT (1) ANY DISCUSSION OF U.S. FEDERAL TAX ISSUES IN THIS OFFICIAL STATEMENT IS NOT INTENDED OR WRITTEN TO BE RELIED UPON, AND CANNOT BE RELIED UPON BY A PURCHASER, FOR THE PURPOSE OF AVOIDING PENALTIES THAT MAY BE IMPOSED ON A PURCHASER UNDER THE INTERNAL REVENUE CODE OF 1986, AS AMENDED (THE "CODE"), (2) SUCH DISCUSSION IS WRITTEN IN CONNECTION WITH THE PROMOTION OR MARKETING OF THE TRANSACTIONS OR MATTERS ADDRESSED HEREIN AND (3) A PURCHASER SHOULD SEEK ADVICE BASED ON ITS PARTICULAR CIRCUMSTANCES FROM ITS OWN INDEPENDENT TAX ADVISORS.

General

The following is a summary of certain U.S. federal income tax consequences relating to the purchase, ownership and disposition of the Series 2013 Bonds. It does not provide a complete analysis of

all potential tax considerations relating to the purchase, ownership and disposition of the Series 2013 Bonds. This summary is based on the provisions of the Code, Treasury regulations promulgated or proposed thereunder, judicial authority and administrative rulings and practice, all as of the date of this Official Statement and all of which are subject to change, possibly retroactively, or to different interpretation.

This summary applies only to “holders” (as defined below) who purchase the Series 2013 Bonds at their original issuance at their “issue price” within the meaning of Section 1273 of the Code and who hold the Series 2013 Bonds as capital assets within the meaning of Section 1221 of the Code. A capital asset generally is an asset held for investment rather than as inventory or as property used in a trade or business. This summary does not discuss all of the aspects of U.S. federal income taxation that may be relevant to investors in light of their particular investment or other circumstances. This summary also does not discuss the particular tax consequences that might be relevant to investors that are subject to special rules under the U.S. federal income tax laws, including: tax-exempt investors; foreign investors; banks, thrifts, insurance companies, regulated investment companies or other financial institutions or financial service companies; brokers or dealers in securities, commodities or foreign currencies; U.S. persons that have a functional currency other than the U.S. dollar; U.S. persons that hold the Series 2013 Bonds through a foreign entity or foreign account; real estate investment trusts; financial asset securitization investment trusts; subchapter S corporations; persons subject to alternative minimum tax; persons who own the Series 2013 Bonds as part of a straddle, hedging transaction, constructive sale transaction or other risk-reduction transaction; persons who have ceased to be U.S. citizens or to be taxed as resident aliens; persons who acquire the Series 2013 Bonds in connection with their employment or other performance of services; or entities or arrangements classified as partnerships for U.S. federal income tax purposes or other pass-through entities and investors therein.

The following summary does not address all possible tax consequences. In particular, it does not discuss any estate, gift, generation skipping, or transfer tax consequences and it does not consider any tax consequences arising under the laws of any foreign, state (except to the extent discussed under the subheading “-State Taxation,” below), local or other jurisdiction. No ruling from the Internal Revenue Service (the “IRS”) has been or will be sought with respect to the statements made and the conclusions reached in the following summary, and there is no assurance that the IRS will agree with those statements and conclusions. For all these reasons, each prospective investor is urged to consult its own tax advisor with respect to the U.S. federal income tax and other tax consequences of the purchase, ownership and disposition of the Series 2013 Bonds.

As used herein, a “holder” is a beneficial owner of the Series 2013 Bonds who, for U.S. federal income tax purposes, is (i) a citizen or individual resident of the United States, (ii) a corporation (or other entity taxable as a corporation for U.S. federal income tax purposes) that is created or organized in or under the laws of the United States, any state thereof or the District of Columbia, (iii) an estate the income of which is subject to U.S. federal income taxation regardless of its source, or (iv) a trust if it (a) is subject to the primary supervision of a court within the United States and one or more U.S. persons have the authority to control all substantial decisions of the trust or (b) has a valid election in effect under applicable Treasury regulations to be treated as a U.S. person.

If a partnership, including an entity or arrangement treated as a partnership for U.S. federal income tax purposes, is a holder of a Series 2013 Bond, the U.S. federal income tax treatment of a partner in the partnership generally will depend on the status of the partner and the activities of the partnership. Partnerships and their partners are urged to consult their own tax advisors as to the particular U.S. federal income tax consequences applicable to them of an investment in the Series 2013 Bonds.

Tax Status of the Series 2013 Bonds

THE SERIES 2013 BONDS ARE TAXABLE BONDS FOR U.S. FEDERAL INCOME TAX PURPOSES. AS SUCH, INTEREST ON THE SERIES 2013 BONDS IS NOT EXCLUDABLE FROM THE GROSS INCOME OF HOLDERS. SEE THE DISCUSSION BELOW FOR A MORE DETAILED SUMMARY OF CERTAIN U.S. FEDERAL INCOME TAX CONSEQUENCES OF OWNING THE SERIES 2013 BONDS.

Interest

A holder of a Series 2013 Bond generally will be required to include in its gross income the stated interest on a Series 2013 Bond at the time it is paid or accrued in accordance with the holder's method of accounting for U.S. federal income tax purposes. Such income will be treated as ordinary interest income. It is anticipated, and the rest of this summary assumes, that the Series 2013 Bonds will be issued without original issue discount or, if issued at a discount from the stated principal amount of the Series 2013 Bonds, with an amount of discount that is less than the statutory de minimis amount.

Sale, Exchange or Redemption of Series 2013 Bonds

A holder generally will recognize gain or loss upon the sale, exchange or other taxable disposition (including redemption or retirement) of a Series 2013 Bond measured by the difference between (i) the amount of cash proceeds and the fair market value of any property received (except to the extent attributable to accrued but unpaid interest) and (ii) the holder's adjusted tax basis in such Series 2013 Bond. A holder's adjusted tax basis in a Series 2013 Bond generally will equal its cost of the Series 2013 Bond, reduced by the amount of amortized premium. Any amount that is attributable to accrued but unpaid interest, to the extent not previously included in income, will be taxable as interest. Gain or loss on the disposition of Series 2013 Bonds generally will be a capital gain or loss and will be a long-term capital gain or loss if the Series 2013 Bonds have been held for more than one year at the time of disposition. Long-term capital gains of individuals and other non-corporate taxpayers generally are eligible for preferential rates of taxation. The deductibility of capital losses is subject to limitations. Prospective investors are urged to consult their own tax advisors regarding the treatment of capital gains and losses.

Defeasance

Defeasance of any Series 2013 Bond may result in a deemed disposition of such Series 2013 Bond and a deemed reissuance of a "new" Series 2013 Bond to the holder for U.S. federal income tax purposes, in which case a holder would recognize taxable gain or loss equal to the difference between the amount realized from the deemed exchange and the holder's adjusted tax basis in the Series 2013 Bond. The "new" Series 2013 Bond deemed reissued in such a defeasance may be treated as issued with original issue discount in an amount equal to the excess, if any, of the stated redemption price at maturity of the "new" Series 2013 Bond over its deemed issue price. Prospective investors are urged to consult their own tax advisors regarding the tax consequences of a defeasance of the Series 2013 Bonds.

Additional Medicare Tax on Net Investment Income

For taxable years beginning after December 31, 2012, certain holders who are individuals, estates, or trusts and whose income exceeds certain thresholds will be subject to a Medicare tax on net investment income. A holder's net investment income will generally include its gross interest income and its net gains from the disposition of the Series 2013 Bonds, unless such interest or net gains are derived in the ordinary course of the conduct of a trade or business (other than a trade or business that consists of certain passive or trading activities). Prospective investors that are individuals, estates, or trusts are urged to

consult their own tax advisor regarding the applicability of the Medicare tax to the income and gains in respect of an investment in the Series 2013 Bonds.

Information Reporting and Backup Withholding Tax

In general, information reporting requirements will apply to payments to certain holders of principal and interest on a Series 2013 Bond and the proceeds of a sale, exchange or other taxable disposition (including a retirement or redemption) of a Series 2013 Bond. A holder may be subject to backup withholding on such payments. In general, a holder can avoid this backup withholding by properly executing under penalties of perjury an IRS Form W-9 or substantially similar form that provides (1) such holder's correct taxpayer identification number; and (2) a certification that such holder (a) is exempt from backup withholding, (b) has not been notified by the IRS that it is subject to backup withholding, or (c) has been notified by the IRS that it is no longer subject to backup withholding.

Backup withholding does not apply to payments made to certain holders, provided their exemptions from backup withholding are properly established. Backup withholding is not an additional tax. Any amount withheld from a payment to a holder under the backup withholding rules will generally be allowed as a credit against such holder's U.S. federal income tax liability and may entitle such holder to a refund, provided that the required information is timely furnished to the IRS. The amount of any "reportable payments" for each calendar year and the amount of tax withheld, if any, with respect to those payments will be reported to the holders of Series 2013 Bonds and to the IRS. Prospective investors are urged to consult their own tax advisors regarding the application of backup withholding rules in their particular situations, the availability of an exemption from backup withholding and the procedure for obtaining such an exemption, if applicable.

State Taxation

Interest on the Series 2013 Bonds and gain from the sale, exchange or other taxable disposition of a Series 2013 Bond is not exempt from present New York taxation. Purchase, ownership and disposition of a Series 2013 Bond may result in other state and local tax consequences to certain prospective purchasers. Prospective purchasers of the Series 2013 Bonds are urged to consult their own tax advisors concerning any specific state and local tax consequences.

THE PRECEDING SUMMARY OF CERTAIN U.S. FEDERAL INCOME TAX CONSIDERATIONS IS FOR GENERAL INFORMATION ONLY AND IS NOT LEGAL OR TAX ADVICE. ACCORDINGLY, PROSPECTIVE INVESTORS SHOULD CONSULT THEIR OWN ADVISORS REGARDING THE U.S. FEDERAL, STATE AND LOCAL, AND FOREIGN TAX CONSEQUENCES OF THEIR PURCHASE, OWNERSHIP AND DISPOSITION OF THE SERIES 2013 BONDS, AND ON THE CONSEQUENCES OF ANY CHANGES IN APPLICABLE LAW.

RATINGS

Moody's Investors Service, Inc. and Standard and Poor's Rating Services, a division of The McGraw-Hill Companies, Inc., have assigned ratings of "Aaa" and "AA+", respectively, to the Series 2013 Bonds prior to the date of delivery of the Series 2013 Bonds. Such ratings reflect only the views of such organizations and any explanation of the significance of such ratings should be obtained from the rating agency furnishing the same. Generally, a rating agency bases its rating on the information and materials furnished to it and on investigations, studies and assumptions of its own. There is no assurance that either or both of such ratings will continue for any given period of time or that either or both of such ratings will not be revised downward or withdrawn entirely by the ratings agencies, if in the judgment of such rating agencies, circumstances so warrant. Any such downward revision or withdrawal of such ratings may have an adverse effect on the market price of the Series 2013 Bonds.

GNMA, HUD AND FHA NOT LIABLE ON THE SERIES 2013 BONDS

The Series 2013 Bonds do not constitute an obligation or indebtedness of, and the payment of the Series 2013 Bonds is not insured or guaranteed by, the United States of America or any agency or instrumentality thereof, including GNMA, HUD or FHA. The Series 2013 Bonds are payable solely from the amounts available under the Indenture, primarily constituting moneys derived from the GNMA Securities and certain funds held by the Trustee, including the investment income thereon.

LEGAL MATTERS

Certain legal matters incidental to the authorization and issuance of the Series 2013 Bonds by the Issuer are subject to the approval of Eichner Norris & Neumann PLLC, Transaction Counsel, whose approving opinion will be delivered with the Series 2013 Bonds. The proposed form of Transaction Counsel's approving opinion is set forth in Appendix H hereto.

Certain legal matters will be passed upon for the Issuer by its counsel, Arent Fox LLP, and by its Senior Vice President for Legal Affairs and General Counsel, Joyce A. Leahy, Esq., for the Underwriter by its counsel, Harris Beach PLLC, and for the Lender by its special counsel, Krooth & Altman LLP.

UNDERWRITING

Merrill Lynch, Pierce, Fenner & Smith Incorporated (the "Underwriter"), has agreed, subject to certain conditions, to purchase the Series 2013 Bonds from the Issuer at a purchase price equal to \$103,748,098.60, representing the principal amount of the Series 2013 Bonds (\$100,535,000), plus original issue premium of \$3,213,098.60 subject to the receipt of an Underwriter's fee of \$733,905.50 on the date of delivery of the Series 2013 Bonds, and to make a public offering of the Series 2013 Bonds at prices that are not in excess of the public offering price stated on the cover page of this Official Statement. The Underwriter will be obligated to purchase all such Series 2013 Bonds if any are purchased. The Series 2013 Bonds may be offered and sold to certain dealers (including the Underwriter) at prices lower than such public offering price, and such public offering price may be changed, from time to time, by the Underwriter.

The Underwriter and its respective affiliates are full service financial institutions engaged in various activities, which may include securities trading, commercial and investment banking, financial advisory, investment management, principal investment, hedging, financing and brokerage services. The Underwriter and its respective affiliates have, from time to time, performed, and may in the future perform, various financial advisory and banking services for the Issuer, for which they received or will receive customary fees and expenses.

In the ordinary course of their various business activities, the Underwriter and its respective affiliates may make or hold a broad array of investments and actively trade debt and equity securities (or related derivative securities, which may include credit default swaps) and financial instruments (including bank loans) for their own account and for the accounts of their customers and may at any time hold long and short positions in such securities and instruments. Such investment and securities activities may involve securities and instruments of the Issuer.

The Underwriter and its respective affiliates may also communicate independent investment recommendations, market color or trading ideas and/or publish or express independent research views in respect of such assets, securities or instruments and may at any time hold, or recommend to clients that they should acquire, long and/or short positions in such assets, securities and instruments.

Related Parties

Merrill Lynch, Pierce, Fenner & Smith Incorporated and Bank of America, N.A., as provider of certain FHA mortgage banking services to the Issuer relating to obtaining HUD approval for the refinancing of Series 1996 Bonds and the Series 2004 Bonds issued by DASNY, are each receiving a separate fee for services rendered, and are both wholly-owned, indirect subsidiaries of Bank of America Corporation.

CONTINUING DISCLOSURE

On the date of delivery of the Series 2013 Bonds, the Issuer has agreed to enter into a written agreement (the “Continuing Disclosure Agreement”) for the benefit of Series 2013 Bondholders to provide to Digital Assurance Certification LLC (“DAC”) certain updated operating data and financial information described below on a quarterly and annual basis. Pursuant to the Continuing Disclosure Agreement, the Issuer will agree to provide to DAC (i) updated operating data (including aggregate inpatient discharges, patient days, average length of stay, emergency room visits, outpatient visits and outpatient procedures; and discharges by major payor source for the fiscal year) and financial information relating to the Issuer (including a balance sheet, statement of operations, changes in net assets and cash flow for the fiscal year) not later than 135 days after the end of its fiscal year in each year commencing with the fiscal year ending December 31, 2013 (collectively, the “Annual Report”), and (ii) certain interim operating data (including aggregate inpatient discharges, patient days, average length of stay, emergency room visits, outpatient visits and outpatient procedures; and discharges by major payor source for the fiscal quarter) and financial information relating to the Issuer (including a balance sheet, statement of operations, changes in net assets and cash flow as of the end of the fiscal quarter) not later than 60 days after the end of the first three quarters in each of its fiscal years (collectively, the “Quarterly Report”) commencing with the fiscal quarter ending June 30, 2013. Information so provided to DAC is available to the public on DAC’s website. In addition, the Continuing Disclosure Agreement will require the Issuer to timely provide to DAC notices of the occurrence of certain enumerated events (the “Listed Events Notices”) and such Listed Events Notices will also be available to the public on DAC’s website. See “APPENDIX G — FORM OF CONTINUING DISCLOSURE AGREEMENT” for additional information regarding the Issuer’s continuing disclosure obligation with respect to the Series 2013 Bonds.

If, and only if, and to the extent that it receives the Annual Reports and Quarterly Reports described above from the Issuer, DAC has agreed to established a website for all of the Issuer’s filings to be held in DAC’s secured centralized database and searchable by CUSIP, the Issuer’s name, full issue description, bond type and other identifying factors. The Annual Reports and the Quarterly Reports can be accessed without cost by registering with DAC at www.dacbond.com. In addition, DAC has agreed to establish an automated tickler system with reporting due dates as stated in the Continuing Disclosure Agreement.

DAC has no duty with respect to the content of any disclosure or Listed Events Notices made pursuant to the terms of the Continuing Disclosure Agreement and DAC has no duty or obligation to review or verify any information contained in the Annual Reports or the Quarterly Reports or Listed Event Notices or any other information, disclosures or notices provided to it by the Issuer and shall not be deemed to be acting in any fiduciary capacity for the Issuer, the Holders of the Series 2013 Bonds or any other party. DAC has no responsibility for the Issuer’s failure to provide to DAC a Listed Events Notice required by the Continuing Disclosure Agreement or duty to determine the materiality thereof. DAC shall have no duty to determine or liability for failing to determine whether the Issuer has complied with the Continuing Disclosure Agreement.

The Issuer has not in the previous five years failed to comply, in all material respects, with any previous undertakings pursuant to Rule 15c2-12. Although the Series 2013 Bonds are not subject to Rule 15c2-12, the Issuer has voluntarily agreed to provide ongoing disclosure with respect to the Series 2013 Bonds pursuant to the Continuing Disclosure Agreement.

ABSENCE OF LITIGATION

There is not now pending (as to which the Issuer has received service of process) or, to the Issuer's knowledge, threatened against the Issuer any legal proceedings seeking to restrain or enjoin the issuance, sale or delivery of the Series 2013 Bonds or questioning or affecting the validity of the Series 2013 Bonds or the proceedings and authority under which they are to be issued.

On the date of delivery of the Series 2013 Bonds, the Issuer will deliver a certificate stating, among other things, that there are no legal proceedings pending (as to which the Issuer has received service of process) or, to the Issuer's knowledge, threatened against the Issuer seeking to restrain or enjoin the issuance, sale or delivery of the Series 2013 Bonds, the proceedings and authority under which they are issued, the payment, collection or application of the proceeds thereof or of the revenues and other moneys and securities pledged or to be pledged under the Indenture or in any way contesting or affecting any authority for or the validity of the Series 2013 Bonds or the Indenture.

MISCELLANEOUS

References in this Official Statement to the Indenture, the Financing Agreement, the FHA Documents, the GNMA Securities and the GNMA Guaranty do not purport to be complete. Refer to the Indenture, the FHA Documents, the Financing Agreement, the GNMA Securities and the GNMA Guaranty for full and complete details of their provisions. Copies of the Indenture, the primary FHA Documents, the Financing Agreement, the GNMA Securities and the form of the GNMA Guaranty are on file with DAC and the Trustee. Information concerning the GNMA program is on file with the Lender.

Any statement in this Official Statement involving matters of opinion, whether or not expressly so described, are intended merely as expressions of opinion and not as representations of fact.

The information regarding the Lender was supplied by the Lender. The Issuer believes that this information is reliable, but the Issuer makes no representations or warranties as to the accuracy or completeness of this information.

The information regarding DTC and DTC's book-entry only system has been furnished by DTC. The Issuer believes that this information is reliable, but makes no representations or warranties whatsoever to the accuracy or completeness of this information.

"Appendix D – Certain Definitions", "Appendix E – Summary of Certain Provisions of the Indenture", "Appendix F – Summary of Certain Provisions of the Financing Agreement and "Appendix H – Form of Approving Opinion of Transaction Counsel" have been prepared by Eichner Norris & Neumann PLLC, Transaction Counsel.

The Issuer has agreed to indemnify the Underwriter and certain others against losses, claims, damages and liabilities arising out of any untrue statements of a material fact contained in the Official Statement or any omission of statements of any material fact from the Official Statement relating to the Issuer, its operations, its financial condition and the Projects.

The execution and delivery of this Official Statement by an Authorized Officer have been duly authorized by the Issuer.

MAIMONIDES MEDICAL CENTER

By: /s/ Robert Naldi
Executive Vice President and
Chief Financial Officer

APPENDIX A

MAIMONIDES MEDICAL CENTER

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MAIMONIDES MEDICAL CENTER

Introduction

Maimonides Medical Center (“Maimonides” or the “Medical Center”) is a voluntary, not-for-profit, teaching hospital located in the Borough Park section of southwestern Brooklyn in Kings County, New York, and is licensed to operate 711 beds. Maimonides is one of the largest independent teaching hospitals in the United States. The Medical Center is an organization exempt from federal income tax pursuant to Section 501(c)(3) of the Internal Revenue Code of 1986, as amended (the “Code”).

The Medical Center was founded in 1911, absorbed several smaller dispensaries and, in 1919, became Israel Hospital of Brooklyn. In 1920, it merged with Zion Hospital and became United Israel Zion Hospital. Maimonides Hospital traces its roots to a 1947 merger between Beth Moses Hospital and United Israel Zion Hospital. Its status as a teaching hospital was established in 1948, and in 1966 its name was changed to Maimonides Medical Center to reflect the broad spectrum and comprehensive nature of the research, educational and medical activities being performed.

Today, the Medical Center provides a comprehensive array of acute care inpatient services to residents of southern Brooklyn and tertiary care inpatient services to the entire Borough of Brooklyn and surrounding metropolitan area. With a medical staff numbering 1,226 (as of December 31, 2011) and graduate medical residents numbering 480 (as of June 2012), Maimonides serves as a teaching affiliate of the Albert Einstein College of Medicine (“Einstein”), located in Bronx, New York, NYU School of Medicine, located in Manhattan, and SUNY Downstate College of Medicine, located in Brooklyn, New York. Under a recently signed affiliation agreement, Maimonides will become a university hospital and the Brooklyn campus of Einstein.

On February 26, 2013, Maimonides announced in a press release that it had entered into an initial three year strategic alliance agreement with Montefiore Medical Center (“Montefiore”) located in The Bronx, New York, to leverage the strengths of both institutions and to allow both institutions to better prepare for new models of health care delivery in the future by establishing programs of clinical excellence, expanding research capabilities and ensuring patient access to the full range of clinical services – from patient care to advanced specialty care. Maimonides and Montefiore will remain independent; each will continue to be solely responsible for its own assets, operations and liabilities; and neither Maimonides nor Montefiore will incur any legal responsibility for the liabilities and expenses of the other institution.

For 2013, Maimonides made America’s 100 Best Hospitals list as determined by HealthGrades, an independent ratings organization. The Medical Center was one of only three New York State hospitals/hospital systems that received this award. In 2010, 2011, 2012 and 2013, the Medical Center received the Distinguished Hospital Award for Clinical Excellence from HealthGrades. Maimonides was one of only two New York City hospitals/hospital systems that received this award in 2013.

The Medical Center continues to be a recognized national leader in the provision of cardiac-related services, having been the recipient of the Cardiac Care Excellence Award from

HealthGrades for the past three years, as one of only a few New York City hospitals so awarded and recognized by the Department of Health for being one of the few hospitals in New York State that achieved statistically significant excellent results in interventional cardiology. Maimonides was also awarded the HealthGrades' Coronary Interventional Excellence Award and Women's Health Excellence Award for the past three years, the Gastrointestinal Care Excellence Award for 2013, and the Pulmonary Care Excellence Award and Stroke Care Excellence Award for 2011 and 2012. Recently, Maimonides was ranked among the top hospitals in Brooklyn (recipient of Modern Healthcare's 2011-2012 Consumer Choice Award) for overall quality and image. In addition, the Medical Center's Left Ventricular Assist Device (LVAD) Program, the only such program serving Brooklyn and Staten Island, was accredited by The Joint Commission in 2012. See "Centers of Excellence" below.

Facilities

Maimonides is located on a 6.67 acre campus at 4802 Tenth Avenue in the Borough Park section of Brooklyn. The Medical Center's campus consists of two city blocks, bounded by Ninth Avenue and Fort Hamilton Parkway between 47th and 49th Streets.

The main hospital campus consists of four inpatient buildings and numerous ambulatory and ancillary buildings totaling approximately 1,000,000 square feet. Visitor and staff parking is provided in a separate, six-level parking garage consisting of approximately 200,000 square feet.

In 2009, a new building was placed into service by the Medical Center as part of its \$100+ million modernization program. The modernization program relocated selected inpatient services from the Gellman, Kronish and Aron Buildings providing much needed expansion space for existing acute care services, simplifying service delivery to patients through consolidation of services and upgrading clinical departments. Specifically, this modernization program resulted in an enlargement of both ambulatory and inpatient surgery facilities, new and improved state-of-the-art medical technologies, and the reorganization and enlargement of three of Maimonides' centers of excellence – the Cardiac Institute, the Infants and Children's Hospital, and the Emergency Department.

In 2011, the freestanding Cancer Center, located approximately one mile from the Maimonides main campus, was enhanced with the addition of the Breast Cancer Center which provides state-of-the-art multi-modality imaging of the breast and surgical consultative services.

Services and Programs

Maimonides offers a broad range of diagnostic and therapeutic services for adults and children on an inpatient and outpatient basis. As of December 31, 2011, its certified bed complement (excluding bassinets) was allocated among the following services:

Medical-Surgical	459
Psychiatric	70
Maternity	60
Adult Intensive Care	40
Pediatric	32
Neonatal	31*
Pediatric Intensive Care	11
Coronary Care	<u>8</u>
TOTAL	711

* Includes 10 beds in Intensive Care, 12 beds in Intermediate Care and 9 beds in Continuing Care.

The Medical Center's comprehensive range of specialties and subspecialties includes: anesthesiology, breast surgery, cardiology, cardiothoracic surgery, colorectal surgery, critical care medicine, dentistry, dermatology, diagnostic radiology, emergency services, endocrinology, fetal medicine, gastroenterology, general surgery, geriatrics, hematology, immunology, infectious diseases, internal medicine, interventional cardiology, interventional neuroradiology, laboratory medicine, medical genetics, metabolic diseases, neonatology, nephrology, neurology, neurosurgery, nuclear medicine, obstetrics and gynecology, oncology, ophthalmology, oral surgery, orthopedic surgery, otolaryngology, pathology, pediatrics and pediatric medical and surgical subspecialties, surgery, podiatry, psychiatry, pulmonary diseases, radiation oncology, rehabilitation medicine, reproductive endocrinology, rheumatology, thoracic surgery, urogynecology, urologic oncology, urology and vascular surgery.

In addition, Maimonides provides acute renal dialysis, ambulatory surgery, audiology, cardiac catheterization, cystoscopy, family planning, geriatric day care, outpatient dentistry, outpatient psychiatry, physical medicine and rehabilitation, physical therapy, primary medical care services and social work services at various outreach sites throughout Brooklyn. These outreach sites are primarily neighborhood-based clinics owned and operated by the Medical Center as well as offices of physicians affiliated with the Medical Center.

Licensure and Accreditation

Maimonides has operating certificates from the New York State Department of Health Office of Health Systems Management and is accredited by The Joint Commission for a three year period commencing February 11, 2012 and was accredited by The Joint Commission for its Stroke Program, high technology home care agency and, most recently, for its LVAD Program in 2012. Maimonides is certified by the U.S. Centers for Medicare & Medicaid Services ("CMS") for participation in the Medicare and Medicaid programs.

Community Outreach Programs

The Medical Center strives to provide exceptional, personalized care to its diverse communities by extending its medical services into the surrounding neighborhoods. Its thirteen (13) ambulatory health care facilities make primary and specialty care more accessible to all

residents of Brooklyn. In addition to offering primary care, many of the Medical Center's facilities offer specialty services. These ambulatory medical care facilities are fully equipped with state-of-the-art information technology to allow streamlined communication with the Medical Center and to provide quick access to laboratory and radiology reports. In addition, the Maimonides network of ambulatory facilities is neighborhood-based and organized to meet the language and cultural needs of its patients. Primary care services are targeted to specific segments of the community, such as the elderly, who comprise the highest percentage of clinic patients. The primary care centers are designated Level 3 Patient Centered and Medical Homes by the National Committee for Quality Assurance (NCQA).

In addition, the Medical Center welcomes new, emerging immigrant populations in Brooklyn, including Asian, Caribbean, Pakistani and others via the "We Speak Your Language" campaign. Medicaid patients constitute approximately 78% of clinic patients and 58% of the patients using the outpatient services of the Community Mental Health Center.

Examples of the Medical Center's community outreach programs include a physician referral service, which enables patients to locate physician services or specific medical care services in their local community. A speaker's bureau reaches out to local community organizations and groups with prominent physicians who educate consumers on various current topics in health promotion and treatment. Neighborhood-based health fairs and screenings are conducted in schools, parks and community centers. A cardiac rehabilitation and exercise program provides EKG-monitored exercises guided by specialists and lectures by cardiac-care physicians, diagnostic screening examinations at community fairs and interactive self-management programs for patients with cardiovascular disease and their families. The Community Residence Support Program, a case-management program, coordinates medical, psychiatric and entitlement programs, and assists mentally ill adults living in shelters, supportive apartments and supervised residences.

Organization and Affiliates

The following is a description of the affiliated corporations under common control with or by the Medical Center, and its related organizations. Any payments made by the Medical Center to these affiliated corporations are at fair market value for services rendered to the Medical Center.

MAIMONIDES HEALTH RESOURCES, INC.: A not-for-profit corporation organized under the laws of the State of New York, a §501(c)(3) organization under the Code and a supporting organization under §509(a)(3) of the Code, it is the holding company for Maimonides and MMC Holding of Brooklyn, Inc. and is organized and operated to support the Medical Center's hospital, educational, scientific and charitable purposes. Maimonides Health Resources, Inc. is the sole corporate member of the Medical Center.

MAIMONIDES RESEARCH AND DEVELOPMENT FOUNDATION (the "Foundation"): A New York not-for-profit corporation, a §501(c)(3) organization under the Code and a supporting organization under §509(a)(3) of the Code, it is organized and operated exclusively for the educational, scientific, medical and charitable purposes carried on by the Medical Center. Its by-laws require that one more than the number which constitutes a quorum of the board of directors must consist of persons who are trustees of Maimonides. At the present

time, the Foundation uses the interest and dividend income from its funds to provide research grants to applicants from Maimonides' medical staff.

MMC HOLDING OF BROOKLYN, INC. ("MMC Holding"): A New York for-profit corporation which owns 100% of the capital stock of, and acts as the executive and accounting offices for, each of the following five (5) active for-profit subsidiaries:

- (i) **MMC PHARMACY, INC.**, which operates a retail pharmacy serving the Medical Center's outpatients and employees, and the community at large;
- (ii) **MMC MANAGEMENT CORP.**, which provides real estate management services to the Medical Center, including eleven (11) apartment buildings, approximately 300,000 square feet of leased commercial space and seven (7) parking lots;
- (iii) **INFUSION OPTIONS, INC.**, which serves as a wholesale pharmacy and distributor of infusion therapy and nursing services, with related medical equipment, to patients living at home and hospice patients;
- (iv) **EXECUTIVE PHYSICIAN SYSTEMS, INC.**, which provides billing services to the Medical Center and affiliated physicians, and other clients; and
- (v) **DIRECT STAFFING SERVICES, INC.**, which provides personnel leasing services that includes human resources, payroll, benefits administration and accounting to the Medical Center, and affiliated entities and physicians.

The salaries of the subsidiaries' employees are paid through MMC Holding and allocated to the respective subsidiary based on time worked. All benefit plans are in the name of MMC Holding. Management fees charged to the subsidiaries are paid to MMC Holding, and executive offices are shared by MMC Holding and the subsidiaries. Expenses incurred by the Medical Center provided by MMC Holdings of Brooklyn, Inc. through its various subsidiaries, less related revenues, was \$19.8 million in 2009, \$19.6 million in 2010 and \$19.4 million in 2011.

FIRST TO CARE HOME CARE, INC.: The Medical Center is a joint venture owner of First to Care Home Care, Inc., a New York not-for-profit corporation providing home care services, with Metropolitan Jewish Geriatric Corporation. In 2011, the Medical Center recorded a loss of approximately \$1.5 million as a result of its ownership in this joint venture. The Medical Center entered into an agreement to (i) terminate the Medical Center's funding obligations effective immediately and (ii) divest the Medical Center's interest in this joint venture with Metropolitan Jewish Geriatric Corporation which is expected to be implemented within the next twelve months.

FOJP-RELATED ENTITIES: The Medical Center retains ownership in three captive insurance companies affiliated with the Federation of Jewish Philanthropies of New York (see "Insurance" below).

OTHER RELATED ORGANIZATIONS: The Medical Center is one of 18 owner sponsor hospitals of Healthfirst, a not-for-profit managed care organization. As of June 2012, Maimonides serviced 3.2% of the Healthfirst member population. The Medical Center is one of

six members of the Brooklyn Health Information Exchange (BHIX), a New York not-for-profit corporation that supports the advancement of health care information technology and maintains a clinical information exchange for numerous hospitals, health centers and other providers of health care in Brooklyn. In addition, the Medical Center is one of six contributing members and is a managing member of Southwest Brooklyn Health Home LLC (see “Centers of Excellence – Health Home”).

The Medical Center is also affiliated with MMC Community Horizons, Inc., a New York not-for-profit corporation, that provides consulting and administrative services without charge, primarily to entities that deliver health care on a not-for-profit basis with a focus on the Medical Center and its affiliated entities. MMC Community Horizons, Inc. is one of the managing members of, and has a 5% membership interest in, Brooklyn SC, L.L.C., an entity formed to own and operate an ambulatory surgical center in Brooklyn.

For information concerning the Medical Center’s transactions with affiliates, see Note 8 of the Audited Financial Statements of the Medical Center for the years ended December 31, 2011 and 2010 included in Appendix B.

None of the affiliated corporations are obligated with respect to payments under the Series 2013 Bonds, the Financing Agreement or the Mortgage, which are solely obligations of the Medical Center.

In addition, the Medical Center has had, and continues to have, discussions with other health care institutions with respect to possible affiliations, joint ventures and other similar arrangements (also see “Educational Programs” below).

Governance

The Medical Center is governed by a Board of Trustees (the “Board”) consisting of not less than three nor more than fifty members who serve staggered one to three-year terms and are elected at the annual meeting. As of October 19, 2012, there were 32 trustees on the Board. Regular meetings of the Board are scheduled at least four times per year. Officers of the Board are elected at the Board’s annual meeting generally held in April of each year. Board members monitor Maimonides’ operations through a varied and active group of committees.

As of October 19, 2012, the members of the Board, their year of first appointment, the year of expiration of their current term and their principal occupations are as follows:

Officers and Voting Members of the Board of Trustees

<u>Name/Office</u>	<u>First Year Appointed</u>	<u>Year Term Expires</u>	<u>Occupation</u>
Martin D. Payson <i>Chairman</i>	1993	2013	Retired
Mortimer N. Klaus <i>Vice Chairman</i>	1980	2015	President and Chief Executive Officer, Burma Bibas (manufacturer)
Chaim H. Leshkowitz <i>Vice Chairman</i>	1995	2014	Partner, Leshkowitz & Company (CPA)
Robert Machinist <i>Vice Chairman</i>	2008	2015	Chief Executive Officer, Ironwood Investment Management, LLC and MB Investment Partners, Inc.

<u>Name/Office</u>	<u>First Year Appointed</u>	<u>Year Term Expires</u>	<u>Occupation</u>
Daniel Z. Nelson <i>Vice Chairman</i>	1987	2014	Of Counsel, Akerman Senterfitt LLP
Moshe H. Wieder <i>Secretary</i>	2003	2015	Chief Executive Officer, Global Business Institute
Pamela S. Brier <i>President</i>	<i>Ex officio</i>	-	President and Chief Executive Officer, Maimonides Medical Center
Edward Fitzpatrick, M.D. <i>President of Medical Staff</i>	<i>Ex officio</i>	2014	President of the Medical Staff, Maimonides Medical Center
John Abi-Habib	2007	2013	Chief Executive Officer, MSI Net. Inc.
Steven Adelsberg	2009	2013	Partner, Adelsburg & Co.
Donna Aston, M.D.	2010	2013	Orthopaedic Surgeon
Irwin M. Birnbaum	1989	2015	Chief Operating Officer, Yale School of Medicine (retired)
Rev. Monsignor David Cassato	2002	2014	Pastor, St. Athanasius Catholic Church
David L. Diamond, M.D.	2012	2013	Physician, Boro Park Pediatric Associates
Richard D. Forman	2008	2013	Managing Partner, Health Venture Group, LLC
Marvin Hellman	1992	2015	Chairman, Hellman Management Company (real estate) (retired)
Jack A. Hidary	2009	2013	Chief Executive Officer, Hidrock Realty Inc.
Elly Kleinman	2007	2013	President and Chief Executive Officer, The Americare Companies
Andrew Kohen	1994	2013	President, Eastern Poly Packing Co. (plastics manufacturer)
Frank A. Naccarato	2005	2014	Insurance and Financial Services Broker, The Prudential Insurance Company
Stephen D. Oppenheim	2004	2014	Partner, Faust Oppenheim, LLP (law firm)
Peter Rebenwurz	2007	2013	President, Coney Management, LLC
Erminia Rivera	2010	2015	Park Avenue Building & Roofing Supplies, LLC
Alex Rovt, Ph.D.	2012	2013	President, IBE Trade Corp. (chemical fertilizers)
Veronica C. Santilli, M.D.	1991	2013	Physician, Brook Island Pediatrics
Alfred Schonberger	1992	2013	Owner, Amber Court Assisted Living Communities
Steven R. Shamah	1991	2013	President, Eccolo Ltd.
Al Sutton	1983	2013	Private Investor
Isaac M. Sutton	2000	2015	Investment Broker, Silver Rock
Professor Aaron D. Twerski	1995	2014	Professor, Brooklyn Law School
Howard Weiss	2007	2013	Senior Vice President, and Chief Financial and Administrative Officer, Castle Harlan, Inc. (private equity firm)
Michael L. Ziegler	2007	2015	Partner, Duane Morris, LLP

The Board has eleven (11) standing committees, which are the Executive Committee, Audit and Legal Committee, Budget and Finance Committee, Compensation Committee, Development Committee, Investment Committee, Joint Conference Committee, Medical Matters & Quality of Care Committee, Nominating Committee, Physical Facilities and Planning Committee, and Religious Observance Committee. The Executive Committee may exercise all

powers of the Board. The Audit and Legal Committee is responsible for compliance matters for the Medical Center (see “Compliance” below).

A conflict of interest policy for all trustees, officers, employees and members of the medical staff of the Medical Center requires any potential conflict of interest to be disclosed. Any transaction with the Medical Center or its affiliates in which a Board member has a financial interest must be disclosed to the Board.

In addition, as part of the recently executed affiliation agreement with Yeshiva University through its Albert Einstein College of Medicine, two individuals recommended by Einstein will serve on the Maimonides Board and two individual recommended by Maimonides will serve on the Einstein board. However, both Maimonides and Einstein will each continue to be autonomous and governed independently by their respective governing bodies and administrations; and will each remain independent corporations.

Management

The senior management of the Medical Center consists of the President and Chief Executive Officer, three (3) executive vice presidents, the Chief Medical Officer, fifteen (15) senior vice presidents and several vice presidents, and are summarized as follows:

Pamela S. Brier, M.P.H., President and Chief Executive Officer (67). Ms. Brier has been President and Chief Executive Officer of the Medical Center since May 2003. Previously, Ms. Brier served as Executive Vice President/Chief Operating Officer of the Medical Center since May 1995. Prior to joining the Medical Center, Ms. Brier served in numerous capacities in the New York City Health and Hospitals Corporation from 1977 through 1995, most recently as Chief Executive Officer of Bellevue Hospital Center (1992-95) and Jacobi Medical Center (formerly Bronx Municipal Hospital Center) (1989-92). From 1976 to 1977, Ms. Brier was an Associate Management Analyst for the New York City Department of Health, and from 1973 to 1975 she was a Health Plan Analyst for the Kaiser Foundation Health Plan, Inc. She was appointed by Mayor Bloomberg to the New York City Board of Health (2003-present) and the New York City Workforce Investment Board, and currently serves on the New York City Board of Corrections and the Fund for Public Health in New York. Ms. Brier is a Fellow of The New York Academy of Medicine, and served as a member of the Community Service Board/New York City Department of Health (1995-2000), the Commonwealth Fund Commission on Women’s Health (1993-96) and the New York State Hospital Review and Planning Council (1991-96). In 2009 and 2011, she was recognized by *Crain’s New York Business* as one of the “50 Most Powerful Women in New York.” From 1996 to 2003, Ms. Brier had served as Chair of the Board of Directors of Housing Works, Inc. and on the Board of the Primary Care Development Corporation. Currently, she is Vice Chair of the Board of The New York eHealth Collaborative (NYeC), a statewide public/private partnership organization devoted to information technology promulgation and adoption, and a board member of the UNITE HERE/Union Health Center.

Ms. Brier also served as a member of the adjunct faculty of the Wagner School of Public Service, New York University and has lectured at the Zicklin School of Business Executive Program at Baruch College. She received her M.P.H. in Health Administration from the School of Public Health, University of California, Los Angeles in 1971 and her B.A. degree from the University of California, Berkeley in 1967.

Dominick Stanzione, Executive Vice President and Chief Operating Officer (61). Mr. Stanzione joined the Medical Center in August 2011 as Executive Vice President and Chief Operating Officer. Prior to joining the Medical Center, for a period of three (3) years, he served as Interim President and Chief Restructuring Officer at Long Island College Hospital which recently merged with State University of New York Downstate Medical Center. Previously, he worked in senior roles at several hospital systems in the Bon Secours Health System, the Sisters of Charity Health Care System, and the Catholic Medical Centers of Brooklyn and Queens. Mr. Stanzione received a B.S. in Health Sciences in physical therapy at State University of New York Downstate in 1976 and his Master of Public Administration degree at New York University in 1981.

Robert Naldi, Executive Vice President and Chief Financial Officer (63). Mr. Naldi joined the Medical Center in 2001 as Executive Vice President and Chief Financial Officer. Prior to joining the Medical Center, he held the dual role of Senior Vice President, Finance of St. Luke's-Roosevelt Medical Center and Beth Israel Medical Center. Prior positions held at Beth Israel Medical Center include Director of Financial Operations and Budget Director. He is a Board member of HANYS Insurance Company. Mr. Naldi received his Master of Business Administration degree from Fordham University Graduate School of Business in 1974 and his B.S. degree in Accounting from Lehman College of the City University of New York in 1972.

David I. Cohen, M.D., M.Sc., Executive Vice President for Clinical Affairs and Affiliations, and Senior Vice Chairman of the Department of Medicine (64). Dr. Cohen joined the Medical Center in 1998. In addition to his other positions, he is currently responsible for Academic Affairs and the Medicaid Health Homes Program at the Medical Center. Prior to joining the Medical Center, he served as the first full-time Medical Director at Bellevue Hospital Center (1991-98), as Deputy Dean for Clinical Affairs at the City University of New York Sophie Davis School of Biomedical Education (1990-91), as Vice President for Medical Operations of the New York City Health and Hospitals Corporation (1988-90) and as Director of Ambulatory Care and Associate Dean for Graduate Medical Education at Mt. Sinai Medical Center (1986-88). In 1981, Dr. Cohen was appointed Chief of the Division of General Internal Medicine at Cleveland Metropolitan General Hospital (1981-86). In this position, he helped to develop the "firm system" as a model for patient care, clinical education and health services research. Prior to this, he was a faculty member of Case Western Reserve University (1979-80) and served as Special Assistant to the Medical Director of the Harvard Community Health Plan (1980-81). Dr. Cohen received his B.A. degree in 1970 and his M.D. degree in 1974 from the University of Pennsylvania, and his M.Sc. degree in Epidemiology and Health from McGill University where he was a Robert Wood Johnson Clinical Scholar (1977-79). He completed his residency in internal medicine at St. Luke's Hospital Center in New York City and his clinic research fellowship at the Royal Victoria Hospital in Montreal.

Samuel Kopel, M.D., Chief Medical Officer (67). In 1995, Dr. Kopel assumed the position of Chief Medical Officer of Maimonides; previously, he served as the Medical Director of Maimonides for six years and also as Associate Director of the Division of Hematology/Oncology of Maimonides. Since 1978, he has been an Assistant Professor of Clinical Medicine at SUNY Downstate Medical Center. Dr. Kopel came to the Medical Center from Mount Sinai Hospital in New York where he completed his fellowships in research hematology and oncology. He also completed his internship, residency in internal medicine and fellowship in clinical hematology at the Jewish Hospital and Medical Center (now Kingsbrook

Jewish Medical Center) in Brooklyn. Dr. Kopel received his B.A. degree from Brooklyn College in 1967 and his M.D. from the University of Bologna, Italy in 1972. He is board certified in hematology and medical oncology. Dr. Kopel is a member of the American Medical Association, the American College of Physician Executives, The Medical Society of the State of New York, The New York Academy of Sciences, as well as the American Society of Clinical Oncology, the American Society of Hematology, the New York Society of Medical Oncologists and Hematologists, The Society for Study of Blood and the World Federation of Hemophilia.

Marcel Biberfeld, Senior Vice President – Psychiatry & Community Services (73). Dr. Biberfeld joined the Medical Center in 1972 and served in a variety of positions, including Director of the Community Mental Health Center, Administrator of the Outpatient Department and Vice President of the Medical Library, prior to his promotion in 1991 to his present position. He is also the Vice President of Chaplaincy Services and oversees the Women, Infants and Children (WIC) Program at the Medical Center. Prior to joining the Medical Center, he was Director of Social Work at Sheepshead Nursing Home (1971-75) and Founding Director of the Jewish Family Services, Inc. of Boro Park (1968-69). In addition, he is a member of the Board of Directors at Ohel Children's and Family Services, and Bikur Cholim/Guardians of the Sick of Boro Park. Dr. Biberfeld received his B.A. degree from Brooklyn College in 1965, and his Masters degree in Social Work in 1971 and his D.S.W. degree in 1983 from Yeshiva University.

Martin A. Cammer, Senior Vice President and Chief Compliance Officer (62). Mr. Cammer joined the Medical Center in 1980 as the Administrator of Community Medicine and Ambulatory Services and has served in various administrative capacities during his tenure. Prior to joining the Medical Center, Mr. Cammer was the Administrator for Ambulatory Care at Jamaica Hospital and an Administrative Resident at LaGuardia Hospital. Mr. Cammer received his M.B.A. degree in Hospital Administration from the Columbia University School of Business Administration in 1977 and his B.A. degree from City College, City University of New York in 1972. Mr. Cammer is currently certified in Health Care Compliance (CHC).

Steven J. Davidson, M.D., Senior Vice President and Chief Medical Informatics Officer (62). Dr. Davidson's position focuses on facilitating patient and patient-caregiver engagement in their care through the use of mobile health and social media technologies, as well as supporting clinical leaders in their use of information to evaluate and improve clinical processes, outcomes and operational effectiveness. Prior to this position, he served for 15 years as founding Chairman of Emergency Medicine at the Medical Center upon his arrival at the Medical Center in 1995. He came to the Medical Center from the Medical College of Pennsylvania in Philadelphia where he served his residency and trained in Emergency Medicine (1975-78) and subsequently served on the faculty becoming Professor of Emergency Medicine (with tenure). Dr. Davidson received his AB degree in Chemistry from Temple University in 1971, his M.D. from Temple University School of Medicine in 1975 and his MBA from The Wharton School of the University of Pennsylvania in 1989.

Tom Doherty, Senior Vice President for Human Resources (66). Mr. Doherty joined the Medical Center in January 2012. From 1996 until joining the Medical Center, he provided management consulting services in the human resources area to various hospitals and healthcare organizations which included serving as a labor mediator appointed by both Local 1199SEIU and the Catholic Medical Center of Brooklyn and Queens to mediate all labor disputes, and as a labor arbitrator for the New York City Transit Authority and the Transit Workers Union in

disciplinary proceedings. Mr. Doherty previously spent 15 years at the New York City Health and Hospitals Corporation where he held human and labor-related positions, including Senior Vice President for Administration, Vice President for Human Resources and Assistant Vice President of Labor Relations. He attended Manhattan College and received a certificate in labor relations from the School of Industrial and Labor Relations at Cornell University in 1981.

Walter J. Fahey, Senior Vice President and Chief Information Officer (64). As Chief Information Officer, Mr. Fahey's responsibilities include information technology and telecommunications at Maimonides. In this regard, he helped Maimonides earn accolades for successful health care system implementation and compliance (see "Centers of Excellence – Health Care Information Technology"). He was also instrumental in the development of the Heal 1, 5, 10 and 17 Grant applications to develop the Brooklyn Health Information Exchange (BHIX). Mr. Fahey arrived at Maimonides in 1995 from The Hospital for Joint Diseases where he worked as Director of Management Information Systems from 1991 to 1995. He also held information technology leadership positions at St. Vincent's Hospital in Manhattan (1990-91) and The New York Methodist Hospital (1986-90). Earlier in his professional information technology career, he worked in manufacturing, retail and computer services. Mr. Fahey is a Board Member of the American Health Association, and a member of the Allscripts Executive Advisory Group, the Aruba Advisory Group, the College of Healthcare Information Management Executives (CHIME), the Gershon Lehrman Group Councils of Advisors, the Healthcare Association of New York State (HANYS), the Healthcare Information and Management Systems Society (HIMSS), the HIT Strategy Group, the Unisys Advisory Group and the Verizon Healthcare Advisory Group, and a former member of the NextGen Advisory Board. Mr. Fahey received his MBA from Farleigh Dickinson University (New Jersey) in 2001.

Lillian Fraidkin, Senior Vice President and Chief of Staff (74). Ms. Fraidkin joined the Medical Center in 1982 as Associate Director of the Emergency Department and Assistant Director of Nursing, and served as Administrative Director of the Operating Room, Post Anesthesia Care Unit and the Emergency Department (1985-88) and Administrator of the Emergency Room, Operating Room, Critical Care and Clinical Services, and Associate Director of Nursing (1988-94) prior to appointment to her present position in 1994. Prior to joining Maimonides, Ms. Fraidkin was in charge of the Emergency Department at Beth Israel Medical Center in New York City from 1965 to 1982. Ms. Fraidkin earned her Nursing Diploma from the Beth Israel School of Nursing, her B.A. degree from Marymount College in 1978 and her M.S. degree in Health Care Administration from Long Island University in 1986.

Derek D. Goins, Senior Vice President – Facilities & Support Services, and Vice President of MMC Management Corp. (60). Mr. Goins joined the Medical Center in 1972 and has served in various capacities, including Administrator of MMC Management Corp. (1989-1991) and Assistant Director of Plant Management (1984-85). He was promoted to his present position in 1991 and, as Vice President of MMC Management Corp., is responsible for the management of all of the real estate holdings of the Medical Center. Mr. Goins attended St. Joseph's College for Hospital Administration and holds a Registered Apartment Manager Certificate from New York University.

Douglas Jablon, Senior Vice President of Patient Relations and Volunteer Services; Special Assistant to President (62). In this position, Mr. Jablon focuses on the social, economic and cultural needs of patients and their families, and oversees the activities of over 1,800

volunteers and numerous volunteer programs surrounding the Medical Center. Mr. Jablon has a 36-year service history with the Medical Center and the community it serves. As an advocate for patient care and patients' rights and professional courteous customer service, Mr. Jablon is involved in every aspect of care provided by the Medical Center and, in this role, acts as the "sounding board" for patient, family and community concerns. He received his Associate of Applied Science degree from New York Community College in 1971, his BA degree in Sociology from the Rabbinical Assembly in 1973 and his Master's degree in Education, Guidance and Counseling from Long Island University in 1977.

Joyce A. Leahy, Esq., Senior Vice President for Legal Affairs and General Counsel; Assistant Secretary (59). Ms. Leahy joined the Medical Center in 1987 as General Counsel. In 1991, she was appointed Assistant Secretary and her title was changed to Vice President for Legal Affairs and General Counsel and, in 2012, she was promoted to Senior Vice President for Legal Affairs and General Counsel. She also is the Privacy Officer for the Medical Center. Prior to joining the Medical Center, Ms. Leahy was an associate with the law firm of De Forest & Duer, New York, New York (1978-87). She received her B.A. degree from Harvard University in 1975 and her J.D. degree from New York University School of Law in 1978. She is a member of the Bars of New York and Massachusetts, and is admitted to practice before the United States District Courts for the Southern and Eastern Districts of New York.

Damiano Marino, Senior Vice President, Financial Operations/Controller (62). Mr. Marino joined the Medical Center in 1998 and previously served as Vice President for Finance/Controller and Assistant Vice President/Controller. Prior to his joining the Medical Center, Mr. Marino served as Associate Director of Finance, Controller, and Senior Budget and Reimbursement Accountant at Catholic Medical Center of Brooklyn and Queens (1980-98), served as Budget and Reimbursement Accountant (1977-80) at Methodist Hospital and Staff Accountant at Sterling Drugs (1975-77). Mr. Marino received his B.S. in Accounting from St. Francis College in 1975.

Sheila Namm, R.N., Senior Vice President for Professional Affairs (65). Ms. Namm's responsibilities in this position include organizational performance, risk management, medical staff peer review, regulatory affairs, bioethics, credentialing, emergency management, safety, patient safety, disclosure/apology, Joint Commission readiness and claims management. She also leads the Bioethics Consultation Service at Maimonides. Ms. Namm joined the Medical Center in 1975 as Nurse Clinician for inpatient psychiatry. After holding several different positions throughout the Medical Center, she was appointed Vice President for Professional Affairs in 2001. Prior to joining the Medical Center, Ms. Namm was a Nurse Clinician at Kings County Hospital for child and adolescent psychiatry. She received her B.S. degree in Nursing from Hunter College in 1968, her M.A. degree in Child and Adolescent Mental Health Nursing from New York University in 1970 and her J.D. degree from New York Law School in 1995, a certificate in Bioethics and Medical Humanities from Montefiore Medical Center/Albert Einstein School of Medicine in 2007 and a certificate in Just Culture from Outcomes Engineering in 2010. Ms. Namm is a member of the Bars of New York and New Jersey, and is licensed as a registered nurse in New York State.

Karen Nelson, M.D., M.P.H., Senior Vice President for Integrated Delivery Systems and Executive Director of the Southwest Brooklyn Health Home (56). Dr. Nelson re-joined the Medical Center staff in April 2012 where she had been the Vice Chair of Medicine from 1996 to

1999. Between 1999 and 2012, she was the Chief Executive Officer and Medical Director of the Union Health Center which provides primary and specialty care to immigrant and low wage union members and their families in New York City. She was the Medical Director of Dorchester House Community Health Center in Boston, Massachusetts from 1988 to 1995. Dr. Nelson received her B.A. degree from Yale University in 1978 and her M.D. from the Mt. Sinai School of Medicine in 1985. She received her Masters of Public Health from Boston University School of Public Health in 1996 where she was a Robert Wood Johnson Scholar from 1994 to 1996.

John Scanlan, Senior Vice President for Finance (50). In this position for the past four (4) years, Mr. Scanlan oversees accounting, accounts payable, budget, chargemaster, decision support/cost accounting, financial planning, managed care contracting, payroll and reimbursement at the Medical Center. Since joining the Medical Center in 1997, he served as Vice President for Finance, Associate Vice President for Finance and Director of Reimbursement. Mr. Scanlan served as Manager of Reimbursement, Supervisor of Reimbursement and Staff Accountant at Catholic Medical Center of Brooklyn and Queens (1985-97). He is currently a fellow in the Healthcare Financial Management Association (HFMA) and is a Past President of the Metropolitan New York Chapter, who received the Medal of Honor, the Muncie Gold, the Reeves Silver and the Follmer Bronze Founders Awards. He also previously served as Vice President of Education, Treasurer and Secretary, and currently is the Chair of the Audit, Certification Coaching and Continuing Professional Education Committees of the HFMA Metropolitan New York Chapter. He has served as Chair of the Finance and General Education Committees as well as the Joseph A. Levi Annual Institute for the HFMA Metropolitan New York Chapter. Mr. Scanlan has served on the national Board of Examiners of HFMA. He is currently serving on the Board of Directors of the Bay Ridge Center as Finance Chair. Mr. Scanlan is also an Associate of the American College of Healthcare Executives. Mr. Scanlan received his B.S. in Finance from St. John's University in 1985 and his MBA in healthcare administration from Adelphi University in 1990.

Tom Smith, R.N., DNP, Senior Vice President – Nursing and Chief Nursing Officer (62). Mr. Smith joined the Medical Center in this position in January 2010. At Maimonides, he is responsible for nursing practice and patient care delivery. Mr. Smith was Chief Nursing Officer and Senior Vice President at Cambridge Health Alliance (2006-10) and Senior Vice President for Nursing at The Mount Sinai Hospital (1996-2005), the first full-service hospital in Manhattan to receive the Magnet Award for nursing excellence, which was received during his tenure. Mr. Smith has previously held leadership positions at Massachusetts General Hospital, Boston Medical Center (formerly Boston City Hospital) and University of Massachusetts-Boston. He has served as a Commissioner for the Commission on Collegiate Nursing Education (CCNE) of the American Association of Colleges of Nursing and is an active member of the American Organization of Nurse Executives. Mr. Smith received his B.S. in Nursing from the University of the State of New York in 1980, his M.S. in cardiopulmonary nursing from Boston University in 1985 and his Doctor of Nursing Practice (DNP) from Case Western Reserve University in 2010. He is a fellow in the American Academy of Nursing.

Harold Wodinsky, Senior Vice President, Physician Enterprises (57). Mr. Wodinsky joined the Medical Center in 2012 as Senior Vice President, Physician Enterprise. Prior to joining the Medical Center, he held the dual role of a Senior Vice President, Operations for the Advisory Board Company (Washington, DC) and Chief Administrative Officer, Staten Island

Physician Practice. Mr. Wodinsky received his Master of Health Sciences degree from the University of Toronto Graduate School of Health Administration in 1981 and his B.A. degree from Brandeis University (Waltham, MA) in 1979.

Eric Scalettar, Vice President – Planning (60). Mr. Scalettar joined the Medical Center in 1988 as Director of Institutional Planning and was promoted to his present position in 1991 where, among other things, he formulates the Medical Center's strategic plan and facility master plan, and planning and implementation of major construction projects. Prior to joining the Medical Center, Mr. Scalettar served as Administrative Director of Radiotherapy and Nuclear Medicine, Mount Sinai Medical Center (1985-88), Assistant Administrator, Maimonides Medical Center (1981-85), Administrative Resident, Long Island Jewish-Hillside Medical Center (1980-81), and Research Assistant, Mount Sinai Medical Center (1977-79). Mr. Scalettar received his M.P.H. degree from the Department of Epidemiology and Public Health, Yale University School of Medicine in 1985 and his B.A. degree from Harpur College, State University of New York at Binghamton in 1976.

Medical Staff

As of December 31, 2011, there were 1,226 physician members on the medical staff of the Medical Center holding appointments in three categories: employed (429), voluntary (777) and honorary (20). In 2011, approximately 600 physicians admitted patients to Maimonides. The medical staff is organized into 11 clinical departments. The Department of Medicine is comprised of 13 clinical divisions: Allergy and Immunology, Cardiology, Dermatology, Gastroenterology, Geriatric Medicine, Hematology/Oncology, Infectious Diseases, Internal Medicine, Metabolic Endocrinology, Nephrology, Neurology, Pulmonary Diseases and Critical Care, and Rheumatology and Immunology. The Department of Surgery is comprised of 12 clinical divisions: Colorectal Surgery, Dentistry, General Surgery, Neurosurgery, Ophthalmology, Otolaryngology, Pediatric Surgery, Plastic Surgery, Surgical Critical Care, Thoracic and Cardiovascular Surgery, Urology, and Vascular Surgery. The Department of Pediatrics is comprised of 14 clinical divisions: Adolescent Medicine, Cardiology, Critical Care, Dermatology, Endocrinology, Gastroenterology, General Pediatrics, Genetics, Hematology/Oncology, Infectious Diseases, Neonatology, Nephrology, Neurology and Pulmonary. The Department of Obstetrics and Gynecology has 3 clinical divisions: Gynecology/Oncology, Maternal and Fetal Medicine, and Reproductive Endocrinology. The Department of Orthopedic Surgery and Musculoskeletal Services has 3 clinical divisions: Orthopedic Surgery, Podiatry and Rehabilitation Medicine. The Department of Psychiatry also includes the Division of Psychology. The Department of Radiology is comprised of 9 clinical divisions: Body Imaging, Diagnostic Radiology, Emergency Radiology, Interventional Radiology, Mammography, Musculoskeletal Radiology, Nuclear Medicine, Pediatric Radiology and Radiotherapy. There are also Departments of Anesthesia, Emergency Medicine, Pathology and Laboratory Medicine, and Radiation Oncology. In 2011, the average age of the medical staff was 51 years old.

The following is a summary by department of the number of physicians, their average age and the percentage of department staff who are board certified as of December 31, 2011:

Medical Staff Composition

	<u>Number of Physicians</u>	<u>Average Age</u>	<u>% Board Certified</u>
MEDICINE			
Cardiology	66	56	88%
Dermatology	8	60	83%
Gastroenterology	36	50	94%
Hematology/Oncology	21	56	86%
Infectious Diseases	9	51	100%
Internal Medicine	243	52	75%
Metabolism and Endocrinology	7	60	100%
Nephrology	15	51	93%
Neurology	23	54	100%
Pulmonary Diseases and Critical Care	27	54	93%
Rheumatology and Immunology	<u>7</u>	<u>52</u>	<u>86%</u>
	462	53	84%
SURGERY			
Colorectal Surgery	2	53	100%
Dentistry	30	57	43%
General Surgery	21	57	71%
Neurosurgery	14	49	86%
Ophthalmology	24	49	88%
Otolaryngology	7	48	86%
Pediatric Surgery	1	48	100%
Plastic Surgery	12	46	92%
Thoracic and Cardiovascular Surgery	15	53	93%
Urology	41	54	93%
Vascular Surgery	<u>6</u>	<u>54</u>	<u>100%</u>
	173	53	80%
OTHER			
Anesthesiology	41	48	83%
Emergency Medicine	54	42	91%
Obstetrics and Gynecology	72	51	92%
Orthopedics	31	52	81%
Pathology and Laboratory Medicine	8	57	100%
Pediatrics	221	51	89%
Podiatry	40	58	68%
Psychiatry	60	52	83%
Radiology	47	51	98%
Rehabilitation Medicine	<u>17</u>	<u>51</u>	<u>88%</u>
	<u>591</u>	<u>51</u>	<u>87%</u>
TOTAL	1,226	51 (average)	84% (average)

Note: The Board Certification category does not include medical staff members who are board eligible but have not yet received board certification.

Nursing

As of September 30, 2012, the Medical Center employed approximately 2,294 full-time equivalent ("FTE") employees on its nursing/patient care staff. This includes approximately 1,212 FTE registered nurses (excluding advanced practice nurses), 37 FTE licensed practical nurses, 488 FTE patient care technicians/mental health workers, 124 FTE ward clerks/information specialists, and 60 FTE nursing supervisors and administrators. An additional

373 FTE employees include advanced practice nurses, technicians from other areas (e.g. operating room, heart monitoring and hemodialysis) and a variety of other specialty roles such as nurse educators. In recent years, the Medical Center has been able to successfully recruit and retain an adequate number of registered nurses despite periodic shortages and fluctuations in the supply of registered nurses. The current vacancy rate of registered nurses is less than 2 percent.

Employees

As of December 31, 2011, the Medical Center employed approximately 5,621 FTE employees, which include 231 FTE employees related to specific purpose programs and contracts. The number of FTE employees has increased in the last year by approximately 1.2% as a result of expanded programs and higher acuity patients.

Employee Benefits

The Medical Center offers to eligible employees health insurance covering hospitalization, major medical expenses, prescription drugs, and dental, health care and dependent care reimbursement accounts; short and long term disability; life insurance; pension plan; and tuition reimbursement. Both the New York State Nurses Association Pension Plan and the 1199 SEIU Health Care Employees Pension Fund are multiemployer defined benefit pension plans (see “Pension Plans and Post-Retirement Health Care Benefits” below).

Collective Bargaining Agreements/Employee Relations

Maimonides has three collective bargaining agreements covering approximately 71% of its employees: an agreement with the Committee of Interns and Residents covers 434 residents and fellows, and expires on October 31, 2013; an agreement with the New York State Nurses Association covers approximately 1,281 registered nurses and expires on January 1, 2014; and an agreement with Local 1199 covers approximately 2,703 service, clerical and technical employees and expires on April 30, 2015. Maimonides considers its relations with its employees to be very good.

Educational Programs

Maimonides serves as a major teaching affiliate of the Albert Einstein College of Medicine of Yeshiva University in The Bronx and the NYU School of Medicine in Manhattan, and is also a teaching affiliate of the SUNY Downstate College of Medicine in Brooklyn. Maimonides recently signed an affiliation agreement with Yeshiva University through Einstein to strengthen Maimonides’ teaching programs and fortify physician recruitment at Maimonides. Einstein operates one of the largest post-graduate medical training programs in the United States. Maimonides physicians who teach and supervise Einstein students will receive faculty appointments at Einstein, and will participate on faculty committees and in faculty development programs.

The Medical Center is a primary participant in three major integrated programs with SUNY Downstate College of Medicine: Endocrinology, Otolaryngology, and Pediatric Dermatology and Pediatric Endocrinology. Maimonides sponsors 22 residency and fellowship programs fully accredited by the Accreditation Council for Graduate Medical Education (ACGME). Maimonides also sponsors an Osteopathic Internship program fully accredited by the American Osteopathic Association (AOA). As a direct result of this commitment to

physician education, the Medical Center provides clinical training to approximately 480 residents in numerous specialties and over 1,000 medical students.

The Medical Center also partners with various social service, medical service, and nursing home providers by entering into linkage or back-up medical services agreements wherein the Medical Center agrees to make its facilities available as a back-up medical resource to the patients or members of the other entity in cases where it is beyond the scope of such entity to provide the contemplated medical services.

In addition, the Medical Center maintains educational affiliations with numerous institutions including, but not limited to, the following institutions: Bellevue Hospital Center, Brookdale Medical Center, College of Mount Saint Vincent, College of New Rochelle, Columbia University, Coney Island Hospital, Connecticut Children's Medical Center, CUNY – Borough of Manhattan Community College, CUNY – LaGuardia Community College, CUNY – Kingsborough Community College, East Stroudsburg University of Pennsylvania, Emory University, Excelsior College, Hartford Hospital, Hofstra University, Kaplan University, Kean University, Kings County Hospital Center, Long Island University – C.W. Post, Lutheran Medical Center, Memorial Sloan Kettering Cancer Center, Midwood Dialysis Center, Molloy College, Montefiore Medical Center – Henry and Lucy Moses Division, Mount Sinai Medical Center, New York City Poison Control Center, North Shore – Long Island Jewish Health System, NYU Langone Medical Center, Pace University, Quinnipiac University, Richmond University Medical Center, SUNY – Canton, SUNY – Downstate Medical Center, Touro College, University of Florida, University of Maryland Shock Trauma Center, Wagner College, Walden University, Weber State University and Yale University.

Graduate Medical Education Funded Positions*

<u>Program</u>	<u>Number of Residents/Fellows Starting July 2010</u>	<u>Number of Residents/Fellows Starting July 2011</u>	<u>(Budgeted) Number of Residents/Fellows Starting July 2012</u>
Anesthesiology	27.0	26.8	29.5
Cardiology	20.9	21.0	21.0
Cardiology - Interventional	3.0	3.0	3.0
Cardiothoracic Surgery	--	--	1.0
Critical Care	6.0	6.0	6.0
Dentistry	22.0	20.0	23.0
Emergency Medicine	42.0	42.5	46.0
Gastroenterology	7.0	8.0	8.0
General Surgery	38.5	38.7	42.3
Geriatrics Medicine	2.0	1.9	2.1
Hematology/Oncology	9.0	9.0	9.0
Infectious Diseases	4.9	5.9	6.0
Internal Medicine	91.0	93.0	96.5
Nephrology	2.0	1.9	2.0
Obstetrics/Gynecology	20.0	19.8	20.0
Orthopaedics Surgery	12.0	14.0	14.0
Osteopathic Medicine	12.0	14.0	8.0
Otolaryngology	--	--	1.0
Pediatrics	74.1	68.0	80.0
Psychiatry	35.1	34.5	35.6
Pulmonary	4.0	4.0	4.0

<u>Program</u>	Number of Residents/Fellows Starting July 2010	Number of Residents/Fellows Starting July 2011	(Budgeted) Number of Residents/Fellows Starting July 2012
Radiology	16.0	16.0	16.0
Transitional	4.0	--	--
Urology	4.5	4.0	4.0
Vascular Surgery	<u>2.0</u>	<u>2.0</u>	<u>2.0</u>
TOTAL	459.0	454.0	480.0

* Includes residents paid by Coney Island Hospital in Maimonides' programs

Maimonides' programs in dentistry and oral surgery are approved by the American Dental Association (ADA). Maimonides participates in the National Resident Matching Program and is a member of the Council of Teaching Hospitals and the Association of American Medical Colleges. All of Maimonides' programs are certified, licensed or otherwise accredited based on the requirements of the field of inquiry.

The Medical Center maintains affiliations with numerous institutions providing clinical training to social work interns, psychology interns, licensed creative arts therapists, dance therapists, radiology technicians, radiotherapy assistants, midwives, pharmacists and physical therapists in affiliation with area colleges and universities, and provides clinical training to approximately 400 nursing students on both the graduate and undergraduate levels on an annual basis. The Medical Center does not make any payment to the schools on behalf of the nursing students nor does it pay the nursing students a salary or stipend.

Centers of Excellence

Maimonides is acknowledged by health care providers as a center of excellence in various specialties, including its Emergency Center, Cancer Center, Cardiac Institute, Infants & Children's Hospital, and Psychiatric Services.

Comprehensive Medical Services: Maimonides' reputation as a leader in medical excellence is evident throughout its many clinical divisions and subspecialties. The Medical Center's tradition of excellence in delivery of primary care, acute inpatient care, medical training and innovative research is consistent with its commitment to addressing its patient's diverse healthcare needs from heart care to pediatric medicine. In 2011, the Medical Center was awarded the General Surgery Excellence Award and the Critical Care Excellence Award by HealthGrades.

Emergency Room: The Medical Center is a designated 911-EMS and a New York State Psychiatric Receiving Hospital (pursuant to New York State Mental Hygiene Law) which provided care to 126,539 patients, including 14,182 obstetrics visits in 2011. In 2011, 33,449 patients were admitted to the Medical Center through the Emergency Room. All Emergency Room physicians are board-certified or board-eligible in emergency medicine and/or internal or family medicine, and maintain advanced cardiac and advanced trauma life support certification.

Bariatric Surgery: Maimonides has been named as a Bariatric Surgery Center of Excellence by the American Society for Metabolic and Bariatric Surgery. This designation

recognizes surgical programs with a demonstrated track record of favorable outcomes in bariatric surgery.

Cancer Center: Brooklyn's only dedicated freestanding multidisciplinary cancer center combines leading-edge cancer treatments, integrated multidisciplinary care, and the very latest in diagnostic and treatment technologies with a compassionate, patient-centered environment. In 2011, the Maimonides Cancer Center was expanded with the state-of-the-art Gilbert Rivera Maimonides Breast Center, the only accredited Breast Imaging Center of Excellence in Brooklyn offering digital mammography, sonography and magnetic resonance imaging (MRI) of the breast. See "Radiation Oncology" below.

Cardiac Institute: As one of New York City's largest providers of cardiology, cardiac catheterization and cardiac surgery services, Maimonides' Cardiac Institute is the primary source of cardiac care for Brooklyn residents and one of the most comprehensive cardiac care centers in the nation. In 1967, the Medical Center performed the first human heart transplant operation in the United States and continues as an innovative force in developing new approaches to heart disease. More recently, Maimonides is the only center in Brooklyn and one of the select few cardiac care centers in the nation to offer transcatheter aortic valve replacements. Since 2009, averages of 300 open-heart operations and 4,700 cardiac catheterization procedures have been performed annually. Maimonides is a widely recognized leader in cardiac catheterization, bringing new technologies to the patient both in diagnostic angiography and therapeutic interventions, and is at the cutting edge of developing and implementing new angioplasty, atherectomy, and stent technologies. In addition to clinical work, the Maimonides' Cardiac Institute has developed and implemented a variety of educational outreach activities, including health fairs and forums, video series, smoking cessation programs and family support groups. Highlighting the Cardiac Institute's commitment to treating patients with advanced heart failure, Maimonides' LVAD Program, the only such program serving Brooklyn and Staten Island, was recently accredited by The Joint Commission.

Geriatrics: Maimonides was recently recognized as a NICHE-designated hospital for its comprehensive geriatric program. NICHE (Nurses Improving Care for Healthsystem Elders) is awarded by the Hartford Institute for Geriatric Nursing at NYU College of Nursing. Brooklyn has the largest percentage of elderly people in New York City, of which over 40% have a disability, including mobility and self-care limitations.

Health Care Information Technology: Maimonides has earned accolades for successful health care system implementation and compliance with The Leapfrog Group standards, and has been named to the American Hospital Association's "Most Wired" and/or "Most Wireless" lists from 1999 to 2011, more than any other hospital in New York State. In addition, Maimonides won The Nicholas E. Davies Award in 2002 and the Computerworld Smithsonian Award in 1998.

Health Home: Building upon a model developed under two New York State HEAL grants, Maimonides developed the Southwest Brooklyn Health Home, designated by New York State in December 2011 as one of four Medicaid Health Homes in Brooklyn. The Southwest Brooklyn Health Home is comprised of 50 member organizations and serves patients throughout Brooklyn with serious mental illness and multiple chronic medical conditions. A key component of the Southwest Brooklyn Health Home is the core interdisciplinary care team that includes a

care manager, care navigator and primary care provider, supplemented by psychiatrists and therapists, who work on a collaborative basis with the patient and family to create and implement the care plan. This core Maimonides team is linked electronically to members of the extended care team including specialty providers, residential providers and home care providers.

Infants & Children's Hospital: The Maimonides Infants & Children's Hospital is the first children's hospital in Brooklyn accredited by the National Association of Children's Hospitals and Related Institutions. Serving a community with one of the highest birth rates in the country, the Infants & Children's Hospital has become the leading pediatric hospital in Brooklyn, specifically focusing on the treatment and care of infants, children and adolescents. Providing more than 30 pediatric surgical, medical and behavioral subspecialties, the Infants and Children's Hospital embraces the family-centered model of care supporting the excellence of Maimonides' community-based primary care physicians. See "Neonatology" below.

Neonatology: The 31 bassinet NICU is a Level III service which provides support to the approximately 8,000 deliveries performed annually at the Medical Center, as well as three other local hospitals with major obstetrical services linked to Maimonides as the New York State Department of Health designated Regional Perinatal Center (RPC) for Southwest Brooklyn. Staffed 7 days a week on a 24 hour a day basis by board certified neonatologists covering the NICU, and the Labor and Delivery Suite, the neonatology staff is trained and capable in delivering all methods of intervention and treatment of newborns in distress due to prematurity and congenital diseases.

Orthopedic Surgery: In addition to the comprehensive array of orthopedic surgeries performed, the orthopedic surgery program at Maimonides is the only program in Brooklyn providing endoscopic spine surgery and complex pediatric orthopedic surgery.

Psychiatric Services: The Department of Psychiatry operates two inpatient units with a 70-bed capacity for acutely ill patients admitted on a voluntary, involuntary or emergency basis. The active diagnostic and therapeutic services, including psychotherapy, psychopharmacology and psychiatric nursing, are provided in a therapeutic milieu designed to alleviate the exacerbated symptoms of the psychiatric disorder and are individualized to meet the specific needs of patients. The Department of Psychiatry also provides numerous outpatient services to those patients who are able to live in the community, including an acute treatment day program which provides short-term, intensive and highly-structured services to meet the daily treatment needs of severely and persistently ill patients in crisis. The Medical Center's Liaison Psychiatry Service provides psychiatric consultations and care to any patient in the Medical Center's clinics, Emergency Room or medical/surgical wards and provides a Crisis Intervention Unit to members of the community.

Radiation Oncology: The development of the freestanding multidisciplinary Cancer Center included the addition of radiation oncology to the array of advanced, integrated therapies available to Maimonides physicians in the diagnosis and treatment of a wide array of cancer diseases. The Department of Radiation Oncology at the Cancer Center has the equipment, board certified physicians and board certified medical physicists to provide its patients with state-of-the-art evidence-based care.

Reproductive Endocrinology: The Division of Reproductive Endocrinology assists patients in overcoming reproductive problems, utilizing a full spectrum of assisted reproductive technologies, including in vitro fertilization, which is located in an outpatient setting that is separate from the Medical Center’s other perinatal services.

Stroke Center: The Jaffe Stroke Center was awarded the Gold Plus Award for excellence in performance in the “Get With the Guidelines Program” of the American Heart Association and the American Stroke Association. In addition, Maimonides was named as one of the four Brooklyn hospitals to the Target: Stroke Honor Roll by the American Heart Association and the American Stroke Association in May 2012, one of only 50 hospitals in the country to achieve this recognition. In addition, the stroke program is accredited by The Joint Commission.

Vascular Surgery: The vascular surgery program provides inpatient and outpatient diagnostic and therapeutic interventions in peripheral vascular diseases, including limb salvage in difficult diabetic disease management, with extremely low rates of morbidity and mortality.

Service Area

The Medical Center’s primary service area consists primarily of the Borough Park, Ocean Parkway, Sunset Park, Windsor Terrace, Bay Ridge, Dyker Heights, Bensonhurst, Bath Beach, Midwood, Flatbush and Kensington sections of Brooklyn. The population of these areas according to the 2010 population data from the New York City Department of City Planning (the “2010 Population Data”) was 768,586; area residents accounted for approximately 55.4% of the Medical Center’s inpatient discharges in 2011.

The Medical Center’s secondary service area consists primarily of the Brighton Beach, Sheepshead Bay, Gerritsen Beach, Homecrest, Gravesend and Coney Island sections of Brooklyn. The population of these areas was 266,439 according to the 2010 Population Data; residents of this area accounted for approximately 21.8% of the Medical Center’s inpatient discharges in 2011.

The following table shows the approximate percentage of total inpatient discharges for residents of the Medical Center’s primary and secondary service areas for the years ended December 31, 2009, 2010 and 2011.

Percent of Total Medical Center Discharges by Service Area *

	<u>2009</u>	<u>2010</u>	<u>2011</u>
Service area			
Primary service area	54.9%	55.7%	55.4%
Secondary service area	21.9	22.0	21.8
Other service areas	<u>23.2</u>	<u>22.3</u>	<u>22.8</u>
TOTAL	100.0%	100.0%	100.0%

* Excludes Normal Newborn Nursing

Source of DRG Data: Hospital Association of New York State, Market Expert Product Lines

The recent storm in Brooklyn did not result in the loss of service at the Medical Center although it did result in the temporary closing of certain service area health care institutions.

Other Area Hospitals

The following table sets forth the number of certified beds, the occupancy rate and number of discharges for 2010 for Maimonides and certain other hospitals located in and around the primary service area of Maimonides:

Summary of Historical Utilization of Certain Other Area Hospitals for 2010

<u>Hospital</u>	<u>Certified Beds</u>	<u>Occupancy Rate</u>	<u>Discharges</u>	<u>Average Length of Stay</u>
Maimonides Medical Center	705	84.4%	45,658	5.68
Beth Israel Brooklyn	212	89.5%	11,313	6.12
Coney Island Hospital	371	83.7%	18,323	6.60
Kings County Hospital Center	695	69.4%	24,637	7.82
Lutheran Medical Center	468	78.4%	27,678	5.62
New York Community Hospital of Brooklyn	134	90.3%	7,142	6.19
SUNY Downstate Medical Center	376	74.8%	17,828	6.14

Source: At the Brink of Transformation: Restructuring the Healthcare Delivery System in Brooklyn Group,” Report of the Brooklyn Health Systems Redesign Work Group, dated November 28, 2011.

Utilization

A summary of the Medical Center’s historical utilization data for the years ended December 31, 2009, 2010 and 2011, and the eleven-month periods ended November 30, 2011 and 2012 is presented in the following table:

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Historical Utilization

	<u>Year Ended December 31,</u>			<u>Eleven-Month Period Ended</u>	
	<u>2009</u>	<u>2010</u>	<u>2011</u>	<u>November 30, 2011</u>	<u>November 30, 2012</u>
Licensed Beds ¹	705	711	711	711	711
Inpatient Discharges	46,696	46,685	46,209	42,114	41,406
Patient Days	239,240	245,553	230,702	210,277	209,672
Patient Days ¹	222,340	229,172	213,903	194,930	193,631
Licensed bed days available ¹	257,325	258,420	259,515	237,474	238,185
Average length of stay ²	5.12	5.26	4.99	4.99	5.06
Average length of stay ^{1, 2, 3}	5.21	5.39	5.15	5.14	5.26
Medicare case mix index ³	1.69	1.67	1.73	1.74	1.85
Non-Medicare case mix index ³	1.25	0.95	1.00	1.02	0.98
Average occupancy rate ^{1, 2}	93%	93%	87%	87%	85%
Medical/Surgical occupancy rate ^{1, 2, 3, 4}	96%	96%	89%	89%	88%
Maternity occupancy rate ²	101%	103%	105%	105%	109%
Psychiatry occupancy rate ²	91%	94%	87%	88%	85%
Emergency room visits (less admissions)	90,571	89,608	93,090	84,854	89,649
Outpatient visits ⁵	307,750	322,162	337,883	309,659	319,289
Psychiatry outpatient visits	65,050	61,158	60,186	55,689	54,266
Outpatient procedures	7,643	8,177	8,273	7,817	8,128

¹ Excludes Nursery

² Calculated based upon beds in service and not licensed beds

³ Excludes Psychiatry

⁴ Excludes Pediatric units

⁵ Includes Cancer Center from August 2009

A summary of the Medical Center's discharges by major specialties for the years ended December 31, 2009, 2010 and 2011, and the eleven-month periods ended November 30, 2011 and 2012 is presented in the following table:

Discharges by Major Specialty Groupings

<u>Specialty Grouping</u>	<u>Year Ended December 31,</u>			<u>Eleven-Month Period Ended</u>	
	<u>2009</u>	<u>2010</u>	<u>2011</u>	<u>November 30, 2011</u>	<u>November 30, 2012</u>
Medicine					
General Medicine	18,929	19,071	18,178	16,617	15,263
Neurology	308	336	418	378	413
Total Medicine	19,237	19,407	18,596	16,995	15,676
Surgery					
General Surgery	2,582	2,532	2,487	2,248	2,243
Thoracic Surgery	969	767	759	693	916
Urology	739	699	636	585	578
Neurosurgery	552	543	617	572	514
Orthopedics	728	700	627	575	483
Vascular Surgery	800	772	616	569	401
Ear, Nose & Throat	113	91	124	110	145
Plastic Surgery	26	27	23	20	38
Total Surgery	6,509	6,131	5,889	5,372	5,318

Obstetrics	8,320	8,375	8,525	7,769	8,122
Gynecology	769	723	730	661	571
Total OB/GYN	9,089	9,098	9,255	8,430	8,693
Pediatrics	2,731	2,765	2,878	2,556	2,623
Neonatal	603	620	643	580	689
Inpatient Subtotal	38,169	38,021	37,261	33,933	32,999
Nursery	7,254	7,290	7,462	6,814	7,044
Psychiatric	1,273	1,374	1,486	1,367	1,363
Total	46,696	46,685	46,209	42,114	41,406

Management's Discussion of Utilization

During the past three years, the Medical Center has experienced relatively consistent inpatient volume. In 2011, total discharges decreased by 1% as compared to 2010 while remaining stable from 2009 to 2010.

From 2010 to 2011, general medicine and surgery discharges decreased. The reduction in volume was partially attributable to evolving changes in the health care industry where select groups of surgical cases are being performed in outpatient ambulatory surgery settings rather than as inpatient admissions. Largely a consequence of improvements in surgical technologies, techniques and improvements in the effectiveness, duration and delivery of pharmaceuticals and anesthetic agents, these factors have also resulted in reductions in lower acuity medical volume. Finally, the challenging economic environment has caused patients to defer seeking select elective and non-urgent medical care. Offsetting the decreases were increases in obstetrics, pediatrics and nursery discharges due to the Medical Center's growing position as a leading newborn and pediatric center. Increases in psychiatric discharges also offset the decrease in 2011 general medicine and surgery discharges. Decreases in discharges for the comparative eleven-month periods are consistent with the explanation for 2011.

For 2010, as in 2011, decreases in surgery discharges were offset by increases in ambulatory procedures as well as discharges in medicine, obstetrics and psychiatric cases leading to stable overall inpatient discharges from 2009. The Medical Center's stable inpatient utilization and decreased average length of stay ("ALOS") have been coupled with increases in Medicare case mix indices related to the complexity of the illnesses of the patients served.

ALOS, excluding nursery and psychiatry, decreased in 2011 by 0.24 days compared to 2010; however, ALOS, excluding nursery and psychiatry, increased by 0.18 days from 2009 to 2010 and by 0.12 days for the eleven-month periods November 30, 2011 to 2012. The improvement in ALOS for 2011 from 2010 was mainly attributable to decreases in medicine and surgery. The 2011 results are consistent with the Medical Center's investments in primary care and clinical referral programs as well as greater efficiency in the use of Medical Center beds. The increase in ALOS for the eleven-month period ended November 30, 2012 was primarily due to fewer shorter stay medicine cases.

Average occupancy in 2011 of 87% decreased from 2010 and 2009 levels. Average occupancy also decreased in 2012 from the eleven-month period ended November 30, 2011, all

primarily as a result of decreases in medical/surgical occupancy due to the general economic conditions delaying elective procedures as well as evolving changes in the health care industry as mentioned above. Psychiatry occupancy also experienced decreases in 2012 as well as in 2011. Occupancies in the maternity area have consistently exceeded 100% for all reported periods.

Consistent with Maimonides' outreach to traditional and new communities, utilization of ambulatory care services has seen steady growth over the past three years. For all reporting periods, outpatient visits increased in the children's and adult psychiatric clinics, other outpatient visits and the multispecialty Cancer Center. Emergency visits also increased for all reported periods due to an increase in capacity as a result of the major renovations to the Emergency Room.

All utilization statistics and related information in this Official Statement represent historical data and may not be indicative of future activity. The Medical Center cannot assess or predict the ultimate effect on its operations that may result from future changes in utilization.

Summary of Historical Operations

The following Summary of Historical Operations for each of the years ended December 31, 2009, 2010 and 2011 was derived from the financial statements of the Medical Center which have been audited by Ernst & Young LLP, independent auditors. The financial statements of the Medical Center for the years ended December 31, 2011 and 2010, which have been audited by Ernst & Young LLP, are included in Appendix B.

The Summary of Historical Operations for the eleven-month periods ended November 30, 2011 and 2012 was derived from the Medical Center's unaudited financial statements. The unaudited financial information includes all adjustments, consisting of normal recurring accruals, which Medical Center management considers necessary for a fair presentation of the financial position and the results of operations for these periods. Operating results for the eleven-month periods ended November 30, 2011 and 2012 are not necessarily indicative of the results that may be expected for the entire years ending December 31, 2011 and 2012, as the case may be. The unaudited financial statements for the eleven-month periods ended November 30, 2011 and 2012 are included in Appendix C.

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The following data should be read in conjunction with the financial statements, related notes, and other information included herein.

Summary of Historical Operations*
(in thousands)

	Year Ended December 31,			(Unaudited) Eleven-Month Period Ended	
	<u>2009</u>	<u>2010</u>	<u>2011</u>	<u>November 30,</u>	<u>November 30,</u>
				<u>2011</u>	<u>2012</u>
<u>Operating Revenue</u>					
Net patient service revenue	\$ 888,664	\$ 950,578	\$ 942,798	\$ 851,179	\$ 888,163
Provision for bad debt	(38,150)	(18,770)	(15,122)	(11,257)	(8,793)
Net patient service revenue less provision for bad debt	850,514	931,808	927,676	839,922	879,370
Other revenue	36,441	44,341	48,654	39,664	32,123
Net assets released from restrictions	3,037	2,337	1,765	1,550	1,513
Total Operating Revenue	<u>889,992</u>	<u>978,486</u>	<u>978,095</u>	<u>881,136</u>	<u>913,006</u>
<u>Operating Expenses</u>					
Salaries and wages	442,433	472,167	476,048	430,970	455,163
Employee benefits	121,315	133,241	142,293	128,623	137,558
Supplies and other expenses	266,024	292,125	296,516	266,567	271,835
Depreciation and amortization	38,229	41,507	46,098	40,933	43,256
Interest and amortization of deferred financing costs	13,472	11,471	10,608	9,723	8,355
Total Operating Expenses	<u>881,473</u>	<u>950,511</u>	<u>971,563</u>	<u>876,816</u>	<u>916,167</u>
Excess (deficiency) of operating revenue over operating expenses before other items	8,519	27,975	6,532	4,320	(3,161)
Other items	28,467	21,046	(12,110)	(9,503)	16,146
Unrealized gain (loss) on investments and change in fair value of swap agreements	<u>734</u>	<u>169</u>	<u>1,248</u>	<u>440</u>	<u>(72)</u>
Excess (deficiency) of revenue over expenses	37,720	49,190	(4,330)	(4,743)	12,913
Change in accrued pension and post retirement benefits liabilities to be recognized in future periods	(485)	(2,659)	(11,099)	---	---
Grants for capital asset acquisitions and net assets released from restrictions used for capital asset acquisitions	<u>832</u>	<u>140</u>	<u>8,863</u>	<u>7,670</u>	<u>873</u>
Increase (decrease) in unrestricted net assets	<u>\$ 38,067</u>	<u>\$ 46,671</u>	<u>(\$ 6,566)</u>	<u>\$ 2,927</u>	<u>\$ 13,786</u>

* A recently issued accounting standard update requires the provision for bad debts to be presented as a reduction from net patient service revenue rather than as an operating expense. The Medical Center adopted this standard update in 2012 and modified the statements of operations for the prior periods presented above to conform to this standard update. See the notes included with the unaudited financial statements in Appendix C for further information. The audited financial statements included in Appendix B do not include the modified presentation for the years ended December 31, 2011 and 2010.

Management's Discussion and Analysis of Recent Financial Performance

Eleven-Month Period Ended November 30, 2012 compared to Eleven-Month Period Ended November 30, 2011:

For the eleven-month period ended November 30, 2012, after taking into account other items, the Medical Center reported an increase in unrestricted net assets of \$13,786,000 as compared to an increase of \$2,927,000 for the same period in 2011. The Medical Center recorded a deficiency of operating revenue over operating expenses of \$3,161,000 in 2012 as compared to an excess of operating revenue over operating expenses of \$4,320,000 in 2011. In 2012, the Medical Center's financial performance has been, for the most part, in line with the operating plan for the Medical Center. This has been achieved through increases in case mix indices, and retroactive rate adjustments which offset lower than expected volume. Offsetting the loss from operations were earnings from the Medical Center's captive insurance program (the "Captive Insurance Program") of \$13,887,000 and the recognition of a medical resident FICA refund of \$2,259,000 that positively impacted the increase in unrestricted net assets.

Total current assets at November 30, 2012 was \$386,705,000, an increase of \$8,533,000 from December 31, 2011. This was primarily due to an increase in net account receivables for patient care of \$7,081,000. Overall, total assets increased by \$35,162,000 to \$1,005,336,000 as a result of an increase in other assets of \$13,822,000 related to funds received from a lender that are available to be spent on capital leased fixed assets as well as an increase to the equity in the Captive Insurance Program of \$8,120,000. Total liabilities increased by \$21,977,000, primarily due to increases in short-term and long-term debt and capital lease obligations of \$30,407,000, and an increase in accounts payable and accrued expenses of \$9,095,000, offset by a decrease in other noncurrent liabilities of \$15,722,000.

Year Ended December 31, 2011 compared to Year Ended December 31, 2010:

For 2011, the Medical Center reported a decrease in unrestricted net assets of \$6,566,000. Although the Medical Center recorded an excess of operating revenue over operating expenses of \$6,532,000, a shortfall related to investment earnings in the Captive Insurance Program of \$10,383,000 and a decrease in net assets from the change of \$11,099,000 in accrued pension and post-retirement benefits liabilities to be recognized in future periods related to reductions in the discount rate utilized were the main causes for the reduction in unrestricted net assets.

Total current assets at December 31, 2011 were \$378,172,000, an increase of \$26,573,000 from 2010. This increase was mainly attributable to increases of net account receivables for patient care of \$12,223,000, cash, cash equivalents and short-term investments of \$6,999,000 and other current assets of \$6,423,000. Overall, total assets increased by \$22,795,000 to \$970,174,000 primarily as a result of the increase in total current assets. Total liabilities increased by \$29,539,000, mainly due to increases in accrued pension and post-retirement benefits of \$12,823,000, professional liabilities of \$16,768,000 and other current liabilities of \$21,506,000, offset by a decrease in long-term debt and capital lease obligations of \$15,582,000.

Year Ended December 31, 2010 compared to Year Ended December 31, 2009:

For 2010, the Medical Center reported an increase in unrestricted net assets of \$46,671,000, after taking into account other items. The Medical Center recorded an excess of operating revenue over operating expenses of \$27,975,000 related to renegotiated managed care contracts, receipt of new funding from the New York State Department of Health, other operating income related to government grant income, a settlement related to a capital advance to Victory Memorial Hospital prior to its bankruptcy and an increase in insurance company surplus distributions. Along with the excess of operating revenue over operating expenses, the Medical Center had earnings from the Captive Insurance Program of \$10,323,000, and \$10,723,000 related to the accrual of a refund from the Internal Revenue Service for FICA taxes on medical residents which contributed to the increase in unrestricted net assets.

Total current assets at December 31, 2010 were \$351,599,000, an increase of \$88,534,000 from 2009. This was primarily a result of increases of \$23,685,000 in cash, cash equivalents and short-term investments, net accounts receivables for patient care of \$11,304,000, a \$23,368,000 increase in other current assets mainly due to the accrual of the anticipated medical resident FICA refund and \$28,205,000 from a change in accounting principle related to the recognition of the current portion of estimated insurance claims receivable. Total assets in 2010 of \$947,379,000 increased by \$279,225,000 from 2009. This increase related to the aforementioned increase in total current assets as well as an increase in net property, plant and equipment of \$11,476,000, an increase of other assets of \$13,104,000 mainly related to funds received from a lender that are available to be spent on capital leased fixed assets and \$159,826,000 from a change in accounting principle related to the recognition of estimated long-term insurance claims receivable. Total liabilities increased by \$232,167,000, mainly related to increases in accrued salaries and related liabilities of \$10,916,000, increases in short-term and long-term debt and capital leases of \$15,768,000, other current liabilities increases of \$13,480,000 primarily related to the medical resident FICA refund and \$188,031,000 in estimated insurance claims liabilities from the change in accounting principle referred to above.

Discussion of Outstanding Indebtedness

Long-Term Debt

The following is a summary of long-term debt as of December 31, 2009, December 31, 2010, December 31, 2011 and November 30, 2012 (in thousands):

	Interest Rates	Maturity	December 31, 2009	December 31, 2010	December 31, 2011	(Unaudited) November 30, 2012
1996 FHA Section 242 insured mortgage loan (a)	6.55%	10/01/22	\$ 11,939	\$ 11,325	\$ 10,669	\$ 10,029
2004 FHA Section 241 insured mortgage loan (a)	5.45%	3/01/32	98,868	96,520	94,041	91,647
2006 New York City Capital Resource Corporation loan/ Syndicated bank credit facility (b)	1-month LIBOR+ 1.70%	5/31/19	28,455	27,430	26,345	26,531

	<u>Interest Rates</u>	<u>Maturity</u>	<u>December 31, 2009</u>	<u>December 31, 2010</u>	<u>December 31, 2011</u>	<u>(Unaudited) November 30, 2012</u>
Construction bank loans	1.87% ^(c)	7/2016 ^(c)	5,150	3,986	2,822	20,674
Capital lease obligations	2.65% ^(c)	4/2015 ^(c)	34,983	55,902	45,836	61,239
Less current portion			(16,124)	(21,049)	(21,181)	(26,162)
TOTAL			<u>\$ 163,271</u>	<u>\$ 174,114</u>	<u>\$ 158,532</u>	<u>\$ 183,958</u>

- (a) This loan is to be refinanced by the loan made in connection with the issuance of the Series 2013 Bonds
- (b) The 2006 New York City Capital Resource Corporation loan was prepaid and refinanced with a syndicated bank credit facility on June 1, 2012
- (c) Multiple loans/obligations with weighted average interest rates (based upon 1-month LIBOR) and maturities provided as of November 30, 2012

The Medical Center has not increased its outstanding long-term debt obligations since November 30, 2012.

In connection with various financing agreements, the Medical Center is required to maintain certain financial covenants. The Medical Center was not in compliance with the current ratio covenant of 1.50:1 related to its FHA mortgage loans at December 31, 2010 and 2011. The Medical Center notified the U.S. Department of Housing and Urban Development of the non-compliance; this non-compliance does not constitute an event of default under the terms of the related agreements.

Interest Rate Swaps

Currently, the Medical Center has five interest rate swap agreements to manage risk associated with changes in interest rates associated with its long-term debt as follows (as of November 30, 2012):

<u>Counterparty</u>	<u>Notional Amount</u>	<u>Maturity Date</u>	<u>Fixed Payment Rate</u>	<u>2012 Fair Value (All Liabilities)</u>
Bank of America, N.A.	\$ 26,531,128	6/01/27	3.95%	(\$ 5,327,330)
Israel Discount Bank of New York	18,919,630	6/03/19	1.186%	(389,569)
Bank of America, N.A.	7,959,338	6/01/19	1.28%	(168,944)
Bank of America, N.A.	1,066,666	1/31/14	6.19%	(31,408)
Bank of America, N.A.	687,500	8/15/15	5.985%	(40,599)
	<u>\$ 55,164,262</u>			<u>(\$ 5,957,850)</u>

Lines of Credit

The Medical Center entered into an unsecured revolving line of credit with a syndicate of three banks on June 1, 2012 in the aggregate amount of \$30,000,000 at a rate of 1-month LIBOR plus 1.50%. As of November 30, 2012, \$17,976,000 was outstanding on this revolving line of credit. It is anticipated that this revolving line of credit will be utilized by the Medical Center for interim financing of capital expenditures and working capital needs.

As of November 30, 2012, the Medical Center also had an unsecured letter of credit relating to the Captive Insurance Program in an amount of \$1 million at a rate of 1% which remains undrawn.

Pension Plans and Post-Retirement Health Care Benefits

The following set forth the funded status of the Medical Center's pension benefits and post-retirement health care benefits as of December 31, 2009, 2010 and 2011 (in thousands):

	<u>Pension Benefits</u>			<u>Post-Retirement Health Care Benefits</u>		
	<u>2009</u>	<u>2010</u>	<u>2011</u>	<u>2009</u>	<u>2010</u>	<u>2011</u>
<u>Benefit Obligation</u>						
Benefit obligation, beginning of year	\$73,412	\$84,644	\$98,873	\$8,107	\$10,540	\$12,279
Costs, actuarial losses and other adjustments	12,578	15,585	25,148	2,862	2,211	653
Net benefits paid and expected expenses	<u>(1,346)</u>	<u>(1,356)</u>	<u>(3,255)</u>	<u>(429)</u>	<u>(472)</u>	<u>(597)</u>
Benefit obligation, end of year	84,644	98,873	120,766	10,540	12,279	12,335
<u>Change in Plan Assets</u>						
Fair value of plan assets, beginning of year	\$43,577	\$53,243	\$64,339	\$ --	\$ --	\$ ---
Actual return and contributions	11,012	12,452	12,319	429	472	597
Benefits paid and actual expenses	<u>(1,346)</u>	<u>(1,356)</u>	<u>(3,302)</u>	<u>(429)</u>	<u>(472)</u>	<u>(597)</u>
Fair value of plan assets, end of year	<u>53,243</u>	<u>64,339</u>	<u>73,356</u>	<u>--</u>	<u>--</u>	<u>--</u>
<u>Unfunded Status</u>	<u>(\$31,401)</u>	<u>(\$34,534)</u>	<u>(\$47,410)</u>	<u>(\$10,540)</u>	<u>(\$12,279)</u>	<u>(\$12,335)</u>

At December 31, 2011, the unfunded status of (i) the pension benefits are all noncurrent accrued benefit liabilities and (ii) 95% of the post-retirement health care benefits are noncurrent accrued benefit liabilities.

The Medical Center participates in two multiemployer defined benefit pension plans. The risks of participating in a multiemployer plan are (i) that assets contributed to a plan by one employer may be used to provide benefits to employees of other participating employers, (ii) if a participating employer stops contributing to a plan, the unfunded obligations of the plan may be borne by the remaining participating employers and (iii) if the Medical Center chooses to stop participating in a plan, the Medical Center may be required to pay the plan an amount based on the unfunded status of the plan, referred to as a withdrawal liability. The Medical Center was listed by the New York State Nurses Association Pension Plan as providing more than 5% of the total contributions of the plan for the plan years ended December 31, 2010 and 2011.

For additional information on the Medical Center's pension plans and post-retirement health care benefits, see Note 6 of the Audited Financial Statements of the Medical Center for the years ended December 31, 2011 and 2010 included in Appendix B.

Sources of Patient Service Revenue

The major portion of revenue received by the Medical Center is derived from third party payors. Maimonides is a provider under the Medicare and Medicaid programs, and receives payment from Empire BlueCross and BlueShield (“Blue Cross”), other commercial insurance companies and various managed care plans. The following table shows the approximate percentage distribution of total patient discharges (excluding psychiatry) by payor source for the years ended December 31, 2009, 2010 and 2011, and the eleven-month period ended November 30, 2012.

Patient Discharges by Payor Source *

	<u>Year Ended December 31,</u>			<u>Eleven-Month Period Ended November 30, 2012</u>
	<u>2009</u>	<u>2010</u>	<u>2011</u>	
Medicaid HMO	36.5%	38.1%	40.4%	43.2%
Medicare	24.7	25.0	23.9	23.0
Medicare HMO	8.7	8.5	8.2	8.0
Other Managed Care	8.3	7.6	7.6	7.6
Blue Cross	8.3	7.7	7.9	7.0
Medicaid	8.3	8.0	7.6	6.1
PPO	2.1	2.0	2.0	2.0
Self Pay	1.4	1.2	1.1	1.6
Commercial	1.4	1.6	1.1	1.2
Other	0.3	0.3	0.2	0.3
TOTAL	100.0%	100.0%	100.0%	100.0%

* Excludes Psychiatry

Revenue from the Medicare and Medicaid programs, on a combined basis, accounted for a significant portion of the Medical Center’s net patient service revenue – approximately 72% in 2009, and approximately 75% in 2010 and 2011.

All revenue statistics and reimbursement information in this Official Statement represent historical data and may not be indicative of future activity. The Medical Center cannot assess or predict the ultimate effect on its operations that may result from existing or future reimbursement legislation or regulations.

Patient Insurers/Managed Care

Maimonides has responded to the changing health care delivery system in New York State by entering into contractual agreements with the major managed care organizations in New York. Managed care cases for 2011 comprise approximately 56% of the Medical Center’s inpatient discharges (excluding psychiatry discharges).

The following eleven (11) organizations insure the vast majority of the patients treated by the Medical Center: Aetna Health Plans, Affinity Health Plan, Amerigroup (including the newly acquired HealthPlus), EmblemHealth (including Group Health Inc. (GHI) and Health Insurance Plan of New York (HIP)), 1199SEIU, Empire BlueCross BlueShield, Fidelis Care, Healthfirst, MagnaCare, MultiPlan and United HealthCare (including Oxford Health Plans, United

Community Plan (formerly Americhoice) and United HealthCare). The Medical Center is an owner hospital of Healthfirst (see “Organization and Affiliates” above).

The Medical Center has experienced growth in managed care inpatient and outpatient utilization over the past several years. To date, it has approximately 20 contracts in place with managed care organizations. The growth in the number of managed care contracts is attributed primarily to the growing number of employer groups opting for some level of health insurance for their employees, the increasing number of Medical Center physicians participating in managed care plans, and a shift from fee for service Medicaid and Medicare to managed care plans.

The Medical Center is actively pursuing innovations in comprehensive patient care management strategies in support of CMS’s three primary goals – better care for individuals, better health for populations and reducing per-capita costs. Using Healthfirst, a full risk financial model, the patient centered medical home (PCMH), the Southwest Brooklyn Health Home and the Brooklyn Health Information Exchange (BHIX), Maimonides is actively engaged with its medical staff and community partners to develop care management strategies to increase the effectiveness and efficacy of health care to the community.

Insurance

The Medical Center maintains comprehensive all-risk form property insurance as well as general liability and professional liability coverage, and workers’ compensation insurance. Property insurance is purchased from commercial carriers and is subject to deductibles. Workers’ compensation insurance is purchased from commercial carriers for statutory limits.

General liability and professional liability insurance is purchased from an insurance program for institutions that are affiliated with Federation of Jewish Philanthropies of New York (the “FOJP Program”). The FOJP Program is a pooled professional and general liability program with certain other health care facilities (primarily Beth Israel Medical Center, Montefiore Medical Center and Mount Sinai Medical Center) that has been approved by the New York State Department of Health. The FOJP Program is underwritten by many insurance carriers which include both commercial foreign and domestic insurance carriers and captive insurance carriers utilizing occurrence based type coverage. The Medical Center retains ownership in three captive insurance companies affiliated with the FOJP Program (the “Captive Insurers”).

The Medical Center, as an owner of the Captive Insurers, guarantees a certain level of investment return. The Medical Center was required to fund its share of market losses (\$10.4 million for 2011) over a period of four years beginning in 2012. In 2011, the Medical Center increased its ownership percentage of the Captive Insurers as one of the member hospitals dropped out of the FOJP Program, increasing the Medical Center’s equity in the Captive Insurers by \$9.7 million and its liabilities by \$8.5 million. It is not anticipated that any other member hospitals will drop out of the FOJP Program. For additional information on the FOJP Program and professional liability coverage, see Note 7 of the audited financial statements of the Medical Center for the years ended December 31, 2011 and 2010 included in Appendix B.

In 1998, a portion of the Medical Center’s professional liability insurance was self-retained. At December 31, 2011, the financial statements reflected professional liabilities of \$4.9

million for the actuarial present value of the self-retained component of professional liability insurance based on a study proposed by independent consulting actuaries discounted at 5.0%.

In addition, all non-Medical Center medical staff members with admitting privileges are required to maintain professional liability insurance coverage in amounts not less than \$2.3 million per occurrence and \$6.9 million in the aggregate.

Material Pending Audits

As of January 31, 2013, the Medical Center was the subject of audits under the Medicare Recovery Audit Contractor (“RAC”) program relating to \$35,857,782 of original payments, the bulk of which related to short-term stay cases in 2009 that Performant Recovery, Inc., as the RAC contractor (the “RAC Contractor”), believes should have been reimbursed as outpatient rather than inpatient services. The Medical Center is making a concerted effort to screen short-term stays to minimize future reimbursement disputes.

The RAC Contractor has not made any determination whether or not there have been overpayments with respect to \$6,181 of the original payments and the RAC Contractor has identified no issues with respect to \$19,958,652 of the original payments. Of those payments originally identified by the RAC Contractor as overpayments, the Medical Center is planning not to dispute \$3,668,866 in overpayments, and the RAC Contractor has overturned or rescinded \$1,481,631 in overpayments. In summary, the RAC Contractor has either identified no issues, or overturned or rescinded overpayments, in an aggregate amount of \$21,440,283 constituting 59.8% of the original payments subject to audit.

The Medical Center’s Case Management Team and the Chief Medical Officer believe that the remaining \$10,742,452 in originally identified overpayments will be decided upon appeal in favor of the Medical Center. The Medical Center has hired outside firms to assist in the appeal process of the RAC audits.

Generally, RAC audits have been expanding nationwide for all health care institutions and are expected to continue to increase due to the enactment of the Affordable Care Act. See “RISK FACTORS AND REGULATORY PROVISIONS WHICH MAY AFFECT THE MEDICAL CENTER OR THE SERIES 2013 BONDS—Health Care Reform—Fraud and Abuse” herein.

Compliance

Maimonides maintains a corporate compliance program that is designed to assist the Board and staff to meet or exceed applicable standards established by federal and state laws and regulations. In this regard, the Medical Center has adopted a code of conduct and compliance program guidelines.

The Compliance Oversight Committee oversees all compliance activities at the Medical Center. It is chaired by the Chief Compliance Officer and includes the President and Chief Executive Officer, the Chief Operating Officer, the Chief Financial Officer, the Chief Medical Officer, the Chief of Staff, the Chief Nursing Officer, the Chief Information Officer, the Executive Vice President for Clinical Affairs and Affiliations, the Senior Vice Presidents for Legal Affairs, Professional Affairs, Physician Enterprises and Human Resources, and the Vice

President for Internal Audit. In addition, there is a Compliance Committee that reviews routine compliance activities on an ongoing basis. The Chief Compliance Officer reports directly to the Audit and Legal Committee of the Board and the Chief Executive Officer of the Medical Center, and reports periodically to the full Board.

The Code of Conduct establishes standards of behavior and provides guidance on legal, regulatory and ethical requirements with respect to, among other matters (i) patient care and rights, (ii) emergency care, (iii) patient privacy and confidentiality, (iv) coding and billing for services, (v) patient referrals/fraud and abuse, (vi) relationships and selection of vendors, (vii) gifts and entertainment, (viii) conflicts of interest, (ix) books and records, (x) corporate confidential information, (xi) software and intellectual information, (xii) electronic media, (xiii) sexual harassment/discrimination, (xiv) workplace violence, (xv) prescription drugs/controlled substances, (xvi) health and safety, (xvii) substance and alcohol abuse, (xviii) personal use of company assets, (xix) marketing and advertising, and (xx) political activities.

All employees are trained in the Medical Center's compliance policies and attest to their agreement to abide by the Fraud and Abuse Policies of the Medical Center. The Chief Compliance Officer personally attends all new employee orientation sessions to discuss the code of conduct and the compliance program guidelines and, on an annual basis, all employees must complete a mandatory training program which includes a compliance module. Medical Center physicians are required to take specialized training in compliance which, among other things, focuses on coding, and anti-kickback and Stark regulations.

The Medical Center routinely checks all employees, medical staff, Board members, students, volunteers and vendors against the federal and the New York State sanction lists. In addition to a robust internal audit function, the Compliance Department has specially trained auditors who review the billing practices of both the Medical Center and its physicians.

The Medical Center has not experienced any significant fines, penalties or other enforcement actions with respect to its legal, regulatory or ethical compliance.

Litigation

The Medical Center has no litigation, including medical malpractice litigation, or proceedings pending or, to its knowledge, threatened against it except: (i) litigation being defended by insurance companies on behalf of the Medical Center, the probable recoveries in which and the estimated costs and expenses of defense of which, in the opinion of counsel to the Medical Center for such matters, will be entirely within the Medical Center's applicable insurance policy limits (subject to applicable deductibles); (ii) litigation, the probable recoveries in which and the estimated costs and expenses of defense of which, in the opinion of management of the Medical Center, will not materially and adversely affect the Medical Center's operations or financial condition; and (iii) litigation, the probable recoveries in which and the estimated costs and expenses of defense of which after exhaustion of available insurance proceeds, if any, in the opinion of management of the Medical Center, will not materially and adversely affect the Medical Center's operations or financial conditions.

Capital Expenditures/Future Plans

The table below sets forth the project name, status and estimated project cost for the projects included in the Medical Center's capital budget for 2012:

<u>Project</u>	<u>CON Status</u>	<u>Estimated Project Cost</u>
Information technology	N/A	\$10,000,000
Various other equipment	N/A	7,050,000
Lobby renovations (main hospital lobby and Community Mental Health Center)	Approved	7,000,000
Bay Parkway ambulatory surgery/endoscopy site	N/A	6,500,000
Hybrid operating room	Approved	6,350,000
Various other construction projects	N/A	4,600,000
Electrophysiology system labs	Approved	3,300,000
Cancer Center Infusion Center	Pending	<u>1,300,000</u>
Total Project Costs		\$46,100,000

The bulk of these projects are expected to be completed by the end of the second quarter of 2013. Funding has been obtained through a taxable equipment financing in an amount of \$30,836,685 closed in August 2012 and a portion of the loan proceeds from a syndicated bank credit facility closed in June 2012 as well as cash reserves from the Medical Center (see "Discussion of Outstanding Indebtedness – Long-Term Debt" above).

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APPENDIX B

**AUDITED FINANCIAL STATEMENTS
OF THE MEDICAL CENTER AND AUDITOR'S REPORT
AS OF AND FOR THE YEARS ENDED DECEMBER 31, 2011 AND 2010**

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FINANCIAL STATEMENTS

Maimonides Medical Center
Years Ended December 31, 2011 and 2010
With Report of Independent Auditors

Ernst & Young LLP

 **ERNST & YOUNG**

Maimonides Medical Center

Financial Statements

Years Ended December 31, 2011 and 2010

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Report of Independent Auditors

The Board of Trustees
Maimonides Medical Center

We have audited the accompanying statements of financial position of Maimonides Medical Center (the “Medical Center”) as of December 31, 2011 and 2010, and the related statements of operations and changes in net assets and cash flows for the years then ended. These financial statements are the responsibility of the Medical Center’s management. Our responsibility is to express an opinion on these financial statements based on our audits.

We conducted our audits in accordance with auditing standards generally accepted in the United States. Those standards require that we plan and perform the audit to obtain reasonable assurance about whether the financial statements are free of material misstatement. We were not engaged to perform an audit of the Medical Center’s internal control over financial reporting. Our audits included consideration of internal control over financial reporting as a basis for designing audit procedures that are appropriate in the circumstances, but not for the purpose of expressing an opinion on the effectiveness of the Medical Center’s internal control over financial reporting. Accordingly, we express no such opinion. An audit also includes examining, on a test basis, evidence supporting the amounts and disclosures in the financial statements, assessing the accounting principles used and significant estimates made by management, and evaluating the overall financial statement presentation. We believe that our audits provide a reasonable basis for our opinion.

In our opinion, the financial statements referred to above present fairly, in all material respects, the financial position of Maimonides Medical Center at December 31, 2011 and 2010, and the results of its operations and changes in net assets and its cash flows for the years then ended in conformity with accounting principles generally accepted in the United States.

As discussed in Notes 1 and 7 to the accompanying financial statements, in 2011 the Medical Center changed its method of accounting and reporting for estimated insurance claims receivable and estimated insurance claims liabilities with the adoption of Accounting Standards Update No. 2010-24, *Presentation of Insurance Claims and Related Insurance Recoveries*.

Ernst & Young LLP

April 30, 2012

Maimonides Medical Center

Statements of Financial Position

	December 31	
	2011	2010
	<i>(In Thousands)</i>	
Assets		
Current assets:		
Cash and cash equivalents	\$ 49,188	\$ 43,996
Short-term investments <i>(Notes 1 and 3)</i>	93,294	91,487
Total cash, cash equivalents and short-term investments	142,482	135,483
Assets limited as to use, current portion <i>(Notes 3 and 5)</i> :		
Board designated	13,757	13,908
Receivables for patient care, net of allowance for doubtful accounts (2011 – \$83,819; 2010 – \$76,844) <i>(Note 2)</i>	131,916	119,693
Due from affiliates, net, current portion <i>(Note 8)</i>	7,300	4,919
Supplies, at average cost	8,851	9,948
Estimated insurance claims receivable, current portion <i>(Notes 1 and 7)</i>	28,000	28,205
Other current assets <i>(Note 1)</i>	45,866	39,443
Total current assets	378,172	351,599
Assets limited as to use, less current portion <i>(Notes 3, 5 and 7)</i> :		
Sinking funds	12,883	10,944
Collateral	2,630	2,630
Self-insurance trust	3,917	3,958
Long-term investment funds <i>(Notes 1 and 3)</i>	1,102	1,384
Due from affiliates, net, less current portion <i>(Note 8)</i>	2,607	2,462
Property, plant and equipment, net <i>(Notes 4 and 5)</i>	340,954	331,664
Equity in captive insurance program <i>(Note 7)</i>	52,707	47,256
Estimated insurance claims receivable, less current portion <i>(Notes 1 and 7)</i>	158,668	159,826
Other assets <i>(Note 1)</i>	16,534	35,656
Total assets	\$ 970,174	\$ 947,379

	December 31	
	2011	2010
	<i>(In Thousands)</i>	
Liabilities and net assets		
Current liabilities:		
Accounts payable and accrued expenses	\$ 54,948	\$ 56,076
Construction payable	7,291	8,280
Accrued salaries and related liabilities	74,152	74,089
Current portion of long-term debt and capital lease obligations <i>(Note 5)</i>	21,181	21,049
Line of credit borrowings <i>(Note 5)</i>	18,408	16,408
Estimated insurance claims liabilities, current portion <i>(Notes 1 and 7)</i>	28,000	28,205
Other current liabilities <i>(Notes 1 and 7)</i>	65,260	43,754
Total current liabilities	269,240	247,861
Long-term debt and capital lease obligations, less current portion <i>(Note 5)</i>	158,532	174,114
Accrued pension and postretirement benefits <i>(Note 6)</i>	59,170	46,347
Other noncurrent liabilities <i>(Notes 2 and 5)</i>	104,430	109,121
Professional liabilities <i>(Note 7)</i>	41,912	25,144
Estimated insurance claims liabilities, less current portion <i>(Notes 1 and 7)</i>	158,668	159,826
Total liabilities	791,952	762,413
Commitments and contingencies <i>(Notes 2, 5, 6, 7, 9, 14 and 15)</i>		
Net assets <i>(Note 9)</i> :		
Unrestricted	162,652	169,218
Temporarily restricted	14,438	14,616
Permanently restricted	1,132	1,132
Total net assets	178,222	184,966
Total liabilities and net assets	\$ 970,174	\$ 947,379

See accompanying notes.

Maimonides Medical Center

Statements of Operations and Changes in Net Assets

	Year Ended December 31	
	2011	2010
	<i>(In Thousands)</i>	
Operating revenue:		
Net patient service revenue <i>(Note 2)</i>	\$ 942,798	\$ 950,578
Other revenue <i>(Note 10)</i>	48,654	44,341
Net assets released from restrictions	1,765	2,337
Total operating revenue	993,217	997,256
Operating expenses:		
Salaries and wages	476,048	472,167
Employee benefits	142,293	133,241
Supplies and other expenses	311,638	310,895
Depreciation and amortization	46,098	41,507
Interest and amortization of deferred financing costs	10,608	11,471
Total operating expenses	986,685	969,281
Excess of operating revenue over operating expenses before other items	6,532	27,975
Change in captive insurance program interest rate shortfall <i>(Note 7)</i>	(10,383)	5,249
Change in equity in captive insurance program <i>(Note 7)</i>	(1,727)	5,074
Medical residents FICA refund <i>(Note 1)</i>	—	10,723
(Deficiency) excess of revenue over expenses before net change in unrealized gains and losses on investments and change in fair value of interest rate swap agreements	(5,578)	49,021
Net change in unrealized gains and losses on investments and change in fair value of interest rate swap agreements <i>(Note 5)</i>	1,248	169
(Deficiency) excess of revenue over expenses	(4,330)	49,190
Change in accrued pension and postretirement benefits liabilities to be recognized in future periods	(11,099)	(2,659)
Grants for capital asset acquisitions and net assets released from restrictions used for capital asset acquisitions	8,863	140
(Decrease) increase in unrestricted net assets	(6,566)	46,671
Temporarily restricted net assets:		
Restricted contributions, grants and other receipts	1,937	2,864
Net assets released from restrictions:		
Operating expenses	(1,765)	(2,337)
Capital asset acquisitions	(350)	(140)
(Decrease) increase in temporarily restricted net assets	(178)	387
(Decrease) increase in net assets	(6,744)	47,058
Net assets at beginning of year	184,966	137,908
Net assets at end of year	\$ 178,222	\$ 184,966

See accompanying notes.

Maimonides Medical Center

Statements of Cash Flows

	Year Ended December 31	
	2011	2010
	<i>(In Thousands)</i>	
Operating activities		
(Decrease) increase in net assets	\$ (6,744)	\$ 47,058
Adjustments to reconcile (decrease) increase in net assets to net cash provided by operating activities:		
Change in accrued pension and postretirement benefits liabilities to be recognized in future periods	11,099	2,659
Depreciation and amortization	46,098	41,507
Amortization of deferred financing costs	359	365
Net change in unrealized gains and losses on investments and change in fair value of interest rate swap agreements	(1,248)	(169)
Restricted contributions, grants and other receipts	(1,937)	(2,864)
Changes in operating assets and liabilities:		
Receivables for patient care	(12,223)	(11,304)
Due from affiliates	(2,526)	(632)
Other assets	(11,276)	(30,481)
Accounts payable and accrued expenses and accrued salaries and related liabilities	(1,065)	10,486
Other current liabilities	21,506	13,480
Other liabilities	11,733	2,797
Net cash provided by operating activities	<u>53,776</u>	<u>72,902</u>
Investing activities		
Redemption of investments	307	3,951
Acquisitions of property, plant and equipment – net	(29,851)	(32,189)
Redemption (purchase) of assets limited as to use	378	(1,203)
Purchase of sinking funds	(1,630)	(1,450)
Net cash used in investing activities	<u>(30,796)</u>	<u>(30,891)</u>
Financing activities		
Repayments of long-term debt and capital lease obligations	(21,725)	(16,994)
Restricted contributions, grants and other receipts	1,937	2,864
Line of credit drawdown (repayments)	2,000	(1,700)
Proceeds from financing	–	153
Net cash used in financing activities	<u>(17,788)</u>	<u>(15,677)</u>
Net increase in cash and cash equivalents	5,192	26,334
Cash and cash equivalents at beginning of year	43,996	17,662
Cash and cash equivalents at end of year	<u>\$ 49,188</u>	<u>\$ 43,996</u>
Supplemental disclosures of cash flow information		
Cash paid for interest	\$ 10,302	\$ 11,123
Capital lease arrangements	\$ 6,275	\$ 32,762

See accompanying notes.

Maimonides Medical Center

Notes to Financial Statements

December 31, 2011

1. Organization and Summary of Significant Accounting Policies

Organization

Maimonides Medical Center (the “Medical Center”) is a not-for-profit membership corporation, organized under the New York State not-for-profit corporation law, whose sole member is Maimonides Health Resources, Inc. (“MHRI”). Located in Brooklyn, New York, the Medical Center provides health care and related services to residents of the metropolitan New York area. The accompanying financial statements do not include the accounts of MHRI, the Maimonides Research and Development Foundation, a not-for-profit corporation which solicits funds and awards grants primarily to the Medical Center for research purposes, or MMC Holding of Brooklyn, Inc., a for-profit company, which provides certain support services to the Medical Center and the surrounding community (see Note 8).

Use of Estimates

The preparation of financial statements in conformity with accounting principles generally accepted in the United States requires management to make estimates and assumptions that affect the reported amounts of assets and liabilities, such as estimated uncollectibles for accounts receivable for services to patients, estimated settlements with third-party payors, estimated insurance claims liabilities and receivables and retirement liabilities, and disclosure of contingent assets and liabilities at the date of the financial statements. Estimates also affect the amounts of revenues and expenses reported during the period. Actual results could differ from those estimates.

Cash and Cash Equivalents

The Medical Center considers all highly liquid investments with a maturity of three months or less at the date of purchase to be cash equivalents. Cash and cash equivalents exclude assets limited as to use.

Investments

Investments consist of cash equivalents, U.S. government obligations, corporate bonds, a mutual fund, a certificate of deposit and alternative investments. Investments, other than alternative investments, are recorded at fair value based on quoted market prices.

Maimonides Medical Center

Notes to Financial Statements (continued)

1. Organization and Summary of Significant Accounting Policies (continued)

Alternative investments (nontraditional, not readily marketable securities) consist of three investment vehicles: an equity fund, a fund of funds and participation in a bank note/certificate receivable investment program. The Medical Center's risk is limited to the carrying amount in each investee company.

The Medical Center's ownership of the equity fund and the fund of funds is carried on the equity method of accounting at net asset value per share. Individual investment holdings within these alternative investments may include investments in both nonmarketable and market-traded securities and interests in other alternative investments. The amounts used to report the Medical Center's alternative investment interests may be determined through a combination of information provided by the investment manager, fair value valuations of underlying securities held by the alternative investment investee company, fair value valuations of other financial instruments held by the alternative investment investee company such as derivative instruments, or other estimates that require varying degrees of judgment. Generally, the value of the Medical Center's interest reflects net contributions to the investee and an ownership share of realized and unrealized investment income and expenses. The Medical Center's holdings of these alternative investment interests may indirectly expose the Medical Center to securities lending, short sales of securities, and trading in futures and forward contracts, options and other derivative products. The financial statements of these alternative investment investee companies are audited annually by independent auditors, although the timing of such audits does not coincide with the Medical Center's year-end financial statement reporting. The Medical Center's ownership structure does not provide the Medical Center with control over the alternative investment investee companies. A portion of the fund of funds is an illiquid asset and, therefore, is classified as long-term at December 31, 2011 and 2010.

The investment in the bank note/certificate receivable investment program is valued by the investee monthly, based on the performance of an underlying insurance fund. The valuation provided monthly to the Medical Center approximates fair value, which was written down to zero at December 31, 2010. Additionally, during 2011, the Medical Center liquidated its investment in the equity fund.

Investment income, which comprises interest and realized gains and losses on investments as well as the change in equity in alternative investments, is reported as a component of other revenue. The net change in unrealized gains and losses on investments is reported as a separate component of the (deficiency) excess of revenue over expenses.

Maimonides Medical Center

Notes to Financial Statements (continued)

1. Organization and Summary of Significant Accounting Policies (continued)

Net Patient Service Revenue and Accounts Receivable

Net patient service revenue is reported at the estimated net realizable amounts from patients, third-party payors, and others for services rendered, including estimated retroactive revenue adjustments due to ongoing and future audits, reviews and investigations. The Medical Center has agreements with third-party payors that provide for payments to the Medical Center at amounts different from its established rates. Payment arrangements include prospectively determined rates per discharge, reimbursed costs, discounted charges and per diem payments. Retroactive adjustments are considered in the recognition of revenue on an estimated basis in the period the related services are rendered and such amounts are adjusted in future periods as adjustments become known or as years are no longer subject to such audits, reviews and investigations.

Patient accounts receivable results from the health care and related services provided by the Medical Center. Additions to the allowance for doubtful accounts result from the provision for bad debts. Accounts written off as uncollectible are deducted from the allowance for doubtful accounts. The amount of the allowance for doubtful accounts is based upon management's assessment of historical and expected net collections, business and economic conditions, trends in Medicare and Medicaid health care coverage and other collection indicators.

Assets Limited as to Use

Assets limited as to use include assets designated by the Board of Trustees, over which the Board retains control and which the Board, at its discretion, may designate for use for other purposes; assets held by trustees under bond indenture agreements; assets restricted for self-insurance; and assets restricted by donors for specific purposes or endowment. Assets limited as to use are carried at fair value based on quoted market prices. Amounts which are available to be used to fund current liabilities are reported as current assets.

Property, Plant and Equipment

Property, plant and equipment are stated at cost, except for donated equipment, which is recorded at the fair value established at the date of the gift. Assets acquired through capitalized lease obligations are recorded at the present value of the future minimum lease payments. The carrying amounts of assets and the related accumulated depreciation are removed from the

Maimonides Medical Center

Notes to Financial Statements (continued)

1. Organization and Summary of Significant Accounting Policies (continued)

accounts when such assets are disposed of, and any resulting gain or loss is included in operations. Depreciation is provided on a straight-line basis over the estimated useful lives of the assets.

Equipment acquired under capital lease obligations is amortized on the straight-line method over the shorter period of the lease term or the estimated useful life of the equipment. Such amortization is included in depreciation and amortization expense in the accompanying statements of operations and changes in net assets. Interest cost incurred on borrowed funds during the period of construction of capital assets is capitalized as a component of the cost of acquiring those assets.

Other Assets

Included in other assets are deferred costs, pledges receivable, other noncurrent investments and funding available to be spent on capital leased fixed assets. Deferred costs are approximately \$4.2 million and \$4.6 million at December 31, 2011 and 2010, respectively. Deferred costs include costs incurred with respect to debt financing and are amortized using the effective interest method over the period the applicable obligation is outstanding. Included in the \$4.2 million of deferred costs at December 31, 2011 are approximately \$0.4 million related to the 1996 FHA Section 242 mortgage loan (see Note 5), \$0.9 million related to a loan from the New York City Capital Resource Corporation (see Note 5), and \$2.9 million related to the 2004 FHA Section 241 mortgage loan (see Note 5). At December 31, 2011 and 2010, the accumulated amortization for deferred costs was approximately \$3.2 million and \$2.8 million, respectively. Amortization of deferred financing costs is included in interest and amortization of deferred financing costs in the accompanying statements of operations and changes in net assets. Pledges receivable are approximately \$708,000 and \$875,000 at December 31, 2011 and 2010, respectively.

Pledges receivable are unconditional promises to give cash, which are reported at the present value of their estimated cash flows at the date the promise is received. Funding available to be spent on capital leased fixed assets was approximately \$1.3 million at December 31, 2011 (\$20.6 million at December 31, 2010).

Maimonides Medical Center

Notes to Financial Statements (continued)

1. Organization and Summary of Significant Accounting Policies (continued)

Equity Investments

Included in other assets is an amount related to the Medical Center's investment in a limited liability company. The Medical Center accounts for this investment using the equity method of accounting. During 2011 and 2010, the Medical Center recognized income of approximately \$3.0 million and \$2.5 million, respectively, and received distributions of approximately \$1.7 million and \$401,000, respectively.

Additionally, the Medical Center has an investment in First to Care Home Care, Inc., a joint venture with Metropolitan Jewish Geriatric Corporation. As a result of its ownership in this joint venture, the Medical Center has recorded a loss of approximately \$1.5 million and \$67,000 in its statements of operations and changes in net assets for the years ended December 31, 2011 and 2010, respectively.

Temporarily and Permanently Restricted Net Assets

The Medical Center separately accounts for and reports upon donor restricted and unrestricted net assets.

Temporarily restricted net assets are those whose use is temporarily limited by the donor. Permanently restricted net assets have been restricted by donors to be maintained by the Medical Center in perpetuity. When a donor restriction expires (that is, when a stipulated time restriction ends or purpose of restriction is accomplished), temporarily restricted net assets are reclassified as unrestricted net assets and reported in the statements of operations and changes in net assets as net assets released from restrictions.

Faculty Practice Revenue

All employed physicians may participate in the Medical Center's faculty practice plan. Plan participants are authorized to conduct faculty practices and engage in professional consultation in accordance with established institutional guidelines. Professional service fee receipts are recorded and deposited in faculty practice funds established by the Medical Center for each individual participant or group practice when received by the Medical Center. Portions of these receipts are used to reimburse the Medical Center for costs incurred in supporting plan activities. The remaining amounts, after direct plan expenses, provide participant salary supplements and support departmental activities. Faculty practice revenue activities are included in net patient

Maimonides Medical Center

Notes to Financial Statements (continued)

1. Organization and Summary of Significant Accounting Policies (continued)

service revenue and are recorded on a modified cash basis, which approximates the accrual basis with respect to the accompanying statements of operations and changes in net assets. Physicians comprising the faculty practices participate in the Medical Center's professional and general liability insurance programs.

Performance Indicator

The statements of operations and changes in net assets include the (deficiency) excess of revenue over expenses as the performance indicator. Certain items that are peripheral to the Medical Center's operations (e.g., change in captive insurance program interest rate shortfall, change in equity in captive insurance program, medical residents FICA refund and net change in unrealized gains and losses on investments and change in fair value of interest rate swap agreements) are considered non-operating for purposes of financial statement presentation. Changes in unrestricted net assets which are excluded from the (deficiency) excess of revenue over expenses include contributions of long-lived assets (including grants for capital asset acquisitions and net assets released from restriction used for capital asset acquisitions) and the change in accrued pension and postretirement benefits liabilities to be recognized in future periods.

Uncompensated Care

For financial reporting purposes, the Medical Center reports as uncompensated care amounts related to care provided for which the patient's payment obligation has not been fully satisfied. Uncompensated care is the sum of the Medical Center's charity care, including free and reduced price medical care, and bad debt expense. During the registration, billing and collection process, a patient's eligibility for charity care is determined. For patients who are determined to be eligible for charity care under the Medical Center's charity care and financial aid policy, care given but not paid for is classified as charity care. The Medical Center amended its charity care and financial aid policy in order to authorize use of additional financial information for uninsured or under-insured patients who have not supplied the requisite information to qualify for charity care. The additional information obtained is used by the Medical Center to determine whether to qualify patients for charity care and/or financial aid in accordance with the Medical Center's policies. For patients who were determined by the Medical Center to have the ability to pay but did not, the uncollected amounts are classified as bad debt expense. Distinguishing between bad debt and charity care is difficult in part because services are often rendered prior to full evaluation of a patient's ability to pay.

Maimonides Medical Center

Notes to Financial Statements (continued)

1. Organization and Summary of Significant Accounting Policies (continued)

The Medical Center's estimated costs for charity care were \$37.0 million for 2011 and \$30.0 million for 2010. This does not include the service loss of treating the Medicaid population. The cost of charity includes the direct and indirect cost of providing charity care services. The cost is estimated by utilizing a ratio of cost to gross charges applied to the gross uncompensated charges associated with providing charity care. Net of pool contributions, funds received from the New York State Indigent Care Pool to offset bad debts and charity care provided totaled \$9.5 million and \$9.4 million for the years ended December 31, 2011 and 2010, respectively. The charity care component of the indigent care pool payments is estimated utilizing a ratio of charity care charges to total charity care and bad debt charges applied to the indigent care pool reimbursement and excludes amounts designated for teaching programs.

Additionally, patients who do not qualify for charity care and all uninsured inpatients who do not qualify for Medicaid assistance are billed at the Medical Center's rates. Uncollected balances for these patients are categorized as bad debts. Total uncompensated care as a result of bad debts for all patient services approximated \$15.0 million in 2011 and \$19.0 million in 2010.

Interest Rate Swap Agreements

The Medical Center uses interest rate swap agreements, which are derivative financial instruments, for interest rate risk exposure-management purposes. The Medical Center recognizes derivative instruments as either an asset or liability in the statements of financial position at fair value. The fair value of derivative instruments is determined using forward interest rate estimates and present value techniques and considers the risk of nonperformance by the parties, which the Medical Center considers to be low. The change in fair value is recognized as a component of the (deficiency) excess of revenue over expenses. The fair value of the interest rate swap agreements is included in other noncurrent liabilities in the accompanying statements of financial position.

Tax Status

The Medical Center is a Section 501(c)(3) organization exempt from Federal income taxes under Section 501(a) of the Internal Revenue Code. It also is exempt from New York State and City income taxes.

Maimonides Medical Center

Notes to Financial Statements (continued)

1. Organization and Summary of Significant Accounting Policies (continued)

New Accounting Pronouncements

In August 2010, the Financial Accounting Standards Board (“FASB”) issued Accounting Standards Update No. (“ASU”) 2010-23, *Measuring Charity Care for Disclosure*. The guidance requires that the level of charity care provided be presented based on the direct and indirect costs of the charity services provided. Separate disclosure of the amount of any cash reimbursements received for providing charity care must also be disclosed. The new disclosure requirements became effective for the Medical Center on January 1, 2011 and are included in the accompanying financial statements for all periods presented.

In September 2011, the FASB issued ASU 2011-09, *Disclosures about an Employer’s Participation in a Multiemployer Plan*, that requires employers to provide additional quantitative and qualitative disclosures about an employer’s involvement in multiemployer pension plans. The new disclosure requirements are included in the accompanying financial statements (see Note 6).

In July 2011, the FASB issued ASU 2011-07, *Presentation and Disclosure of Patient Service Revenue, Provision for Bad Debts, and the Allowance for Doubtful Accounts for Certain Health Care Entities*. The new guidance will require the Medical Center to change the presentation of its statement of operations and changes in net assets by reclassifying the provision for bad debts associated with patient service revenue (net of contractual allowances and discounts). Additionally, the Medical Center is required to provide enhanced disclosures about its policies for recognizing revenue and assessing bad debts, as well as qualitative and quantitative information about changes in the allowance for doubtful accounts. The new guidance is required to be adopted by the Medical Center as of January 1, 2012. The adoption of this guidance is not expected to affect the Medical Center’s financial condition, results of operations or cash flows.

Change in Accounting Principle

In August 2010, the FASB issued ASU 2010-24, *Presentation of Insurance Claims and Related Insurance Recoveries*, which provides clarification to companies in the health care industry on the accounting for and presentation of professional and similar contingent liabilities. Under the guidance, these liabilities should not be presented net of insurance recoveries and an insurance recovery receivable should be recognized on the same basis as the liabilities, subject to the need

Maimonides Medical Center

Notes to Financial Statements (continued)

1. Organization and Summary of Significant Accounting Policies (continued)

for a valuation allowance for uncollectible accounts. The guidance became effective for the Medical Center as of January 1, 2011. The change resulted in an increase to current assets and liabilities of approximately \$28.0 million and \$28.2 million as of December 31, 2011 and 2010, respectively, and an increase to long-term assets and liabilities of approximately \$158.7 million and \$159.8 million as of December 31, 2011 and 2010, respectively. As permitted, the Medical Center's statement of financial position for 2010 has been restated to reflect this accounting change, which had no effect on the Medical Center's net assets or on its statement of operations and changes in net assets.

Medical Residents FICA Refund

In March 2010, the Internal Revenue Service ("IRS") announced that, for periods ending before April 1, 2005, medical residents would be eligible for the student exception of Federal Insurance Contributions Act ("FICA") taxes. Under the student exception, FICA taxes do not apply to wages for services performed by students employed by a school, college, or university where the student is pursuing a course of study. As a result, the IRS will allow refunds for institutions that file timely FICA refund claims and provide certain information to meet the requirements of perfection, established by the IRS, for their claims applicable to periods prior to April 1, 2005. Institutions are potentially eligible for medical resident FICA refunds for both the employer and employee portions of FICA taxes paid, plus statutory interest.

In the fourth quarter of 2010, the Medical Center recorded estimated net revenue of approximately \$10.7 million related to FICA medical resident refund claims and accumulated interest that are expected to meet the IRS requirements to be eligible for refunds. At December 31, 2010, the Medical Center recorded a receivable of approximately \$13.2 million, included in other current assets (in addition to the \$10.7 million of net revenue recognized), and a liability of approximately \$13.2 million, included in other current liabilities, related to the portion of the refunds to be collected on behalf of and, therefore, to be remitted to the medical residents. The Medical Center established these estimates based on information presently available; the estimates are subject to change as the IRS adjudicates the claims.

Reclassifications

Certain reclassifications have been made to amounts previously reported in 2010 to conform with the current year's presentation.

Maimonides Medical Center

Notes to Financial Statements (continued)

1. Organization and Summary of Significant Accounting Policies (continued)

Subsequent Events

Generally accepted accounting principles establish standards of accounting and disclosure requirements for subsequent events, events that occur after the balance sheet date but before the financial statements are issued. In accordance with these requirements, the Medical Center has evaluated the impact of subsequent events through April 30, 2012, representing the date on which the financial statements were issued.

2. Third-Party Payor System

Non-Medicare Reimbursement: In New York State, hospitals and all non-Medicare payers, except Medicaid, workers' compensation and no-fault insurance programs, negotiate hospitals' payment rates. If negotiated rates are not established, payers are billed at hospitals' established charges. Medicaid, workers' compensation and no-fault payers pay hospital rates promulgated by the New York State Department of Health. Effective December 1, 2009, the New York State payment methodology was updated such that payments to hospitals for Medicaid, workers' compensation and no-fault inpatient services are based on a statewide prospective payment system, with retroactive adjustments; prior to December 1, 2009, the payment system provided for retroactive adjustments to payment rates, using a prospective payment formula. Outpatient services also are paid based on a statewide prospective system, APGs (Ambulatory Payment Group), that was effective December 1, 2008. Medicaid rate methodologies are subject to approval at the Federal level by the Centers for Medicare and Medicaid Services ("CMS"), which may routinely request information about such methodologies prior to approval. Revenue related to specific rate components that have not been approved by CMS is not recognized until the Medical Center is reasonably assured that such amounts are realizable. Adjustments to the current and prior years' payment rates for those payers will continue to be made in future years.

Medicare Reimbursement: Hospitals are paid for most Medicare inpatient and outpatient services under the national prospective payment system and other methodologies of the Medicare program for certain other services. Federal regulations provide for certain adjustments to current and prior years' payment rates, based on industry-wide and hospital-specific data.

The Medical Center has established estimates, based on information presently available, of amounts due to or from Medicare and non-Medicare payers for adjustments to current and prior years' payment rates, based on industry-wide and Medical Center-specific data. Medicare cost reports,

Maimonides Medical Center

Notes to Financial Statements (continued)

2. Third-Party Payor System (continued)

which serve as the basis for final settlement with the Medicare program, have been audited by the Medicare fiscal intermediary and settled through 2004. Other years remain open for audit and settlement as are numerous issues related to the New York State Medicaid program for prior years. As a result, there is at least a reasonable possibility that recorded estimates will change by a material amount when open years are settled and additional information is obtained.

There are various proposals at the Federal and State levels, including health care reform enacted by the Federal and State governments, that could, among other things, significantly reduce payment rates or modify payment methods. The ultimate outcome of these proposals and other market changes cannot presently be determined. Future changes in the Medicare and Medicaid programs and any reduction of funding could have an adverse impact on the Medical Center. Additionally, certain payers' payment rates for various years have been appealed by the Medical Center. If the appeals are successful, additional income applicable to those years might be realized.

For the years ended December 31, 2011 and 2010, revenue from the Medicare and Medicaid programs on a combined basis accounted for approximately 75% of the Medical Center's net patient service revenue. In 2010, net patient service revenue includes increased revenue from Medicaid outliers, which were included in the Medical Center's Medicaid reimbursement rates as a result of the Medical Center's appeal to the New York State Department of Health. The current Medicaid, Medicare and other third-party payor programs are based upon extremely complex laws and regulations that are subject to interpretation. The Medical Center is not aware of any allegations of noncompliance that could have a material adverse effect on the accompanying financial statements and believes that it is in compliance, in all material respects, with all applicable laws and regulations. Action for noncompliance could result in repayment of amounts improperly reimbursed, fines, penalties and exclusion from such programs.

Maimonides Medical Center

Notes to Financial Statements (continued)

3. Investments and Assets Limited as to Use

A summary of investments and assets limited as to use is as follows:

	December 31	
	2011	2010
	<i>(In Thousands)</i>	
Short-term investments:		
U.S. government obligations	\$ 66,936	\$ 62,481
Corporate bonds	25,647	24,769
Alternative investments	12	3,535
Accrued interest	699	702
Total short-term investments	<u>\$ 93,294</u>	<u>\$ 91,487</u>
Assets limited as to use – current portion:		
Board designated:		
Cash and cash equivalents	\$ 6,156	\$ 6,759
U.S. government obligations	7,504	7,044
Accrued interest	97	105
Total assets limited as to use – current portion	<u>\$ 13,757</u>	<u>\$ 13,908</u>
Assets limited as to use – noncurrent portion:		
Sinking funds:		
Cash and cash equivalents	\$ 381	\$ 676
U.S. government obligations	12,419	10,195
Accrued interest	83	73
Total sinking funds	<u>12,883</u>	<u>10,944</u>
Collateral for standby letter of credit (<i>Note 5</i>):		
Certificate of deposit	2,630	2,630
Self-insurance trust:		
Mutual fund	3,917	3,958
Total assets limited as to use – noncurrent portion	<u>\$ 19,430</u>	<u>\$ 17,532</u>
Long-term investment funds:		
Alternative investments	<u>\$ 1,102</u>	<u>\$ 1,384</u>

Maimonides Medical Center

Notes to Financial Statements (continued)

3. Investments and Assets Limited as to Use (continued)

Investment income comprises the following for the years ended:

	December 31	
	2011	2010
	<i>(In Thousands)</i>	
Interest income	\$ 2,696	\$ 2,849
Net realized gains (losses) on investments reported at fair value and equity in earnings of alternative investments	5,659	(2,972)
Equity in earnings of investees	1,480	2,385
	<u>\$ 9,835</u>	<u>\$ 2,262</u>

4. Property, Plant and Equipment

A summary of property, plant and equipment is as follows:

	December 31	
	2011	2010
	<i>(In Thousands)</i>	
Land	\$ 4,980	\$ 4,980
Buildings and building improvements	393,396	371,227
Equipment	516,789	470,986
	<u>915,165</u>	<u>847,193</u>
Less accumulated depreciation and amortization	576,564	530,466
	<u>338,601</u>	<u>316,727</u>
Construction in progress	2,353	14,937
	<u>\$ 340,954</u>	<u>\$ 331,664</u>

Substantially all property, plant and equipment are pledged as collateral under various loan agreements (see Note 5).

Maimonides Medical Center

Notes to Financial Statements (continued)

4. Property, Plant and Equipment (continued)

Capitalized leases, included in property, plant and equipment, are as follows:

	December 31	
	2011	2010
	<i>(In Thousands)</i>	
Assets recorded under capital leases	\$ 274,574	\$ 243,278
Less accumulated amortization	198,894	184,217
	<u>\$ 75,680</u>	<u>\$ 59,061</u>

5. Long-Term Debt, Capital Lease Obligations, Other Borrowings and Related Matters

A summary of long-term debt is as follows:

	December 31	
	2011	2010
	<i>(In Thousands)</i>	
1996 FHA Section 242 insured mortgage loan (a)	\$ 10,669	\$ 11,325
2004 FHA Section 241 insured mortgage loan (b)	94,041	96,520
Construction bank loans (c)	2,822	3,986
Capital lease obligations (d)	45,836	55,902
2006 New York City Capital Resource Corporation loan (e)	26,345	27,430
	<u>179,713</u>	<u>195,163</u>
Less current portion	21,181	21,049
	<u>\$ 158,532</u>	<u>\$ 174,114</u>

- (a) In 1977 and 1983, the Medical Center entered into mortgage loan agreements to expand its facilities. Repayment of the loans was insured under the provisions of the Federal Housing Administration ("FHA") Hospital Mortgage Loan Program, with the Dormitory Authority of the State of New York ("DASNY"). In April 1996, the Medical Center entered into an FHA Section 242 mortgage loan agreement to borrow \$32.7 million. Some of the proceeds from this agreement (\$15.8 million) were used to refinance the 1977 mortgage loan agreement. Under the terms of the agreement, for this element of the loan, monthly installments approximating \$210,000 were applied to principal and interest, at 5.75% per annum, through August 2005. This portion of the mortgage was satisfied in 2005.

Maimonides Medical Center

Notes to Financial Statements (continued)

5. Long-Term Debt, Capital Lease Obligations, Other Borrowings and Related Matters (continued)

Other proceeds from the 1996 mortgage loan (\$16.8 million) were used to finance the expansion and modernization of the Medical Center's facilities. For this element of the loan, monthly payments of approximately \$115,000 are applied to principal and interest, at 6.55% per annum, through October 2022.

As a condition of these borrowings, the Medical Center is required to establish and maintain a sinking fund. Amounts deposited into the sinking fund, together with investment earnings thereon, are available for principal payments and purchases of specified levels of capital assets. Assets on deposit in the sinking fund at December 31, 2011 and 2010 are in compliance with the required amounts.

- (b) In June 2004, the Medical Center entered into an FHA Section 241 loan in the amount of \$104.7 million. Proceeds of the loan, which were raised by DASNY through the issuance of tax-exempt revenue bonds, supported the Medical Center's major modernization project. The loan is insured by FHA. The loan bore interest at 7.5% during the construction period (based on amounts drawn) until final endorsement and thereafter at a rate of 5.45% with monthly payments commencing April 1, 2007 through 2032. This project was completed in 2009. Although final endorsement is expected to occur in 2012, effective November 2009, DASNY approved the monthly interest rate of 5.45%.

Under the terms of the agreements, the Medical Center was required to maintain three standby letters of credit. The first letter of credit was cancelled in 2009, based upon the approval of DASNY. The second letter of credit has a current availability of \$2.6 million and has been extended to February 2, 2013. The third letter of credit in the amount of \$2.2 million expires on February 1, 2016, and is fully collateralized.

As a condition of this borrowing, the Medical Center is required to establish and maintain a sinking fund. Amounts deposited into the sinking fund, together with investment earnings thereon, are available for principal payments. Assets on deposit in the sinking fund at December 31, 2011 and 2010 are in compliance with the required amounts.

Maimonides Medical Center

Notes to Financial Statements (continued)

5. Long-Term Debt, Capital Lease Obligations, Other Borrowings and Related Matters (continued)

- (c) The Medical Center entered into a ten-year, \$8.0 million construction bank loan in February 2004 for the renovation of the property to house a comprehensive cancer center. The interest rate is variable and was 1.51% at December 31, 2011 and 2010. Monthly payments are due through January 2014.

During 2005, the Medical Center entered into a second construction loan for ten years in the amount of \$2.5 million for additional renovations of the cancer center. The interest rate for this loan is variable and was 1.52% and 1.61% at December 31, 2011 and 2010, respectively. Monthly payments are due through August 2015.

- (d) The Medical Center has various capital lease obligations with interest rates ranging up to 6.7%.
- (e) On May 25, 2006, the Medical Center entered into a loan agreement in the amount of \$31.2 million. The proceeds of the loan, which were raised by the New York City Capital Resource Corporation through the issuance of tax-exempt revenue bonds, are to be used to renovate, improve, repair and equip the Medical Center's facilities. The loan bears interest at a variable rate, which was 1.82% and 2.37% (including letter of credit fees) at December 31, 2011 and 2010, respectively, with monthly payments through 2026. A portion of the proceeds was utilized to repay a line of credit which was used for interim capital financing. The loan is secured by a letter of credit of approximately \$26.6 million which expires on May 25, 2016. The Medical Center is in the process of refinancing this debt which is approximately \$25.2 million and anticipates it will close the transaction in the second quarter of 2012.

Maimonides Medical Center

Notes to Financial Statements (continued)

5. Long-Term Debt, Capital Lease Obligations, Other Borrowings and Related Matters (continued)

Scheduled principal payments on long-term debt and capital lease obligations are as follows:

	Loans Payable	Capital Leases	Total
	<i>(In Thousands)</i>		
2012	\$ 5,627	\$ 16,879	\$ 22,506
2013	5,890	12,499	18,389
2014	5,327	8,579	13,906
2015	5,459	7,576	13,035
2016	5,597	2,383	7,980
Thereafter	105,977	702	106,679
Total minimum payments	133,877	48,618	182,495
Less amounts representing interest	—	2,782	2,782
Total long-term debt	133,877	45,836	179,713
Less current portion of long-term debt	5,627	15,554	21,181
Long-term debt, net of current portion	<u>\$ 128,250</u>	<u>\$ 30,282</u>	<u>\$ 158,532</u>

Required sinking fund balances for FHA Sections 241 and 242 loan agreements for the next five years are as follows (in thousands):

2012	\$ 13,612
2013	14,933
2014	16,221
2015	17,468
2016	18,665

As discussed in Note 7, the Medical Center also has a letter of credit from a bank of \$1.0 million at December 31, 2011, for the benefit of one of its captive insurance companies. This letter of credit expires on November 1, 2012. During the years ended December 31, 2011 and 2010, no drawdowns were made under this letter of credit agreement. The Medical Center expects to extend this letter of credit in 2012.

Maimonides Medical Center

Notes to Financial Statements (continued)

5. Long-Term Debt, Capital Lease Obligations, Other Borrowings and Related Matters (continued)

In addition, the Medical Center maintains two lines of credit with two banks, in the amount of \$20.0 million each. At December 31, 2011 and 2010, approximately \$18.4 million and \$16.4 million were outstanding on these two lines of credit, respectively. The drawdowns were for interim financing of capital expenditures and working capital needs. The average interest rate charged in 2011 was 2.3% on one line (2.3% in 2010) and 2.2% on the other (2.3% in 2010). These lines of credit expire on June 22, 2012 and July 31, 2012, respectively. The Medical Center intends to renew these lines of credit in 2012.

In connection with various debt agreements, the Medical Center is required to maintain certain financial covenants. The Medical Center was not in compliance with the current ratio financial covenant related to its FHA mortgage loans at December 31, 2011 and 2010. The Medical Center has notified the U.S. Department of Housing and Urban Development of the noncompliance for 2011 and 2010. This noncompliance does not constitute an event of default under the terms of the related agreements.

Interest Rate Swap Agreements

The Medical Center has three interest rate swap agreements to manage risk associated with changes in interest rates associated with its long-term debt. Under the terms of the interest rate swaps, the Medical Center pays fixed rates monthly and receives from the counterparties variable payments based on percentages of LIBOR. Interest payments and the change in fair value of the swaps (loss of approximately \$1.1 million in 2011 and \$0.6 million in 2010) are recorded in the (deficiency) excess of revenue over expenses.

At December 31, 2011 and 2010, the swaps are reported in other noncurrent liabilities and are summarized as follows:

Notional Amount	Maturity Date	Fixed Payment Rate	2011 Fair Value (All Liabilities)
\$ 26,345,000	January 1, 2026	4.14%	\$ 4,974,997
1,904,762	January 31, 2014	6.19	92,080
916,667	August 15, 2015	5.985	66,221
			<u>\$ 5,133,298</u>

Maimonides Medical Center

Notes to Financial Statements (continued)

5. Long-Term Debt, Capital Lease Obligations, Other Borrowings and Related Matters (continued)

Notional Amount	Maturity Date	Fixed Payment Rate	2010 Fair Value (All Liabilities)
\$ 27,430,000	January 1, 2026	4.14%	\$ 3,779,114
2,819,047	January 31, 2014	6.19	183,053
1,166,667	August 15, 2015	5.985	92,571
			<u>\$ 4,054,738</u>

6. Pension Plans and Postretirement Health Care Benefits

The Medical Center provides retirement and similar benefits to its union employees through several defined benefit multiemployer pension plans and to its nonunion employees through a noncontributory defined benefit pension plan, tax deferred annuity plans, and a nonqualified defined contribution plan covering certain employees. Payments to the defined benefit multiemployer union plans are made in accordance with contractual arrangements under which contributions are generally based on gross salaries and are funded on a current basis. The Medical Center contributes amounts to the nonunion plan sufficient to meet the minimum funding requirements set forth in the Employee Retirement Income Security Act of 1974. The Medical Center's pension expense under all existing plans aggregated approximately \$35.0 million and \$31.6 million for the years ended December 31, 2011 and 2010, respectively.

In relation to the two multiemployer defined benefit pension plans that cover the Medical Center's union-represented employees, the risks of participating in these multiemployer plans are different from single-employer plans in the following respects:

- Assets contributed to a multiemployer plan by one employer may be used to provide benefits to employees of other participating employers.
- If a participating employer stops contributing to a plan, the unfunded obligations of the plan may be borne by the remaining participating employers.
- If the Medical Center chooses to stop participating in some of its multiemployer plans, the Medical Center may be required to pay those plans an amount based on the underfunded status of the plan, referred to as a withdrawal liability.

Maimonides Medical Center

Notes to Financial Statements (continued)

6. Pension Plans and Postretirement Health Care Benefits (continued)

The Medical Center's participation in these plans for the years ended December 31, 2011 and 2010 is outlined in the table below. The "EIN Number" column provides the Employer Identification Number ("EIN"). Unless otherwise noted, the most recent Pension Protection Act ("PPA") zone status available in 2011 and 2010 is for a plan's year-end at December 31, 2011 and 2010, respectively. The zone status is based on information that the Medical Center received from the plans and is certified by the plans' actuaries. Among other factors, plans in the red zone are generally less than 65% funded, plans in the yellow zone are less than 80% funded, and plans in the green zone are at least 80% funded. The "FIP/RP Status Pending/Implemented" column indicates plans for which a financial improvement plan ("FIP") or a rehabilitation plan ("RP") is pending or has been implemented. The last column lists the expiration dates of the collective bargaining agreements to which the plans are subject.

The number of employees covered by the Medical Center's multiemployer plans did not change significantly from 2010 to 2011. Contribution rates required to be paid to the plans have increased from 2010 to 2011.

Pension Fund	EIN Number	Plan Number	Pension Protection Act Zone Status		FIP/RP Pending/Implemented	Contributions by the Medical Center		Surcharge Imposed	Expiration Date of Collective Bargaining Agreement
			2011	2010		2011	2010		
The New York State Nurses Association Pension Plan	13-6604799	001	Green	Green	No	\$ 10,172,000	\$ 7,445,000	No	1/01/2014
1199 SEIU Health Care Employees Pension Fund	13-3604862	001	Green	Red	Rehabilitation Plan adopted in 2010	10,959,000	9,360,000	No	4/30/2015

The Medical Center was listed in The New York State Nurses Association Pension Plan Form 5500 as providing more than 5% of the total contributions of the plan for plan years ended December 31, 2011 and 2010.

In addition to the plans described above, the Medical Center sponsors a defined benefit health care plan that provides postretirement medical, dental and life insurance benefits to certain full-time employees hired prior to July 1, 1984 and who have worked ten years and attained age 65 while in service with the Medical Center. The plan contains cost-sharing features such as deductibles and coinsurance.

The Medical Center uses a December 31 measurement date for its pension and postretirement benefits plans.

Maimonides Medical Center

Notes to Financial Statements (continued)

6. Pension Plans and Postretirement Health Care Benefits (continued)

The Medical Center recognizes the funded status (i.e., the difference between the fair value of plan assets and the projected benefit obligations) of the defined benefit plans in its statements of financial position. Net unrecognized actuarial losses and the net unrecognized prior service costs at the reporting date will be subsequently recognized in the future as net periodic benefit cost pursuant to the Medical Center's accounting policy for amortizing such amounts. Further, actuarial gains and losses that arise in subsequent periods and are not recognized as net periodic benefit cost in the same periods will be recognized as a component of unrestricted net assets.

Included in other changes in unrestricted net assets at December 31, 2011 and 2010 are the following amounts that have not yet been recognized in net periodic pension and postretirement cost:

	Pension Benefits		Postretirement Health Care Benefits	
	2011	2010	2011	2010
	<i>(In Thousands)</i>			
Unrecognized prior service (cost) credit	\$ (278)	\$ (335)	\$ 198	\$ 113
Unrecognized actuarial loss	(41,140)	(29,829)	(3,950)	(4,020)
	<u>\$ (41,418)</u>	<u>\$ (30,164)</u>	<u>\$ (3,752)</u>	<u>\$ (3,907)</u>

The net prior service cost and actuarial loss included in unrestricted net assets and expected to be recognized in net periodic pension and postretirement cost during the year ending December 31, 2012 is approximately \$3.6 million.

Maimonides Medical Center

Notes to Financial Statements (continued)

6. Pension Plans and Postretirement Health Care Benefits (continued)

The following table sets forth the change in benefit obligations, change in plan assets and the funded status of the plans as of December 31, 2011 and 2010:

	Pension Benefits		Postretirement Health Care Benefits	
	2011	2010	2011	2010
	<i>(In Thousands)</i>			
Change in benefit obligation:				
Benefit obligation, beginning of year	\$ 98,873	\$ 84,644	\$ 12,279	\$ 10,540
Service cost	5,546	4,862	190	161
Interest cost	4,990	4,671	555	631
Plan amendments	—	—	(158)	—
Medicare Part D reimbursements	—	—	20	38
Actuarial losses	14,612	6,052	46	1,381
Net benefits paid and expected expenses	(3,255)	(1,356)	(597)	(472)
Benefit obligation, end of year	120,766	98,873	12,335	12,279
Change in plan assets:				
Fair value of plan assets, beginning of year	64,339	53,243	—	—
Actual return on plan assets	6,021	6,452	—	—
Contributions	6,298	6,000	597	472
Benefits paid and actual expenses	(3,302)	(1,356)	(597)	(472)
Fair value of plan assets, end of year	73,356	64,339	—	—
Unfunded status	\$ (47,410)	\$ (34,534)	\$ (12,335)	\$ (12,279)

The following table provides the amounts recognized in the statements of financial position:

	Pension Benefits		Postretirement Health Care Benefits	
	2011	2010	2011	2010
	<i>(In Thousands)</i>			
Accrued benefit liability – noncurrent	\$ (47,410)	\$ (34,534)	\$ (11,760)	\$ (11,813)
Accrued benefit liability – current	—	—	(575)	(466)
	\$ (47,410)	\$ (34,534)	\$ (12,335)	\$ (12,279)

Maimonides Medical Center

Notes to Financial Statements (continued)

6. Pension Plans and Postretirement Health Care Benefits (continued)

The projected benefit obligation, accumulated benefit obligation and fair value of pension plan assets are as follows:

	December 31	
	2011	2010
	<i>(In Thousands)</i>	
Projected benefit obligation	\$ 120,766	\$ 98,873
Accumulated benefit obligation	105,205	85,170
Fair value of plan assets	73,356	64,339

Net periodic pension costs, included in employee benefits expense, consist of the following components:

	Pension Benefits		Postretirement Health Care Benefits	
	2011	2010	2011	2010
	<i>(In Thousands)</i>			
Service cost	\$ 5,546	\$ 4,862	\$ 190	\$ 161
Interest cost on projected benefit obligation	4,990	4,671	555	631
Expected return on plan assets	(5,049)	(4,192)	—	—
Net amortization and deferral	2,433	2,421	43	93
Net periodic pension cost	\$ 7,920	\$ 7,762	\$ 788	\$ 885

Maimonides Medical Center

Notes to Financial Statements (continued)

6. Pension Plans and Postretirement Health Care Benefits (continued)

The following are assumptions used in the measurement of the Medical Center's benefit obligations and net periodic benefit cost:

	Pension Benefits		Postretirement Health Care Benefits	
	2011	2010	2011	2010
Weighted-average assumptions used to determine benefit obligations as of December 31:				
Discount rate	4.25%	5.25%	4.25%	5.25%
Rate of compensation increase (2011)	4.0	2.5	4.0	2.5
Rate of compensation increase (2012 and future)	4.0	4.5	4.0	4.5
Weighted-average assumptions used to determine net periodic benefit cost for years ended December 31:				
Discount rate	5.25%	5.75%	5.25%	5.75%
Expected return on plan assets	8.0	8.0	N/A	N/A
Rate of compensation increase (2011)	4.0	2.5	4.0	2.5
Rate of compensation increase (2012 and future)	4.0	4.5	4.0	4.5

The overall long-term rate of return was developed by estimating the expected long-term real return for each asset class within the portfolio, computing an average weighted real rate of return for the portfolio as a whole, reflecting both the plan's expected asset class allocation and the correlations between the various asset classes and adding that expected real rate of return to the expected long-term rate of inflation component per year.

Maimonides Medical Center

Notes to Financial Statements (continued)

6. Pension Plans and Postretirement Health Care Benefits (continued)

	2011	2010
Assumed health care cost trend rates at December 31:		
Pre-65 Medical:		
Initial rate for health care costs	12.9%	7.3%
Ultimate rate for health care costs	4.3%	4.7%
Ultimate year of health care increase	2083	2082
Post-65 Medical:		
Initial rate for health care costs	9.0%	7.3%
Ultimate rate for health care costs	4.3%	4.7%
Ultimate year of health care increase	2083	2082
Dental:		
Initial rate for health care costs	94.0%	5.0%
Ultimate rate for health care costs	4.3%	4.7%
Ultimate year of health care increase	2083	2082

Assumed health care cost trend rates may have a significant effect on the amounts reported for the health care plans. A 1% change in assumed health care cost trend rates would have the following effects at December 31, 2011:

	1% Increase	1% Decrease
	<i>(In Thousands)</i>	
Effect on total of service and interest cost components of net periodic postretirement health care benefit cost	\$ 89	\$ (75)
Effect on health care components of the accumulated postretirement benefit obligation	1,501	(1,269)

Maimonides Medical Center

Notes to Financial Statements (continued)

6. Pension Plans and Postretirement Health Care Benefits (continued)

Plan Assets

The Medical Center's pension plan weighted-average asset allocation at December 31, 2011 and 2010 by asset category is as follows:

	Plan Assets as of December 31	
	2011	2010
Asset category:		
Equities	46%	52%
Fixed income	52	47
Other	2	1
Total	100%	100%

Refer to Note 13 for fair value measurement information related to the defined benefit plan asset categories noted in the table above.

Investment Strategy

The overall investment philosophy of the Medical Center is to maximize return while minimizing risk in a diversified portfolio. The rate of return of the total fund is compared to the return of a policy portfolio consisting of 50% of Barclays Capital Bond Index, 35% of Russell 3000 Index and 15% of the Morgan Stanley Capital International Europe, Australia and Far East Index. The target asset allocation permissible ranges by asset category are as follows:

Asset Category	Permissible Range
Equities – Domestic	30% to 40%
Equities – Non-U.S.	12% to 18%
Fixed income	45% to 55%

Cash Flows

Contributions: The Medical Center expects to contribute \$6.0 million to its defined benefit pension plan in 2012.

Maimonides Medical Center

Notes to Financial Statements (continued)

6. Pension Plans and Postretirement Health Care Benefits (continued)

Expected Future Benefit Payments: The following benefit payments, which reflect expected future service, as appropriate, are expected to be paid:

	Pension Benefits	Postretirement Health Care Benefits
	<i>(In Thousands)</i>	
Year:		
2012	\$ 6,700	\$ 608
2013	7,288	613
2014	6,192	644
2015	6,490	756
2016	7,341	762
2017 to 2021	43,056	4,560

Medicare Prescription Drug, Improvement and Modernization Act of 2003

In December 2003, the Medicare Prescription Drug, Improvement and Modernization Act of 2003 became law. It introduced a prescription drug benefit under Medicare (“Medicare Part D”) as well as a federal subsidy to sponsors of retiree health care benefit plans that provide a benefit that is at least actuarially equivalent to Medicare Part D. The Medical Center follows guidelines for companies that sponsor defined benefit postretirement health care plans that provide prescription drug benefits under Medicare Part D. Those guidelines address situations in which a company has determined that the benefit provided is at least actuarially equivalent to Medicare Part D benefits. The Medical Center determined that, as of January 1, 2004, the benefit provided to retirees is actuarially equivalent to Medicare Part D and should be accounted for as such. Accordingly, the effect to the accumulated postretirement benefit obligation was a reduction of approximately (in thousands):

2012	\$ 21
2013	20
2014	21
2015	21
2016	20
Thereafter	92

Maimonides Medical Center

Notes to Financial Statements (continued)

7. Professional Liabilities

Beginning in 1977, the Medical Center has participated in a pooled professional and general liability program with certain other health care facilities (principally hospitals) affiliated with the Federation of Jewish Philanthropies of New York (the “FOJP Program”). This participation has been with captive and commercial insurance companies utilizing occurrence basis type coverage. At December 31, 2011, the Medical Center retained ownership in three captive insurance companies affiliated with the FOJP Program. The captive insurance companies are primarily accounted for using the equity method.

In 2011, certain member hospitals of the captive insurance companies purchased the interest of another of its members under an agreement that requires payments to be made over the next three years. This resulted in a change of ownership percentage for each of the remaining members. As a result, the Hospital’s equity in the captive insurance companies increased by approximately \$9.7 million and its liabilities increased by \$8.5 million. These amounts are included in equity in captive insurance program, other current assets, other current liabilities and long-term professional liabilities in the accompanying statement of financial position at December 31, 2011.

The aggregate net carrying value of the Medical Center’s interests in the insurance program was approximately \$52.7 million and \$47.3 million at December 31, 2011 and 2010, respectively, which is included in equity in captive insurance program in the accompanying statements of financial position.

The Medical Center changed its malpractice insurance program for the period from January 1, 1998 through December 31, 1998. Under the terms of the revised program, a portion of the Medical Center’s coverage became self-retained. Effective January 1, 1999, this program was terminated prospectively and the Medical Center no longer maintains a self-retained component. At December 31, 2011 and 2010, the Medical Center’s statements of financial position reflected professional liabilities of approximately \$4.9 million and \$4.5 million, respectively, for the actuarial present value of the self-retained component of malpractice insurance based on a study prepared by independent consulting actuaries discounted at 5.0%. In conjunction with this program, the Medical Center, together with several other hospitals, invested approximately \$6.7 million at December 31, 1998 (Medical Center’s portion) in pooled investment unit trusts. At December 31, 2011 and 2010, the fair value of these units is approximately \$3.9 million and \$4.0 million, respectively.

Maimonides Medical Center

Notes to Financial Statements (continued)

7. Professional Liabilities (continued)

Effective January 1, 1999, the Medical Center's malpractice insurance program reverted to a coverage agreement similar to the arrangement that existed prior to January 1, 1998. This program, however, provides for a deferral of premium payments through 2014 for years subsequent to 2009. At December 31, 2011 and 2010, approximately \$35.4 million and \$32.3 million, respectively, represent the estimated present value of future payments. Amounts due in 2012 are included in other current liabilities and the balance is included in long-term professional liabilities in the accompanying statements of financial position.

The Medical Center, as part owner of its malpractice captive, guarantees a certain level of investment return. As a result of market losses in 2011, the Medical Center is required to fund its share of market losses of \$10.4 million. The liability is to be paid over a period of four years beginning in 2012 and is included in other current liabilities and long-term professional liabilities in the accompanying 2011 statement of financial position. At December 31, 2011, approximately \$1.1 million of accrued interest remains from prior years market losses and is included in long-term professional liabilities in the accompanying statements of financial position.

As discussed in Note 1, in 2011, the Medical Center changed the presentation of its statement of financial position to present on a gross basis its professional liabilities and insurance recoveries receivable. The estimate of professional liabilities and the estimate for incidents that have been incurred but not reported is included in estimated insurance claims liabilities in the accompanying statements of financial position at the actuarially determined present value of approximately \$186.7 million and \$188.0 million based on a discount rate of 3.0% at December 31, 2011 and 2010, respectively. The Medical Center has recorded related estimated insurance claims receivable of approximately \$186.7 million and \$188.0 million at December 31, 2011 and 2010, respectively, in consideration of the expected insurance recoveries. The current portion of estimated insurance claims liabilities and the related estimated insurance claims receivable represents an estimate of expected settlements and insurance recoveries over the next 12 months.

At December 31, 2011 and 2010, the Medical Center had an unsecured letter of credit for the benefit of one of its captive insurance companies from a bank in the amount of \$1.0 million. During the years ended December 31, 2011 and 2010, no drawdowns were made under this letter of credit agreement.

Maimonides Medical Center

Notes to Financial Statements (continued)

8. Transactions with Affiliates

Services provided by MMC Holding of Brooklyn, Inc. through its various subsidiaries to the Medical Center include pharmaceuticals, medical supplies and equipment, technical support services, contracted services and management reviews for various Medical Center properties. Expenses incurred by the Medical Center for such services approximated \$19.9 million and \$20.2 million during 2011 and 2010, respectively. Additionally, MMC Holding of Brooklyn, Inc. purchases various goods and services from the Medical Center. Revenue generated from such activity approximated \$516,000 and \$616,000 during 2011 and 2010, respectively.

The net amounts due from affiliates in the accompanying statements of financial position include a revolving loan receivable and accrued interest thereon from MMC Holding of Brooklyn, Inc. of approximately \$2.6 million and \$2.5 million at December 31, 2011 and 2010, respectively. The loan bears interest at 5.75% at December 31, 2011 and 2010 and has no specified repayment date. Also included in the balances each year are amounts due from and to MMC Holding of Brooklyn, Inc. and its subsidiaries for services provided by and to the Medical Center, as previously described.

9. Temporarily and Permanently Restricted Net Assets

Temporarily restricted net assets are available for the following purposes at December 31:

	2011	2010
	<i>(In Thousands)</i>	
Research and education	\$ 1,115	\$ 1,346
Plant replacement and expansion	13,323	13,270
	\$ 14,438	\$ 14,616

Permanently restricted net assets at December 31, 2011 and 2010 consist of investments to be held in perpetuity, the income from which is to be used for health care related services.

Maimonides Medical Center

Notes to Financial Statements (continued)

10. Other Revenue

Other revenue consisted of the following for the years ended December 31:

	2011	2010
	<i>(In Thousands)</i>	
Government grant income	\$ 6,762	\$ 6,935
Real estate rentals	4,703	4,743
Investment income	9,835	2,262
Special funds income	9,921	12,957
Dining room and parking lot income	1,458	1,421
Grants and contributions	2,211	1,220
Settlements	4,275	4,500
Insurance company surplus distribution	4,649	6,768
Other	4,840	3,535
	\$ 48,654	\$ 44,341

On April 23, 2010, the Medical Center received a settlement from Victory Memorial Hospital of Brooklyn in the amount of \$4.5 million. The settlement was for repayment of cash advances and payment for management support services provided from 2005 to 2007.

As indicated in Note 1, the Medical Center held an investment in a bank/note certificate receivable investment program which was written down to zero at December 31, 2010. In May 2011, the Medical Center reached an agreement with the bank related to the valuation of the investment and received a settlement in the amount of \$2.0 million. The Medical Center also settled a legal matter with a utility company during 2011 in the amount of \$2.3 million related to incorrect charges.

11. Functional Expenses

Functional expenses for the years ended December 31 are as follows:

	2011	2010
	<i>(In Thousands)</i>	
Health care and related services	\$ 790,413	\$ 768,639
Program support and general	196,272	200,642
	\$ 986,685	\$ 969,281

Maimonides Medical Center

Notes to Financial Statements (continued)

12. Concentration of Credit Risk

The Medical Center grants credit without collateral to its patients, most of whom are local residents and are insured under third-party payor agreements. Net accounts receivable by payor were as follows:

	December 31	
	2011	2010
Medicare and Medicaid	42%	40%
Commercial and managed care	53	55
Self pay	5	5
	100%	100%

At December 31, 2011 and 2010, substantially all of the Medical Center's cash and cash equivalents were held in custodial accounts at four financial institutions. Management believes that credit risk related to these deposits is minimal.

The Medical Center routinely invests its operating cash in money market funds. These funds generally invest in highly liquid U.S. government and agency obligations. Investments in money market funds are not insured or guaranteed by the U.S. government.

13. Fair Value Measurements

For assets and liabilities required to be measured at fair value, the Medical Center measures fair value based on the price that would be received to sell an asset or paid to transfer a liability in an orderly transaction between market participants at the measurement date. Fair value measurements are applied based on the unit of account from the Medical Center's perspective. The unit of account determines what is being measured by reference to the level at which the asset or liability is aggregated (or disaggregated) for purposes of applying other accounting pronouncements.

The Medical Center follows a valuation hierarchy that prioritizes observable and unobservable inputs used to measure fair value into three broad levels, which are described below:

Level 1: Quoted prices (unadjusted) in active markets that are accessible at the measurement date for identical assets or liabilities. The fair value hierarchy gives the highest priority to Level 1 inputs.

Level 2: Observable inputs that are based on inputs not quoted in active markets, but corroborated by market data.

Maimonides Medical Center

Notes to Financial Statements (continued)

13. Fair Value Measurements (continued)

Level 3: Unobservable inputs are used when little or no market data is available. The fair value hierarchy gives the lowest priority to Level 3 inputs.

A financial instrument's categorization within the valuation hierarchy is based upon the lowest level of input that is significant to the fair value measurement. In determining fair value, the Medical Center utilizes valuation techniques that maximize the use of observable inputs and minimize the use of unobservable inputs to the extent possible as well as considers counterparty credit risk in its assessment of fair value. Financial assets and liabilities, including the defined benefit plan assets, carried at fair value as of December 31, 2011 are classified in the table below in one of the three categories described above:

	Level 1	Level 2	Level 3	Total
	<i>(In Thousands)</i>			
Assets				
Cash and cash equivalents	\$ 55,725	\$ —	\$ —	\$ 55,725
U.S. government notes and bonds	86,859	—	—	86,859
Corporate bonds	—	25,647	—	25,647
Certificate of deposit	2,630	—	—	2,630
Mutual funds	—	3,917	—	3,917
Total assets at fair value	<u>\$ 145,214</u>	<u>\$ 29,564</u>	<u>\$ —</u>	<u>\$ 174,778</u>
Defined benefit plan assets				
Cash and cash equivalents	\$ 1,517	\$ —	\$ —	\$ 1,517
Mutual funds – equity	33,376	—	—	33,376
Mutual funds – fixed income	38,463	—	—	38,463
Total defined benefit plan assets at fair value	<u>\$ 73,356</u>	<u>\$ —</u>	<u>\$ —</u>	<u>\$ 73,356</u>
Liabilities				
Interest rate swap agreements	\$ —	\$ 5,133	\$ —	\$ 5,133

Maimonides Medical Center

Notes to Financial Statements (continued)

13. Fair Value Measurements (continued)

Financial assets and liabilities carried at fair value as of December 31, 2010 are classified in the table below in one of the three categories described above:

	Level 1	Level 2	Level 3	Total
	<i>(In Thousands)</i>			
Assets				
Cash and cash equivalents	\$ 51,431	\$ —	\$ —	\$ 51,431
U.S. government notes and bonds	79,720	—	—	79,720
Corporate bonds	—	24,769	—	24,769
Certificate of deposit	2,630	—	—	2,630
Mutual funds	—	3,958	—	3,958
Total assets at fair value	<u>\$ 133,781</u>	<u>\$ 28,727</u>	<u>\$ —</u>	<u>\$ 162,508</u>
Defined benefit plan assets				
Cash and cash equivalents	\$ 581	\$ —	\$ —	\$ 581
Mutual funds – equity	33,376	—	—	33,376
Mutual funds – fixed income	30,382	—	—	30,382
Total defined benefit plan assets at fair value	<u>\$ 64,339</u>	<u>\$ —</u>	<u>\$ —</u>	<u>\$ 64,339</u>
Liabilities				
Interest rate swap agreements	\$ —	\$ 4,055	\$ —	\$ 4,055

The following is a description of the Medical Center's valuation methodologies for assets measured at fair value. Fair value for Level 1 is based upon quoted market prices. Fair value for Level 2 is based on quoted prices for similar instruments in active markets, quoted prices for identical or similar instruments in markets that are not active, and model-based valuation techniques for which all significant assumptions are observable in the market or can be corroborated by observable market data for substantially the full term of the assets. Inputs are obtained from various sources including market participants, dealers and brokers. The methods described above may produce a fair value that may not be indicative of net realizable value or reflective of future fair values. Furthermore, while the Medical Center believes its valuation methods are appropriate and consistent with other market participants, the use of different methodologies or assumptions to determine the fair value of certain financial instruments could result in a different estimate of fair value at the reporting date.

Maimonides Medical Center

Notes to Financial Statements (continued)

13. Fair Value Measurements (continued)

The following table is a rollforward of the statement of financial position amounts for financial instruments (bank note/certificate receivable) classified by the Medical Center in Level 3 of the valuation hierarchy defined above for the year ended December 31, 2010 (in thousands):

Fair value at January 1	\$ 4,835
Unrealized gains	165
Realized losses	(5,000)
Fair value at December 31	<u>\$ —</u>

The Medical Center's long-term debt obligations are reported at carrying value. The fair value of the Medical Center's long-term debt obligations is approximately \$185.5 million and \$197.9 million at December 31, 2011 and 2010, respectively, based on quoted market prices and other valuation considerations.

14. Operating Lease Obligations

The Medical Center has entered into various agreements under noncancellable operating leases. During 2003, the Medical Center entered into an operating lease agreement for a building to provide a site for the development of a comprehensive cancer center. Rent payments are recognized as expense on a straight-line basis over the term of the related lease. At December 31, 2011 and 2010, the Medical Center had an asset of approximately \$3.3 million and \$2.7 million, respectively, representing rental payments made in excess of rent expense on a straight-line basis.

Future minimum payments under noncancellable operating leases with initial or recurring terms of one year or more are as follows (in thousands):

2012	\$ 10,804
2013	10,188
2014	9,408
2015	8,317
2016	6,843
Thereafter	42,623
Total minimum operating lease payments	<u>\$ 88,183</u>

Total rental expense charged to operations for the years ended December 31, 2011 and 2010 aggregated approximately \$12.8 million and \$11.9 million, respectively.

Maimonides Medical Center

Notes to Financial Statements (continued)

15. Commitments and Contingencies

Various investigations, lawsuits and claims arising out of the normal course of operations are pending or on appeal against the Medical Center. While the ultimate effect of such actions cannot be determined at this time, it is the opinion of management that the liabilities which may arise from such actions would not materially affect the Medical Center's financial position or results of operations.

Approximately 71% of the Medical Center's employees are members of various unions. The Local 1199 union contract expires on April 30, 2015. The New York State Nurses Association contract expires on January 1, 2014 and the Committee on Interns and Residents contract expires on October 31, 2013.

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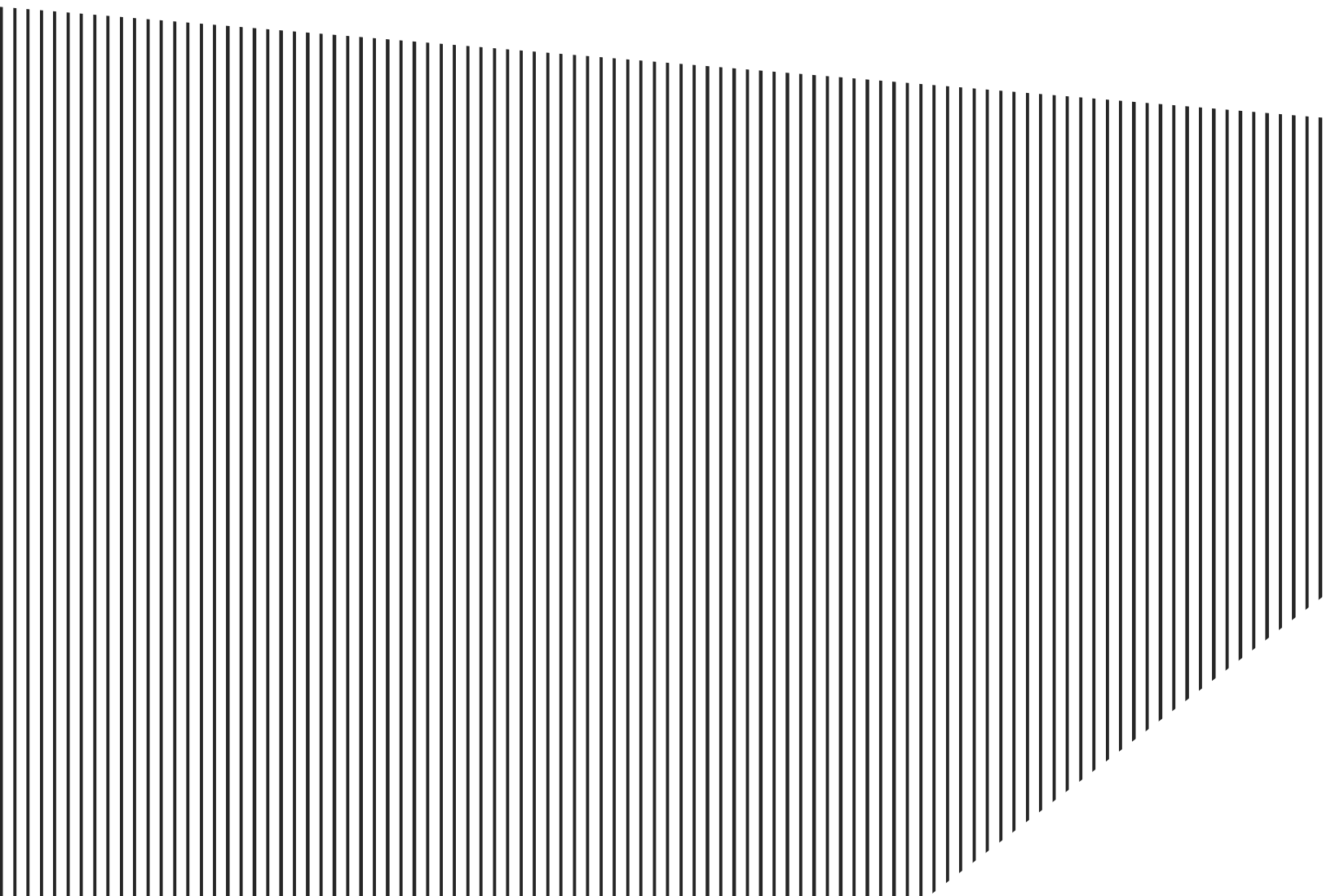
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APPENDIX C

**UNAUDITED FINANCIAL STATEMENTS OF THE MEDICAL CENTER
AS OF NOVEMBER 30 2012 AND FOR THE
ELEVEN MONTH PERIODS ENDED NOVEMBER 30, 2012 AND 2011**

Maimonides Medical Center

Financial Statements

As of November 30, 2012 and for the eleven month periods ended November 30, 2012 and 2011

(Unaudited)

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Maimonides Medical Center
Statements of Financial Position

	(Unaudited) November 30, 2012	(Audited) December 31, 2011
	(In Thousands)	
Assets		
Current assets:		
Cash and cash equivalents	\$ 46,518	\$ 49,188
Short-term investments	96,714	93,294
Total cash, cash equivalents and short-term investments	<u>143,232</u>	<u>142,482</u>
Assets limited as to use, current portion:		
Board designated	14,389	13,757
Medical resident FICA refund (Note H)	10,881	—
Receivables for patient care, net of allowance for doubtful accounts (Note D)	138,997	131,916
Due from affiliates, net, current portion	9,530	7,300
Supplies, at average cost	9,272	8,851
Estimated insurance claims receivable, current portion	28,000	28,000
Other current assets	32,404	45,866
Total current assets	<u>386,705</u>	<u>378,172</u>
Assets limited as to use, less current portion:		
Sinking funds	14,008	12,883
Collateral	2,630	2,630
Self-insurance trust	3,917	3,917
Long-term investment funds	—	1,102
Due from affiliates, net, less current portion	2,739	2,607
Property, plant and equipment, net	345,486	340,954
Equity in captive insurance program	60,827	52,707
Estimated insurance claims receivable, less current portion	158,668	158,668
Other assets	30,356	16,534
Total assets	<u><u>\$ 1,005,336</u></u>	<u><u>\$ 970,174</u></u>

See accompanying notes.

	(Unaudited) November 30, 2012	(Audited) December 31, 2011
	<i>(In Thousands)</i>	
Liabilities and net assets		
Current liabilities:		
Accounts payable and accrued expenses	\$ 64,043	\$ 54,948
Construction payable	7,690	7,291
Accrued salaries and related liabilities	79,064	74,152
Current portion of long-term debt and capital lease obligations <i>(Note F)</i>	26,162	21,181
Line of credit borrowings	17,976	18,408
Estimated insurance claims liabilities, current portion	28,000	28,000
Other current liabilities	67,120	65,260
Total current liabilities	290,055	269,240
Long-term debt and capital lease obligations, less current portion <i>(Note F)</i>	183,958	158,532
Accrued pension and postretirement benefits <i>(Note G)</i>	59,734	59,170
Other noncurrent liabilities	88,708	104,430
Professional liabilities	32,806	41,912
Estimated insurance claims liabilities, less current portion	158,668	158,668
Total liabilities	813,929	791,952
Commitments and contingencies <i>(Notes D, I and J)</i>		
Net assets:		
Unrestricted	176,438	162,652
Temporarily restricted	13,837	14,438
Permanently restricted	1,132	1,132
Total net assets	191,407	178,222
Total liabilities and net assets	\$ 1,005,336	\$ 970,174

See accompanying notes.

Maimonides Medical Center

Statements of Operations and Changes in Net Assets

	(Unaudited) Eleven Months Ended November 30,	
	2012	2011
	(In Thousands)	
Operating revenue:		
Net patient service revenue	\$ 888,163	\$ 851,179
Provision for bad debts <i>(Notes C and D)</i>	(8,793)	(11,257)
Net patient service revenue less provision for bad debts	879,370	839,922
Other revenue	32,123	39,664
Net assets released from restrictions	1,513	1,550
Total operating revenue	913,006	881,136
Operating expenses:		
Salaries and wages	455,163	430,970
Employee benefits	137,558	128,623
Supplies and other expenses	271,835	266,567
Depreciation and amortization	43,256	40,933
Interest and amortization of deferred financing costs	8,355	9,723
Total operating expenses	916,167	876,816
(Deficiency) excess of operating revenue over operating expenses before other items	(3,161)	4,320
Change in captive insurance program interest rate shortfall	5,767	(9,745)
Change in equity in captive insurance program	8,120	242
Medical resident FICA refund	2,259	—
Excess (deficiency) of revenue over expenses before net change in unrealized gains and losses on investments and change in fair value of interest rate swap agreements	12,985	(5,183)
Net change in unrealized gains and losses on investments and change in fair value of interest rate swap agreements	(72)	440
Excess (deficiency) of revenue over expenses	12,913	(4,743)
Grants for capital asset acquisitions and net assets released from restrictions used for capital asset acquisitions	873	7,670
Increase in unrestricted net assets	13,786	2,927
Temporarily restricted net assets:		
Restricted contributions, grants and other receipts	1,392	2,093
Net assets released from restrictions:		
Operating expenses	(1,513)	(1,550)
Capital asset acquisitions	(480)	(279)
(Decrease) increase in temporarily restricted net assets	(601)	264
Increase in net assets	13,185	3,191
Net assets at beginning of year	178,222	184,966
Net assets at end of period	\$ 191,407	\$ 188,157

See accompanying notes.

Maimonides Medical Center
Statements of Cash Flows

	(Unaudited) Eleven Months Ended November 30,	
	2012	2011
	<i>(In Thousands)</i>	
Operating activities		
Increase in net assets	\$ 13,185	\$ 3,191
Adjustments to reconcile increase in net assets to net cash provided by operating activities:		
Depreciation and amortization	43,256	40,933
Amortization and write-off of deferred financing costs	1,206	334
Net change in unrealized gains and losses on investments and change in fair value of interest rate swap agreements	72	(440)
Restricted contributions, grants and other receipts	(1,392)	(2,093)
Changes in operating assets and liabilities:		
Receivables for patient care	(7,081)	(9,538)
Due from affiliates	(2,362)	(3,467)
Other assets	(7,431)	(23,547)
Accounts payable and accrued expenses and accrued salaries and related liabilities	14,007	(2,904)
Other current liabilities	1,860	9,989
Other liabilities	(24,690)	31,511
Net cash provided by operating activities	<u>30,630</u>	<u>43,969</u>
Investing activities		
(Purchases) redemptions of investments	(1,512)	1,162
Acquisitions of property, plant and equipment – net	(31,235)	(38,254)
(Purchases) redemptions of assets limited as to use	(11,531)	334
Purchases of sinking funds	(1,160)	(1,608)
Net cash used in investing activities	<u>(45,438)</u>	<u>(38,366)</u>
Financing activities		
Repayments of long-term debt and capital lease obligations	(47,429)	(19,923)
Restricted contributions, grants and other receipts	1,392	2,093
Line of credit (repayments) drawdown	(432)	2,000
Proceeds from financings	58,607	12,429
Net cash provided by (used in) financing activities	<u>12,138</u>	<u>(3,401)</u>
Net (decrease) increase in cash and cash equivalents	(2,670)	2,202
Cash and cash equivalents at beginning of year	49,188	43,996
Cash and cash equivalents at end of period	<u>\$ 46,518</u>	<u>\$ 46,198</u>
Supplemental disclosures of cash flow information		
Capital lease arrangements	<u>\$ 4,291</u>	<u>\$ 3,187</u>

See accompanying notes.

Maimonides Medical Center

Notes to Interim Financial Statements

November 30, 2012

(Information pertaining to the eleven months ended November 30, 2012 and 2011 is unaudited)

Note A - Basis of Presentation

The accompanying unaudited financial statements have been prepared in accordance with accounting principles generally accepted in the United States applied on a basis consistent with that of the 2011 audited financial statements of Maimonides Medical Center (the “Medical Center”), except for the required adoption of Accounting Standards Update (“ASU”) No. 2011-07, *Presentation and Disclosure of Patient Service Revenue, Provision for Bad Debts, and the Allowance for Doubtful Accounts for Certain Health Care Entities* (“ASU 2011-07”), as disclosed in Note C. The Medical Center presumes that users of this interim financial information have read or have access to the Medical Center’s audited financial statements and that the adequacy of additional disclosures needed for a fair presentation may be determined in that context. Information contained in the Medical Center’s audited financial statements for the years ended December 31, 2011 and 2010 is incorporated herein. Footnotes and other disclosures that would substantially duplicate the disclosures contained in the Medical Center’s most recent audited financial statements have been omitted. Accordingly, these financial statements do not include all the information and footnotes required by U.S. generally accepted accounting principles for complete financial statements. In the opinion of management, all transactions considered necessary for a fair presentation have been included. Certain material estimates are updated only annually at year-end, including liabilities related to the defined pension and postretirement benefits and estimated insurance claims liabilities and related claims receivable. The effects of these year-end update calculations could be material at December 31, 2012 as compared to amounts reflected in the accompanying financial statements.

Patient volumes and net operating revenue and results are subject to seasonal variations caused by a number of factors. Monthly and periodic operating results are not necessarily representative of operations for a full year for various reasons, including the level of occupancy and other patient volumes, interest rates, unusual or infrequent items and other seasonal fluctuations. These same considerations apply to year-to-year comparisons.

Maimonides Medical Center

Notes to Interim Financial Statements (continued)

Note B – Use of Estimates

The preparation of financial statements in conformity with accounting principles generally accepted in the United States requires management to make estimates and assumptions that affect the reported amounts of assets and liabilities, such as estimated uncollectibles for accounts receivable for services to patients, estimated settlements with third-party payors, estimated insurance claims liabilities and receivables and retirement liabilities, and disclosure of contingent assets and liabilities at the date of the financial statements. Estimates also affect the amounts of revenues and expenses reported during the period. Actual results could differ from those estimates (see Note A).

Note C – Change in Accounting Principle

In July 2011, the Financial Accounting Standards Board issued ASU 2011-07. ASU 2011-07 requires that the provision for bad debts related to patient revenue be presented as a deduction from patient revenue in the statement of operations and changes in net assets rather than as an operating expense, with enhanced disclosure of the policies for recognizing patient service revenue and assessing bad debts. The guidance is effective for fiscal years and interim periods within those years beginning after December 15, 2011. The Medical Center adopted the provisions of ASU 2011-07 in 2012 and retrospectively applied the presentation requirements in the accompanying financial statements.

Note D – Net Patient Service Revenue and Accounts Receivable

Net patient service revenue is reported at the estimated net realizable amounts from patients, third-party payors, and others for services rendered, including estimated retroactive revenue adjustments due to ongoing and future audits, reviews and investigations. Retroactive adjustments are considered in the recognition of revenue on an estimated basis in the period the related services are rendered and such amounts are adjusted in future periods as adjustments become known or as years are no longer subject to such audits, reviews and investigations.

The Medical Center recognizes accounts receivable and patient service revenue associated with services provided to patients who have third-party payor (insurance) coverage on the basis of contractual rates, governmental rates or established charges for the services rendered. For uninsured patients who are ineligible for any government assistance program, the Medical Center provides services without charge or at amounts less than its established rates to patients who meet the criteria of its charity care policy. For patients who were determined by the Medical Center to have the ability to pay but do not, including those covered by insurance who might be responsible for copayments and deductibles, the estimated uncollectible amounts are recorded as

Maimonides Medical Center

Notes to Interim Financial Statements (continued)

Note D – Net Patient Service Revenue and Accounts Receivable (continued)

the provision for bad debts. In distinguishing charity care from the provision for bad debts, a number of factors are considered, certain of which require a high degree of judgment.

Patient revenue for the eleven months ended November 30, 2012, net of contractual and charity care allowances, but before the provision for bad debts, from third-party payors (insured patients) was approximately \$866.7 million and from uninsured patients was approximately \$21.5 million.

Together, charity care and the provision for bad debts represent uncompensated care. The allowance for doubtful accounts represents the Medical Center's estimate of the uncollectible accounts receivable related to uncompensated care. Additions to the allowance for doubtful accounts result from the provision of uncompensated care. Accounts written off as uncollectible are deducted from the allowance for doubtful accounts. The allowance for doubtful accounts is based upon the Medical Center's assessment of historical and expected net collections, business and economic conditions, trends in health care coverage and other collection indicators.

For receivables associated with services provided to patients who have third-party coverage, the Medical Center analyzes amounts due from third-parties and provides an allowance for doubtful accounts and a provision for bad debts, if necessary (for example, for third-party payors who are known to be having financial difficulties that make the realization of amounts due unlikely). For receivables associated with self-pay patients, which includes both patients without insurance and patients with deductible and copayment balances due after third-party coverage, the Medical Center records an allowance for doubtful accounts and provision for uncompensated care in the period of service based on historical experience. The allowance for doubtful accounts for self-pay patients was approximately 86% of the gross self-pay accounts receivable as of November 30, 2012.

The Medical Center's allowance for doubtful accounts relating to accounts receivable totaled approximately \$92.5 million at November 30, 2012.

Maimonides Medical Center

Notes to Interim Financial Statements (continued)

Note E – Fair Value Measurements

For assets and liabilities required to be measured at fair value, the Medical Center measures fair value based on the price that would be received to sell an asset or paid to transfer a liability in an orderly transaction between market participants at the measurement date. Fair value measurements are applied based on the unit of account from the Medical Center's perspective. The unit of account determines what is being measured by reference to the level at which the asset or liability is aggregated (or disaggregated) for purposes of applying other accounting pronouncements.

The Medical Center follows a valuation hierarchy that prioritizes observable and unobservable inputs used to measure fair value into three broad levels, which are described below:

- *Level 1:* Quoted prices (unadjusted) in active markets that are accessible at the measurement date for identical assets or liabilities. The fair value hierarchy gives the highest priority to Level 1 inputs.
- *Level 2:* Observable inputs that are based on inputs not quoted in active markets, but corroborated by market data.
- *Level 3:* Unobservable inputs are used when little or no market data is available. The fair value hierarchy gives the lowest priority to Level 3 inputs.

A financial instrument's categorization within the valuation hierarchy is based upon the lowest level of input that is significant to the fair value measurement. In determining fair value, the Medical Center utilizes valuation techniques that maximize the use of observable inputs and minimize the use of unobservable inputs to the extent possible as well as considers counterparty credit risk in its assessment of fair value. Financial assets, excluding the defined benefit plan assets, and liabilities carried at fair value as of November 30, 2012 are classified in the table below in one of the three categories described above:

Maimonides Medical Center

Notes to Interim Financial Statements (continued)

Note E – Fair Value Measurements (continued)

	Level 1	Level 2	Level 3	Total
	<i>(In Thousands)</i>			
Assets				
Cash and cash equivalents	\$ 64,402	\$ –	\$ –	\$ 64,402
U.S. government notes and bonds	90,053	–	–	90,053
Corporate bonds	–	27,348	–	27,348
Certificate of deposit	2,630	–	–	2,630
Mutual funds	–	3,917	–	3,917
Total assets at fair value	<u>\$ 157,085</u>	<u>\$ 31,265</u>	<u>\$ –</u>	<u>\$ 188,350</u>
Liabilities				
Interest rate swap agreements	<u>\$ –</u>	<u>\$ 5,958</u>	<u>\$ –</u>	<u>\$ 5,958</u>

Financial assets, excluding the defined benefit plan assets, and liabilities carried at fair value as of December 31, 2011 are classified in the table below in one of the three categories described above:

	Level 1	Level 2	Level 3	Total
	<i>(In Thousands)</i>			
Assets				
Cash and cash equivalents	\$ 55,725	\$ –	\$ –	\$ 55,725
U.S. government notes and bonds	86,859	–	–	86,859
Corporate bonds	–	25,647	–	25,647
Certificate of deposit	2,630	–	–	2,630
Mutual funds	–	3,917	–	3,917
Total assets at fair value	<u>\$ 145,214</u>	<u>\$ 29,564</u>	<u>\$ –</u>	<u>\$ 174,778</u>
Liabilities				
Interest rate swap agreements	<u>\$ –</u>	<u>\$ 5,133</u>	<u>\$ –</u>	<u>\$ 5,133</u>

The following is a description of the Medical Center's valuation methodologies. Fair value for Level 1 is based upon quoted market prices. Fair value for Level 2 is based on quoted prices for similar instruments in active markets, quoted prices for identical or similar instruments in markets that are not active, and model-based valuation techniques for which all significant assumptions are observable in the market or can be corroborated by observable market data for substantially the full term of the assets. Inputs are obtained from various sources including market participants, dealers and brokers. The methods described above may produce a fair value that may not be indicative of net realizable value or reflective of future fair values. Furthermore,

Maimonides Medical Center

Notes to Interim Financial Statements (continued)

Note E – Fair Value Measurements (continued)

while the Medical Center believes its valuation methods are appropriate and consistent with other market participants, the use of different methodologies or assumptions to determine the fair value of certain financial instruments could result in a different estimate of fair value at the reporting date.

The Medical Center's long-term debt obligations are reported at carrying value. The fair value of the Medical Center's long-term debt obligations is approximately \$215.2 million and \$185.5 million at November 30, 2012 and December 31, 2011, respectively, based on quoted market prices and other valuation considerations. These fair values are categorized as Level 2 in the fair value hierarchy described above.

Note F – Long-Term Debt, Capital Lease Obligations, Other Borrowings and Related Matters

A summary of long-term debt is as follows:

	November 30, 2012	December 31, 2011
	<i>(In Thousands)</i>	
1996 FHA Section 242 insured mortgage loan	\$ 10,029	\$ 10,669
2004 FHA Section 241 insured mortgage loan	91,647	94,041
Construction bank loans (a)	20,674	2,822
Capital lease obligations (b)	61,239	45,836
2006 New York City Capital Resource Corporation loan (a)	–	26,345
Syndicated bank credit facility (a)	26,531	–
	<u>210,120</u>	<u>179,713</u>
Less current portion	26,162	21,181
	<u>\$ 183,958</u>	<u>\$ 158,532</u>

(a) On May 25, 2006, the Medical Center entered into a loan agreement in the amount of \$31.2 million. The proceeds of the loan, which were raised by the New York City Capital Resource Corporation through the issuance of tax-exempt revenue bonds, were used to renovate, improve, repair and equip the Medical Center's facilities. This loan was prepaid and refinanced on June 1, 2012 with a syndicated bank credit facility that bears interest at November 30, 2012 at

Maimonides Medical Center

Notes to Interim Financial Statements (continued)

Note F – Long-Term Debt, Capital Lease Obligations, Other Borrowings and Related Matters (continued)

1.91%, with monthly payments through 2019. The syndicated arrangement also includes a construction loan of \$20.0 million that bears interest at November 30, 2012 at 1.91%, with monthly payments through 2019. In connection with the refinancing, the Medical Center wrote off deferred financing costs of approximately \$883,000, which is included in interest and amortization of deferred financing costs in the accompanying financial statements. As part of these financing transactions, the Medical Center restructured its existing lines of credit with two banks (\$40.0 million) to one line of credit for \$30.0 million; as of November 30, 2012, approximately \$18.0 million was outstanding.

(b) The Medical Center has various capital lease obligations with interest rates ranging up to 6.7%.

Annual maturities on all long-term debt for the years 2013 through 2017 are as follows (in thousands):

2013	\$ 26,162
2014	22,672
2015	21,866
2016	17,617
2017	14,355

Note G – Pension Plans and Postretirement Health Care Benefits

The Medical Center provides retirement and similar benefits to its union employees through several defined benefit multiemployer pension plans and to its nonunion employees through a noncontributory defined benefit pension plan, tax deferred annuity plans, and a nonqualified defined contribution plan covering certain employees. The Medical Center contributes amounts to the nonunion plan sufficient to meet the minimum funding requirements set forth in the Employee Retirement Income Security Act of 1974. The Medical Center's pension expense under all existing plans aggregated approximately \$34.5 million and \$29.6 million for the eleven months ended November 30, 2012 and 2011, respectively.

In addition to the plans described above, the Medical Center sponsors a defined benefit health care plan that provides postretirement medical, dental and life insurance benefits to certain full-time employees hired prior to July 1, 1984 and who have worked ten years and attained age 65 while in service with the Medical Center. The plan contains cost-sharing features such as deductibles and coinsurance.

Maimonides Medical Center

Notes to Interim Financial Statements (continued)

Note G – Pension Plans and Postretirement Health Care Benefits (continued)

The following table provides an estimate of the components of the net periodic costs for pension and postretirement benefits for the eleven months ended November 30, 2012 and 2011:

	Pension Benefits		Postretirement Health	
	2012	2011	2012	2011
	<i>(In Thousands)</i>			
Service cost	\$ 6,060	\$ 5,083	\$ 210	\$ 174
Interest cost on projected benefit obligation	4,691	4,575	469	509
Expected return on plan assets	(5,350)	(4,628)	–	–
Net amortization and deferral	3,587	2,230	121	39
Net periodic pension cost	<u>\$ 8,988</u>	<u>\$ 7,260</u>	<u>\$ 800</u>	<u>\$ 722</u>

Note H – Medical Resident FICA Refund

At December 31, 2011, the Medical Center had receivables recorded related to FICA medical resident refund claims for employer and employee portions of taxes paid. In 2012, the Medical Center received payment for a portion of the refund claims and recorded cash received of approximately \$10.9 million, included in assets limited as to use, and a liability of approximately \$10.9 million, included in other current liabilities, related to the portion of the refunds collected on behalf of and, therefore, to be remitted to the medical residents.

Note I – Commitments and Contingencies

The Medical Center is involved in various litigation and claims that are not considered unusual given the nature, complexity and size of the Medical Center's business. While the ultimate effect of such actions cannot be determined at this time, management believes that the ultimate resolution of these matters will not have a material effect on the accompanying financial statements or on the Medical Center's financial position or results of operations.

Note J – Subsequent Events

The Medical Center has evaluated events and transactions subsequent to November 30, 2012 through February 20, 2013, the date the financial statements were issued. No events have occurred that require disclosure in, or adjustment to, the financial statements.

APPENDIX D
CERTAIN DEFINITIONS

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APPENDIX D

CERTAIN DEFINITIONS

The following is a summary, which does not purport to be comprehensive or definitive, of certain definitions under the Indenture, which is qualified in its entirety by reference to the Indenture.

“Acquisition Fund” means the trust fund by that name established pursuant to the Indenture.

“Act of Bankruptcy” means the filing of a petition in bankruptcy (or other commencement of a bankruptcy or similar proceeding) by or against the Issuer, any guarantor of the Issuer or such other Person, as applicable, under any applicable bankruptcy, insolvency or similar law as now or hereafter in effect.

“Authorized Denomination” means \$5,000 or any integral multiple of \$5,000 in excess thereof except as otherwise permitted under the Indenture.

“Authorized Issuer Representative” means the President or Executive Vice President/Chief Financial Officer of the Issuer, or such other person as shall be designated to the Trustee by a Certificate of the Issuer (as defined in the Indenture) containing the specimen signature of such person, which certificate may designate an alternate or alternates.

“Bond Fund” means the trust fund by that name established pursuant to the Indenture.

“Bondholder” or *“holder”* or *“registered owner,”* when used with respect to any Bond, means the person or persons in whose name(s) such Bond is registered.

“Bond Obligation” means, as of any date of calculation, the aggregate principal amount of all outstanding Bonds.

“Bond Purchase Agreement” means the Bond Purchase Agreement with respect to the Bonds between the Issuer and the Underwriter (as defined in the Indenture).

“Bond Register” and *“Bond Registrar”* have the respective meanings specified in the Indenture.

“Bond Resolution” means the Resolution, adopted by the Governing Body (as defined in the Indenture) of the Issuer on November 5, 2012, authorizing the issuance of the Bonds and the execution of this Indenture, the Financing Agreement and any other documents in connection therewith.

“Bonds” means the Issuer’s \$100,535,000 aggregate principal amount of Maimonides Medical Center GNMA Collateralized Taxable Revenue Bonds, Series 2013.

“*Business Day*” means a day, other than a Saturday or Sunday, on which (a) banks located in New York, New York, or in the city in which the Trust Office of the Trustee is located, are not required or authorized by law or executive order to close for business and (b) the New York Stock Exchange is not closed.

“*Bylaws*” means the Issuer’s Bylaws as amended on March 12, 2012.

“*Certificate of the Issuer*,” “*Request of the Issuer*,” “*Requisition of the Issuer*” and “*Statement of the Issuer*” mean, respectively, a written certificate, request, requisition or statement signed in the name of the Issuer by an Authorized Issuer Representative. Any such instrument and supporting opinions or representations, if any, may, but need not, be combined in a single instrument with any other instrument, opinion or representation, and the two or more so combined shall be read and construed as a single instrument.

“*Closing Date*” means March 21, 2013, the date of issuance and delivery of the Bonds in exchange for the purchase price thereof.

“*Costs of Issuance*” means, with respect to any Bonds, all expenses incurred in connection with the authorization, sale, issuance and delivery of the Bonds to the extent not paid from other sources, including, without limitation, the Underwriter’s spread, discount or fees (including any premium on the resale of the first delivery of the Bonds), Counsel (as defined in the Indenture) fees and expenses (including Underwriter’s counsel, Issuer’s counsel, Transaction Counsel (as it relates to the Bonds), Trustee’s counsel, as well as any other specialized Counsel fees and expenses incurred in connection with the issuance of the Bonds), rating agency fees, accountant fees related to issuance of the Bonds and printing costs (for the Bonds and preliminary and final offering materials) and Trustee’s acceptance and first year annual administration fees, title insurance fees, and recording and filing fees.

“*Costs of Issuance Fund*” means the trust fund by that name established pursuant to the Indenture.

“*Counsel*” means an attorney or firm whose members are attorneys duly admitted to practice law before the highest court of any state of the United States of America and, without limitation, may include independent legal counsel, Issuer attorney or general counsel for the Issuer or the Trustee, as appropriate.

“*DASNY*” means the Dormitory Authority of the State of New York.

“*Defaulted Interest*” means any interest on any Bond which is payable, but is not punctually paid or duly provided for, on any Interest Payment Date (as defined in the Indenture).

“*Event of Default*” means any of the events so specified or defined in the Indenture.

“*FHA*” means the Federal Housing Administration, together with its successors and assigns.

“*FHA Loan Documents*” means, collectively, the Mortgage Notes, the Mortgages, the FHA Regulatory Agreement and all other documents required in connection with the

modification of the Mortgage Loans by FHA for Mortgage Insurance (as defined in the Indenture), and any amendments thereto.

“FHA Loan Modification Documents” means, collectively, all documents required in connection with the modification of the Mortgage Loans by FHA for Mortgage Insurance for the subject transaction.

“FHA Regulations” means the regulations promulgated by FHA regarding insurance under Section 241 and/or Section 242 of the National Housing Act, as applicable.

“FHA Regulatory Agreement” means the regulatory agreement entered into between the Issuer and HUD relating to the Mortgage Loans as it has and may be amended.

“Financing Agreement” means that certain Financing Agreement of even date with the Indenture, together with any and all Supplements thereto, among the Issuer, the Trustee and the Lender.

“Financing Documents” means this Indenture, the Financing Agreement, the Bond Purchase Agreement, the GNMA Guaranty Agreement, the FHA Loan Documents and the GNMA Securities (all as defined in the Indenture).

“GNMA” means the Government National Mortgage Association, together with its successors and assigns.

“GNMA Document or Documents” means any document, or, collectively, documents which have been or are required by GNMA to be executed by the Issuer, FHA, GNMA and/or the Lender in connection with the Project.

“GNMA Guaranty Agreement” means the Schedule of Subscribers and GNMA Guaranty/Contractual Agreement between GNMA and the Lender, together with all Supplements thereto.

“GNMA Security” or *“GNMA Securities”* means one or more fully modified pass-through securities in the form of a PLC (as defined in the Indenture) issued by the Lender, registered in the name of the Trustee or its designee and guaranteed by GNMA as to timely payment of principal and interest, pursuant to Section 306(g) of the National Housing Act, and the regulations promulgated thereunder, backed by the Mortgage Loans made by the Lender to finance the Project in accordance with the Financing Agreement and the FHA Loan Documents, which Mortgage Loans are insured by the Secretary of HUD acting by and through the FHA.

“GNMA Securities Delivery Date” means the earlier of (a) the date on which the GNMA Securities are delivered to the Trustee or (b) April 4, 2013, or such later date as may be permitted by the provisions of the Indenture.

“GNMA Securities Issue Date” means the first day of the month in which the GNMA Securities are issued, but in no event later than April 1, 2013, unless extended pursuant to the provisions of the Indenture.

“*Governing Body*” of the Issuer means its board members or any body succeeding to the functions thereof.

“*Government Obligations*” means bonds, notes and other evidences of indebtedness of the United States of America or of any agency or instrumentality thereof backed by the full faith and credit of the United States of America and guaranteed as to full and timely payment of principal thereof and interest thereon.

“*HUD*” means the United States Department of Housing and Urban Development, together with any authorized representative thereof or any successor thereto.

“*Indenture*” has the meaning set forth in the introductory paragraph thereto.

“*Interest Payment Date*” means each March 20 and September 20, commencing September 20, 2013.

“*Issuer*” has the meaning set forth in the introductory paragraph to the Indenture.

“*Lender*” means Prudential Huntoon Paige Associates, LLC, together with its successors and assigns.

“*Lender Commitment*” means the financing commitment letter relating to the Mortgage Loans from the Lender to the Issuer.

“*MBS Submission Schedule*” means the Document Delivery Schedule for Ginnie Mae I Serial Note, Pools issued by GNMA from time to time.

“*Moody’s*” means Moody’s Investors Service Inc., and its successors and assigns, to the extent they are providing a rating on the Bonds.

“*Mortgages*” means the mortgages on the Project securing the Mortgage Notes, as amended.

“*Mortgage Insurance*” means the insurance of the Mortgage Loans provided by the FHA, as evidenced by the endorsed Mortgage Notes.

“*Mortgage Loans*” means the two existing mortgage loans with the Issuer in aggregate principal amounts up to \$90,746,371.22 and \$9,787,310.39.

“*Mortgage Notes*” means the Issuer’s mortgage notes, as each has and may be amended.

“*National Housing Act*” means the National Housing Act of 1934, as amended.

“*Outstanding*,” when used with respect to the Bonds, means all Bonds theretofore authenticated and delivered under this Indenture, except:

- (a) Bonds theretofore cancelled by the Trustee or theretofore delivered to the Trustee for cancellation;
- (b) Bonds for the payment or redemption of which moneys or obligations shall have been theretofore deposited with the Trustee in accordance with Article IX;
- (c) Bonds in exchange for or in lieu of which other Bonds have been authenticated and delivered under this Indenture; and
- (d) Bonds replaced pursuant to the Indenture.

“*Pass-Through Rate*” means the rate of interest on the GNMA Securities, which shall be 3.345% or such other rate as permitted under the Mortgage Notes.

“*Person*” means any natural person, firm, association, corporation or public body.

“*PLC*” means a project loan certificate (which may bear an “LM” designation) which is a GNMA Security which shall bear interest at the Pass-Through Rate and shall be in a principal amount equal to the full principal amount of the Mortgage Loans.

“*Prior Bonds*” means the Dormitory Authority of the State of New York Maimonides Medical Center FHA-Insured Mortgage Hospital Revenue Bonds, Series 1996A and 1996B and the Dormitory Authority of the State of New York Maimonides Medical Center FHA-Insured Mortgage Hospital Revenue Bonds, Series 2004.

“*Project*” means the construction and renovation of a hospital facility known as the Maimonides Medical Center.

“*Property*” means any and all rights, title and interests in and to any and all assets of Issuer, whether real or personal, tangible or intangible which secures the Mortgages.

“*Qualified Investments*” means any of the following which at the time of investment are legal investments under the laws of the State of New York for the investment of the Issuer’s funds:

- (a) Government Obligations;
- (b) senior, unsecured obligations issued by the Federal Home Loan Bank;
- (c) interest-bearing time deposits, reverse repurchase agreements, rate guarantee agreements or other similar banking arrangements with a bank or trust company having capital and surplus aggregating at least \$50 million or with any government bond dealer reporting to, trading with and recognized as a primary dealer by the Federal Reserve Bank of New York having capital aggregating at least \$50 million or with any corporation which is subject to registration with the Board of Governors of the

Federal Reserve System pursuant to the requirements of the Bank Holding Company Act of 1956 and whose unsecured or uncollateralized long-term debt obligations are assigned a rating by the Rating Agencies (as defined in the Indenture) of at least the long-term sovereign credit rating of the United States of America; provided that each such interest-bearing deposit, repurchase agreement, guarantee agreement or other similar banking arrangement shall (a) be approved in writing by the Rating Agencies and (b) permit the moneys so placed to be available for use at the time provided with respect to the investment or reinvestment of such moneys; and

(d) no-load, open-end money market mutual funds (including those of the Trustee and its affiliates) registered under the Investment Company Act of 1940, provided the portfolio of such fund is limited to Government Obligations and such fund has been assigned a rating by the Rating Agencies of at least the long-term sovereign credit rating of the United States of America.

Qualified Investments shall not include the following: (i) any investments with a final maturity or any agreements with a term greater than 365 days from the date of the investments (except (A) obligations that provide for the mandatory tender, at par, by the holder thereof at least once within 365 days of the date of purchase, (B) any investments listed in subparagraph (b) above that are irrevocably deposited with the Trustee for payment of Bonds pursuant to the Indenture, and (C) agreements listed in subparagraphs (c) and (d) above), (ii) any obligation with a purchase price greater than the par value of such obligation, (iii) mortgage-backed securities, real estate mortgage investment conduits or collateralized mortgage obligations, (iv) interest-only or principal-only stripped securities, (v) obligations bearing interest at inverse floating rates, or (vi) investments which may be prepaid or called at a price less than its purchase price prior to stated maturity.

“*Rating Agencies*” means Standard & Poor’s Rating Services, a division of The McGraw-Hill Companies, Inc., and Moody’s Investors Service Inc., and their successors and assigns, to the extent they are providing a rating on the Bonds.

“*Regular Record Date*” means, with respect to an Interest Payment Date, the close of business on March 5 or September 5, as the case may be, next preceding such Interest Payment Date, whether or not a Business Day.

“*Responsible Officer*” of the Trustee shall mean and include any Vice President, Assistant Vice President, Trust Officer or other officer of the Trustee having regular responsibility for corporate trust matters related to this Indenture.

“*Retirement Date*” means the date on which the last Bond is retired.

“*Revenues*” means, the revenues, receipts, interest, income, investment earnings and other moneys received or to be received by the Issuer or the Trustee under the Financing Documents (subject to the limitations contained in the Financing Documents and the FHA Loan Documents), including moneys received or to be received from the GNMA Securities and all investment earnings derived or to be derived on any moneys or investments held by the Trustee

under the Indenture, but excluding amounts paid as fees or reimbursement for expenses or for indemnification of the Trustee.

“S&P” means Standard & Poor’s Rating Services, a division of The McGraw-Hill Companies, Inc., and its successors and assigns, to the extent they are providing a rating on the Bonds.

“*Seasoned Funds*” means (a) any moneys received as a payment under a GNMA Security (including moneys representing the payment of premium on the Mortgage Notes) (b) moneys with respect to which there has been delivered to the Trustee an opinion of nationally recognized bankruptcy Counsel in a form acceptable to the Rating Agencies to the effect that payment of such moneys to the Bondholders in payment of principal of, premium, if any, or interest on the Bonds will not constitute a preferential payment recoverable under Section 547 of the United States Bankruptcy Code and will not be subject to, or will promptly be released from, the automatic stay or transfer provisions provided for in Sections 362(a) and 550(a), respectively, of the United States Bankruptcy Code in the event of the bankruptcy of the Person providing such funds, (c) moneys deposited on behalf of the Issuer by a Person other than the Issuer with the Trustee and so designated by the Issuer, which moneys shall have been held by the Trustee for at least 366 days prior to the date such moneys are to be used to make payments on the Bonds, provided that no Act of Bankruptcy shall have occurred with respect to such Person during such 366-day period after such moneys were deposited with the Trustee (as evidenced by a certificate of such Person to the effect that no Act of Bankruptcy has occurred during such period), (d) proceeds from any letter of credit, (e) proceeds from the sale of bonds (including any premium thereon) or (f) earnings on any amounts listed in (a) through (e) above.

“*Section 241 Mortgage Loan*” means bonds issued in 2004 by DASNY to finance a mortgage loan to the Issuer for FHA Project No. 012-10030 in the amount of \$104,704,000, which mortgage loan is secured by a FHA-insured Section 241 mortgage.

“*Section 242 Mortgage Loan*” means bonds issued in 1996 by DASNY to finance a mortgage loan to the Issuer for FHA Project No. 012-13058 in the amount of \$32,676,000, which mortgage loan is secured by a FHA-insured Section 242 mortgage.

“*Special Record Date*” (a) in the case of Defaulted Interest, has the meaning specified in the Indenture, and (b) in the case of each redemption, such record date as shall be specified by the Trustee in the notice of redemption required by the Indenture, provided that such record date shall be not less than 15 calendar days before the mailing of such notice of redemption. However, the Trustee may specify a record date that is less than 15 calendar days before the mailing of such notice of redemption if the circumstances do not permit the Trustee to specify a record date in accordance with clause (b) of the preceding sentence.

“*State*” means the State of New York.

“*Supplements*” means all extensions, renewals, modifications, amendments, supplements and substitutions.

“Transaction Counsel” means Eichner Norris & Neumann PLLC, Washington, D.C., or any other firm of attorneys selected by the Issuer, duly admitted to practice law before the highest court of any state of the United States of America.

“Trustee” has the meaning set forth in the introductory paragraph to the Indenture.

“Trustee Fee” means the annual fees and expenses of the Trustee with respect to the Bonds, payable annually, in advance, on the Closing Date and each September 20 thereafter commencing September 20, 2013, in an amount equal to \$3,500, provided, however, in the event of any unscheduled prepayment of the GNMA Securities, such fee shall be proportionally reduced with the difference being immediately due and payable by the Trustee to the Issuer.

“Trust Estate” means the property rights, money, securities and other amounts pledged and assigned to the Trustee pursuant to the granting clauses of the Indenture.

“Trust Office” means the appropriate trust office of the Trustee located at the address set forth in the Indenture, or such other offices as may be specified in writing to the Issuer by the Trustee.

“Underwriter” means Merrill Lynch, Pierce, Fenner & Smith Incorporated.

APPENDIX E

SUMMARY OF CERTAIN PROVISIONS OF THE INDENTURE

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APPENDIX E

SUMMARY OF CERTAIN PROVISIONS OF THE INDENTURE

The following is a summary, which does not purport to be comprehensive or definitive, of certain provisions of the Indenture, which is qualified in its entirety by reference to the Indenture.

Security for the Bonds

Under the Indenture, as security for payment of the principal of, premium, if any, and interest on the Bonds and for funds advanced by the Trustee pursuant thereto, the Issuer bargains, sells, conveys, pledges and assigns to, and grants a security interest to the Trustee in, the following described property:

(a) All right, title and interest of the Issuer in and to all Revenues (as defined in the Indenture), derived or to be derived by the Issuer or the Trustee for the account of the Issuer under the terms of the Indenture and the Financing Agreement, together with all other Revenues received by the Trustee for the account of the Issuer arising out of or on account of the Trust Estate;

(b) All right, title and interest of the Issuer in and to the GNMA Securities, including all payments and proceeds with respect thereto and any interest, profits or other income derived from the investment thereof; and

(c) All funds, moneys and securities and any and all other rights and interests in property whether tangible or intangible from time to time by delivery or by writing of any kind conveyed, mortgaged, pledged, assigned or transferred as and for additional security under the Indenture for the Bonds by the Issuer or by anyone on its behalf or with its written consent to the Trustee, which is authorized under the Indenture to receive any and all such property at any and all times and to hold and apply the same subject to the terms of the Indenture.

Establishment of Funds

The following funds and accounts shall be established and maintained by the Trustee under the Indenture for the benefit of the Bonds:

- (a) Acquisition Fund;
- (b) Bond Fund; and
- (c) Costs of Issuance Fund.

The funds and accounts created under the Indenture shall be held in trust in the custody of the Trustee. The Issuer authorizes and directs the Trustee to withdraw moneys from said funds and accounts for the purposes specified in the Indenture, which authorization and direction the Trustee thereby accepts. All moneys required to be deposited with or paid to the Trustee under any provision of the Indenture shall be held by the Trustee in trust and shall, while held by the Trustee, constitute a part of the Trust Estate and be subject to the lien thereof.

Acquisition Fund

(a) The Trustee shall deposit into the Acquisition Fund the amounts required by the Indenture and any amounts paid to the Trustee for deposit into the Acquisition Fund in accordance with the Indenture and shall invest such proceeds in Qualified Investments at the specific written direction of the Issuer. The Trustee shall request funds invested in Qualified Investments in accordance with the terms thereof such that funds will be timely available in advance of the date such funds are needed under the Indenture.

(b) Subject to the requirements of the Financing Agreement, moneys in the Acquisition Fund shall be disbursed by the Trustee to acquire GNMA Securities from the Lender. Immediately upon delivery of the GNMA Securities to the Trustee, the Trustee shall remit to the Lender as payment for the GNMA Securities, an amount equal to the current balance on the GNMA Securities, plus accrued and unpaid interest thereon at the Pass Through Rate. Accrued and unpaid interest on the GNMA Securities shall be paid from the Bond Fund.

(c) If the GNMA Securities are not delivered to the Trustee on or before the GNMA Securities Delivery Date (or such later date as may be established as described in paragraph (d) below), the Trustee shall, on the Business Day immediately after the GNMA Securities Delivery Date (or such later date as may be established as described in paragraph (d) below), transfer to the Bond Fund amounts on deposit in the Acquisition Fund for application to the mandatory redemption of Bonds in accordance with the Indenture.

(d) The GNMA Securities Delivery Date shall be extended and such corresponding transfer and redemption described in (c) above shall be delayed one or more times, provided that the Trustee shall have received no later than the Business Day next preceding the foregoing date, respectively (or any date to which such date has been extended pursuant to the provisions of the Indenture), a request from either the Lender or the Issuer for such delay (whether or not a conflicting request is received from such other party) accompanied by (i) a cash flow projection demonstrating that the sum of (A) the amount in the Acquisition Fund and the Bond Fund, (B) the investment earnings to accrue on the amounts held in the Acquisition Fund and the Bond Fund during the period ending 30 days after the end of any period of delay requested, (C) any additional sums paid to or held by the Trustee by or on behalf of the Issuer or the Lender for deposit into the Acquisition Fund or Bond Fund (which are in the form of Seasoned Funds) and (D) all scheduled payments on the GNMA Securities assuming its issuance on the last day of such extension, will be at least equal to (1) the debt service on the Bonds as originally scheduled plus originally scheduled and accrued Trustee Fees and (2) will also be at least equal to, without regard to scheduled payments on the GNMA Securities, the debt service on the Bonds through the date which is 15 days after the end of any such extension period, plus originally scheduled and accrued unpaid Trustee fees (assuming redemption of all Bonds on the date set forth in this clause (2)) and any other amounts which were shown to be available at such time for debt service on the Bonds in the original cash flows prepared in connection with the issuance of the Bonds; (ii) instructions satisfactory to the Trustee for the making of the investments contemplated by the cash flow projection; and (iii) written notice from the Rating Agencies that the rating then assigned to the Bonds will not be lowered, suspended or withdrawn as a result of such extensions. Upon the receipt of the documents and upon the arrangements listed in this subsection, the Trustee shall permit the extension(s); provided, however, that if such documents have not been received and such arrangements have not been made by the Business Day next preceding the GNMA Securities Delivery Date (or any date to which such date has previously been extended), then the moneys remaining on deposit in the Acquisition Fund on such date shall be transferred to the Bond Fund on the Business Day next preceding the foregoing date, and applied to the

redemption of Bonds on such dates (except as such dates may be extended pursuant to the provisions of the Indenture).

The Trustee shall provide the Lender with the Trustee's written consent to the extension upon its receipt of the items described in this section in (i) through (iii) above.

On the GNMA Securities Delivery Date, amounts remaining in the Acquisition Fund after purchase of the GNMA Securities shall be transferred to the Bond Fund.

(e) The Trustee shall not be required to acquire a GNMA Security unless the GNMA Security pays interest at the Pass Through Rate and matures on March 15, 2032. All GNMA Securities shall be registered in the name of the Trustee or its designee.

(f) If the GNMA Securities are not delivered by the GNMA Securities Delivery Date, as such date may be extended pursuant to the Indenture, the Trustee shall transfer the proceeds remaining in the Acquisition Fund to the Bond Fund pursuant to the Indenture to redeem Bonds in accordance therewith.

(g) The Trustee shall compare each GNMA Security or its book-entry form with the GNMA prospectus relating to the GNMA Securities and GNMA Guaranty Agreement provided by the Lender to assure delivery of correct GNMA Securities.

Bond Fund

(a) The Trustee shall deposit, upon receipt, into the Bond Fund (i) the amounts required by the Indenture; (ii) all income, revenues, proceeds and other amounts received from or in connection with the GNMA Securities; (iii) all earnings and gains from the investment of moneys held in the Bond Fund and the Acquisition Fund; and (iv) any other amounts received by the Trustee which are subject to the lien and pledge of the Indenture.

(b) The Trustee shall apply moneys on deposit in the Bond Fund to pay accrued interest, if any, upon its acquisition of the GNMA Securities.

(c) The Trustee shall apply all other amounts on deposit in the Bond Fund after application of (b) above on each Interest Payment Date (or any other date on which Bonds are to be redeemed) to first pay the principal of and premium, if any, and interest on the Bonds becoming due and payable in accordance with the Indenture.

(d) The Trustee shall pay the Trustee Fee, when due.

(e) Following each scheduled redemption under the Indenture, the Trustee shall transfer any amounts remaining in the Bond Fund in excess of \$750,000 (or such other amount permitted by the Rating Agencies and as notified in writing to the Trustee) to the Issuer or its designee.

(f) The GNMA Securities shall be held at all times for the benefit of the Bond Fund. If the Trustee does not receive a payment on any of the GNMA Securities when due by the close of business on the sixteenth day of any month, the Trustee shall promptly notify and demand payment from the Lender. If the Lender shall not make such payment by the next Business Day, the Trustee shall promptly notify and demand payment from GNMA.

(g) The Trustee shall deposit into the Bond Fund Seasoned Funds representing payments made to the Trustee by the Issuer for deposit therein as specified by the Issuer. Such Seasoned Funds shall be applied only to pay principal of and premium, if any, and interest on the Bonds as the same shall become due and payable in accordance with the Indenture.

(h) If any GNMA Securities are in book-entry form, the GNMA Securities must be registered in the name of the Trustee at the Federal Reserve Bank at the time of purchase of the GNMA Securities by the Trustee, and the Trustee shall have a first lien position perfected security interest in the GNMA Securities.

(i) The Trustee shall not transfer any moneys from the Bond Fund except in accordance with the provisions of the Indenture.

Costs of Issuance Fund

The Trustee shall deposit in the Costs of Issuance Fund the amount set forth in the Indenture and shall pay costs of issuance and defeasance of the Prior Bonds promptly upon the written direction of the Issuer. Any amounts remaining in the Costs of Issuance Fund 90 days after the Closing Date, and not specifically committed to the payment of costs of issuance and defeasance of the Prior Bonds, shall be released to the Issuer.

Investment of Funds

The Trustee shall invest all moneys in the Acquisition Fund and the Bond Fund in Qualified Investments at the direction of the Issuer. Subject to the provisions of the foregoing sentence, any moneys held as part of any fund created by the Indenture shall be invested or reinvested from time to time by the Trustee upon receipt by the Trustee of the written direction of the Issuer in Qualified Investments having a maturity not exceeding the date on which such funds may be needed under the Indenture. In no event shall a maturity be longer than the longest maturity of the Bonds. The investments so made shall be held by the Trustee and shall be deemed at all times to be a part of the fund (which shall mean the applicable subaccount to the extent applicable) in which such moneys were held. The Trustee shall sell and reduce to cash a sufficient amount of such investments whenever the cash balance in any fund shall be insufficient to cover a proper disbursement therefrom. For the purpose of determining the amount in any fund, Qualified Investments credited to such fund or account shall be valued at their cost (exclusive of accrued interest after the first payment of interest following acquisition) or market value, whichever is less. The Issuer shall invest at the highest yields prudently available consistent with the Indenture.

The Trustee will furnish the Issuer with periodic cash transaction statements which include details for all investment transactions made by the Trustee under the Indenture.

All investment earnings shall be deposited as provided in the Indenture.

No Disposition of GNMA Securities

The Trustee hereby agrees that any GNMA Securities delivered to the Trustee under the Indenture shall at all times be held by the Trustee for the sole benefit of the holders of the Bonds. There shall be no disposition of GNMA Securities for an amount less than an amount sufficient, together with other amounts then held under the Indenture and available for the payment of the principal of, premium, if any, and interest on the Bonds, to provide for the payment of the Bonds in accordance with the Indenture without the consent of the holders of 100% of the Bonds.

Events of Default

Each of the following shall be an “Event of Default” under the Indenture:

- (a) default in the due and punctual payment of any interest on any Bond;
- (b) default in the due and punctual payment of the principal of or premium, if any, on any Bond whether at the stated maturity thereof, or on proceedings for redemption thereof, or on the maturity thereof by declaration;
- (c) default in the performance or observance of any other of the covenants, agreements or conditions on the part of the Issuer in the Indenture or in the Bonds and such default continues for a period of thirty (30) days; provided however, if such default cannot be cured within such thirty (30) period, the same shall not constitute an “Event of Default” so long as the Issuer has commenced prompt corrective action and is diligently pursuing such action (subject to the Indenture).

The Issuer’s failure to pay obligations owing to the Lender under the Mortgage Loans or to otherwise comply with the terms of the FHA Loan Documents shall not constitute an Event of Default under the Indenture.

The Trustee shall give written notice to the Rating Agencies of the occurrence of any Event of Default described in subsection (a) or (b) above, within 15 days after a Responsible Officer of the Trustee has actual notice or knowledge thereof.

Acceleration

If an Event of Default described in subsection (a) or (b) above has occurred and is continuing with respect to a Bond, the Trustee may, and upon the written request of the holders of a majority of the Bond Obligation the Trustee shall, by notice in writing delivered to the Issuer, declare the principal of all Bonds then outstanding and the interest accrued thereon immediately due and payable without premium, and such principal and interest shall thereupon become and be immediately due and payable.

The foregoing provisions of the Indenture, however, are subject to the condition that if at any time after the principal of the Bonds shall have been so declared due and payable, and before any judgment or decree for the payment of the money due shall have been obtained or entered as provided in the Indenture, there shall be paid or deposited with the Trustee a sum sufficient to pay all principal of the Bonds matured (or due upon mandatory redemption) prior to such declaration and all matured installments of interest (if any) upon all the Bonds, with interest at the rate borne by the Bonds on such overdue principal and premium, if any, and (to the extent legally enforceable) on such overdue installments of interest (other than in the payment of principal of and interest on the Bonds due and payable solely by reason of such declaration), and the reasonable fees and expenses of the Trustee shall have been made good or cured or adequate provisions shall have been made therefor, then, and in every case, the holders of at least a majority of the Bond Obligation, by written notice to the Trustee and the Issuer, may direct the Trustee on behalf of the holders of all the Bonds to rescind and annul such declaration and its consequences, but no such rescission and annulment shall extend to or shall affect any subsequent default, nor shall it impair or exhaust any right or power consequent thereon. Nothing in the Indenture shall be construed to obligate the Issuer to make a payment or deposit referred to in the Indenture from any revenues other than the revenues derived from the Trust Estate.

Notwithstanding anything to the contrary in the Indenture, no acceleration of the Bonds shall result in an acceleration or prepayment of the Mortgage Loans and/or the GNMA Security.

Remedies

Upon the occurrence of an Event of Default, the Trustee shall have the power to proceed with any right or remedy granted by the Constitution and laws of the State, as it may deem best, including, without limitation, any suit, action or special proceeding in equity or at law for the specific performance of any covenant or agreement contained in the Indenture or under the GNMA Securities or for the enforcement of any proper legal or equitable remedy as the Trustee shall deem most effectual to protect the rights aforesaid, insofar as such may be authorized by law, and specifically including the right to bring action on behalf of Bondholders against third parties.

No remedy by the terms of the Indenture conferred upon or reserved to the Trustee or to the Bondholder is intended to be exclusive of any other remedy, but each and every such remedy shall be cumulative and shall be in addition to any other remedy given to the Trustee or to the Bondholders under the Indenture or now or hereafter existing at law or in equity or by statute. No delay or omission to exercise any right or power accruing upon any default or Event of Default shall impair any such right or power or shall be construed to be a waiver of any such default or Event of Default or acquiescence therein, and every such right and power may be exercised from time to time and as often as may be deemed expedient. No waiver of any default or Event of Default under the Indenture, whether by the Trustee or by the Bondholders, shall extend to or shall affect any subsequent default or Event of Default or shall impair any rights or remedies consequent thereto.

Rights of Bondholders

If any Event of Default shall have occurred and, except as provided in the Indenture, if requested in writing so to do by the holders of not less than a majority of the Bond Obligation, and if indemnified as provided in the Indenture, the Trustee shall be obligated to exercise such one or more of the rights and powers conferred by the Indenture and to proceed to protect its rights and the rights of the Bondholders under applicable law, the GNMA Securities, the GNMA Guaranty Agreement, the Financing Agreement and the Indenture, as the Trustee, being advised by Counsel, shall deem most expedient in the interest of the Bondholders. Anything in the Indenture to the contrary notwithstanding, but subject to the provisions of the Indenture, the holders of a majority of such Bond Obligation shall have the right at any time, by an instrument in writing executed and delivered to the Trustee, to direct the time, method and place of conducting all proceedings to be taken in connection with the enforcement of the terms and conditions of the Indenture, or for the appointment of a receiver or any other proceedings under the Indenture, in accordance with the provisions of law, the Indenture and the GNMA Securities.

Supplemental Indentures Not Requiring Consent of Bondholders

The Issuer and the Trustee may, without the consent of or notice to any of the Bondholders, enter into an indenture or indentures supplemental to the Indenture as shall not be inconsistent with the terms and provisions thereof or materially adverse to the interests of the holders of the Bonds, including, without limitation, for any one or more of the following purposes:

- (a) to cure any ambiguity or to cure or correct any defect or inconsistent provisions contained in the Indenture or to make such provisions in regard to matters or questions arising under the Indenture as may be necessary or desirable and not contrary to or inconsistent with the Indenture or materially adverse to the Bondholders;
- (b) to change or modify any provision of the Indenture so as to harmonize to the maximum extent practicable the provisions thereof with existing rules, regulations and procedures of FHA;

(c) to add to the covenants and agreements of the Issuer in the Indenture other covenants and agreements, to surrender any right or power reserved or conferred upon the Issuer or amend or supplement any other provision thereof if the foregoing shall not, in the judgment of the Trustee, materially adversely affect the interests of the Bondholders, the Trustee being authorized to conclusively rely on an opinion of Counsel (including Counsel to the Issuer) with respect thereto;

(d) to confirm, as further assurance, any pledge of or lien on the Financing Agreement or the Revenues or of any other moneys, securities or funds subject to the lien of the Indenture;

(e) to modify any of the provisions of the Indenture relating to the use of a book-entry system for registration of the Bonds;

(f) to subject to the lien and pledge of the Indenture additional revenues, properties or collateral;

(g) to grant to or confer upon the Trustee for the benefit of the Bondholders any additional rights, remedies, powers or authority that may lawfully be granted to or conferred upon the Bondholders or the Trustee or any of them; or

(h) to modify, amend or supplement the Indenture or any indenture supplemental thereto in such manner as to permit the qualification thereof and under the Trust Indenture Act of 1939 or any similar federal statute hereafter in effect or under any state securities laws.

Supplemental Indentures Requiring Consent of Bondholders

With the consent of the holders of not less than a majority of the Bond Obligation, the Issuer and the Trustee may, from time to time, enter into supplemental indentures for the purpose of modifying, altering, amending, adding to or rescinding any of the terms or provisions contained in the Indenture or in any supplemental indenture; provided, however, that nothing contained in this paragraph shall permit, or be construed as permitting, (a) an extension of the stated maturity of or a reduction in the principal amount of or reduction in the interest rate on, or an extension of time of payment of interest on, or reduction of any premium payable on the redemption of, any Bonds, without the consent of the registered owner of such Bonds; (b) the creation of any lien on all or any portion of the Trust Estate prior to or on a parity with the lien of the Indenture, without the consent of the holders of all of the Bonds; (c) a reduction in the amount of Bond Obligation, the holders of which are required to approve any such supplemental indenture, without the consent of the holders of all the Bonds at the time Outstanding which would be affected by the action to be taken; (d) a privilege or priority of any Bond over any other Bonds without the consent of the holders of all Bonds adversely affected thereby; or (e) an amendment to the Indenture relating to disposition of the GNMA Securities or relating to acceleration upon an Event of Default under the Indenture, without in each case the consent of the holders of all the Bonds then Outstanding.

If at any time the Issuer shall request the Trustee to enter into any such supplemental indenture for any of the purposes described in this section, the Trustee shall, upon being satisfactorily indemnified with respect to expenses, cause notice of the proposed execution of such supplemental indenture to be mailed, postage prepaid, to all Bondholders. Such notice shall be prepared by the Issuer and shall briefly set forth the nature of the proposed supplemental indenture and shall state that copies thereof are on file at the Trust Office for inspection by all Bondholders. If, within 60 days following the mailing of such notice, the holders of the required portion of Bonds at the time of the execution of any such supplemental indenture shall have consented to and approved the execution thereof as provided in the Indenture, no

holder of any Bond shall have any right to object to any of the terms and provisions contained therein, or the operation thereof, or in any manner to question the propriety of the execution thereof, or to enjoin or restrain the Trustee or the Issuer from executing the same or from taking any action pursuant to the provisions thereof. Upon the execution of any such supplemental indenture as is in the Indenture permitted and provided, the Indenture shall be and be deemed to be modified and amended in accordance therewith.

Amendment of Certain Documents

(a) The Issuer and the Trustee may, with the consent of the Lender, make or consent to any amendment, change or modification of the Financing Agreement and the GNMA Securities, for the purpose of curing any ambiguity, or of curing, correcting or supplementing any defective or inconsistent provision contained therein, or in regard to matters or questions arising under said documents, as the Issuer and the Trustee may deem necessary or desirable and not inconsistent with said documents or the Indenture and which shall not materially adversely affect the interests of the holders of the Bonds.

(b) Except for the amendments, changes or modifications as provided in subsection (a) above, the Trustee shall not consent to or approve any other amendment, change or modification of any of the Financing Documents (other than the Indenture) without notice to and the written approval or consent of the Issuer, the Lender and the beneficial owners of not less than a majority of the Bond Obligation given and procured as provided in the Indenture. If, at any time, the Issuer shall request the consent to or approval of the Trustee to any such proposed amendment, change or modification of any of the Financing Documents (other than the Indenture), the Trustee shall, upon being satisfactorily indemnified by the Issuer with respect to expenses, cause notice of such proposed amendment, change or modification to be given in the same manner as provided in the Indenture with respect to supplemental indentures. Such notice shall be prepared by the Issuer and shall briefly set forth the nature of such proposed amendment, change or modification and shall state that copies of the instrument embodying the same are on file at the Trust Office of the Trustee for inspection by all Bondholders. If, within 60 days or such longer period as shall be prescribed by the Trustee following the giving of such notice, the holders of not less than a majority of the Bond Obligation at the time of the execution of such proposed amendment shall have consented to and approved the execution thereof as provided in the Indenture, subject to the provisions thereof, no holder of any Bond shall have any right to object to any of the terms and provisions contained therein, or the operation thereof, or in any manner to question the propriety of the execution thereof, or shall have any right to object to any of the terms and provisions contained therein, or the operation thereof, or in any manner to question the propriety of the execution thereof, or to enjoin or restrain the Trustee from agreeing to the execution thereof. Upon the execution of any such amendment as in the Indenture is permitted and provided, the Financing Documents shall be and be deemed to be modified and amended in accordance therewith.

(c) Anything in the Indenture to the contrary notwithstanding, the Issuer and the Trustee shall not consent to any amendment, change or modification of the GNMA Securities which would reduce the Lender's obligations to make payments thereunder or GNMA's guarantee of such payments (other than as a result of a prepayment of the GNMA Securities permitted pursuant to the terms thereof) without the consent of the holders of 100% of the Bond Obligation.

(d) Notwithstanding anything to the contrary in the Indenture, no amendment to the Financing Documents or the FHA Loan Documents which adversely affects the rights of HUD or

the Lender thereunder shall be effective without the written consent of HUD or the Lender, as appropriate.

Amendment by Unanimous Consent

Notwithstanding any other provision of the Indenture, the Issuer and the Trustee may consent to any Supplement to any Financing Document upon receipt of the consent of the Lender and the holders of all Bonds then Outstanding.

Satisfaction and Discharge of Indenture

If the Issuer (a) shall pay or cause to be paid to the holders of the Bonds the principal, interest and premium, if any, to become due thereon at the times and in the manner stipulated therein and in the Indenture, and shall pay or cause to be paid all fees and expenses of the Trustee, and (b) shall keep, perform and observe all and singular the covenants and promises in the Bonds and in the Indenture expressed as to be kept, performed and observed by it or on its part, then these presents and the estate and rights hereby granted shall cease, terminate and be void, and thereupon the Trustee shall cancel and discharge the lien of the Indenture and execute and deliver to the Issuer such instruments in writing as shall be requisite to satisfy the lien of the Indenture, shall convey to the Issuer the estate thereby conveyed and shall assign and deliver to the Issuer any interest in property at the time subject to the lien of the Indenture which may then be in its possession, except amounts held by the Trustee for the payment of principal of, premium, if any, and interest on the Bonds.

All Outstanding Bonds shall, prior to the maturity or redemption date thereof, be deemed to have been paid within the meaning and with the effect expressed in item (a) of the first paragraph of this section if the following conditions shall have been fulfilled: (i) there shall be on deposit with the Trustee either moneys which are Seasoned Funds or direct noncallable Government Obligations purchased with Seasoned Funds in an amount sufficient, as certified by a firm of certified public accountants, to pay when due the principal or redemption price, if applicable, and interest due and to become due on the Bonds on and prior to the redemption date or maturity date thereof, as the case may be; (ii) if any of the Bonds are to be redeemed on any date prior to their maturity, the Issuer shall have given to the Trustee, in form satisfactory to it, irrevocable instructions to mail, as provided in the Indenture, notice of redemption of such Bonds on such date; (iii) the Issuer shall have given the Trustee irrevocable instructions to mail, as soon as practicable, in the manner prescribed by the Indenture, a notice to the holders of such Bonds that the deposit required by this paragraph has been made with the Trustee and that the Bonds are deemed to have been paid in accordance with the Indenture and stating the redemption date upon which moneys are to be available for the payment of the principal or redemption price, if applicable, on said Bonds; and (iv) the Trustee shall have received the opinion required by the Indenture.

No deposit under this heading shall be made or accepted under the Indenture and no use made of any such deposit unless the Trustee shall have received (a) an opinion of Transaction Counsel to the effect that such deposit and use is permitted by the Indenture and that upon such deposit the lien of the Indenture shall be discharged in accordance with the Indenture and (b) executed irrevocable instructions with respect to the use of such deposit from the Issuer.

The Trustee

The Trustee hereby accepts the trust imposed upon it by the Indenture and agrees to perform said trusts upon the following terms and conditions:

(a) The Trustee may execute any of the trusts or powers under the Indenture and perform any of its duties by or through attorneys, agents, receivers or employees and shall be entitled to advice of Counsel concerning all matters of the trusts and the duties under the Indenture and may in all cases pay such reasonable compensation to all such attorneys, agents, receivers and employees as may be reasonably employed in connection with the trusts of the Indenture. The Trustee may act upon the opinion or advice of any attorney, who may be the attorney or attorneys for the Issuer, and the Trustee shall not be responsible for any loss or damage resulting from any action or inaction taken in good faith in reliance upon such opinion or advice.

(b) The Trustee shall not be responsible for any recital in the Indenture, or in the Bonds (except in respect to the authentication certificate of the Trustee endorsed on the Bonds), or for the validity of the execution by the Issuer of the Indenture or of any supplements thereto or instruments of further assurance, or for the sufficiency of the security for the Bonds issued under the Indenture or intended to be secured by the Indenture.

(c) The Trustee may become the owner or pledgee of the Bonds secured by the Indenture and otherwise deal with the Issuer with the same rights which it would have if not Trustee.

(d) To the extent permitted under the Indenture, the Trustee may conclusively rely and shall be protected in acting upon any notice, request, consent, certificate, order, affidavit, letter, telegram or other paper or document reasonably believed to be genuine and correct and to have been signed or sent by the proper person or persons, and the Trustee shall not be bound to make any investigation into the facts or matters stated in any such document so delivered and signed unless requested in writing so to do by the holders of a majority of the Bond Obligation.

(e) The permissive rights of the Trustee to do things enumerated in the Indenture shall not be construed as a duty unless so specified therein.

(f) At any and all reasonable times, the Trustee and its duly authorized agents, attorneys, experts, engineers, accountants and representatives shall have the right, but shall not be required, to inspect the Project fully, including all books, papers and records of the Issuer relating to the Project and the Bonds, and to take such memoranda from and in regard thereto as may be desired.

(g) Notwithstanding anything elsewhere in the Indenture contained, the Trustee shall have the right, but shall not be required, to reasonably demand, in respect of the authentication of any Bonds, the withdrawal of any moneys, the release of any interest in property or any action whatsoever within the purview of the Indenture, any showings, certificates, opinions, appraisals or other information, or official action or evidence thereof, in addition to those required in the Indenture.

(h) Before taking any action under the Indenture, other than any action involved in accelerating Bonds, calling Bonds for mandatory redemption, seeking payment from the Lender or GNMA and making payments of principal and interest on Bonds, the Trustee may require that

an indemnity bond satisfactory to it be furnished for the reimbursement of all expenses to which it may be put and to protect it against all liability, except liability which resulted from its negligence, bad faith or willful misconduct by reason of any action so taken.

(i) All moneys received by the Trustee or any paying agent shall, until used or applied or invested as provided in the Indenture, be held in trust for the purposes for which they were received but need not be segregated from other funds except to the extent required by law or by the Indenture. Neither the Trustee nor any paying agent shall be under any liability for interest on any moneys received under the Indenture except liability which resulted in its negligence, bad faith or willful misconduct and such as otherwise may be agreed upon in writing with the Issuer.

(j) The Trustee undertakes to perform only such duties as are specifically set forth in the Indenture and shall not be answerable for other than its negligence or bad faith in the performance of those express duties. In case an Event of Default has occurred which has not been cured, the Trustee shall exercise the rights, duties and powers vested in it by the Indenture in good faith and with that degree of diligence, care and skill which a reasonable person would exercise under similar circumstances in like situations.

(k) The Trustee shall not be responsible for insuring the Project or for collecting any insurance moneys.

(l) The Trustee shall not be required to give any bond or surety with respect to the performance of its duties or the exercise of its powers under the Indenture.

(m) In the event the Trustee receives inconsistent or conflicting requests and indemnity from two or more groups of Bondholders, each representing less than a majority of the aggregate principal amount of Bonds then Outstanding, the Trustee, in its sole discretion, may determine what action, if any, shall be taken, including the right to interplead said parties in any court of applicable jurisdiction.

(n) No personal recourse may be taken, directly or indirectly, against any officer, director, employee or agent of the Trustee with respect to the obligations of the Trustee under the Indenture, the Financing Agreement or any certificate or other writing delivered in connection therewith.

(o) The Trustee's immunities and protections from liability and its right to indemnification in connection with the performance of its duties and functions under the Indenture and the Financing Agreement shall extend to the Trustee in each of its capacities under the Indenture and to the Trustee's officers, directors, employees and agents.

(p) The Trustee's immunities and protections from liability and its right to payment of compensation and indemnification in connection with performance of its duties and functions under the Indenture and the Financing Agreement shall survive the Trustee's resignation or removal and the final payment of the Bonds.

(q) The Trustee shall have no responsibility for any information in any offering memorandum or other disclosure material distributed with respect to the Bonds, and the Trustee shall have no responsibility for compliance with any state or federal securities laws in connection with the Bonds.

(r) In acting or omitting to act under the Financing Agreement, the Trustee shall be entitled to all of the rights, protections and immunities, and to its rights of indemnification, accorded to it under the Indenture.

(s) Whether or not expressly provided for in the Indenture, every provision of the Indenture relating to the conduct of or affecting the liability of the Trustee shall be subject to the provisions of the Indenture.

(t) The Trustee shall not in any event be responsible for ensuring that the rate of interest due and payable on the Bonds under the Indenture does not exceed the highest legal rate of interest permissible under federal or state law applicable thereto.

(u) The Trustee shall not be responsible for the application of any of the proceeds of the Bonds or any other moneys deposited with it and paid out, withdrawn or transferred under the Indenture if such application, payment, withdrawal or transfer shall be made in accordance with the provisions of the Indenture.

(v) No provision of the Indenture shall require the Trustee to expend or risk its own funds or otherwise incur any financial liability in the performance of any of its duties under the Indenture, or in the exercise of any of its rights or powers.

GNMA/FHA Documents and Regulations Control; FHA Requirements

(a) To the extent that there is any conflict, inconsistency or ambiguity between or among the Indenture and (i) any applicable FHA mortgage insurance, or other applicable FHA or GNMA statutory, regulatory, administrative requirements; (ii) any of the FHA Loan Documents which have been or are required by FHA and/or the Lender to be executed by the Issuer, FHA and/or the Lender in connection with the subject transaction; or (iii) any of the GNMA Documents which have been or are required by GNMA to be executed by the Issuer, FHA, GNMA and/or the Lender in connection with the subject transaction, said FHA mortgage insurance and other applicable FHA and GNMA statutory, regulatory and administrative requirements and said FHA Loan Documents and GNMA Documents will be deemed to be controlling and any such ambiguity or inconsistency will be resolved in favor of, and pursuant to the FHA mortgage insurance, and other applicable FHA and GNMA statutory, regulatory or administrative requirements and the terms of the FHA Loan Documents and GNMA Documents, as applicable. For purposes of the Indenture, the reference to FHA's statutory, regulatory and administrative requirements shall be deemed to include, but shall not be limited to, any statutory, regulatory or administrative requirements pertaining to Sections 241 and/or 242 of the National Housing Act, as may be applicable. The parties to the Indenture agree to amend the instrument as may be necessary or required by FHA, GNMA or the Lender to conform the instrument to the above cited requirements and FHA Loan Documents and GNMA Documents. In addition, it is understood and agreed that any default under the Indenture shall not constitute a default under the FHA Loan Documents or the GNMA Documents; and further, that nothing in the Indenture contained shall be construed to limit or affect the Lender's rights under the FHA Loan Documents or the GNMA Documents.

(b) No amendment to the Indenture shall conflict with the provisions of the National Housing Act, any applicable HUD regulations, related administrative requirements, the FHA Loan Documents, any applicable GNMA regulations, related administrative requirements and the GNMA Documents.

(c) The Lender will hold escrow funds and any letters of credit required under the FHA Loan Documents as required by FHA and/or GNMA outside the terms of the Indenture, and any Mortgage

Loans funds held by the Lender will not be deposited in any of the funds or accounts created by the Indenture or under the control of the Trustee.

(d) Nothing contained in the Indenture shall inhibit or impair the right of the Lender or the Issuer to require or agree to any amendment, charge or modification of the FHA Loan Documents or the GNMA Documents for the purpose of curing any ambiguity, or of curing, correcting or supplementing any defective or inconsistent provision contained therein, or in regard to matters or questions arising under said FHA Loan Documents or the GNMA Documents so long as any such amendment, change or modification shall not adversely affect the payment terms of the Bonds.

(e) The Indenture shall not be construed to restrict or adversely affect the duties and obligations of the Lender under the contract of mortgage insurance between FHA and the Lender with respect to the Mortgage Loans, or with respect to GNMA under the GNMA Documents.

(f) Neither the Trustee nor any of the Bondholders has or shall be entitled to assert any claim against the Project, the Mortgage Loans proceeds, any reserves or deposits required by HUD in connection with the Mortgage Loans transaction, or the revenues, receipts, rents or deposits or other income of the property other than available "Residual Receipts" (as defined in the FHA Regulatory Agreement) authorized for release by HUD.

(g) Proceeds from any condemnation award or from the payment of a claim under any hazard insurance policy relating to the Project will not be payable to the Trustee, but will be payable to the Lender, and the Bonds will be subject to redemption in whole or in part, prior to their stated maturity date, in said event from payments the Trustee receives on the GNMA Security representing and subsequent to a prepayment of the Mortgage Notes and the Mortgages from such proceeds or claims payment.

(h) In the event of any conflict between the provisions under this heading and the provisions contained in any other Section of the Indenture, the provisions under this heading shall govern and be controlling in all respects.

(i) Nothing under this heading shall be interpreted to increase the obligations of the Issuer under the Indenture or otherwise impose additional obligations on the Issuer.

(j) The parties hereto acknowledge and agree that an indemnification obligation of Issuer hereunder shall not succeed to or become an obligation of FHA and/or HUD.

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APPENDIX F

SUMMARY OF CERTAIN PROVISIONS OF THE FINANCING AGREEMENT

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APPENDIX F

SUMMARY OF CERTAIN PROVISIONS OF THE FINANCING AGREEMENT

The following is a summary, which does not purport to be complete, comprehensive or definitive, of certain provisions of the Financing Agreement, which is qualified in its entirety by reference to the Financing Agreement.

Financing Structure

(a) The Lender agrees, subject to the terms and conditions of the Financing Agreement and the Lender Commitment, to make a loan to the Issuer in an amount equal to the outstanding principal amount of the Mortgage Loans as of the Closing Date, which will be evidenced by the Mortgage Notes, as amended. The Issuer agrees, subject to the terms and provisions of the Financing Agreement, to issue the Bonds and to make the proceeds thereof available to purchase the GNMA Securities. The Issuer agrees, subject to the terms and conditions of the Financing Agreement, to use the proceeds of the Mortgage Loans to refinance the Prior Bonds and pay certain costs in connection therewith.

(b) Upon issuance of the GNMA Securities by the Lender, the Trustee, on behalf of the Issuer, has been authorized and directed under the Indenture to purchase the GNMA Securities from the Lender with funds in the Acquisition Fund and with respect to the payment of accrued and unpaid interest on the GNMA Securities with funds in the Bond Fund in accordance with the Financing Agreement and the Indenture, at a price specified in the Financing Agreement. The Trustee agrees to acquire the GNMA Securities as set forth in the Financing Agreement and in the Indenture.

(c) Pursuant to the GNMA Mortgage-Backed Securities Guide, the Lender agrees to make all payments on the GNMA Securities in accordance with their terms, provided such agreement shall not be construed as the Lender's guaranty.

(d) Nothing in the Financing Agreement shall be construed to impose a duty on the Trustee to purchase, or on the Lender to issue, GNMA Securities in a principal amount in excess of the outstanding balance of the Mortgage Notes as of the Closing Date.

(e) The Issuer agrees to remit to the Lender any amounts which the Trustee is required to pay the Lender, but for which sufficient funds to make such payments are not available under the Indenture.

Delivery of the GNMA Securities

The Issuer and the Lender agree under the Financing Agreement to use their best efforts to deliver to the Trustee the GNMA Securities as soon as practicable after the Closing Date. The Financing Agreement provides that the Issuer and the Lender expect to deliver the GNMA Securities to the Trustee on or before April 4, 2013. The Financing Agreement further states that neither the Issuer nor the Lender has actual knowledge of a material fact that causes it to believe that the GNMA Securities will not be delivered to the Trustee on or before such date.

Sufficiency of Funds

The Issuer agrees that if the Issuer should pay any costs relating to the acquisition of the GNMA Securities other than from the proceeds of the Bonds, the Issuer shall not be entitled to any reimbursement therefor from the Lender, the Trustee or the Bondholders.

Operation of the Project

The Financing Agreement provides that the Issuer shall operate or cause the Project to be operated as an eligible project under Sections 241 and 242 of the National Housing Act.

Events of Default; Remedies

Upon receipt by a Responsible Officer of the Trustee of notice of a violation by the Issuer of, or default by the Issuer under, any of the provisions of the Financing Agreement, the Trustee shall give written notice thereof to the Issuer by certified mail, postage prepaid, return-receipt requested. If a violation or default by the Issuer of any of the provisions of the Financing Agreement is not corrected to the reasonable satisfaction of the Trustee within 30 days after the date such notice is received, or if the violation or default (other than a payment default) cannot be corrected within such period, within such longer period as may be necessary, in the reasonable opinion of the Trustee, to correct such violation, provided that the Issuer has commenced and is diligently pursuing appropriate action to correct such violation and there will be no material adverse effect on the rights of the Trustee, the Lender or the Bondholders under the Financing Agreement, any of the FHA Loan Documents or the Indenture as a result of such extension, without further notice, the Trustee may declare a default under the Financing Agreement effective on the date of such declaration of default, and, upon such default, the Issuer, the Lender or the Trustee may apply to any state or federal court having jurisdiction (a) for specific performance of the Financing Agreement or for an injunction against any violation of the Financing Agreement, since the injury to the Issuer, the Lender and the Trustee arising from a default under any of the terms of the Financing Agreement would be irreparable, and the amount of damage would be difficult to ascertain, or (b) for other relief in law or equity which may be appropriate. A default under the Financing Agreement shall not constitute an Event of Default under the Indenture, the FHA Loan Documents or the documents relating to the GNMA Securities. Notwithstanding the occurrence of any event of default under the Financing Agreement, the Trustee shall continue to purchase the GNMA Securities from the Lender and to make disbursements pursuant to the Financing Agreement, and the funds on deposit in the Acquisition Fund and the Bond Fund shall remain available for such purpose.

GNMA/FHA Loan Documents and Regulations Control; FHA Requirements

(a) To the extent that there is any conflict, inconsistency or ambiguity between or among the Financing Agreement and (i) any applicable FHA mortgage insurance, or other applicable FHA or GNMA statutory, regulatory, administrative requirements; (ii) any of the FHA Loan Documents which have been or are required by FHA and/or the Lender to be executed by the Issuer, FHA and/or the Lender in connection with the subject transaction or (iii) any of the GNMA Documents which have been or are required by GNMA to be executed by the Issuer, FHA, GNMA and/or the Lender in connection with the subject transaction, said FHA mortgage insurance and other applicable FHA and GNMA statutory, regulatory and administrative requirements and said FHA Loan Documents and GNMA Documents will be deemed to be controlling and any such ambiguity or inconsistency will be resolved in favor of, and pursuant to, the FHA mortgage insurance, and other applicable FHA and GNMA statutory, regulatory or administrative requirements and the terms of the FHA Loan Documents and GNMA Documents, as applicable. For purposes of the Financing Agreement, the reference to FHA's statutory, regulatory and administrative requirements shall be deemed to include, but shall not be limited to, any statutory,

regulatory or administrative requirements pertaining to Sections 241 and 242 of the National Housing Act, as may be applicable. The parties to the Financing Agreement agree to amend the instrument as may be necessary or required by FHA, GNMA or the Lender to conform the instrument to the above-cited requirements and FHA Loan Documents and GNMA Documents. In addition, it is understood and agreed that any default under the Financing Agreement shall not constitute a default under the FHA Loan Documents or the GNMA Documents; and further, that nothing in the Financing Agreement contained shall be construed to limit or affect the Lender's rights under the FHA Loan Documents or the GNMA Documents.

(b) No amendment to the Financing Agreement shall conflict with the provisions of the National Housing Act, any applicable HUD regulations, related administrative requirements, the FHA Loan Documents, any applicable GNMA regulations, related administrative requirements and the GNMA Documents.

(c) The Lender will hold escrow funds required under the Mortgage Loans as required by FHA and/or GNMA outside the terms of the Indenture and the Financing Agreement, and any Project funds held by the Lender will not be deposited in any of the funds or accounts created by the Financing Agreement or under the control of the Trustee.

(d) Nothing contained in the Financing Agreement shall inhibit or impair the right of the Lender to require or agree to any amendment, charge or modification of the FHA Loan Documents or the GNMA Documents for the purpose of curing any ambiguity, or of curing, correcting or supplementing any defective or inconsistent provision contained in the Financing Agreement, or in regard to matters or questions arising under said FHA Loan Documents or the GNMA Documents so long as any such amendment, change or modification shall not adversely affect the payment terms of the Bonds.

(e) The Financing Agreement shall not be construed to restrict or adversely affect the duties and obligations of the Lender under the contract of mortgage insurance between FHA and the Lender with respect to the Mortgage Loans.

(f) The Financing Agreement shall not entitle the Trustee, or any of the Bondholders, to assert any lien against the Project, the Mortgage Loans proceeds, any reserves or deposits required by HUD in connection with the Mortgage Loans transaction, or the revenues, receipts, rents or deposits or other income of the Project.

(g) Proceeds from any condemnation award or from the payment of a claim under any hazard insurance policy relating to the Project will not be payable to the Trustee, but will be payable to the Lender to the extent proceeds of any casualty insurance, condemnation award or other amounts received following a partial or total destruction or condemnation of the Project are not applied to the repair or restoration of the Project, and the Bonds will be subject to redemption in whole or in part, prior to their stated maturity date, in said event from payments the Trustee receives on the GNMA Securities representing and subsequent to a prepayment of the Mortgage Notes and the Mortgages from such proceeds or claims payment.

(h) In the event of any conflict between the provisions of this Section 9.10 and the provisions contained in any other Section of the Financing Agreement, the provisions of this Section 9.10 shall govern and be controlling in all respects.

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APPENDIX G

FORM OF CONTINUING DISCLOSURE AGREEMENT

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CONTINUING DISCLOSURE AGREEMENT

This Continuing Disclosure Agreement (the “Disclosure Agreement”), dated March 21, 2013, is by and between **MAIMONIDES MEDICAL CENTER** (the “Issuer”) and **U.S. BANK NATIONAL ASSOCIATION**, as Trustee (defined herein) on behalf of the holders of the Issuer’s \$100,535,000 aggregate principal amount of GNMA Collateralized Taxable Revenue Bonds, Series 2013 (the “Bonds”). The Bonds are being issued by the Issuer and sold to the underwriter (defined below) as described in the official statement, dated February 27, 2013 (the “Official Statement”). The Bonds are being issued pursuant to a Trust Indenture dated as of March 1, 2013 (the “Indenture”) between the Issuer and U.S. Bank National Association, as trustee (the “Trustee”). The proceeds of the Bonds are being loaned to the Issuer pursuant to a Financing Agreement dated as of March 1, 2013 (the “Financing Agreement”) among the Issuer, the Trustee and Prudential Huntoon Paige Associates, LLC, a Delaware limited liability company (the “Lender”). The Issuer and Trustee covenant and agree as follows:

Section 1. Purpose of this Disclosure Agreement. This Disclosure Agreement is being executed and delivered by the Issuer and the Trustee for the benefit of the holders of the Bonds. The Issuer and the Trustee acknowledge that neither the Issuer nor the Trustee shall have any monetary liability to any Person, including any holder of the Bonds or Beneficial Owner, with respect to any reports, notices or disclosures required to be provided pursuant to this Disclosure Agreement.

Section 2. Definitions. In addition to the definitions set forth in the Indenture, which apply to any capitalized term used in this Disclosure Agreement unless otherwise defined in this Section, the following capitalized terms shall have the following meanings:

“*Annual Report*” shall mean any Annual Report provided by the Issuer pursuant to, and as described in, Sections 3 and 4 of this Disclosure Agreement.

“*Beneficial Owner*” shall mean any Person which (a) has the power, directly or indirectly, to vote or consent with respect to, or to dispose of ownership of, any Bonds (including Persons holding Bonds through nominees, depositories or other intermediaries), or (b) is treated as the owner of any Bonds for federal income tax purposes.

“*DAC*” shall mean the Digital Assurance Certification LLC, or such other Person as the Issuer shall designate in writing to the Trustee from time to time.

“*Disclosure Representative*” shall mean the Chief Financial Officer of the Issuer or his or her designee, or such other Person as the Issuer shall designate in writing to the Trustee from time to time.

“*Listed Events*” shall mean any of the events listed in Section 5(a) of this Disclosure Agreement.

“*Quarterly Report*” shall mean any Quarterly Report provided by the Issuer pursuant to, and as described in, Sections 3 and 4 of this Disclosure Agreement.

“*Underwriter*” means Merrill Lynch, Pierce, Fenner & Smith Incorporated and its successors and assigns.

Section 3. Provision of Reports. (a) The Issuer will provide to DAC for electronic posting on DAC’s website, not later than 135 days following the end of the Issuer’s fiscal year, commencing with the fiscal year ending in December 31, 2013, an Annual Report which is consistent with the requirements set forth in Section 4(a) below. The Issuer will also provide to DAC for electronic posting on DAC’s website, not later than 60 days following the end of the Issuer’s first three fiscal quarters, commencing with the fiscal quarter ending June 30, 2013, a Quarterly Report which is consistent with the requirements

set forth in Section 4(b) below. In each case, the Annual Report and/or Quarterly Report may be submitted as a single document or as separate documents comprising a package and may cross reference other information, provided that the audited financial statements for the prior fiscal year of the Issuer or the unaudited financial statements for the fiscal quarter of the Issuer, as the case may be, may be submitted separately from the balance of the Annual Report and/or Quarterly Report. The Issuer covenants and agrees to provide the Trustee with an electronic copy of every Annual Report and Quarterly Report at the same time that it provides such information to DAC.

(b) If the Trustee has not received an electronic copy of the Annual Report and/or Quarterly Report from the Issuer by the applicable date specified in subsection (a), the Trustee shall (i) notify the Disclosure Representative that the Issuer is not in compliance with subsection (a), and (ii) upon confirmation of such non-compliance shall provide DAC with an electronic notice to such effect for posting on DAC's website.

(c) Upon the timely receipt of each Annual Report and/or Quarterly Report, the Issuer shall request from DAC a written confirmation of receipt of the Annual Report and/or Quarterly Report, including the date it was received by DAC.

Section 4. Content of Reports. (a) The Issuer's Annual Report will contain or incorporate by reference the following:

(i) the audited financial statements of the Issuer for its prior fiscal year, prepared in accordance with generally accepted accounting principles as promulgated from time to time by the Financial Accounting Standards Board (or any successor entity), and modified as necessary to comply with HUD reporting. If the Issuer's audited financial statements are not available by the time the Annual Report is required to be filed, the Annual Report will contain unaudited financial statements in a format similar to the financial statements contained in the Official Statement (*i.e.*, including a balance sheet, statement of operations, changes in net assets and cash flow for the fiscal year), and the audited financial statements will be filed in the same manner as the Annual Report when they become available; and

(ii) certain annual financial information and operating data relating to it for its prior fiscal year, in scope, content and presentation similar to the information set forth in the Official Statement (*i.e.*, including aggregate inpatient discharges, patient days, average length of stay, emergency room visits, outpatient visits and outpatient procedures; and discharges by major payor source for the fiscal year), with an explanation of any significant trends or changes.

(b) The Issuer's Quarterly Report will contain or incorporate by reference the following:

(i) the Quarterly Report will contain unaudited financial statements of the Issuer for its prior fiscal quarter in a format similar to the unaudited financial statements contained in the Official Statement (*i.e.*, including a balance sheet, statement of operations, changes in net assets and cash flow as of the end of the fiscal quarter); and

(ii) interim financial information and operating data relating to it for its prior fiscal quarter, in scope, content and presentation similar to the information set forth in the Official Statement (*i.e.*, including aggregate inpatient discharges, patient days, average length of stay, emergency room visits, outpatient visits and outpatient procedures; and discharges by major payor source for the fiscal quarter), with an explanation of any significant trends or changes.

Section 5. Reporting of Significant Events. (a) The Issuer will give, or cause to be given, notice of the occurrence of any of the following events with respect to the Bonds:

- (i) principal and interest payment delinquencies;
- (ii) non payment related defaults, if material;
- (iii) [Reserved];
- (iv) unscheduled draws on credit enhancements reflecting financial difficulties;
- (v) substitution of credit or liquidity providers or their failure to perform;
- (vi) [Reserved];
- (vii) modifications to rights of Bondholders, if material;
- (viii) Bond calls, if material and tender offers;
- (ix) defeasances;
- (x) release, substitution or sale of property securing repayment of the Bonds;
- (xi) rating changes;
- (xii) bankruptcy, insolvency, receivership of similar event of the Issuer;
- (xiii) the consummation of a merger, consolidation or acquisition involving the Issuer, or the sale of all or substantially all of the assets of the Issuer, other than in the ordinary course of business, the entry into a definitive agreement to undertake such an action or the termination of a definitive agreement relating to any such actions, other than pursuant to its terms, if material; and
- (xiv) appointment of a successor or additional Bond trustee, or the change of name of a Bond trustee, if material.

(b) Upon obtaining knowledge of the occurrence of a Listed Event for which a materiality standard exists, the Issuer agrees to promptly determine if such event is considered material for purposes of applicable federal securities laws.

(c) If the Issuer has determined that knowledge of the occurrence of a Listed Event would be material under applicable federal securities laws, or if the Listed Event does not involve a materiality standard, the Issuer will (i) promptly notify the Trustee in writing, and (ii) electronically file with DAC a notice of the occurrence of such Listed Event for posting on DAC's website.

(d) If in response to a request under subsection (b), the Issuer determines that the Listed Event would not be material under applicable federal securities laws, the Issuer will so notify the Trustee in writing and (except with respect to Section 5(a)(ii) above) provide a concurring opinion of counsel to the Trustee.

Section 6. Amendment; Waiver. Notwithstanding any other provision of this Disclosure Agreement, the Issuer and the Trustee may amend this Disclosure Agreement and any provision of this Disclosure Agreement may be waived, provided that the following conditions are satisfied:

(a) If the amendment or waiver relates to the provisions described under paragraph (a) under “Provision of Reports”, “Contents of Reports” or paragraph (a) under “Reporting of Significant Events”, it may only be made in connection with a change in circumstances that arises from a change in legal requirements, change in law or change in the identity, nature or status of an obligated Person with respect to the Bonds or the type of business conducted; and

(b) The amendment or waiver either (i) is approved by the Holders of the Bonds in the same manner as provided in the Indenture for amendments to the Indenture with the consent of Holders or (ii) does not, in the opinion of Transaction Counsel, materially impair the interests of the Holders or Beneficial Borrowers of the Bonds.

Notwithstanding the foregoing, no amendment or waiver shall be required if there is a change in accounting principles as required under generally accepted accounting principles.

In the event of any amendment or waiver of a provision of this Disclosure Agreement, the Issuer covenants and agrees to (i) describe such amendment in the next Annual Report and (ii) as applicable, a narrative explanation of the reason for the amendment or waiver and its impact on the type (or, in the case of a change of accounting principles, on the presentation) of financial information or operating data being presented by the Issuer. In addition, if the amendment relates to the accounting principles to be followed in preparing financial statements, the Annual Report for the year in which the change is made should present a comparison (in narrative form and also, if feasible, in quantitative form) between the financial statements as prepared on the basis of the new accounting principles and those prepared on the basis of the former accounting principles. Notwithstanding the foregoing, a separate narrative explanation shall not be required if described in the relevant financial statements.

The Issuer will provide a copy of any amendment to this Disclosure Agreement to the Trustee promptly following its execution.

Section 7. Remedies. In the event of a failure by either the Issuer or the Trustee to comply with any provision of this Disclosure Agreement, the non-defaulting party, may, and, at the request of the Underwriter or the Holders of at least 25% aggregate principal amount of Outstanding Bonds, shall, take such actions by mandamus or other suit or proceeding at law or in equity as may be necessary and appropriate against the other party and any of its officers, agents and employees, in order to cause the defaulting party to comply with its obligations under this Disclosure Agreement; provided, however, that the sole and exclusive remedy for breach of this Disclosure Agreement shall be an action to compel specific performance of the obligations of such defaulting party hereunder and no person or entity shall be entitled to recover monetary damages hereunder under any circumstances.

A default under this Disclosure Agreement shall not be deemed an Event of Default under the Indenture or the Financing Agreement.

Section 8. Beneficiaries. This Disclosure Agreement will inure solely to the benefit of the Issuer, the Trustee, the Underwriter and Holders from time to time of the Bonds and will create no rights in any other Person or entity.

Section 9. GNMA/FHA Documents and Regulations Control. Notwithstanding any provision of this Disclosure Agreement to the contrary, the parties hereto acknowledge and agree that all of their respective rights and powers under this Agreement are subordinate and subject to the liens of the Mortgages created by the Issuer in favor of the Lender under the FHA Loan Documents, and to all of the terms and provisions of the Mortgages, and any and all other FHA Loan Documents and GNMA Documents executed by the Issuer, FHA, GNMA and/or the Lender, as required by FHA, GNMA or the Lender in connection with the Mortgage Loans.

Section 10. Additional Information. Nothing in this Disclosure Agreement shall be deemed to prevent the Issuer from disseminating any other information, using the means of dissemination set forth in this Disclosure Agreement or any other means of communication, or including any other information in any Annual Report, Quarterly Report or notice of occurrence of a Listed Event, in addition to that which is required by this Disclosure Agreement. If the Issuer chooses to include any information in any Annual Report, Quarterly Report or notice of occurrence of a Listed Event, in addition to that which is specifically required by this Disclosure Agreement, the Issuer shall have no obligation under this Disclosure Agreement to update such information or include it in any future Annual Report or notice of occurrence of a Listed Event.

Section 11. Duties, Immunities and Liabilities of the Trustee. For purposes of this Disclosure Agreement, the Trustee shall have only such duties as are specifically set forth herein, and the Issuer agrees to indemnify and save the Trustee and its officers, directors, employees and agents, harmless against any loss, expense and liabilities which they may incur arising out of or in the exercise or performance of their rights, obligations, powers and duties hereunder, including the costs and expenses (including reasonable attorneys' fees) of defending against any claim of liability, but excluding liabilities due to the Trustee's negligence or willful misconduct. The obligations of the Issuer under this Section shall survive the termination of this Disclosure Agreement, the resignation or removal of the Trustee hereunder and the payment of the Bonds.

The Trustee shall not be deemed to be acting in any fiduciary capacity for the Issuer or the holders of the Series 2013 Bonds or any other party. The Trustee shall have no duty or obligation (i) with respect to the form or content of any disclosure or Listed Events Notices provided by the Issuer pursuant to the terms of this Disclosure Agreement, or (ii) to review or verify any information contained in the Annual Reports or the Quarterly Reports or Listed Event Notices or any other information, disclosures or notices provided to it by the Issuer. The Trustee shall have no responsibility for the Issuer's failure to electronically provide to DAC any notices of Listed Events required by this Disclosure Agreement or duty to determine the materiality thereof. The Trustee shall have no duty to determine or liability for failing to determine whether the Issuer has complied with this Disclosure Agreement and it may conclusively rely upon certifications of the Issuer with respect to its obligations hereunder. In no event shall the Trustee have any liability to the holders of the Bond or any other party for any monetary damages or financial liability of any kind whatsoever related to or arising from the breach of this Disclosure Agreement.

In the event that the obligations of the Trustee hereunder terminate, the Issuer will either appoint a successor for the benefit of the Series 2013 Bondholders.

No provision of this Disclosure Agreement shall require or be construed to require the Issuer or the Trustee to interpret or provide an opinion concerning any information disclosed hereunder.

The Annual Report may contain such disclaimer language as the Issuer may deem appropriate.

The Issuer hereby agrees to compensate the Trustee for the services provided pursuant to this Disclosure Agreement, in an amount to be agreed upon, and to reimburse the Trustee upon its request for all reasonable expenses, disbursements and advances incurred by it hereunder (including any reasonable compensation and expenses of counsel) except any such expense, disbursement or advance that may be attributable to its negligence or willful misconduct.

The Trustee may consult with counsel of its choice and the written advice of such counsel or any opinion of counsel shall be full and complete authorization and protection in respect of any action taken, suffered or omitted by it hereunder in good faith and in reliance thereon, it being understood that for purposes of this provision, that such counsel may be counsel to the Issuer.

No provision of this Disclosure Agreement shall require the Trustee to expend or risk its own funds or otherwise incur any financial liability in the performance of any of its duties hereunder, or in the exercise of any of its rights of powers.

Section 12. Notices. Any notices or communications to or among any of the parties to this Disclosure Agreement may be given as follows, and shall be effective only upon receipt, with notices to be sent by mail or overnight mail, and facsimile:

To the Issuer:

Maimonides Medical Center
4721 Fort Hamilton Parkway
Brooklyn, NY 11219
Attention: Robert Naldi
Telephone: (718) 283-3909
Facsimile: (718) 283-3930

With a copy to the Issuer's General Counsel

To the Trustee:

U.S. Bank National Association
100 Wall Street, 16th Floor
New York, New York 10005
Attention: Corporate Trust Department
Telephone: (212) 951-8500
Facsimile: (212) 809-4993

Any person may, by written notice to the other persons listed above, designate a different address or telephone number(s) to which subsequent notices of communications should be sent, effective only upon receipt.

Section 13. Governing Law. This Disclosure Agreement shall be governed by the laws of the State of New York.

Section 14. Termination of this Disclosure Agreement. The Issuer or the Trustee may terminate this Disclosure Agreement by giving written notice to the other party at least 30 days prior to such termination. The Trustee shall be fully discharged at the time any such termination is effective.

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Section 15. Counterparts. This Disclosure Agreement may be executed in several counterparts, each of which shall be an original and all of which shall constitute but one and the same instrument.

Dated as of March __, 2013

MAIMONIDES MEDICAL CENTER

By: _____
Authorized Officer

U.S. BANK NATIONAL ASSOCIATION
On behalf of the holders of the Bonds

By: _____
Authorized Officer

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APPENDIX H

FORM OF APPROVING OPINION OF TRANSACTION COUNSEL

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EICHNER NORRIS & NEUMANN PLLC

ATTORNEYS AT LAW
THE JEFFERSON BUILDING
1225 19TH STREET, N.W., 7TH FLOOR
WASHINGTON, D.C. 20036
TEL: (202) 973-0100
FAX: (202) 296-6990

March 21, 2013

Maimonides Medical Center
Brooklyn, New York

Re:

\$100,535,000
Maimonides Medical Center
GNMA Collateralized Taxable Revenue Bonds,
Series 2013

Ladies and Gentlemen:

We have acted as transaction counsel to Maimonides Medical Center (the “Issuer”), a New York not-for-profit corporation organized under the New York State not-for-profit corporation law and a charitable organization within the meaning of Section 501(c)(3) of the Internal Revenue Code of 1986, in connection with the issuance of \$100,535,000 in aggregate principal amount of its GNMA Collateralized Taxable Revenue Bonds, Series 2013, dated the date hereof (the “Bonds”). The Bonds are issued by the Issuer under and pursuant to that certain Trust Indenture dated as of March 1, 2013 (the “Indenture”) between the Issuer and U.S. Bank National Association, as trustee (the “Trustee”).

The proceeds from the sale of the Bonds will be used by the Issuer to purchase fully modified mortgage-backed securities which will be guaranteed as to timely payment of principal and interest by the Government National Mortgage Association from Prudential Huntoon Paige Associates, LLC (the “Lender”) pursuant to that certain Financing Agreement dated as of March 1, 2013 (the “Financing Agreement”) among the Issuer, the Trustee and the Lender.

In our capacity as transaction counsel to the Issuer, we have examined, among other things, proceedings of the Issuer authorizing, among other things, the execution and delivery of the Indenture, the Financing Agreement, and the issuance of the Bonds, a specimen Bond, a certificate of the Trustee regarding the authentication of the Bonds, executed counterparts of the above-referenced documents, the executed opinion of Arent Fox LLP, New York, New York, counsel to the Issuer, dated this date, and such other documents, showings and related matters as we have deemed necessary in order to render this opinion.

Based on the foregoing, but subject to the assumptions, qualifications and limitations set forth below, we are of the opinion that:

1. The Indenture has been duly authorized by the Issuer, has been duly executed and delivered by authorized officers of the Issuer and, assuming due authorization, execution and delivery thereof by the Trustee, constitutes a valid and binding instrument of the Issuer, enforceable against the Issuer in accordance with its terms.

2. The Bonds have been duly authorized by the Issuer, have been duly executed by authorized officers of the Issuer, authenticated by the Trustee and validly issued by the Issuer and constitute the valid and binding limited obligations of the Issuer payable solely from payments and prepayments and from other amounts payable under the under the Indenture and the Bonds are enforceable in accordance with their terms and are entitled to the benefit and security of the Indenture.

3. The Financing Agreement has been duly authorized by the Issuer, has been executed and delivered by authorized officers of the Issuer and, assuming due authorization, execution and delivery thereof by the other parties thereto, constitutes a valid and binding instrument of the Issuer enforceable against the Issuer in accordance with its terms.

The opinions set forth above are subject to the following assumptions, qualifications and limitations:

(a) We have assumed without any independent investigation (i) the genuineness of all signatures on documents examined by us and the authority of the persons to sign such documents (other than the genuineness of the signature of the officers of the Issuer on the Indenture, the Financing Agreement and the Bonds, and the authority of such officers to sign such documents), (ii) the authenticity of all documents submitted to us as originals and the conformity to authentic original documents of all documents submitted to us as certified, conformed, facsimile or photostatic copies, and (iii) the correctness of public files, records and certificates of, or furnished by, governmental or regulatory agencies or authorities.

(b) We have assumed (without expressing any opinion thereon), to the extent the obligations of the Issuer or its performance thereof may be dependent on such matters, that: (i) each party to the Indenture and the Financing Agreement (other than the Issuer) is duly organized, validly existing and in good standing under the laws of its jurisdiction of incorporation, and (ii) any party seeking to enforce its rights against the Issuer under the Indenture or the Financing Agreement is qualified to do business in every state necessary in order for it to realize the practical benefits of enforcing its rights under such document. We also have assumed (i) the due authorization, execution and delivery of the Indenture and the Financing Agreement by all parties (other than the Issuer), (ii) that each party to the Indenture and the Financing Agreement (other than the Issuer) has the requisite organizational power and authority to perform its obligations under such document, and (iii) the validity and binding effect of the Indenture and the Financing Agreement with regard to such parties (other than the Issuer).

(c) Our opinions in paragraphs 2 and 3 are subject, as to enforceability, to the effect of any applicable bankruptcy, insolvency, reorganization, moratorium or similar laws affecting creditors' rights generally from time to time in effect, and to the effect of general principles of equity, including without limitation those related to equitable subordination, matters of charitable trust or public policy, concepts of materiality, reasonableness, good faith and fair dealing (regardless of whether considered in a proceeding in equity or at law) and the possible unavailability of the remedy of specific performance or payment of attorney fees.

(d) We offer no opinion as to whether any authorizations, consents, approvals, licenses, permits, filings or registrations may be required (or whether any fees may be required to be paid in connection therewith) in connection with any legal or other proceedings for enforcement of the Indenture and the Financing Agreement.

(e) We express no opinion as to the applicability of laws relating to fraudulent transfer, fraudulent obligations or preferential transfer, or any laws governing the distribution of assets of the Issuer including without limitation, Section 548 of Chapter 11 of the United States Code.

(f) We express no opinion as to any provision of any document purporting to provide for the payment of interest on interest.

(g) We express no opinion as to the inclusion of interest on the Bonds in the gross income of the owners thereof for federal or state income tax purposes.

These opinions are based solely on the laws of the State of New York and federal laws of the United States of America and no opinion is intended or given with respect to the laws of any other jurisdiction.

This opinion is intended solely for your benefit and may not be relied upon by any other person. We are assuming no professional responsibility to any other person by rendering this opinion.

Respectfully submitted,

EICHNER NORRIS & NEUMANN PLLC

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